

Krishna Institu. (KIMS)

Conference call transcripts, oldest-first. 13 call(s). Generated 2026-06-08.

Call: May 2023

25th May, 2023

The General Manager,
Department of Corporate services
Bombay Stock Exchange Ltd (BSE)
Phiroze Jheejheebhoy Towers,
Dalal Street,
Mumbai - 400 001.
Scrip Code - 543308
ISIN: INE967H01017 The Manager,
Listing Department
National Stock Exchange of India Limited,
Exchange Plaza, 5th Floor,
Plot No.C/1, 'G' Block
Bandra - Kurla Complex
Mumbai - 400 051.
Symbol - KIMS
ISIN: INE967H01017

Dear Sir/ Madam,

Sub: Transcript of Earnings Conference Call with Analyst / Investors.

In continuation to our letter dated 17th May 2023, the Company organized a conference call with the Investors/ Analysts on Friday, 19th May 2023 at 11:00 AM (IST). A copy of the transcript of the conference call held with the Investors/ Analysts is enclosed herewith and the same has also been uploaded on the Company's Website at <https://www.kimshospitals.com/investors> > Disclosures under Regulation 46 of SEBI (LODR) Regulations, 2015 > Analysts Calls schedule, PPT & Transcripts > Transcripts, Audio & Video Recordings > FY 22-23.

Kindly take the same on record and display the same on the website of your exchange.

Thanking you,
Yours truly

For Krishna Institute of Medical Sciences Limited

Uma Shankar Mantha
Company Secretary & Compliance Officer
MANTHA
UMASHANKAR Digitally signed by
MANTHA UMASHANKAR Date: 2023.05.25 12:10:54 +05'30'
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"KIMS Hospitals Q4 FY2023
Financial Results Conference Call"

May 19, 2023

ANALYST: M R. RAHUL JEEWANI – IIFL SECURITIES LIMITED

MANAGEMENT : DR. BHASKAR RAO BOLLINENI – FOUNDER &
MANAGING DIRECTOR – KIMS HOSPITALS
DR. ABHINAY BOLLINENI – EXECUTIVE DIRECTOR &
CHIEF EXECUTIVE OFFICER – KIMS HOSPITALS
MR. VIKAS MAHESHWARI – CHIEF FINANCIAL OFFICER
– KIMS HOSPITALS

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Page 2 of 24 Moderator : Ladies and gentlemen, good day and welcome to the KIMS Hospitals' Q4 FY2023 Financial Results conference call hosted by IIFL Securities Limited. As a reminder, all participants' lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference call is being recorded. I now hand the conference over to Mr. Rahul Jeewani from IIFL Securities Limited. Thank you and over to you Sir!

Rahul Jeewani : Good afternoon, everyone. I am Rahul from IIFL Institutional Equities. I welcome you all to the Quarter Four Earnings Conference Call of KIMS Hospitals. From KIMS, we have with us Dr. Bhaskar Rao Bollineni, Founder and Managing Director, Dr. Abhinay Bollineni, Executive Director and CEO, and Mr. Vikas Maheshwari, Chief Financial Officer. Over to you Sir for your opening comments.

Bhaskar Rao Bollineni : Very good morning and a hearty welcome to you all. Two days back, that is May 16, 2023 is observed as International Day of Families. Please accept my greetings of the lovely occasion. Family denotes a sense of belonging and commitment. Family is not necessarily blood; it is the people in our life. As investors, you belong to KIMS family. As such, we will walk together and grow together.

Now let me have the pleasure of apprising you of the financial and operational highlights for the financial year 2023, which marked another successful and growth oriented year. The robust growth is a reflection of the trust reposed in KIMS by patients and their relatives on account of its patient care, clinical excellence coupled with affordability.

Now coming back to the financial highlights, quarter four FY2023. Consolidated revenue from operations grew by 54.7% year-on-year basis and a 2.4% on a quarter-on-quarter basis to Rs.5759.2 million. Consolidated EBITDA pre Ind-AS and other inco

me grew by 38.8%

on a year-on-year basis and 8.3% on a quarter-on-quarter basis to Rs.1563.3 million.

Consolidated EBITDA margin stands at 27.1%, which has improved by 1.4% over the previous quarter led by operational efficiency. The consolidated PAT grew by 18.4% on a year-on-year basis and 20.6% on a quarter-on-quarter basis to Rs.986 million. The consolidated earnings per share grew by 15.6% on a year-on-year basis, and 22.8% on a quarter-on-quarter basis to Rs.11.7. The financial highlights of the year-on-year financial year 2022 to 2023. Consolidated revenue grew by 33.1% to 21976.7 million in FY2023 from 16508.3 million in FY2022.

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Page 3 of 24 Profit before tax increased by 5.1% to 4700.8 million in FY2023 from 4473.7 million in financial year 2022. EBITDA pre Ind-AS and other income increased by 12.6% to 5766.4 in FY2023 from 5122.4 million in FY2022.

The operational highlights. The average revenue per patient and average revenue per operating bed registered improvements both on quarter-on-quarter and year-on-year basis. The IP and OP volumes displayed a strong growth on year-on-year basis. The expansion project status. KIMS Sunshine Secunderabad Hospital is expected to be completely shifted a new state of the art facility by quarter two financial year 2024. Nashik, the Greenfield project and Bangalore Semi Brownfield are progressing well and are expected to be operational as per schedule. Vizag Gastro Unit is ready and will be operational by quarter one FY2024. Thane projects legal documentation work is in progress, and expected to be

completed by June 2023. The hospital is expected to be operational by quarter two financial year 2025. On the digital side, SAP has been successfully implemented and gone live at a few of our units and is expected to be fully implemented across all units by the end of financial year 2024.

We will let you know about some achievements during this quarter. At KIMS Cuddles Kondapur a marathon followers surgery was performed on a three day old baby to help him continue his breathing. Usually such surgeries are performed only on children at least four weeks old. The Maharashtrian parents were delighted at the rebirth by doctors to the newborn. Such a newborn and such a major operation to perform speaks about the teamwork by the neonatologist, the operation surgeons and the postoperative care. A 12 year old boy suffering from a rebuilding disorder was treated successfully at KIMS Secunderabad. The 58 year old woman was treated successfully after undergoing a complex cancer related surgery lasting 7 hours at KIMS Vishakhapatnam. That means all our peripheral centres are also geared up to take up the complex surgeries and we are building that unit in one of the best unit in that region.

Endovascular coiling was performed on a 46 year old patient successfully at KIMS Vishakhapatnam, thereby preventing disabling coma. Even though it is a very important event of coiling prevent a lot of disabled personality and as well as crippled life. Even if place of Vishakhapatnam we are performing means that the centre has been growing up very well. A Zambian nurse with severe intestinal damage underwent a lifesaving surgery at KIMS Secunderabad after a series of failed surgeries back home. Iraqi boy with a severe knee deformity was successfully operated at KIMS Kondapur making him to stand straight on his legs with dignity. This speaks about how our international patients are improvising after the post COVID. Bilateral retinal case with a brain complication was treated successfully at KIMS Vishakhapatnam to prevent blindness.

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Page 4 of 24 The academic accomplishment and awards. Following are the accomplish pertaining to quarter four financial year 2023. Dr. Sita Jayalakshmi Satuluru was honored by the American Academy of Neurology with 2023 A.B. Baker Teacher Recognition Award. Dr. P Rag

haram, Director of KIMS Ushalakshmi Centre for Breast Diseases, is the first surgeon from Telangana in Andhra Pradesh to be appointed as International Adviser by the Royal College of Surgeons and Physicians of Glasgow from 2023 to 2026. In this role he would be providing strategic guidance and advice to the Royal College from India. Our doctors at KIMS distinguished themselves in academics also with their valuable contributions to national, international journals. The academics is the most important and administrative role that our consultants are taking will help us to build our organization, to expand our organization into different territories.

The Neurology academics Dr. Sita Jayalakshmi Satuluru, the Epilepsy Specialist published papers in our field of Epilepsy Disorders in International Journals of high impact factor. As I already mentioned this is the second largest epilepsy programmes in the country after the first all India Institute which performs. In KIMS, we have performed more than 1000 epilepsy surgeries which are at least 80% of the success rate which is very, very important for the patient is concerned for prevention of epilepsy without any drugs. Dr. Powell a renowned neurologist authored original articles of issues relating to brain stroke that were published in reputed journals and one acclaim. Once you have a brain stroke, then you will be crippled for life long with the hemiplegia or a monoplegia or some sort of events. On that we are doing a lot of research. There are many people who have come with a stroke within time has went back walking. So that means that is the quality of treatment that the patients are getting and our consultants are putting efforts to achieve all these things. Dr. Rajendra Kumar Jain and associates published their research on heart failure studies, Anthem studies in International Journal of Cardiology, how we can able to improvise these heart failures which is very bothering for the society. So we are doing a research on that. In Obstetrics and Gynecology Dr. Vasundhara Devi's original articles on postoperative maternal health was published in international journals to prevent maternal mortality, we are taking steps to find out what will be the reasons and then how we can able to prevent it, and then we also do all these things that will help to give a good quality treatment to our patients. Not only to improvise our existing skills in further, we also conduct some workshops and CMU programmes National and International every quarter at minimum of two. One of that two day workshop was organized at HICC Novatel, Hyderabad and HIP Symposium Basics to revision, total hip replacement and cadaveric by Dr. Uday Krishna Myneni that attracted participants' from across the country and internationally, and I would like to share a piece of information that our CFO, Mr. Vikas Maheshwari had resigned due to personal reasons. We will place on record our appreciation for the services rendered by

him during his tenure. A firm succession plan is in place and we would be having a new
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Page 5 of 24 CFO. Patient safety is our prime concern and crux of all our activities. We at KIMS compete with nobody else, but ourselves in being more and more responsive, responsible and respectful in all our activities. Transparency and value adherence is our mantra in discharging our functions. Your trust and support give us wings to translate our plans into actions. The more we serve, the more we grow. The more we grow, the more we share. Investor interests are close to our heart and a fundamental aspect of our growth journey. We have had a productive and a rewarding year and I can assure you that we will continue with same verve and tempo to scale even greater heights in the days to come. So thank you very much for your patient hearing. If there is anything we can able to answer, we will go for the question and answer session.
Moder

ator : Thank you very much, Sir. Ladies and gentlemen, we will now begin the question and answer session. Take our first question from the line of Dhara Patwa from SMIFS Limited. Please go ahead.

Dhara Patwa : Thanks for the opportunity and congratulations on good set of numbers. I have two questions. First is like Nagpur facility, before the acquisition, it had EBITDA margins of around 10%. Even in the previous quarter it was around 4%, but there was some one off items. So when can we expect the margin to move to something like 12%, 15% margin for this Nagpur facility. First question is that.

Abhinay Bollineni : As far as Nagpur is concerned, it is currently at 4%-5%. We have already identified the opportunity and where we can bring it to 10%-12%, but because it is our first few months into the organization, we are still trying to stabilize things with the doctor's engagements, and the other things. Maybe in the next 6 to 8 months, you will see us get to a 10% kind of a margin, 10%-11% on the current revenue, and that will be the maximum that we will be able to get from the current revenue, and as we start seeing incremental revenue coming in, you will see at least 40% of that flow straight into our EBITDA, and the large areas that we are focusing on in trying to optimize is 2% on the consumable cost, 2% to 3% on the doctor cost and the rest of it is in the other cost, which is HR and the other category.

Dhara Patwa : That is helpful. Second question was on the Thane facility. If you could provide some color on that, how much bed capacity will be added in the Thane or you will be acquiring and what all specialties will be there in that hospital?

Abhinay Bollineni : No, on the Thane hospital it is a 300 bedded hospital with services ranging from accident and emergency care to oncology as well as transplant. So we intent like we said we only focus on multi-specialty. This will be our first hospital that we will have oncology right

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Page 6 of 24 from the beginning and not as an addition subsequently and it will have all the services that we are providing in many of our other hospitals including liver transplant and kidney transplant.

Dhara Patwa : So should we assume that ARPOB the similar level of the company's ARPOB because oncology is already included and it is a micro city so 30000 ARPOB could still be achievable, right?

Abhinay Bollineni : I think what the competitor in the current geography is around 39000-38000 ARPOB. So yes, I think definitely 30000 is something we will see. We should see northwards of 30000 if not in the first year, maybe in the second year onwards.

Dhara Patwa : Capex guidance for FY2024 like we already had 350 Crores for Nashik and Bangalore, how much additional we will be spending on Thane facility, the total Capex for the company?

Vikas Maheshwari : With respect to the Thane project, though we have taken a decision to take this project ahead. We are in touch with one or two institutional investors who can take the assets and it becomes asset light model and niche model business. So that is all still in the discussion and we will finalizing this steps in next two to three weeks times. If the whole land and building and other equipment comes in our books means we could not finalize the investors at a project cost something will be between Rs.400 Crores to Rs.450 Crores overall project cost for 300 beds, if we have been successful negotiating the good deal with the investors who can dig the assets outside of book, then the Capex value will come down by at around 250 Crores. So this is the status as we progress, we will keep all the investors informed on the final structures which we are going ahead.

Dhara Patwa : Total Capex will be 250 Crores for Thane and 350 Crores which is the ongoing Capex for Nashik and Bangalore correct like around 650 Crores, 700 Crores for FY2024.

Vikas Maheshwari : FY2024 you should take yes that

should be the outflow. But Mumbai there is a plus minus depending upon land building how we are negotiating.

Dhara Patwa : Sure sir. Got it. That is it from my side.

Moderator : Thank you. We take our next question from the line of Reshab Sisodiya from Sameeksha Capital. Please go ahead.

Reshab Sisodiya : Hi. Thank you for the opportunity. Congratulations on good set of numbers. I have two questions, one is on the payor mix. So if we see our cash portion has decreased from 55%-60% to odd 50% at the end of Q4 2023 and the similar has been the increase in the

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Page 7 of 24 insurance share. So would that explain our higher debt days for the year and where should we take it forward?

Vikas Maheshwari : The present payor mix cash and insurance is roughly 79%-80% and it is safe to assume that there will be a deviation of 1% or 2% here and there quarter depending on the quarter-on-quarter, but that should be the optimal mix, but as we progress our unit into Bangalore and Mumbai which is more or less is the cash and insurance market. The payor mix will get better, but with the existing units that is the safe to assume this is the mix which will continue to maintain 80% to 20%.

Reshab Sisodiya : So our debtor days should stay in this range at 40 days would that be a good assumption.

Vikas Maheshwari : Yes, that is the safe assumption.

Reshab Sisodiya : The next question is on the KIMS Telangana clusters. For the quarter, we reported 35% margins which were find it that is above quarter and on the other hospital there was slight margin compression on a sequential basis. So is there any seasonality impact on these how would you explain this?

Abhinay Bollineni : As far as Telangana Cluster is concern you have a mixed of, so wherever the passing expansion has happened is because the consumable cost has come down because of typical case mix number one and number two is because of the doctor cost. So you have doctors who are on a fee for service model and a minimum guarantee model. So there have been a lot of doctors in the minimum guarantee model. They started performing off late. I mean it takes him that much time to perform and then we are seeing progress on those doctors in the minimum guarantee. So because of that, there has been a slight margin expansion, but I think we will continue to maintain that 31%-32% kind of a margin at the Telangana Cluster.

Reshab Sisodiya : One last question, so how many beds are we planning to add in FY2024 capacities if you could just give me on Nashik and Bangalore and even Vizag we are expanding next year it would get operationalize in Q1.

Abhinay Bollineni : In FY2024, we have the Vizag Hospital, which is a small gastro hospital, which is 25 bed in size. Then sunshine we are relocating from one campus to others. So there is no incremental bed addition. We are looking at Nashik to be operational, which will be in phase one. We will operationalize. Though the capacity will built for 250-300 beds. We will only operational as 150 beds and Thane hopefully should also be operationalize in FY2024.

Reshab Sisodiya : Okay, Sir. Thank you. That is it from my side.

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Page 8 of 24 Moderator : Thank you. We take our next question from the line of Anuj Suneja from ICICI Prudential. Please go ahead.

Anuj Suneja : Thank you for the opportunity and congratulations to the management on the great set of numbers. My question is broadly on the lines of the interest cost. If I can get a guidance on how the interest cost is moving, because I am seeing a lot of volatility in the last few quarters and even if I go back like two years back. The interest cost was 32 Crores which came down to 16 Crores. Now it is up back again in the 30 range. So one from a quarterly perspective, how should we looking at it and overall how are we looking at the debt numbers going forward and what would be a comfortable debt to equity

ratio that I should

be looking at? So couple of those questions.

Vikas Maheshwari : Let me take this question on the financial cost side. Actually the quarter second there was some adjustment because of Nagpur unit which we acquired and the first few of the high cost loan which we refinanced etc. So that cost has got hit and in the closing interim there was a few of the provisions which we have to take, so we have cleaned up this. So you should assume that run rate of at around Rs.6 Crores per quarter, so Rs.24 Crores is the run rate which we should assume for that interest rate and as far as the debt trajectory is concerned, right now we have a net debt position of roughly Rs.300 Crores and we are generating something like that Rs.400 Crores of the cash every year and in the next two years we plan to deploy the same type of the capital. But in the interim period depending upon the project which we are taking whether it is asset light or asset heavy depending upon the commercials being agreed some debt may come up front. But overall in next two years

times we should be back on the same positions where we are sitting right now. So right now the current balance sheet is very strong, cash flow remains very, very strong and we are deploying the capital for the next leg of the growth and which is very important to keep the growth momentum continuing in the coming quarters. So there will be some debt and our comfort level which our board has defined is 1.5 to 1.75 times of the debt to EBITDA. I think a lot headroom available for us to reach there. So we are very comfortable position on that.

Anuj Suneja : Very helpful. My second question is on the, so last time you had mentioned something on the lines of that you are looking at property in Mumbai somewhere in Andheri or somewhere. So is there any update on that you have that you want to share or you are still counting your properties there?

Abhinay Bollineni : Not in Andheri, but we are looking for something in Malad. We still trying to do some diligence on the line. So nothing conclusive at this point in time.

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Page 9 of 24 Anuj Suneja : Thanks a lot. Very helpful.

Moderator : Thank you. We will take our next question from the line of Mayank Hyanki from Axis AMC. Please go ahead.

Mayank Hyanki : Sir, can you please explain the structure of the Thane hospital that you are going for?

Vikas Maheshwari: Let me take this question. We have definitive agreements and understanding with the seller about the Hiranandani. We had given some advance also, but we are exploring the opportunity where we can have the asset light model so the third-party large investors, institutional investors who can hold this, asset and give us on the lease. So this commercials are still under active discussion and we should be concluding in next two to three weeks' time and then we will communicate that. If we are not getting a good commercial deal if we feel that, that does not work in the long-term for the company we will have the asset in our books. So the asset price is 250 Crores and the other cost will be roughly Rs.200 Crores for equipment, refurbishment of the hospital, etc., is roughly Rs.450 to Rs.475 Crores of the total project costs? So depending upon whether it is asset light or on the lease model that Capex will move?

Mayank Hyanki : So if the advances which we have given right now in case it happens to be an asset light model in that case that will be like a deposit or the rental will be deducted from it...

Vikas Maheshwari: We will get this amount because we have paid to the building owners, land owners, we have paid. So the investors will pay off to the seller and then we will get our money back.

Mayank Hyanki : So this is the exact structure is something which will come out and what is the potential in terms of number of beds in this hospital?

Vikas Maheshwari: So, as Dr. Abhinay told, it is 300 beds which we c

an occupy on this facility.

Abhinay Bollineni: So the kind capacity is built to is 300 beds. There is additional FSI available for an incremental 50-75 beds in the same campus. But fortunately there is a land parcel that can take up an incremental 1 lakh square feet right adjacent into the current hospital.

Mayank Hyanki : Sorry sir, the current building has 300 beds right. For additional you need to take more land nearby and you already have a FSI in place, right?

Abhinay Bollineni: There is SSI in the current campus, which can take up the bed strength from 300 to 375, plus there is an additional land available on which we can build another 1 lakh square feet.

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Page 10 of 24 Mayank Hyanki : And for this additional 375 beds from 300 beds you will need to spend on construction as well, right?

Abhinay Bollineni: Correct.

Mayank Hyanki : Second question is on the Nagpur facility, I mean, between two quarters since you have that under us. So you explained that you are still looking at doctor, equipments and all and that is something because of which the ramp up probably is not happening, and on the margin side you explained I think the cost savings structure which can take the margin to 10% right at current revenue level only. So generally we have seen a very strong business momentum for hospitals across the country and from that perspective what is the difference between us and the competitor who are earning higher ARPOB and in the last six months, has anything structurally happened in that second quarter of which the patient flow or the revenue has not remain to ramp up or it is just because of our internal issues where in certain therapies or department we do not have the doctors.

Abhinay Bollineni: The thing is we have done the 7-8 acquisitions in the past. So there are acquisitions where it is a clean slate like in the case of Vizag, Anantapur, Ongole where there is nothing that we will have to work, it is almost as clean as possible and we just bring new doctors and grow faster. But both what we have seen with Sunshine and Nagpur and I think this will be a

pattern that we will see moving forward. Now as you go in and try to create change within the organization, it kind of leads to resistance, then it kinds of leads to a lot of bad blood. So we need to give the current doctors and the new management sometime for them to understand each other's way of functioning and build confidence and we typically have seen we do at least six months in trying to build that confidence among the current set of doctors and then be able to bring new doctors from outside because there is a name change, then organization cultural change. The doctors outside are also trying to understand what is happening to this company. A new brand is coming, whether it be Sunshine or whether it is Nagpur. So there is a lot of insecurity in the organization because a new brand is coming. So we need to build that confidence, which takes that much time. It takes multiple meetings, multiple discussions with them. Once we have the confidence that things in house are more in control and there is a lot of security among doctors, management and rest of the people that is when we start looking at bringing in new doctors and new talent for us to grow. So that is what we have seen in Nagpur for the last six months, you stabilize things internally. Now we are starting to look at bringing doctors from outside which we will do in the next six to nine months and then we will start seeing the growth happening even on the cost optimization if you go in and try to bring in aggressive measures, do cost optimization it again leads to the same thing is the company more focused on cost than quality? What are they trying to do? What is the direction? So these are all the questions that they keep

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Page 11 of 24 continuously raising. So before we do anything drastic,

we need to win their confidence.

We need to build a relationship with them and then start taking things forward and that is what we have done because this hospital already has 30 doctors in different specialties, certain doctors of high repute. It is important to win their confidence before we drastically make measures in cost optimization and bringing in new doctors.

Mayank Hyanki : Sir, can you just give us probably, the number of doctors who were there in this hospital when you took over and how many are there right now? And has if there has been any attrition among senior doctors in the last six months and how many do you want to finally take this number to?

Abhinay Bollineni: So we currently have around 30-35 doctors in Nagpur and I am talking about the doctors who are admitting patients in the hospital. There are others doctors which I am not talking to you about. Out of the 30-35 doctors, we have lost three doctors when we came in and these three doctors experience are in the range of three to five years, the doctors that we have lost, we immediately replaced them with three doctors of the similar caliber or slightly senior. In addition to these three, there are only two new doctors that we added in different specialties. That is how it has been, so the attrition we have controlled any attrition, we have controlled any exists of the doctors other than the three people who have left and those three left one month after we got into the system.

Mayank Hyanki : And in future plan additions in the next 6 months or 12 months or are we done with doctor appointment now and this is the stable strength.

Abhinay Bollineni: No, so this 30 doctors are enough for running the current volume of the hospital. But if you want to double or increase the revenues by 50%, we will have to bring in more doctors and we intend to bring in at least 12 to 15 doctors in the next six to nine months.

Mayank Hyanki : So that will happen as and when the revenue ramps up, right, the patient flow.

Abhinay Bollineni: No, the doctor comes in first, then the revenue ramp up.

Mayank Hyanki : So we will add more doctors in the next two, three quarters.

Abhinay Bollineni: Correct, two, three quarters we should add more.

Mayank Hyanki : Great. Sir, this is very helpful. Thank you and all the best.

Bhaskar Rao Bollineni : Usually the acquisitions are a double edged pan unless you go very slowly, change the culture, understand the mindset of the employees and the patient requirements and the need

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Page 12 of 24 of the facility of the new therapies and all instead of going faster pace, I think it will be a slow pace that we will be able to yield we are very, very confident there is a great potential and we will be able to achieve that. That is where purposefully we are going very slowly to change the entire thing. We are in the right direction.

Mayank Hyanki : That is very helpful. We have already done multiple successful acquisitions in the past with the past decade or so. Wish you all the best.

Moderator : Thank you. We take our next question from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Ahamed : Good morning. Thanks for the opportunity. So during FY2023 we have acquired minority stakes at various hospitals at Sunshine, at Kondapur. So how much have you spent for these

stake increases and do we intend to continue acquiring minority stakes going forward as well.

Vikas Maheshwari: We have increased our stake in the 3Q which is Sunshine Hospitals and one of our Kondapur facility company, overall the amount of roughly 95 Crores, 96 Crores is to acquire this minority interest and in the previous calls also we have told that yes, we remain inclined to increase our shareholding of these subsidiaries as and when this opportunity arises, we are more than willing to acquire the share based on the agreed value which is a win win situation for all this.

Hari

th Ahamed : And looking at your specialty mix for FY2023, I see that oncology is roughly 5% of our overall revenues and given that this is a high ARPOB specialty are we looking to add oncology centers at some of our hospitals where we do not have this capability currently and 5% seems to be on the lower side when I compare with some of our peers. So that is why the question.

Abhinay Bollineni: So of the current ex Nagpur hospital, eleven facilities, we should add oncology in six hospitals may not be end of this financial year. But maybe in the next 24 months, all the six hospitals should have oncology, but moving forward including Nagpur, Thane, and Bangalore and Nasik. Nashik unfortunately already has oncology. But the other hospitals that we are having, it is designed to have accommodate oncology right from the beginning.

Harith Ahamed : And just a clarification on the Thane project. Are we having any discussions with doctors to co-invest in the project along with us because that has been our template for expansion especially in newer markets, so anything on those lines?

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Page 13 of 24 Abhinay Bollineni: Absolutely. Both in Nashik and in Thane. We are talking to doctors to co-invest along with us and be able to be part of our growth story that we want to create in Maharashtra, Nasik, we are seeing very promising results so far. Thane we have met doctors two years back there was a lot of interest in because the project got delayed we took time for acquiring the project we did not go back today but I think once the work starts we will approach all of them again and we are pretty confident that we will see good traction going forward.

Harith Ahamed : That is all from my side. Thanks for taking my question.

Moderator : Thank you. We will take our next question from the line of Pritesh Chheda from Lucky Investment Managers. Please go ahead.

Pritesh Chheda : Sir, my first question is on your Telangana and AP matured assets, both assets. What is the room for growth in these assets? Because they contribute about a 75%-80% of your EBITDA and when we look at the OR and all it is a fairly boost up assets. So what would be the levers there?

Abhinay Bollineni: As far as Telangana is concerned. We have Secunderabad and Kondapur. So Kondapur where almost doubling or tripling our bed capacity for that facility, we are running at a 90 plus kind of an occupancy. So once there is more capacity the current specialty will continue to grow aggressively plus we will add new specialties like oncology, transplant and so on and so forth. But are still further scale up and reach a higher occupancy from the Kondapur asset. Likewise in Secunderabad, which is our flagship hospital. We have reached a certain size and scale in terms of our occupancy. We are also looking to expand and add more beds in that hospital, there is ample land available and the area of focus is that new expanded hospital will be oncology. We want to scale up oncology in a much large scale and size, mother and child and gastroenterology and obviously we will create more room for the other specialties to continue to grow. That is as far Telangana. AP acquired has four assets which are our legacy assets which is Rajahmundry, Nellore...

Pritesh Chheda : No, I wanted to only know about Telangana and AP matured not AP acquired.

Abhinay Bollineni: I am talking about AP Matured. Which are our legacy assets, which is Nellore, Rajahmundry and Srikakulam and all three of them we are in fact doubling capacity there but one we have limited specialty so we are adding specialties like oncology mother and child once the capacity addition happens. This will help those hospitals grow.

Pritesh Chheda : So these 1200 plus 645, 1845 beds. How much more can it go? Or let us say the operational bed is 1100 plus 600 it is about 1700.

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Page 14 of 24 Abhinay Bollineni: So Telangana 1200 will

go to close to 2000 beds and your Andhra 645 will double to almost 1200 beds.

Pritesh Chheda : So 800 and 600, so then another 1400 beds?

Abhinay Bollineni: Correct.

Pritesh Chheda : So then the next question so whatever your expansions that I can see, which is Bangalore, Nashik and Thane they are slated to come in FY2025, right?

Abhinay Bollineni: 2025.

Pritesh Chheda : FY2025 right all the three, whether it is half yearly or yearend I do not know, but we will be FY2025. So a lot of benefits only will be in FY2026. The existing assets, what is the bed addition that you have planned for 2024 and 2025. So this 1400 beds which you are referring to what is the plan of action in terms of addition?

Abhinay Bollineni: Unfortunately, none of them will happen in this financial year, which is FY2024. In FY2025 Kondapur expansion will happen. In FY2026 you will start seeing bed addition from Telangana and AP mature.

Pritesh Chheda : So then for FY2024, your existing assets have any room for growth?

Abhinay Bollineni: Yes, so existing Telangana, if you look Secunderabad, still is at a 70% occupancy. There is headroom for growth for the next two years. Whatever expansion we are doing, we are anticipating the growth beyond 3-4 years and then doing it today.

Pritesh Chheda : So when you say you have expanded scope for improving the performance of these assets can these assets grow high single digit which is a combination of double digit which is a combination of ARPOB and OR.

Abhinay Bollineni: No, I think so. The EBITDA will continue to grow at a high single digit for Telangana and AP mature, but a double digit growth on the EBITDA front would be a big difficult.

Pritesh Chheda : And my last question is when I look at your Thane investment, and when you mentioned of 450 Crores asset. Do we have an internal ROIC metrics because when you do a 450 Crores asset and if you had to put in the 450 Crores when you are not clear as of now what kind of asset light you want to do but let us say it is a 450 Crores asset, then the ROIC in this asset will be less than 15%. So do we have an internal metrics of deciding on an investment or?

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Page 15 of 24 Abhinay Bollineni: We do have an internal metric and that is much higher than 15% which we have done historically. But as far as Thane is concerned we are pretty confident that this investor will move forward. The dialogues have been ongoing for the last 12 months and because of the legal clearances off late, we are just trying to put these together. So when that does happen it will again put back in perspective and how we historically do something. The other reason why we have decided to move forward on Thane is given that the Maharashtra Cluster that will be fixed it very clearly and we are buying in a lot of time typically hospital in Bombay will take 4-5 years to operationalize. This will be operationalized in 9-12 months and it is important for us to have as it is coming in. So we build more confidence with the doctor community, but when we announced the structure very soon and then maybe in next 35 to 40 days you will see it will be very clearly aligned to what we have historically done.

Pritesh Chheda : And just hypothetical question, if it was a 450 Crores investment from your side which is 1.5 Crores a bed? Then what is the ROIC of multispecialty hospital come out to at a 450 Crores 1.5 Crores investment?

Abhinay Bollineni: I think the hospital is capable of, so how do we look at it, we look at it as one time asset turn 1 to 1.25 time asset turn and a 25% margin. So the hospital still has the ARPOB and bed capacity to achieve a onetime asset turn or a 1.25 asset turn and 25% margin is something we have historically demonstrated we will continue to do that in this facility too.

Pritesh Chheda : So in that case ARPOB is not 40000 when the ARPOB is much higher assumption?

Abhinay Bollineni: It is much higher.

So the current ARPOB in Thane...

Pritesh Chheda : Because when you take 40000 on a 300 bed in to 0.7 as utilization then the revenue is 300 Crores and as soon as you take 400 Crores then the ARPOB is shoots to 50000-55000.

Abhinay Bollineni: So Current ARPOB of FY2023 of a competitor in that market is around Rs.39000 and that is today and you typically assume that the hospital takes 4 to 6 years for maturity and the ARPOB will continue to grow as you start doing more complex work. So today transplant in Thane is very insignificant. So that will grow, oncology is very insignificant, that will grow. So we are pretty confident that as more hospitals come the ARPOB also will grow up.

Pritesh Chheda : And just one last clarification question. When you look at the slide 20 where you have a AP matured asset and the Telangana matured asset for the ARPOB difference the margin

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Page 16 of 24 difference is not there so what happens is just that the whole P&L with respect to the doctor cost changes between the two assets.

Abhinay Bollineni: Sorry could you come again I did not follow your question.

Pritesh Chheda : I said if you look at the slide 20 for our ARPOB difference that we see a 50000 versus

16000 where everything else is the same right the utilization is the same, the losses are actually the same but the margin is also the same so what happens in a P&L of 16000 ARPOB and what happens in a P&L of 47000 ARPOB asset.

Abhinay Bollineni: The cost structures are very different right. So doctor payouts are different, HR payouts are different, local cost of operations are different.

Pritesh Chheda : So basically the surgical or that case mix that you have will be completely different here and hence the doctor cost will be completely different.

Abhinay Bollineni: No case mix will more or less be 70% there in case mix you will not see substantially different but the doctor cost if you are paying a doctor 7, 8 lakhs in a city like Hyderabad you will end up paying 3 lakhs there. If your average cost per employee is 35000 in Hyderabad your average cost of employee there will be around 18000 to 20000. So there is a difference there and the way the kind of technology that you would have invested for and followed by their AMC, CNC contracts will still be very different from what you are doing there. So based on the market that we are operating and the ARPOB that we can expect from the market you will have to tailor make the cost accordingly.

Pritesh Chheda : And lastly what is the debt figure you mentioned for FY2024 when you would get into this Capex for a large Greenfield Capex that was announced. So what will be the debt go up to.

Vikas Maheshwari: What you have to look at is the two years horizon because the whole project which is Bangalore, Nasik and the Thane over those 12 to 15 months' time period so if you look at our cash flow we are generating roughly Rs.400 Crores of the revenue cash flow for the full year and if you take a holistic two years picture we are generating something around Rs.800 Crores cash and that is the amount which will be investing for our growth project which we have taken plus, minus here and there. So some debts may come up front depending upon the projects which we are taking up whether it is like Thane we are not still decided. So based on that some debt may come up front, but over a period of time in next few years times when we are speaking with you in FY2025 we will be in the same situations of very comfortable debt to EBITDA we are a very conscious company and a disciplined company

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Page 17 of 24 and we believe that we should not cross 1.5 to 1.7 times of debt to EBITDA this is what is the board has given us the guidance.

Pritesh Chheda : What is the net debt right now?

Vikas Maheshwari: It is roughly 300 Crores.

Pritesh Chheda : Thank you very much and all the best to you ge

ntlemen. Thank you very much.

Moderator : Thank you. We take our next question from the line of Omkar Kapekar from Pandu Port Pool. Please go ahead.

Omkar Kapekar : First one question with respect to the lease term. So when we take any asset that is on the asset light model, so what is the average lease term approximately.

Abhinay Bollineni: Usually we have a 30 year contract, we only have two leased assets and both of them are on a 30 years contract.

Omkar Kapekar : And is there any provision for you to takeover that asset at the end or there is a renewal period.

Abhinay Bollineni: The current assets we do not have such a provision, but moving forward like for Thane and the other one we are negotiating such that we can buy back at the end of 10th year or whenever we feel appropriate after a lock-in period.

Omkar Kapekar : And with respect to the time to mature? I think you just previously mentioned, it will take approximately 3 to 4 years to mature a hospital fully.

Abhinay Bollineni: Four to six years.

Omkar Kapekar : Four to six years, okay. So with respect to the additions that we are making and expanding in other geographies. So will we continue to expand our base of operations throughout the country or would we like to digest this growth a bit and then move on forward.

Abhinay Bollineni: I think what we have already committed to a substantial number of projects and substantial number of bed addition in new geography, we will first allocate consolidating the position here in the five, six hospitals that we have visibility on and then look at further growth beyond this. But further growth beyond this also will be concentrated in these markets that we opened up now. It will not be that we will go to a completely different geography in

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Page 18 of 24 North or in East somewhere and try to do something very aggressive and big. We will try to do more stuff in Maharashtra and Karnataka.

Omkar Kapekar : So we will do expansion in the current circuits and consolidate our position, and digest the growth and expand it further.

Abhinay Bollineni: At the maturity of the hospital, just to add one point, also depends on the size of the

hospital. So what typically happens is sometimes to size the hospital always corrected is difficult. So sometimes you may invest a little more upfront today knowing that it will take a little longer because you can only build that kind of a structure of a hospital. So ideally should take four to six years, but if you are building a hospital with a larger view of being able to fill it over a period of time, then it take seven to eight years.

Omkar Kapekar : And finally two clarifications. The Thane hospital is 300 beds. So all the beds would be operational or I think you mentioned but I could not get it. All 300 beds will be operational when it comes online.

Abhinay Bollineni: Yes. So everything will be equipped for 300 beds, but we will stack it again only for a 150 beds as we keep occupying we will stack the rest of the bed.

Omkar Kapekar : And final clarification on the doctor payout method. So as you mentioned earlier to the previous colleague of ours in the con call that the payout section differs from hospital to hospital. So is there a policy where we can standardize it because it was previously referred that when the matured assets of Telangana and AP have similar metrics, but they have different ARPOBs. So if there a way by which that could be standardized so it makes it slightly more apple-to-apple comparison with respect to even between assets. So is there something on the cards on that?

Abhinay Bollineni: Are you saying that is the doctor cost payout structure different hospital to hospital. Is that the question?

Omkar Kapekar : Yes.

Abhinay Bollineni: So the fundamental approach to how the payout happens is uniform across the hospitals and across state. There is no difference in that. But in each hospi

tal and based on the maturity of

the hospital and the maturity of the doctors in the hospital, they keep progressing from a minimum guarantee model to a fee for service model. You just said more the mature hospital, the more mature doctors you have and that is why you will see more fee for service doctors. If it is a young hospital, then you will find more doctors on a minimum guarantee model.

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Page 19 of 24 Omkar Kapekar : Okay.

Moderator : Thank you. We take our next question from the line of Sagar Shah from Philip Capital.

Please go ahead.

Sagar Shah : Actually almost all my questions have been answered. Just a couple of questions actually.

So first of all, I missed out on that point. I wanted clarity on the Capex front on FY2024, we have already spent around 570 Crores of Capex in FY2023. So what is the Capex that you are likely to spend in FY2024 and Capex consist of which hospital?

Vikas Maheshwari: There is a two ongoing projects which is the Bangalore and Nashik. On that we should invest further. At the same time we are discussing about the Thane but depending upon the structure that Capex outflow will happen. I think you got the numbers on this. In addition to this Dr. Abhinay spoke about few of the Capex being done in terms of oncology additions and few of Andhra units so those Capex will be there. So you should assume something like that Rs.100 Crores towards oncology bed additions or gas flow units, mother and child unit and the relevant equipment which we are adding the Capex towards that. So overall the Capex 250 towards Bangalore, Nashik plus 100 Crores is on the regular bed additions in the existing unit strengthening of the unit. On that and plus Thane depending upon the structure which is happening.

Sagar Shah : So basically 250 Crores is the Capex that we are likely to spend that is remaining for Nashik and Bangalore, right?

Abhinay Yes.

Sagar Shah : And these two hospitals would be operational by Q1 FY2025.

Abhinay Bollineni: The idea is to do it early, but that is the timelines we are giving to the investors.

Sagar Shah : So out of the total beds, I think 350 beds are from Bangalore and around 350 again are in Nashik. So we will be opening around 150 beds for both of them, right, in the first year FY2025.

Abhinay Bollineni: Correct. We will operationalize 150 and as the occupancy and traction goes up we will be opening up further beds. Though the infrastructure will be ready for all the capacity which we are just doing.

Sagar Shah : Yes in FY2023 can you give a breakup of the Capex that we have spent.

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Page 20 of 24 Abhinay Bollineni: Yes, see that one of the Capex's towards which we did not discuss is this Sunshine Hospital, which we now buying out the building and the Capex is going on. So the majority it is towards that and plus Bangalore and Nashik.

Sagar Shah : So basically now my final question is related to our margin expansion in acquired hospitals which are based in Andhra Pradesh. So basically any update on that, because that is one of your big growth drivers for FY2024 along with Sunshine Hospital. Sunshine Hospital you have already explained what is the driver, but I wanted to understand about the acquired hospital, so anything on that front, Abhinay Sir would like to contribute that?

Moderator : Dr. Abhinay's line got disconnected. We are reconnecting?

Bhaskar Rao Bollineni : What is the question?

Sagar Shah : For acquired hospitals in Andhra Pradesh, are we anticipating increase in ARPOB, the change in payer mix?

Bhaskar Rao Bollineni : You talking about Andhra or only at Telangana? Both.

Sagar Shah : Andhra.

Bhaskar Rao Bollineni : So Andhra acquisitions what we have done is only is almost coming to maturity then definitely we are adding as I mentioned in my initial achievements we are adding more and more complex procedures performing there which was not happening in

the entire Andhra

Pradesh state, they are all coming to either at Telangana, to Chennai, Bangalore. In all those areas, we are planning to improve the transplant programme, oncology programmes, even the complex surgeries in other areas to treat. So that definitely the ARPOBs will definitely increase.

Sagar Shah : Okay got it.

Moderator : Thank you. We take our next question from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal : Sir, two questions. One is, in the current quarter, in the Telangana cluster, there has been a pretty sharp improvement in the operating margins. So any specific drivers for the improvement in the margins about 35% for the quarter?

Vikas Maheshwari: You are talking about, sorry we missed, which cluster you are speaking about?

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Page 21 of 24 Nitin Agarwal : Telangana.

Vikas Maheshwari: Yes, Telangana the margin is because of the throughput basically if you look at the revenue grew by something like that on quarter-on-quarter by Rs.15 Crores or so, and there was some reduction in terms of our consumable costs and the doctor's payout cost. It is a combination of both, we believe that plus, minus 1% we should be able to maintain this EBITDA margin, do not take us for the quarter-on-quarter moment, but overall plus minus 1% we should be there.

Nitin Agarwal : And in Thane, by when do you see the project, the first set of beds getting commission in the Thane project?

Vikas Maheshwari: It should take 12 to 15 months from the time we are speaking. So it should be quarter first of FY2025, which is the April to June of next year. Sometimes like that. Plus, minus one month.

Nitin Agarwal : Just to get a sense of that. Is the civil structure in place for the project.

Vikas Maheshwari: Yes, so the land and building is completely ready. The building was built only for the hospital and it has the necessary requisite approval. So the launch to the hospital time is slightly better. It is beneficial for us. That is where we have gone ahead with this project and we have to do few modification in the building refurbish and put up the medical equipment that is why we are saying that we should be ready with this facility by June 2024 which is first quarter of FY2025.

Nitin Agarwal : Just to confirm, you mentioned that the land building cost of 250 Crores?

Vikas Maheshwari: Yes, the balance will go towards the equipment, putting up all the necessary infrastructure, IT systems, furniture, fixtures, etc., etc.

Nitin Agarwal : This cost of about 200 Crores for a 375 beds hospital in a relatively upmarket place like Thane is not that on the lower side given the fact that most of the competitors have been talking about 1 Crore plus, 1.5 Crore on the brownfield expansions typically in the past.

Vikas Maheshwari: Not sure, but we believe that even if the land and building is coming, if the cost comes to us, it is roughly 450-475 or 300 type of beds is Rs.1.5 Crore including land and building. We believe that yes it is an up market the ARPOBs are high, margins can be better because this is the less competitive scenario right now there. Not sure that the figure which you have got for even if it is brownfield X on the least Rs.1 Crore for bed, not sure about that.

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Page 22 of 24 Nitin Agarwal : Thank you.

Moderator : Thank you, Sir. We take our last question from the line of Alankar Garude from Kotak. Please go ahead.

Alankar Garude : Thank you for the opportunity. Sir, you laid out the progress in Nagpur earlier in the call

quite well. Similarly, can you please update us on the progress in Sunshine because we have seen some fluctuation in margins out there and Karimnagar has also gone out. So broadly how has been the like-to-like revenue performance and also on margins? If you could comment that would be really helpful.

Abhinay Bollineni : I am glad you asked this question. I am waiti

ng for someone to ask me this question, if not I

have to just answered it myself. So on Sunshine, so when we first look at Sunshine as I said we will make that investment, there were two assets that we have our key Secunderabad asset and the Gachibowli asset and so when we look at Gachibowli it is very similar to how our Kondapur is in terms of its size and scale very, very similar, but when we look at it also it was highly competitive and by then larger hospitals in that space were coming up. So we were always aware that it will take us a little longer to attract talent to this hospital because the big guys would not want to come there, because of the size of the facility, we will have to bring in younger talent and give them time to scale up their hospital. So that is exactly what has happened. The last one year, the financial year, we have added close to 8 clinicians. We actually got in talent better than what we had originally planned. But we always thought we will take this as a long-term project. We cannot look at it as a quick turn around and bring doctors immediately and that is what exactly is happening. So we still see gap in 4-5 departments in that hospital if you look at the March number for Sunshine Gachibowli it has done the highest ever around 13.5 Crores of revenue and that is because we fixed one of the key department that is hovering for a long time. So we believe if we add another 5-6 doctors which we are seeing visibility towards the end of this calendar year to happen, that hospital will further scale up from 13.5 to around 16-17 Crores and then moving forward it will further scale up to a typical 18-20 Crores of revenue and this is how we had originally talked through the hospital and maybe there is a six month, nine month delay in trying to get the right talent because hurrying up and getting the wrong talent will further delay the scale up much more than waiting and getting the right talent. So that is as far as Gachibowli is concerned, we are pretty happy with how it is shaping up, because once we get these balance four, five doctors, it will be an auto mode. Coming to Begumpet unit. We were very sure that the current hospital we will not be able to attract talent at all because of the way it is positioned, the way it is located and the infrastructure quality especially having the KIMS flagship of the hospital around and another competitor we are very difficult to attract talent to that hospital. So that is when we decided we have to move KIMS Hospitals

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Page 23 of 24 it to a new campus, redo the entire facility and position it as a multispecialty that is an ortho dominant hospital, which is what we did by going ahead in that buying out this land and building. Unfortunately again six months delay in the project execution which anyway is now has complete. There is a lot of excitement around that hospital. Once we move to that hospital in six months we should be able to bring in 10 clinicians and once that 10 to 15 clinicians join we will see an incremental revenue of 7-8 Crores of revenue, which will then change the EBITDA margin profile of the Begumpet facility likewise when the balance doctor in Gachibowli join they will also change the EBITDA profile of that hospital.

Alankar Garude : This is very helpful Dr. Abhinay. Just one thing is once you had indicated earlier, once the shift happens at least for the period of say 20-30 days, we will have doubling of cost. So possibly we might see that impact either in the first quarter or in the second quarter of FY2024.

Abhinay Bollineni : Correct.

Alankar Garude : The second question is, you mentioned about broadly a six to nine months of stabilization time for all your future acquired assets going forward. As you grow your network, how do you plan to manage this from a leadership bandwidth standpoint?

Abhinay Bollineni : I think we have already identified the key leaders in the organization who

are going to play

key roles in leading these hospitals. So whether it is Thane or Nashik or even the Bangalore cluster, the teams are already on-boarded, they have been running different hospitals for us today and they will be relocating to these new facilities when we go there. So one we have had the experience of working with them for the last 18-24 months and for the next 12 months. So they understand how we want to run the facility, what the culture of KIMS is. So when they relocate to the new hospital they will be able to inculcate the same practice. As far as doctor management is concerned. I think that is something we will continue to be very core to us and that is where our bandwidth and focus is to be able to do it and it is not an exercise that you have to keep continuously doing it. I think every hospital needs attention from the first 6 to 12 months and once you cross that period, the local leadership is capable of handling that and being able to sustain them, it would not consume after the first

9 to 12 months. It will not consume as much bandwidth of the senior leadership as it does in the first few months.

Alankar Garude : Understood. That is quite helpful. Thank you and all the best.

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Page 24 of 24 Moderator : Thank you. Ladies and gentlemen, we have reached the end of the question and answer session and I would now like to hand the conference back over to the management for closing comments. Over to you gentlemen.

Bhaskar Rao Bollineni : Basically whatever is the question that is asked for the last one hour to sum up, basically we are very, very clear understanding and the ROE, ROC. When we go some of the times we need to put an opportunity cost which we need to and when there is a low ARPOB and high ARPOB the EBITDA margins, how we can able to establish looking the number of beds, the per bed, the number of doctors we involve in the treatment and with all these a combination of rather than working as an independent to each unit wise and holistically we will look into that and that will be able to create the leadership recruitment and the opportunities and the succession plans are in line and maybe one quarter, two quarters this way or that way whatever is the leadership that we acquire and to understand it all that we may not be 100% sure that all of them what we have been looked into that that they were able to deliver, but it is a work going on, it is a continuous process. So with all these things I would like to sum up saying that the year gone by proved to be productive and rewarding. We will continue on the same growth trajectory and strive to reap up even greater results in the current year. The road ahead is optimistic, we are critically evaluating new opportunities for expansion and further growth that will result in sound value addition. I will continue to interact with you and keep you posted of developments. I am happy that KIMS had firmly establish its niche as an organization of quality and values and one feels proud to be associated with us. We will continue to uphold and nourish this reputation. Thank you for your trust and investment in KIMS. Thank you.

Moderator: Thank you very much Sir. Ladies and gentleman, on behalf of IIFL Securities Limited that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Call: Aug 2023

11th August, 2023

The General Manager,
Department of Corporate services
Bombay Stock Exchange Ltd (BSE)
Phiroze Jheejheebhoy Towers,
Dalal Street,
Mumbai - 400 001.
Scrip Code - 543308
ISIN: INE967H01017 The Manager,
Listing Department
National Stock Exchange of India Limited,
Exchange Plaza, 5th Floor,
Plot No.C/1, 'G' Block
Bandra - Kurla Complex
Mumbai - 400 051.
Symbol - KIMS
ISIN: INE967H01017

Dear Sir/ Madam,

Sub: Transcript of Earnings Conference Call with Analyst /Investors.

In continuation to our letter dated 1st August 2023, the Company organized a conference call with the Investors/ Analysts on Tuesday, 8th August 2023 at 9:30 AM (IST). A copy of the Transcript of the conference call held with the Investors/ Analysts is enclosed herewith and the same has also been uploaded to the Company's Website at <https://www.kimshospitals.com/investors> > Disclosures under Regulation 46 of SEBI (LODR) Regulations, 2015 > Analysts Calls schedule, PPT & Transcripts > Transcripts, Audio & Video Recordings > FY 23-24.

Kindly take this intimation on record and oblige.

Thanking you,
Yours truly

For Krishna Institute of Medical Sciences Limited

Uma Shankar Mantha
Company Secretary & Compliance Officer
MANTHA
UMASHANKAR Digitally signed by MANTHA
UMASHANKAR
Date: 2023.08.11 11:34:36 +05'30'
Page 1 of 21

"KIMS Hospital
Q1 FY '24 Earnings Conference Call"
August 08, 2023

MANAGEMENT : D R. BHASKAR RAO BOLLINENI – FOUNDER AND
MANAGING DIRECTOR – KIMS HOSPITAL
D R. ABHINAY BOLLINENI – EXECUTIVE DIRECTOR

AND CHIEF EXECUTIVE OFFICER – KIMS HOSPITAL
M R. SACHIN SALVI – CHIEF FINANCIAL OFFICER –
KIMS HOSPITAL

MODERATOR : M R. RAHUL JEEWANI – IIFL SECURITIES LIMITED

KIMS Hospital
August 08, 2023

Page 2 of 21

Moderator: Ladies and gentlemen, good day and welcome to Q1 FY 24 Earnings Conference Call of Kim's Hospitals hosted by IIFL Securities. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Rahul Jeewani from IIFL Securities. Thank you and over to you.

Rahul Jeewani: Good morning everyone. I am Rahul from IIFL Institutional Equities. I welcome you all to the quarter one earnings conference call of Kim's Hospitals. From Kim's, we have with us Dr. Bhaskar Rao Bollineni, Founder and Managing Director, Dr. Abhinav Bollineni, Executive Director and CEO, and Mr. Sachin Salvi, CFO. Over to you, sir, for your listening comments.

Management: Good morning, everybody. A warm welcome to each of you. Heavy rains and floods have battered many parts of our country, causing havoc. My heart goes out to those who suffered losses on account of nature's fury. Kim's is known for its clinical excellence and repeatedly demonstrated the same over the years. I would like to share with you a few of the rare surgeries performed at our various centers which are primarily Tier 2 and Tier 3 towns. It is a matter of great pride that such rare procedures are undertaken in these relatively small towns, wherein previously patients were required to go too far off cities involving heavy expenditure, loss of precious time, and severe strain on account of staying away from home.

This phase marks an advancement in medical reach to the population in urban and rural areas. Time is crucial in many cases and a few hours of delay can make the difference between life and death. I am going to talk about a few rare cases in the different hospitals in our group. In a first at Nagpur, doctors at Kims Kingsway performed an awake bypass surgery on a 72-year-old man without administering general anesthesia the patient was awake and talking during surgery and one more 11 months Anushree was Central India's youngest cochlear transplant recipient. The 11 month old had severe profound hearing loss at birth. She was gifted hearing capability through cochlear implants a very advanced solution in hearing sciences by doctors at Kims Kingsway, Nagpur.

A baby born in the seventh month was cured of a rare CMV infection after treatment for 90 days. He underwent multiple episodes of ventilatory days, lung infection and lung collapse, transfusion-related lung injury, feed intolerance, transient renal tubular acidosis, neonatal jaundice, and low platelets. The doctor successfully treated the boy who was discharged after 90 days of neonatal intensive care unit stay.

Complex heart surgery, an emergency complex heart surgery within 30 hours of birth was performed successfully at Kim's Cuddles, Vizag. It was technically a big challenge for surgeons to perform such a surgery on a neonate whose tissues of the heart are very fragile and tender. Such a treat is one of its kind in the history of Vishakhapatnam. After one month of discharge, the child is gaining weight and spreading joy.

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Kidney and liver transplant to the same patient on the same day. A 50-year-old patient suffering from liver cancer and chronic kidney disease was given a new lease of life by doctors at Kims Icon by transplanting both kidneys and liver. Availability of the state of the art equipment and a dedicated team of doctors enable the hospital to perform more and more organ transplant surgeries.

A hydrated cyst of the kidney is a very rare condition with an incidence of 1 in 5 lakh population.

The surgery is difficult because if the cyst gets ruptured and the contents spill, it can lead to immediate anaphylaxis and the patient can die on the table. The surgery was done free of cost under the Aarogyasri scheme at Anantapur.

A 36-year-old woman from Maharashtra came to the hospital with repeated shocks for ventricular fibrillation, which means it is a rapid heart rhythm leading to cardiac arrest. She

would experience as many as 30 shocks in a single day. The doctors designed a special strategy and a two-hour-long procedure was performed. The patient was able to walk on her own on the next day and she was discharged on the following day. The kidneys of a brain in a dead baby were transplanted successfully to a 60-year-old woman.

This is the youngest kidney of all time in Kim's history and the second such successful transplantation in Telangana and AP states. This is technically challenging in the view of pediatric donors. The extraordinary task could not have been possible but for the kindness and selfishness of the donor family. Their act of humanity exemplifies the power of organ donation and serves as a sterling inspiration.

A complex pancreatic surgery was performed laparoscopically on a 49-year-old patient. It was done in a laparoscopic surgery, not as the ripple procedure where the incision is very small. It causes less pain and enables faster recovery. The patient recovered quickly and was discharged within one week. Due to its technical complexity and skill required, the procedure is done in very few centers in Hyderabad. Better quality of life is also possible in these surgeries compared to open procedures. This has been done in Kims, Konapur.

The patient was suffering from a stiff neck and spine for over 5 years and was not able to walk, erect and bend or sleep properly. A complex surgery called Pedicle Subtraction Osteotomy lasting 8 hours was done by the doctors and the patient at last is able to stand erect and regain his dignity. The deformity is gone.

A 48-year-old woman came to the hospital complaining of severe pain in urinary bladder. There was a tumor and it tested benign. However, before the surgery could be performed, the tumor first slipped down and blocked the urethra. She stopped urinating. Immediate surgery was done and tumor was removed. When it was first sent for testing, it was found to be a rare type of inflammatory pseudotumor. It is rare in medical history. There are only about 100 cases worldwide to date. Such a case was done in KIMS Hospitals Kurnool successfully and the patient was relieved.

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Doctors at KIMS Kurnool treated a 19-year-old boy who was suffering from the rarest NORSE disease (New-onset Refractory Status Epilepticus) which is a rare devastating condition that can abruptly occur in previously healthy people mostly children. Awareness of this disease is very limited even among our medical community. The boy remained in the hospital for 6 weeks before he was treated and discharged. He has also performed in Kurnool. Doctors at KIMS Kurnool performed a complicated surgery on a 43-year-old woman having a tumor-like thing around her liver which is called hemangioma in a medical parlance.

There was swelling in the liver due to internal bleeding. To arrest further bleeding all the blood vessels were blocked and next day surgery was done and tumor removed. It was done free of cost under Aarogyasri scheme again from Kurnool Hospital. An 8-day-old baby successfully treated and discharged. 8-day-old baby was put on peritoneal dialysis for 5 days continuously and successfully treated for renal failure and seizures. The baby was discharged after 21 days of NICU. The treatment was done free of cost under Aarogyasri scheme.

Let me now present the financial highlights for Quarter 1 FY23. Gross revenue at INR609 crores, a

growth of 21.6% on year-on-year and a 4.9% on a quarter-on-quarter basis. Operating EBITDA of INR160 crores, an increase of 12.4% year-on-year and a decrease of 4.7% on quarter-on-quarter. EBITDA margin at 26.3% versus 28.5% in quarter 1 FY23 and 28.9% in quarter 4 FY23. PAT at INR87 crores in quarter 1 financial year 24 against INR79 crores and INR99 crores in quarter 1 FY'23 and quarter 4 FY23 respectively. Conducted 3.83 OPD consultations, consultations in quarter 1 FY24 up by 18% and 0.4% in a year and quarter-on-quarter basis. Bed occupancy in Q1 FY24 at 70.4% up 5% year-on-year and down by 0.4% and quarter-on-quarter basis.

The average revenue per operating bed increased to INR31,697 in quarter 1 FY'24 showing a 4% year-on-year and a 3.7% growth in quarter-on-quarter basis. Cash and cash equivalents at INR178 crore in Q1 financial year 24. Net debt-to-equity ratio at 0.22. Academics, I'm happy to share with you that recently KIMS Hospital is accredited by American Accreditation Commission International for having demonstrated compliance with the international accreditation standard for excellence and patient safety. This is yet another feather in our cap as we dedicatedly continue our march in pursuit of excellence.

The events, the conferences that we have done in KIMS, a two-day Annual Conference of Indian Neuro Gastroenterology and motility association was hosted by KIMS on 29th April, 2023, attended by several experts in the field from throughout the country. The Department of Clinical Immunology and Dermatology, KIMS organized a Lupus ramp walk and Lupus awareness program on the world Lupus day on 10th of May 2023, participated by the local doctors and the

patients who have been cured.

An exclusive inflammatory bowel disease clinic is inaugurated at KIMS Secunderabad and a workshop with wide participation was conducted on 27th April 2023 so that exclusively all the people who are suffering from inflammatory bowel disease will do get benefit out of this. The Department of International Pulmonology conducted a THORACOSCOPY concern with diabetic lectures and hands on workshop on 28th May, 2023. So, that with a minimal invasive KIMS Hospital
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is enable to treat without operations of all the lung conditions. Awards and Honours Dr. Raghu Ram our breast cancer expert surgeon was conferred honorable fellowship by American Surgical Association. This is the highest recognition from ASA and there are only 99 honorary fellow from throughout the world under this.

Two studies of doctors in Euro Gynecology Department has been accepted for presentation International Continence Society Annual Conference 2023 at Toronto. Our DNB students was awarded the best poster award in Euro-Asia Critical Care Conference by the European Society for Critical Care. Our projects are moving a pace as per schedules. This quarter we launched an exclusive single specialty hospital for gastro sciences, went to operation at Vishakhapatnam, Andhra Pradesh.

We launched KIMS Cuddles mother and child brand at Nagpur and Vishakhapatnam, both the places. We have done well in Q1 and do hope the performance will be even better in the upcoming quarters. Winston Churchill said, my tastes are simple, I am easily satisfied with the best. We at KIMS too are satisfied only with the best and best in all aspects to the patients and investors. Thank you for your continued trust and support. We are open for questions.

Moderator: Thank you very much. We have a first question from the line of Dhara Patwa from Smifs Limited. Please go ahead.

Dhara Patwa: Yes, thank you for the opportunity. So I have two questions. First is like, finance cost in this quarter has actually doubled. So what was the driver for this?

Management: Thank you, Dhara, for the question. So finance cost has doubled because we have gone down from our limit

to for our Sunshine units. That is one of the reasons.

Dhara Patwa: Okay, so this will stay for the next few quarters and we should assume the same run rate for the coming years onwards?

Management: It depends upon the requirement Dhara. So there are two options available here. We can use our existing limits to spend on these capital expenditures or we can draw from the available profits.

Dhara Patwa: Okay, so next question, like the Telangana and AP cluster are pretty much stable and have an occupancy of around 80% level. So from where will the next like of growth would come? I understand that we are doing expansion in other regions, but these will take time to scale up to a level, right? So if you could throw some light on this?

Management: So there are like, we have always mentioned, if you look at our Telangana and Andhra cluster even this year, year-on-year basis we have grown at a revenue at 15%. So there is still more opportunity in the same Andhra and Telangana cluster by adding new specialties like oncology, mother and child and many more interventional procedures which will drive growth. So we don't see that Andhra and Telangana are fully saturated. There is a lot of opportunity in our existing hospitals by adding these new specialties to be able to see growth.

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And also there is a lot of margin expansion opportunity in the Andhra Pradesh cluster. If you look at Telangana, it is at 31%, whereas in Andhra it is still at 22%. There is an opportunity for us to improve that by at least 4% to 5% over a period of time. Likewise, there is also opportunity for us to grow Sunshine Hospital, which we recently acquired a while ago. But given there is some delay in the movement into the new facility, there is a delay in wrap-up. But once that happens, there is more opportunity for growth there as well.

Dhara Patwa: So what I understand is the next level of growth, majorly in Telangana and AP would come from ARPOB, right? If we are adding more specialties, it would fetch more ARPOB. So will the occupancy go to like 80%, 85%, 90% or at occupancy of 80% we can expect some improvement in ARPOB going forward?

Management: So there is a lot of bed capacity addition that we are also doing in Andhra and Telangana. We are adding close to 1500 beds over the next 24, 36 months to accommodate the growth that is happening in this market plus to introduce a lot of new specialties. So with that incremental bed we will still be maintaining at around 70%-75% kind of an occupancy.

Dhara Patwa: Okay so just one last question. Recently some North-based players know they are saying that the industry is facing some challenge on the staffing and nursing side, like due to some scarcity of human resources. Since we are mostly based in South India, so are we facing any such issue or challenge, any color on that would be helpful?

Management: There has been a constant challenge on nursing staff and paramedical for a long period of time. It's not something new. It's been there in for a long period. So we are trying to solve that by setting up our own nursing colleges, tying up with the reputed nursing colleges to source staff. So I don't think it's something that is worrying us at this point in time, but the long-term solution is to increase the number of nursing colleges and bring more people into the system.

Moderator: Thank you. We have a next question from the line of Reshab Sisodiya from Sameeksha Capital. Please go ahead.

Reshab Sisodiya: First of all, congrats on a good set of numbers. I have a few questions, sir. So first, on the AP assets that we have combined from this quarter, the reporting, so if I compare it on a like-to-like basis on a rough calculation, so I find that the margin improvement has slightly slowed down. And is it also that because we added capacity in Vizag, if I

am not wrong, for Gastro. Are those of the reasons that is why we see the margin trajectory has slightly slowed down in the AP assets?

Management: Yes, that's the reason why the, because of the capacity addition in Vizag, the new Gastro unit, which will be EBITDA negative for this financial year and should be turning positive towards the end of the year. That's why there is a drag.

Reshab Sisodiya: If I look at our total cluster basis on the overall hospital space, so our ARPP has almost grown 3% quarter-on-quarter. So how much of this would be because of price hike and other difference would be because of change in mix and introducing more speciality. If you could put more color
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on that. And if I look at historic, so our orthopedics as a percentage of total share has grown from 5% to 15% in the last two years. So any specific specialities that you are looking at that you want to grow those further, if you could get some sense?

Management: In Andhra and Telangana, a significant growth in the future will come in from mother and child and oncology. But having said that, there is a lot of focus on pulmonology, gastroenterology and few other services that we are trying to focus. But you will see the bio of mother and child and oncology expand in the Andhra and Telangana cluster. As far as orthopedic expansion, because we acquired Sunshine Hospitals, naturally because it is a very dominant orthopedic brand, the share of orthopedic revenue as a percentage of the overall revenue has increased.

So far, there are few, 50% of the insurance renewals have happened in the last quarter, towards the end of last quarter. And we are expected to complete the rest of them towards the middle of quarter two or the beginning of quarter three. That's the price impact that you could see. But we have not still done a cash price hike so far because we are pretty confident about growth in volume. We don't want to focus on doing a price hike at this point in time, further cash basis.

Reshab Sisodiya: Okay, so just last one if I can squeeze in. The Nagpur hospital, we are facing some doctor attrition over there. So has that issue been resolved? Have we added almost all the doctors that you said that, we are adding in this year for this facility? And the margin expansion looks healthy on a sequential basis. What is the trajectory over here and the same for Sunshine?

Management: So like I explained in my previous call, as far as Nagpur was concerned, there was not a significant attrition. We lost two-three doctors, but the problem there was a lot of insecurity among the doctors, who were practicing. It was more to solve for that problem and stabilize the unit before we go out and acquire new doctors. It took us six, by the time we acquired it and set up our system, it was November, December.

We took good six months to stabilize internal processes to ensure that, the doctors are more comfort with the management before we start introducing a lot of doctors. Having said that, we are pretty positive on how things have shaped up and the confidence level within the hospital. We have recently had one new doctor join us, who is a very prominent cardiac surgeon and doing around 30 to 40 cardiac surgeries a month.

So with him joining, there is a lot more confidence in the market. So, we are anticipating a lot more new doctors to join us in the next two quarters and once they do join us, we will start seeing an improvement in revenue, two quarters, three quarters from there. And automatically, as a result of which, the EBITDA margins also will fall in place.

Reshab Sisodiya: Sure, sir. Overall, doctor attrition in our combined company business would be? Near doctors?

Management: Less than 0.5%.

Reshab Sisodiya: Okay, sir. Sure that's it from my side. Thank you, sir.

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Moderator: Thank you. We have a next question from the line of Nikhil Mathur from HDFC Mutual Fund. Please go ahead.

Nikhil Mathur: Hi, good morning everyone.

Management: Hi Nikhil.

Nikhil Mathur: Hi. Sir, my first question is on the margins for the different clusters. Andhra Pradesh, I understand that because of the gastro unit set up in Vizag, the margins have compressed Y-o-Y. But what is the problem in Telangana because if I look at it on a Q-on-Q basis, the revenue is stable but still the margins are down almost four percentage points?

Management: Right. So, as far as Telangana. Nikhil, we have invested a lot. So because our new expansion is happening and we are starting to acquire a lot of talent. So lot of the corporate, actually all of the corporate cost is booked in the Telangana cluster. So the dip is largely one because of the corporate cost that got added in for the future expansion. Number two, there is a seasonal variation in terms of the electricity cost going up in the first quarter, which is the usual trend across.

And there is some high cost of consumables, which have been used in this quarter because of the case mix change. So that is why the margin kind of went down from 35% from Q4 to 31% in Q1 of this year. But if you look at the full year of FY '23, we were at 31%, which is in line with Q1.

Nikhil Mathur: Right. So, sir, this cost going up, is it more related to expansion or is it some sort of, not abnormal, but slightly higher inflationary pressure that the hospital space is starting to see a bit?

Management: Not really, because the revenue growth has been quite healthy. So, the growth in revenue will take care of the growth in costs that happen on a year-on-year basis. It is largely towards the expansion of new hospitals. There's a seasonal impact. Usually, if you look at, historically, all our Q1s have always had compressed EBITDA margins, when compared to the rest of the year. So that's the impact.

Nikhil Mathur: Right. Secondly, sir, can you revisit some of the expansion initiatives? I know you talk about that in your calls a lot. I can always revisit my notes from last quarter, but it would be helpful, if you can brief us on most of the expansion initiatives.

Management: Sure. Other than the ones that are currently operational, are you referring to those?

Nikhil Mathur: So Nagpur Sunshine, I can see you disclose it separately, but apart from that, Bangalore, Thane, and all those, what are the number of beds and timelines, we are looking at?

Management: Sure. So we are talking about two hospitals in Bangalore, combined bed strength of 750 beds, both of which should be operational in quarter two of next financial year, between quarter one and quarter two of next financial year. The total outlay for both the projects in Bangalore will be around INR650 crores, that is around INR80 lakhs per bed. And both of them should be KIMS Hospital

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operational one months, two months apart from each other. So as far as Bangalore, we're pretty much on track in getting these facilities operational.

As far as Thane, we've started work. We have, like we mentioned in the previous call, we are looking to bring an investor. We have signed an SPA with the investor. We are looking to close the transaction in the next 15 days. When I say close the transaction, we are talking about transferring the money to [Anand Dhani 0:26:37] to completely acquire the asset.

The work on site has already started. So by the end of first quarter and financial year, we should hopefully have that operational as well. Nashik is something that will be operational by Q4 of this financial year or early Q1 of the next financial year. So these are the four projects that we have currently, which are going to be operational in the next 12 months to 18 months.

And then beyond this, we have expansion that's happening in Andhra, which is adding oncology centers across five of our seven facilities, adding mother and child in five of our seven facilities.

And then you have the Kondapur expansion, which is an incremental 400 beds, which is slated to begin two years from today. Yes, that's about the expansion.

Nikhil Mathur: Okay, so we are roughly adding almost 1,700 to 1,800 beds by end of FY '25, right?

Management: FY '25, yes, we should be adding. Correct, you're right.

Nikhil Mathur: And these would be capacity beds, so operational beds would be more like 1,300, 1,400?

Management: Correct, or a little less, because in the first year, we may not even operationalize more than 40% of the bed.

Nikhil Mathur: Okay. So, sir, if I look at your consol EBITDA per bed today, it is somewhere around INR25 lakhs. I know there are some underutilized assets like Sunshine Nagpur, where the cost will still

be pretty high versus the revenue generation. But if I, let's say, were to do a rough math of these incremental bets in FY '27 or FY '28, what kind of EBITDA per bed should we be looking at? Should we be benchmarking these newer assets to, let's say, a mature center like, mature cluster like Telangana or somewhere in between? So any indication of EBITDA per bed of these incremental capacities in FY '27- '28 would be very helpful?

Management: How we typically look at it is, Nikhil, Telangana, whatever incremental revenue comes in, we should assume 40% of that will straightaway flow to EBITDA. As far as Andhra, it's the same, but there is the potential EBITDA margin that we can reach is around 25% to 27%, anywhere between 25% to 27%. As far as Sunshine, we will be in line with, how we are achieving things at Telangana, in Telangana KIMS. We will get to a 30% margin over the next 24 months. And then from then on, whatever incremental revenue comes in, 40% of that will flow through. As far as Nagpur is concerned and Maharashtra, given that the cost structure is slightly higher than Telangana in other expenses, we believe that 25%- 27% EBITDA margin is, what is doable at this point in time. But as we scale, there is potential for the margin to expand as well. And since a lot of the beds are getting added to our Telangana and Andhra cluster, not in the KIMS Hospital
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next 12 months, but beyond, a lot of the beds are getting added to the Andhra and Telangana cluster, you will see a lot of these incremental beds. Even there, 40% of the revenue growth will flow into the EBITDA.

Nikhil Mathur: Okay, sure. And sir, one final question. Now, a couple of Northwest hospitals have reported this quarter, and you have also reported. It seems, they seem to be a bit done.

Management: Sorry, Nikhil, we lost you.

Moderator: Sorry, we lost his connection. We'll take the next question from the line of Dheeresh Pathak from Whiteoak. Please go ahead.

Dheeresh Pathak: Yes, thank you. So, this, you said the Telangana cluster, the EBITDA is lower because there is a higher cost of goods and there is corporate overheads. So I would suggest that...

Management: Can you repeat the question please, sir? Your voice is very feeble. Can you please repeat the question?

Dheeresh Pathak: So, if I understand correctly, you said that, in the Telangana cluster, the main cluster, the EBITDA is lower because there is corporate overheads and higher cost of goods sold. And I am referring mainly sequentially because the revenue is similar, but EBITDA is lower by about INR30 crores odd.

So the question is that, how much of this is dragged from corporate, higher corporate overheads as you are building capabilities and capacities for expanding into newer markets. And it will be better if you can, unallocated corporate overheads, if you can put it in a separate bucket altogether, so that a

at least we get a true picture of the Telangana cluster?

Management: Yes, we'll do that. From next presentation, we'll put all the corporate costs, unallocated corporate costs into a different segment. But at this point in time, you should assume 50% of the dip in margin is because of the corporate cost and the remaining 50% is because of growth in cost of consumables and some seasonal variation because during summers, the electricity costs are much higher in Q1.

Dheeresh Pathak: Okay, understood. And on Sunshine, this maybe, I am not able to recollect properly but on the numbers, it seems that the ramp up is not that much like for the last three quarters- four quarters, the occupied beds seem to be in that same range, revenue seems to be similar INR110 crores although margin profile has improved. So, what is the you know guide path for scaling up of the asset?

Management: So Sunshine has two facilities one in Begumpet and one in Gachibowli. Gachibowli, like I mentioned in the past, it took us one year to get rid of the high cost doctors and then start onboarding doctors at an optimum cost. We have finished almost all the onboarding except for one or two doctors. In my previous quarter, I mentioned there were five, six doctors pending. Most of them have joined. Now, just one or two are left.

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So in the month of July and in the month of August, we are seeing a good improvement in the revenue. It will get reflected in Q2 performance. So we believe over the next 24 months, 36 months, Gachibowli will hit a 70% occupancy. All the doctors for us to be able to achieve that are more or less in place and the EBITDA margin correction is also done. So whatever incremental revenue happens in Gachiboli, almost 50 % of that will flow straight to EBITDA.

There is one other contract that will end in October at Sunshine Hospitals, which is a contract with a third-party diagnostic vendor, where we are losing close to 2%- 3% margin because of that outsourced element. That contract ends in the month of October. After that contract ends, there is an opportunity for the margin to further expand. As far as Begumpet is concerned, whatever margin correction we could do, we did. And the reason we have not been able to scale that hospital is only because of the poor infrastructure that it currently has.

Once we move to the new facility, then we have already identified key departments that, we want to add and key doctors that we want to onboard. So with moving to the new facility, we will start seeing growth happening there. And as we speak right now, we are actually moving into the, we're starting OPD on August 9, and we will move completely into the new facility in the next 30 days-35 days. So from Q3 and Q4, you will start seeing lot of growth in both revenue growth in Sunshine hospitals other than the margin growth.

Dheeresh Pathak: Understood, okay. And in Bangalore, when you said, so just can you refresh the asset status like INR650 crores, INR80 lakh per bed, are we owning the real estate or we are leasing it? If you could just explain that again on the two assets in Bangalore.

Management: So one asset we own 50% of the land and building. The remaining 50% is on lease. The other asset is on a revenue share model. Where we bring in medical equipment and parts of the interiors. Cost is what we incur and rest of it is on a revenue share model.

Dheeresh Pathak: Okay. Thank you so much for taking the question.

Moderator: Thank you. We have a question from the line of Nikhil Mathur from HDFC Mutual Fund. Please go ahead, sir. Mr. Nikhil Mathur, your line is unmuted. Please go ahead. Since there is no response, we'll move on to the next question from the line of Sagar Shah from Philip Capital PGC. Please go ahead.

Sagar Shah: Yes. Good morning, sir. and first of all congratulations for a h

ealthy set of numbers. Now my, I

have just few questions actually on this. First of all, my first question was related to your Sunshine Hospital. Sunshine Hospital, we are facing almost delay in shifting our Secunderabad Hospital and finally, as we have said, we are moving the SKU to FY '24.

So first of all, you have already mentioned the key drivers behind the improvement and margins and occupancy and everything. But I wanted to understand that, basically apart from this, will you be adding other specialties in this year itself to maybe to improve your occupancy at these levels?

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And secondly, also Nagpur, what are the key strategies apart from hiring doctors that you are going to do to actually achieve scale in these two hospitals, which are actually one of the biggest growth drivers for us in the next two years? That is my first question.

Management: This concerned, Gachibowli, we have more or less concluded on the clinical specialties, the talent they've joined, it's more or less in place except for one or two specialties, which we will fix soon. Begumpet, if you look at the revenue today, 70% of the revenue comes from two departments, which is orthopedics and cardiac, which is usually not the case for KIM.

It is quite spread out and we don't have more than 10% of the revenue coming in from any one specialty. But having said that, given the dominance of orthopedics there, it is very difficult to bring it down to 10%. We believe it, we will continue to have a significant share, but the aspiration is to see how we can bring it down to maybe 35% versus the current 55%.

And yes, the idea is to bring in a lot more specialties or a lot more doctors in the current specialties that exist, strengthen them further. The same is the strategy with Nagpur. I think the current set of doctors are extremely talented and very good. It's just that typically any hospital of 350 beds, 400 beds, you need close to 70-80 doctors to ramp up the hospital. That hospital currently has only 30-35 full-time doctors.

The more doctors that we bring in, the incremental revenue will flow in. But before you bring in more doctors, you need to make sure that the current doctors are very secure about their position. They are feeling confident that the management is only trying to grow it and not trying to create insecurity in the hospital. So that is what we have established. And once we bring in these incremental 25- 30 doctors, I'm sure the revenue will start flowing in.

Sagar Shah: Okay, so basically in the last quarter, once you had said, that in the Begumpet Hospital, we were facing the doctors weren't ready to join actually. Something was in the Begumpet Hospital.

Hasn't the issue resolved?

Management: No. So Begumpet, Sunshine has two facilities. So the current facility is a very old facility and we cannot do much beyond operational efficiency, which we have already completed. Now what we are trying to do is, when we move to the new facility because of the size of the new facility, the main road presence of the facility and new infrastructure, a lot of doctors are showing interest to join. But the old facility, no one is willing to join, no big doctors are willing to join. That's the

concern.

Sagar Shah: Okay, got your point. Now my final question was regarding your expansion actually. In your expansion, if I'm not wrong, we are planning for three hospitals. The first one is Bangalore, the second one is Nashik and the third one is the Thane project, which you haven't declared still on Board actually.

So basically, I wanted to understand in the last quarter already you mentioned that, the Thane project that you will be giving the project to Hiranandani builders and the capex will be around INR400 crores, correct me if I am wrong. So that is on course or something like, will there be

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only oncology specialty there or have you changed the strategy and can you give certain number. What is the bed capacity would be over there?

Management: So that is a 300 bedded hospital. It will be a multi-specialty hospital. Unlike our other facilities, where we have always added oncology later, here we will have oncology from day one. So it is going to be like any other KIMS hospital with oncology on day one, focusing on all specialties including transplant.

Sagar Shah: That will be the capacity...

Moderator: Mr. Shah, I request you to join by the queue sir.

Sagar Shah: Okay, sure.

Moderator: Thank you. We have our next question from the line of Ashish Thavkar from 360 One AMC. Please go ahead.

Ashish Thavkar: Yes, hi, thanks. Just wanted to ask this, Mr. Rao's investment in Som Datt Financials. If you guys could provide some color around that, it would be helpful?

Bhaskar Rao: Basically this is an NBFC, we have been taken for one of my cousins and because he's a young guy to grow and second important thing, he cannot able to acquire this because of his financial status and other things. My involvement in that is not there at all, except because to get the permissions from the SEBI and the Reserve Bank, and I have little stake in that, and even my time investment in that is absolutely zero.

It is only to help that person and once it has been established and I know, there is a lot of regulatory things, when compared to the hospitals NBFC has, but we are going to put the total completely professionals on that. They will run on their own. Maybe over a period of time, once it is established on its own, I may be able to step out of that as a Director also in that.

Ashish Thavkar: Yes, okay. So this clarifies a lot of things. Thank you so much. Secondly, on this, so you mentioned about ARPOBs, right? So you aim to be around 70%-75% of the occupancies, but as far as the ARPOBs are concerned, so this time around it was 5% Y-o-Y growth. On a longer term basis, or at least in the near term, FY '24, what kind of ARPOB growth are we building in?

Management: Our ARPOB growth can happen because of case mix change or price change. We are not focusing on increasing prices to the customer, especially in cash patients. We are adding a lot of new clinical therapies, which the ARPOB of those clinical therapies are in line with what we are currently doing. So we should expect maybe around a blended 3% to 5%. Historically, that's how it has moved.

Ashish Thavkar: Yes, and this also includes the pricing benefits, the government laying out new price stack, so it includes all those things, right?

Management: Correct.

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Ashish Thavkar: Okay, fair enough. And so in the longer run, so I guess so as far as FY '24 is concerned, so this fiscal, there are no issues, but since most of the capex starts sitting from next fiscal onwards, so how do we see, are we confident of maintaining EBITDA margin upwards of 35% or you feel at least for that one particular year, where 40% of the beds will get operational, margins could come down below 35%? Is that a fair assessment?

Management: Yes, Shyam.

Management: The way to read, it will be better if we can read it from a cluster point of view. New facilities will keep coming, the turnaround of that will happen, but as long as the other clusters that are already mature or are maturing are showing a progressive trend, I don't think, we should, one should really look at it at a consol level, is my understanding.

But having said that, given the base of the current revenue and EBITDA is so high, that the incremental bids may not have a significant impact on the EBITDA margin. Because there will be growth in Sunshine and Nagpur, which will set off the EBITDA loss that we will

incur from

the new facilities.

Ashish Thavkar: And Sunshine will get operational from next quarter onwards, right? Or as we are speaking, is it operational already?

Management: What are you referring to, sorry, could you come again?

Ashish Thavkar: Does Sunshine want the new asset?

Management: No, from 10th of August, we're trying to move our OPD services, that is day after tomorrow.

And over the next 35, 40 days, we will shift all the IPD as well.

Ashish Thavkar: Okay, perfect. This helps, thank you so much.

Moderator: Thank you. We have our next question from the line of Harsh Bhatia from Bandhan Mutual Fund. Please go ahead.

Harsh Bhatia: Yes, hi, Dr. Abhinav. Good morning. Just two or three quick questions from my side. In terms of the mother and child care aspect that we are trying to bring in the AP and Telangana markets, if you could help us understand the initiatives in terms of the outreach, the awareness, because from an outsider, the patient pool is very different from the multi-specialty business and the pediatric specialty business.

So one would be in terms of your outreach and awareness, the initiatives that we're taking, how are we looking at the overall market dynamics for this particular segment? And lastly, in terms of the talent pool availability in terms of specialty pediatricians, let's say an onco-pediatrician or a neuro-pediatrician, your thoughts would be helpful?

Management: Sure, so we have successfully done one hospital at Kondapur, which is Cuddles, which is part of our Kondapur hospital. In fact today in that micro market, in a 10 kilometer radius, that is the KIMS Hospital

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largest mother and child hospital and we've been able to do that in just four years or five years in spite of dominant standalone mother and child brands being there.

Now we look, after seeing the success there, we're looking to expand, those services to our rest of our hospitals. And the formula, we followed there was very simple to what we do in our adult hospitals. We bring in the right clinical talent, who have some of whom who have practices, some of whom, who have great clinical capabilities to showcase.

So, we continue to bring and acquire these clinical practices, which are probably today in standalone hospitals or in nursing homes, who are willing to consolidate and move to larger facilities. That's how, we're trying to focus and build our obstetric and pediatric brand.

And once we have the right clinical talent, branding and awareness is not going to play as much as a significant role as much as acquiring the right clinical talent. And that's what, we have historically demonstrated and we believe is what will be helpful to grow the business.

Harsh Bhatia: Sure, would it be right to say that, doctor for specialty pediatric care would have or would not have any overlap with the multi-specialty business that you're already doing, the multi-specialty services that you're already providing? So the medical department would be entirely different in that sense.

Management: Yes, so Neonatology is absolutely different. There is no overlap. Pediatrics also up to their certain age group up to 11 years, there is no overlap. But there could be some overlap between the age group of 11 to 16, where historically they have probably been going to adult specialties. Now because we are also offering a larger pediatric service and pediatric sub-specializations and super specialties, there are chances that, there could be that overlap in that age group. But otherwise, there will not be a significant overlap.

Harsh Bhatia: Right, and lastly, I missed your earlier comment in terms of the price hike. So you're seeing cash, you have not taking any price hike, but insurance is where we are taking some bit of price hike.

But would you be able to quantify, what would be the blended price hikes

for this quarter on a

Y-o-Y basis?

Management: Sorry, how much is the impact of price hike?

Harsh Bhatia: Yes, blended price hikes on a year-on-year basis?

Management: So, on this quarter, we would have had a INR3 crore s impact on because of price hike from insurance companies. And CGHS price hike that happened for the full quarter. But given that it happened in the middle of the quarter, we should take INR1.5 crores for this year, for this quarter.

Harsh Bhatia: All right, sir. Thank you.

Moderator: Thank you. We have our next question from the line of Reshab Sisodiya from Sameeksha Capital. Please go ahead.

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Reshab Sisodiya: Hi sir, thank you for the follow-up. Just small detail on the payer mix. So our government share has been reducing, it has reached close to 7%. So is there anything specific, we are doing and the government? Share mix or how is the margin and the data profile moved after the last price is taken by ECGS?

Management: So we have not done anything deliberate to reduce the government scheme patients. I think the growth has largely happened in cash and insurance. And that's why the percentage things are changing. But there is a continuous effort and effort to treat all sorts of patients, whether it's cash insurance or scheme patients. There's no deliberate effort that we have taken so far.

Reshab Sisodiya: So we are not obligated to take any specific government patient to fill up a quota or anything is that in any of our hospitals?

Management: None of our hospitals are obligated to do such thing.

Reshab Sisodiya: Okay sir and as you mentioned in one of your comments that, you are looking at adding colleges, so all of that is on the promoter entity. How is that?

Management: So, whenever we the current hospitals, we have sufficient nursing pool and paramedical pool but moving forward, wherever we are setting up hospitals like in Bangalore in Thane and so on and so forth, it makes sense because these are all long term assets. To also have nursing schools associated with the hospitals and it'll be part of that entity itself.

Reshab Sisodiya: So we'll be having nursing schools within these spaces, right?

Management: Yes, not in the campus, maybe in that geography.

Reshab Sisodiya: Okay, and that will be part of our own books, right?

Management: No, what we have done is, we already having a 5-6 nursing schools and moving forward, every hospital do have required a nursing school and a nursing college and paramedical courses, that's why we created an 100% subsidiary of KIMs education. Now there is a KIMs education society, we have been created. This society is going to run all those schools and all. We will utilize those people into KIMs hospitals.

As a regulatory body that, we cannot run nursing schools and colleges and the corporate hospital.

So that's why, we created this KIMs Educational Society, that is an independent body. Nothing to do with KIMs but only thing is, those nursing students will come to our hospitals to get trained.

In turn we can able to absorb them once they finish their course and they'll stay up most their course for two years and then there will not be any issue, either nursing attrition is concerned.

Reshab Sisodiya: Okay sir. Thank you. That's it is all from my side. Thanks.

Moderator: Thank you. We have our next question from the line of Alankar Garude from Kotak. Please go ahead.

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Alankar Garude: Hi, good morning everyone. So firstly on Sunshine, we have seen a sequential improvement in this quarter and now in the second quarter as you said, August OPD and then in the next 30-35 days, we will be starting operations and maybe we will have the full impact of the doctor additions in the third quarter and hence, the duplication of cost. So

overall should we expect a meaningful dip in the Sunshine margins from the current 23% for the next two quarters and after which, we should see an expansion. So just wanted your thoughts on both the near-term trajectory as well as how should we look at margins for Sunshine in FY '25, so for a quarter?

Management: For the quarter, Alankar, there will be an impact because we will be paying rent on both facilities. We will be maintaining both the facilities for around 45-60 days. But and after that, like you rightly said, we will be onboarding doctors. So the next two quarters will be tricky.

But I don't think, we are hopeful that, we will not see any significant margin dip. But after the two quarters, maybe from Q4 or Q1 of next year, we will start seeing a significant growth in revenue and margin improvement. And from then on, in the next four quarters to six quarters, it should be in line with our KIMs Telangana cluster.

Alankar Garude: Understood. The second question is on Nashik. Broadly similar question. Now, you mentioned about 25% to 27% margins at peak. Currently, we are at 6%. So how should we look at this journey now that the current doctor setup is stabilized, it's more of a question of adding further doctors and specialties. So is it something which can be done in the next two years, two and a half years or could possibly take longer?

Abhinav Bollineni: No, so Alankar, what will happen, there are three things that we need to fix in Nagpur. One is on the cost side and one is on the revenue side. As far as cost is concerned, consumables have been addressed to a large extent. We had seen 4%- 5 % improvement in the consumable cost itself. Doctor cost, we have taken action.

Part of the action is still left because the doctors believe that, if time is given to them, they will start performing and the minimum guarantee will get compensated. So that is a time consuming process. Over the next six months, we will know which direction that is going. If they are not

performing, then their minimum guarantees will be reduced. If they are performing, then it will get compensated.

And the other cost is largely on the nursing cost, which is currently significantly high in Nagpur and in general, in Maharashtra. So we are using our nursing colleges, nursing schools in different parts of Andhra and Telangana to source a lot of these nurses and put them there. And this is going to take time because it is attrition needs to play its role and then we are able to bring in these new nurses, onboard these new nurses at a much lower cost, delivering the same care. So this is an initiative that we need to address on the cost.

On the revenue side, like I said, most of the onboarding, at least 70% of the onboarding should be completed over the next three quarters and then from there in three quarters to four quarters,
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you will start seeing significant improvement in revenue. We have already started seeing good improvement in revenue in the July and August months.

So that is after having two, three doctors on Board. But I'm sure given more and more doctors on Board, the confidence in the local market, confidence in the hospital will only go up and then we'll start seeing an improvement. By the end of the financial year, we should see at least 60%-70% of the doctor on-boarding to be completed.

Alankar Garude: Thank you Dr. Abhinay, that's helpful. And maybe just one final question, Dr. Bhaskar, what is the quantum of investment for acquiring that 64% stake in Som Dutt Finance and would that lead to any increase in pledging in KIMs?

Bhaskar Rao: No, it is only INR20 crores, INR22 crores total, both put together myself and my cousin and we have not taken any pledge from, we have not used any shares to raise this fund.

Alankar Garude: Understood sir. Thank you and all the best.
Mo

Moderator: Thank you. We have our next question from the line of Pallavi Deshpande from Sameeksha Capital. Please go ahead.

Pallavi Deshpande: Yes, sir. Thank you for taking my question. Just wanted to understand in the Telangana cluster, what would be the number of doctors that were added additionally and what were the existing numbers since you mentioned that, there was margin pressure due to that?

Management: Margin pressure is because of the increase in manpower that is largely the corporate cost. Doctor addition has not been more than three or four doctors, revenue contributing doctors in Telangana.

Pallavi Deshpande: Yes, no, so just wanted to understand, like you said, like, the capacity addition, 1,500 beds coming up in the near future. So would you be adding the doctors just now for, that addition? Would there be any excess?

Management: We will not require to add any doctors upfront today. As in when the hospitals get operational, we will keep adding doctors. And it's also part of the capacity will be utilized because we're running short of capacity in the current facilities. So the current doctors are enough to handle a part of the occupancy.

Pallavi Deshpande: And sir, secondly, on the pricing side, like you mentioned, you don't want the height, but in terms of the mix, how much can the mix contribute to the pricing ARPOB increase on the next two year to three year basis?

Management: I'm sorry, could you please repeat the question? Not very clear.

Pallavi Deshpande: Yes, so on the pricing side, how much can the mix change contribute to the ARPOB annual increase? What kind of increase can we see to do a mixed change over the next two year to three years?

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Management: ARPOB will continue to grow at around 3% to 5% growth rate on a year-on-year basis. This could be a function of price hike, could be a function of case mix change and clinical case mix.

Pallavi Deshpande: Sir, I'm just trying to understand, are we being conservative on this guidance because that would be just in line with inflation and if the mix change is coming in, it should be a higher number than that.

Management: Yes, but there is also volume growth that, we are hoping that, there will be more volume growth than price change. Historically, we have delivered more than 5% but we have never modelled more than 3%-5% because just to keep some up.

Pallavi Deshpande: Okay, thank you, sir.

Moderator: Thank you. We have our next question from the line of Bino Pathiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil: Hi, good morning. Thanks for taking my question. First, why have you stopped giving Andhra

Pradesh assets mature and acquired separately and is this going to be the way you report going forward combined?

Management: Yes, we didn't see any need to further separate it because most of the hospitals, both acquired and mature, have reached healthy EBITDA margins. The growth profile has been quite stable. Occupancies have been stable. So we didn't just want to create more confusion by dividing it. And that's why, we had consolidated into Telangana, Andhra, Sunshine, and Nagpur. And as Sunshine matures, we will consolidate that into Telangana. So that was the thought process, but if there is a request for data to be made available, we can do that.

Bino Pathiparampil: Okay, going forward on an ongoing basis, you'll be reporting this component?

Management: As and when, these hospitals start maturing, we will consolidate them into those clusters.

Bino Pathiparampil: Okay. Second, I'm looking at your Telangana numbers. Y-o-Y compared to Q1 FY '23, the occupancy has come down from 74% to 71% in Q1 FY '24. What has led to this?

Management: Sorry, could you please repeat the question again?

Bino Pathiparampil: In your Telangana cluster, occupancy level has come down

from 74% to 71% Y-o-Y.

Management: If you actually look at the IP volume, there is a 7%-8% growth from a Y-on-Y basis. It is because the length of stay has come down from 4.3 to 3.9, that's why you're seeing a dip in the occupancy. So the real indicator would actually be the growth in IP volume versus occupancy or ARPOB.

Bino Pathiparampil: Understood. And finally, just following up, what Nikhil Mathur was saying, it would be great, earlier you had to – you used to give a slide about the upcoming projects and timelines. It would be great if you consistently give that in your presentation.

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Management: Sure.

Bino Pathiparampil: Thank you.

Moderator: Does that answer your question, sir?

Bino Pathiparampil: Yes. Thank you very much.

Moderator: Thank you. We'll take our last question from the line of Namit Arora from Indgrowth Capital.

Request you to ask two questions, sir.

Namit Arora: Yes, thank you for the opportunity and thank you Rahul for hosting this call. My first question is to Dr. Bhaskar and Dr. Abhinay. You clearly have a lot going on both in terms of organic and inorganic, and obviously there's been a lot of success, but do you feel the need to augment the senior leadership team at a group level, either as a chief operating officer or just augmenting the bandwidth, given that there is a lot going on, number one.

And then my second question was that going forward, would you be now in a phase of sort of digesting and integrating and executing all the initiatives that you've launched, or will you continue to look at acquisition opportunities selectively or new Greenfield organic efforts as well? Will that process continue or do you think you already have enough on your plate? Thank you very much.

Management: As far as senior leadership is concerned, what we currently have which is Andhra, Telangana, Sunshine and Nagpur, the team is more or less in place. Things are pretty much in auto mode. Our bandwidth consumption for that is significantly low, except for onboarding new doctors on a day-to-day operations, where hardly our bandwidth gets consumed.

Most of the expansion is where our focus is, but again there is an operating team because these hospitals are not operational. The team hasn't joined us yet. But at the right time, we will onboard the team and we will showcase the organogram in each of those clusters. So we're pretty confident that, with the new team joining, we will not have any bandwidth constraints. As far as – does that answer your question on the bandwidth?

Namit Arora: Very comprehensively, thank you.

Moderator: Thank you. As there are no further questions, I now hand the conference over to the management for closing comments.

Management: Thank you very much for the good questions that you people asked, so that it will be able to excite us to build little better. And as you all know that KIMS is also an affordable quality care. We are all keep mentioning that, all these years in Tier 2 and where we can able to do only the primary, secondary and low-end tertiary care. Whereas in Tier 1 cities, we are doing about high-end tertiary and quaternary care.

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Now with so much of consultations that are available in the district itself, we are trying to

strengthen the existing hospitals by creating a high-end tertiary care, like advanced gastro, women and child, and oncology units and all those things that, we'll be able to do some growth. And it's a continuous journey in the next three years to five years of going organic and inorganic growth. That's why there will be a little bit of fluctuations on the EBITDA and all, but we'll expect a continuous growth on the top line and bottom line. That is one. And also, we are not very averse of the government

schemes. Our aim is to provide a good quality care to each and every person.

When there are many other small hospitals are coming and nearby to that government scheme patients, they will go there. That's why the scheme is coming down. And the other important thing is, as far as the organization is concerned, I feel the entire health care is mostly driven by the doctors, the good, passionate doctors who are able to select.

Then the management will be not a very big issue. The other important thing, a lot of concern on all of you as somebody has pointed out also, the finance company. We have only invested about INR22 crores and that's all our personal to help one of my cousins. And I have nowhere concerned spend any time on that.

The entire professional team, we are going to take it. It is a slow growth on its own, so I don't need to spend much of our time and other things. So to clarify more and more, there is nothing to do. My entire dedication, passion, commitment for lifelong will be for the KIMs hospitals and whatever we do to improvise and with all these things of your investments in this company.

As I mentioned in the patient care small hospitals in type 2 and type 3 places, where we could able to provide a very good quality care otherwise not many chains that can able to provide this type of type 2 type 3 that is a big satisfaction for you as well as for me, we are in this space.

Thank you very much for patient hearing and satisfied with all our answers.

Moderator: Thank you, sir. On behalf of IIFL Securities, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Call: Nov 2023

10th November , 2023

The General Manager,
Department of Corporate services
Bombay Stock Exchange Ltd (BSE)
Phiroze Jheejheebhoy Towers,
Dalal Street,
Mumbai - 400 001.
Scrip Code - 543308
ISIN: INE967H01017 The Manager,
Listing Department
National Stock Exchange of India Limited,
Exchange Plaza, 5th Floor,
Plot No.C/1, 'G' Block
Bandra - Kurla Complex
Mumbai - 400 051.
Symbol - KIMS
ISIN: INE967H01017

Dear Sir/ Madam,

Sub: Transcript of earnings Conference Call with Analyst /Investors.

In continuation to our letter dated 3rd November 2023 , the Company organized a conference call with the Investors/ Analysts on Wednesday , 8th November 2023 at 9:30 AM (IST). A copy of the transcript of the conference call held with the Investors/ Analysts is enclosed herewith and the same has also been uploaded to the Company's Website .

at <https://www.kimshospitals.com/investors> > Disclosures under Regulation 46 of SEBI (LODR) Regulations, 2015 > Analysts Calls schedule, PPT & Transcripts > Transcripts, Audio & Video Recordings > FY 23-24.

Kindly take the same on record and display the same on the website of your exchange.

Thanking you,
Yours truly

For Krishna Institute of Medical Sciences Limited

Uma Shankar Mantha
Company Secretary & Compliance Officer
MANTHA
UMASHANKAR
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MANTHA UMASHANKAR
Date: 2023.11.10 14:02:28 +05'30'
Page 1 of 21

" KIMS Hospitals
Q2 FY2024 Conference Call"

November 08, 2023

ANALYST: M R. RAHUL JEEWANI – IIFL SECURITIES LIMITED

MANAGEMENT : DR. BHASKAR RAO BOLLINENI – FOUNDER &
MANAGING DIRECTOR – KIMS HOSPITALS

DR. ABHINAY BOLLINENI – EXECUTIVE DIRECTOR &
CHIEF EXECUTIVE OFFICER – KIMS HOSPITALS

MR. SACHIN SALVI – CHIEF FINANCIAL OFFICER –
KIMS HOSPITALS

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Page 2 of 21 Moderator : Ladies and gentlemen, good day and welcome to the KIMS Hospital Q2 FY2024 Conference Call hosted by IIFL Securities Limited. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” and then “0” on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Rahul Jeewani from IIFL Securities Limited. Please go ahead Rahul!

Rahul Jeewani : Good morning, everyone. I am Rahul from IIFL Institutional Equities. I welcome you all to the Q2 Earnings Conference Call of KIMS Hospitals. From KIMS, we have with us Dr.

Bhaskar Rao Bollineni, Founder and Managing Director, Dr. Abhinay Bollineni, Executive Director and CEO, and Mr. Sachin Salvi, CFO; over to you for your opening comments.

Bhaskar Rao Bollineni : Good morning and a hearty welcome to the investor's meet for the quarter ending September 2023-2024. It gives me great pleasure to apprise you of the financial and operational results and other developments. I am sure you will find them encouraging. The company continues its onward growth trajectory.

The financial highlights: Gross revenue stood at Rs.655 Crores a growth of 14.3% year-on-year and 7.6% on a quarter-on-quarter basis. Operating EBITDA is Rs.180 Crores reflecting an increase of 11.5% year-on-year basis and 12.5% on a quarter-on-quarter basis. EBITDA margin is at 27.5% versus 28.2% in Q2 FY2023 and 26.3% in Q1 FY2024. PAT at Rs.101 Crores in Q2 FY2024 against Rs.106 Crores in Q2 FY2023 and Rs.87 Crores in Q1 FY2024. Operational highlights both inpatient and outpatient volumes have shown growth. The inpatient volumes increased by 9.7% on a year-on-year basis and 10.6% on a quarter-on-quarter basis. The outpatient volumes also increased by 10.1% on year-on-year basis and 8% on a quarter-on-quarter basis. The average revenue

per operative bed increased by 6.5%

on year-on-year basis and it declined by 1.8% on a quarter-on-quarter basis. The average revenue per patient increased by 5.5% on year-on-year basis and declined by 2.6% on a quarter-on-quarter basis.

Other developments: The Sunshine and KIMS Kingsway, Nagpur are integrating well into the stream of KIMS and progressing well. We have started IVF at Nagpur unit which presents a good potential. Our expansion plans at Bengaluru, Nasik and Mumbai are progressing on schedule. As regards to new technology and development, I am pleased to inform you that KIMS placed an order for procuring radiation therapy systems worth approximately \$38 million to \$40 million from Electa, a leader in precision radiation therapy based in Stockholm, Sweden. The equipment will be deployed in a phased manner

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Page 3 of 21 over three years at different units of KIMS. The initiative will benefit cancer patients by making available advanced precision radiation therapy solutions in all our units with past and present and future units.

Academic activities: There have been a galore of academic activities is ranging from clinical to social during the quarter. These activities help in sharpening the professional skills and shaping knowledge. The first of its kind workshop on NOTSS – Non-Technical Skills for Surgeons was conducted with the participation from across the country and overseas. It was inaugurated by the Governor of Telangana and Puducherry, Smt. Tamilisai

Soundararajan. Surgeons are anyways trained in surgical skills but they may lack non-surgical skills that will enhance the quality of treatment and outcomes. This is a first of its kind workshop conducted with international participation to enhance such non-surgical skills among surgeons.

Many people suffer from unbearable pains without relief. KIMS organized a workshop on acute and chronic pain. Several doctors from within and outside the state participated. This is the first time any hospital has conducted a cadaveric-based hands-on workshop to impart training and improve their skills so that they can be able to practice on the patients. These would allow the professionals to have a realistic approach towards pain intervention and provide relief to patients.

Sudden Cardiac Arrest: Nowadays, we are witnessing many people collapsing suddenly due to heart attacks. In response to the call given by the National Forum of 500 Warriors were trained in CPR to fight sudden cardiac death. Ours was the largest contribution from a single institution. Also CPR workshop was held at Shamshabad Airport to train the CISF staff. These workshops will enable common people to save lives of others before reaching medical help. There are many incidents where we saved lot of sudden cardiac deaths by training all of them either police people or all the public aware of the massage of the cardiac event.

Workshop was conducted on a spinal x-ray, CT and MRI integrating radiology into physiotherapy. When we do the physiotherapy, we need to understand what exactly is happening in the body. With that we can be able to get a good quality of physiotherapy will vastly improve when it is thoroughly synchronized with CT and MRI. This workshop will help improve the quality of physiotherapy.

The main culprit for many problems is lack of awareness on proper and effective waste management. In order to address this issue, KIMS conducted a workshop on waste management in association with World Health Organization and CAHOCON .

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Page 4 of 21 To create a platform to enable research and publications by its doctors, KIMS entered into a research partnership with an Indian Institute of Public Health.

All the said workshops attracted wide participation and facilitated sharing of knowledge and exploration of new avenues in respective faculties and also created an awareness in

the

public itself.

Awards and Accolades: KIMS Hospital is accredited by the American Accreditation Commission of International that recognized the highest professional and ethical standards in healthcare. ASA is one of the world's most experienced organizations in the provision of healthcare organization, accreditation and clinical excellence certification services with high-level benchmarks.

A prestigious magazine conducted a survey and presented a list of best doctors of South for 2023 in its Independence Day issue. The special feature showcases the top names in diverse fields of medicine. Among them are 10 doctors from KIMS. It is a matter of deep professional pride for us.

The clinical accomplishments: I would like to share with you some of clinical accomplishments taking place in various centers of KIMS.

KIMS Saveera in Anantapur :

A premature baby just 10 days old with multi-organ failure and a cardiac arrest was successfully treated by doctors and this budding life was saved.

A 65-year-old woman suffering from a brain stroke was treated by a performing carotid artery stenting feed done for first time in entire Rayalaseema Region. The patient is treated under Arogya Sri Scheme.

KIMS Kurnool,

Doctors treated a young girl of 5 years who was diagnosed with absence seizure a rare problem with as many as 80 to 100 episodes per day. This condition is very rare where the child experiences brief loss of awareness caused by an abnormal electrical activity in brain and such cases are said to be about 5 to 10 per lakh. Such a rare phenomenon was successfully treated in backward area like Kurnool in Andhra Pradesh. These are all done in the tier II and tier III cities which will give a quality care provided by the KIMS Hospitals.

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In a great gesture of humanity parents of 14 MONTH BABY donated her organs that were transplanted in a 58-YEAR-OLD WOMAN who is on dialysis as well as on a pacemaker.

The complex procedure was successfully performed overcoming the problems of age and size of the organ a true testimony to the clinical expertise and state of the art equipment available at the hospital.

In an extremely rare case only 300 reported worldwide and in 20 in India wherein some persons will about have BOTH MALE AND FEMALE REPRODUCTIVE ORGANS were successfully operated in KIMS Secunderabad. In medical terminology it is called as PERSISTENT MULLERIAN DUCT SYNDROME.

KIMS Kondapur

In a FIRST AT HYDERABAD doctors at KIMS Kondapur performed a VERTEBRAL BODY STENTING on a 65-year-old female patient. This highly advanced procedure helps cure osteoporotic fractures within no time. It is a minimally invasive technique where a metal stent is used to stabilize fractures. The entire procedure is done in 20 minutes with no blood losses as against an operation which would last for three to four hours.

The above cases highlight the professional expertise and technology available with KIMS at various centers.

Organ donations:

The organ donation is a highly noble task that results in saving lives of others. KIMS has made a name for itself in this field. There were as many as 16 organ donations in the entire KIMS Group of Hospitals resulting in about 36 transplants and this is probably one among the highest in the country at the hospital.

We are amidst the season of bountiful cricket and delightful festivals. Yesterday's match of Australia against Afghanistan was very, very special. Glenn Maxwell single-handedly led his team to victory in spite of so many odds and physical impairment. He was not able to run due to pain yet he scored with force and success thus planning perseverance and determination are likely to be key to any success. This is true in business also and we at KIMS follow the same traits in our activities. Let us hope that India will bag the World Cup and bring cheer to the whole n

ation. I hereby convey my festival greetings to each of you and your families. Let me end on this happy note. Thank you for your trust and support and
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Page 6 of 21 assuring you of our continued efforts to keep the KIMS flag high in results and rewards. Thank you very much for patient hearing.

Moderator : Thank you very much. We will now begin the question-and-answer session. Ladies and gentlemen, we will wait for a moment while the question queue assembles.

Bhaskar Rao Bollineni : Rahul before any question comes that Thane Hiranandani work is going on. It is all as planned and we need to enter into an MOU that is why it has got delayed. Maybe in a short span of time that also will be done. Work is progressing.

Moderator : The first question we have from the line of Dhara Patwa from SMIFS Limited. Please go ahead.

Dhara Patwa: Thank you for the opportunity and congratulations for the good set of numbers. My first question is Sunshine facility will be operational in November at the new location, so is the relocating or shifting cost included in the quarter or it will come in the next quarter?

Abhinay Bollineni : Part of it is included, but OPD has moved on November 6 completely to the new facility. We are going to do our first surgery on November 15, so in the month of November, we will see a lot more shifting costs.

Dhara Patwa: What could be the quantum of that cost which will be coming in the next quarter?

Abhinay Bollineni : We are anticipating at least Rs.2 Crores to Rs.3 Crores towards the cost of medical equipment and cost of running both the facilities for two months will be another incremental Rs.4 Crores to Rs.5 Crores. This includes the rent that we will have to pay the old facility.

Dhara Patwa: Okay, thank you and we are getting regular updates on acquisition of minority stake in Kondapur Healthcare, can you provide more color on that like what is the thought process on expanding specifically in Kondapur demographic there and what value of growth can Kondapur Healthcare provide to us?

Bhaskar Rao Bollineni : What is happening in Kondapur is there is an existing facility which is a 200-bed hospital that is running at full capacity nearly 90% to 95% occupancy and we are expanding. We are tying with one of the people who are building an asset that is around 6 lakh square feet.

Initially, we thought of taking it once they build a shell and taking into on a lease premises.

Then we worked out the financials of the GST and the lease amount and all, we want to tie up with a long-term credit organization in which they are funding for 25 years, and with

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Page 7 of 21 that, we are adding about an associate company so that tomorrow all these leases will not be

required. If necessary, we can able to merge that in KIMS Kondapur that is the plan.

Dhara Patwa: Okay got it. The next question is on Nagpur Facility. Like we have already reached a matured occupancy of 80% and now we have also started IVF unit at Nagpur, so what could be the ARPOB growth in next two years for this unit specifically for Nagpur?

Abhinay Bollineni : We have reached occupancy of 80% on the operational beds. There are another 70 to 80 beds that can be further operationalized which is where most of the growth will come from.

Dhara Patwa: Could we see more improvement of the ARPOB after including this IVF unit as well like some growth on the ARPOB front as well?

Abhinay Bollineni : Yes ARPOB growth will continue to happen as we start doing more and more quardary tertiary care work. IVF may not be a significant contributor to the growth of ARPOB. It is just completing all the services that we are offering but when we start transplant which we have not started aggressively yet and a lot of the other tertiary care the ARPOB could further increase.

Dhara Patwa: Okay that is helpful. Just one last question like, we are continuously ac

quiring minority

stake from last two years and now we have reduced the EBITDA share of minority significantly to 12.6% now, so is there any target we have internally like till what level we will be consolidating the share?

Bhaskar Rao Bollineni : We would love to have the 100% acquisition. Either we will do a purchase or maybe in the future we will discuss in the board meeting and come back to whether we can able to do all merger. The initial thought process of the subsidiary is where the local doctors will invest. Once it is totally matured beyond that they may not get any value for them. Then they would love to merge either in the KIMS or they may be able to sell out. Those who are willing have any need of money we keep buying them otherwise one day we will be able to merge all the units together.

Dhara Patwa: That is helpful. That was my last question. Thank you.

Moderator : The next question is from the line of Omkar Kamtekar from Bonanza Portfolio. Please go ahead.

Omkar Kamtekar: Thanks for taking my question and congrats on a good set of numbers. Can you please share what is the cash flow from operations generated for Q2 and H1?

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Page 8 of 21 Sachin Salvi : Sure. Cash flow which has been generated from operations for H1 is about Rs.345 odd Crores. This is for H1. Q1 number I will separately give you.

Omkar Kamtekar: Q2?

Sachin Salvi : Q2 number I will give you separately. There is a tax payment during this half year of about Rs.73 Crores. Again this is half a year. I will give you separate Q2 numbers in some time.

Omkar Kamtekar: Okay. We have seen that the ARPOB has marginally declined despite the volumes being increasing in some of the facilities, so any specific reason generally because of the monsoon period, the Q2 is a period where you generally see a lot more diseases and people getting admitted for various reasons, so any reasons for specifically for the ARPOB to marginally decline?

Bhaskar Rao Bollineni : Both the question and answer you gave. You are right. Basically being a seasonal one lot of fever patients, dengue fever and all that is why the volume has increased. Our revenue per operating bed and ARPOB will come down, and also year loss also has decreased. These are all reasons that it has come down.

Omkar Kamtekar: Okay, so because the value because the patients that are being admitted are for lower value or slightly lower margin treatment because of that?

Abhinay Bollineni : More secondary care work so that is when the ARPOB reduces as the result of which everything else falls.

Omkar Kamtekar: Okay and with respect to the KIMS AP unit, so we are seeing very good occupancy levels in the AP region, close to 85% to 84% however still the what we can say the ARPOB here is materially lower? What is the reason behind this? Can you please shed some light on this?

Abhinay Bollineni : There are two reasons. One is it could be 15% to 20% a little higher. There is one of our assets where the ARPOB is lower than 4,000 to 5,000 that is because we have a tie-up with an organization. One reason is this 15,000 on a normalized basis could be around 17,000 to 18,000, but the market in those micro markets in Andhra that we are operating out of that are the healthy ARPOB in that micro market. Our investments for these assets are also significantly lower and it is more a volume-driven game than a pricing game there.

Omkar Kamtekar: Okay, can we expect these kind of level margin or a growth in the AP region in the same?

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Page 9 of 21 Abhinay Bollineni : These cities are growing. They are becoming bigger and bigger. Affordability in cities is

increasing so over a period of time they may not become as rich as Telangana or the other cities, but there will be progress in how the pricing potential here will be.

Omkar Kamtekar: Okay and with respect to the incremental beds, we have shared on the ex

pansion plan the

incremental beds in Nashik, Thane, etc., how much would be the operational beds that would come in on sites say like day one or on first few periods? What would be the operational?

Abhinay Bollineni : Nashik, Mumbai, Bengaluru, and Kondapur 50%; we will start off year one with 35% of those beds being operational. Then we will scale it up to 50 in the basement so the capex for it will be done. It is only about the manpower so we will usually based on the traction that we are getting from doctors and all we will size it up and according to plan so around 30% is where we will look at operationalizing in the first year.

Omkar Kamtekar: All hospitals would be approximately 30% to 35% on year one and then they would scale up approximately?

Abhinay Bollineni : They would scale up but again depends on how the doctor's traction is.

Omkar Kamtekar: Good. Thank you very much. That is it from my side. I will join back in the queue.

Moderator : The next question is from the line of Bino Pathiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil: Good morning. I was comparing the slide on upcoming projects that you have given in your presentation and if I compare back to three quarters back when it was last given I see a delay of almost between one and two years for most of the projects so has the timeline changed and why has it changed?

Abhinay Bollineni : From our deck a year ago we said 36 to 48 months. 36 months complete in the coming April but yes there has been some six to nine months delay because we were first looking at a Greenfield. Then we looked at Brownfield so executing that took us a year time and in Thane we had some legal issues that we had to clear out stuff, but at this point in time the timelines that we have published are more or less work on the ground is happening to meet those timelines. These projects are also getting clearances on these projects.

Bino Pathiparampil: Understood yes just a follow up on Kondapur, you are mentioning creating up a new entity to give credit or so I could not fully understand that? Could you please explain that?

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Page 10 of 21 Abhinay Bollineni : Could you come again please on Kondapur?

Bino Pathiparampil: In answering an earlier question, Dr. Bhaskar had given some details about Kondapur some new entity being created to give credit to the new facility that is coming up or something like that? I did not quite understand what exactly that was? This was answered to the first question?

Bhaskar Rao Bollineni : Actually what we were planning as I mentioned that we have an existing 200 bedded facility which is on a rental basis as of now. Then it is 95% occupancy is running and we are losing a lot more patients because we are not able to provide that is why a couple of years back somebody has acquired a land and they are planning to build the building. Then we asked them to build according to the hospital so that we will be able to take it on a rental basis so when they want to give us a shell then we want to invest a lot more money on the interiors and the equipments and all those things. In that, we are looking at the rental program so that the GST and the rentals and then we calculated for a period of 10 to 15 years the amount of rentals that we are paying along with the GST if you can able to pay a bank loan, which now we are getting funding for 25 long years so it is coming around the same so that the depreciation benefit and other things will come to the KIMS. That is why we reconverted into as an associate company rather than a rental one. Am I clear?

Bino Pathiparampil: Yes, understood. When we say associate company are you going to invest in that company?

Bhaskar Rao Bollineni : Yes that is where we have bought some shares 28.6% so far. Then over a period of time in the existing Kondapur facility and depending upon the cash flows we will try to acquire over a period o

f time majority of the stake.

Bino Pathiparampil: Okay and how large would be the total investment required to buy it out completely?

Bhaskar Rao Bollineni : That will be around Rs.500 Crores for a 600-bed facility total entire thing of equipment, building, and interiors.

Bino Pathiparampil: Okay, so once we spend this Rs.500 Crores the entire thing will be owned by KIMS?

Bhaskar Rao Bollineni : Yes. We will be able to do it in a phased manner like what we are all doing as a subsidiary. Whenever we have excess cash we start using that and keep buying that.

Bino Pathiparampil: Understood Thank you.

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Page 11 of 21 Moderator : The next question is from the line of Omkar Kamtekar from Bonanza Portfolio. Please go ahead.

Omkar Kamtekar: Thank you for the followup. In last call, an update was given with respect to we were scouting a hospital in Malad on a lease basis or on operating long-term lease finance model, any update on that? Is that still on or it is changed?

Abhinay Bollineni : The project is still on. As we also mentioned it will take us a year before we settle all the issues that are there and get all permissions so the project is definitely on. We are waiting for the landlord to revert on one or two points that he needs to settle. Maybe in the next one or two months, he should come back with those points and once those are settled we can close the transaction.

Omkar Kamtekar: Can we expect like close of FY2024 this might be a KIMS Facility? Can we see that?

Bhaskar Rao Bollineni : These are all the Greenfield projects. We were in a phased manner in the next two to three years. We are already engaged in the number of beds required in all these places. We were planning in a phased manner so that if you acquire the land and keep spending without causing any inconvenience to the financial cash flows and at the same time mostly we will be able to do it on our own internal accruals so today next two to three years there is a 2000 to 3000 beds are coming into operational in all the facilities so this will be after that.

Omkar Kamtekar: Okay and one more question?

Bhaskar Rao Bollineni : Legal, will take a little longer time on the land parcel when we want to acquire.

Omkar Kamtekar: Okay understood and on the same line the incremental the new facilities that we are setting up Nashik, Thane and Bengaluru, etc., so these are also part owned or these are like these long-term financial leases where at the end of the period we get ownership? How how are these structured with respect to the ownership?

Abhinay Bollineni : Nashik is a 51:49 joint venture where the company owns the land and building. Bengaluru the company owns 50% of the building 50% is on lease and Thane is O&M format for now.

If there are any changes then we will own the complete land and building.

Omkar Kamtekar: Okay Thank you. That was from me.

Sachin Salvi : With reference to your first question on the cash flow from operation. For the H1 as I said the total cash flow from operation was Rs.340 Crores and for Q2 specifically you have asked it is Rs.197 Crores; just to answer your question.

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Page 12 of 21 Omkar Kamtekar: Rs.197 Crores and Rs.345 Crores for H1. Thank you.

Moderator : The next question is from the line of Alankar Garude from Kotak Institutional Equities. Please go ahead.

Alankar Garude: Good morning everyone. If you look at what we had spoken about when it comes to Bengaluru in the previous call, we had indicated about 750 beds coming up versus the 450 which we have announced now and also in terms of timelines we are now talking about a Q4 FY2025 quarter for operationalization compared to the earlier timeline of Q2 FY2025, so there seems to be a delay of six months plus the number of beds have come down. Maybe we are indicating just one facility out here versus two earlier, so can you please help understand why exa

ctly there has been a delay and also on the bed count please?

Abhinay Bollineni : Alankar, in Bengaluru we are still going to do two more facilities. Right now we have signed the deal on the 450 bedded hospital. The other project also is ongoing. The landlord has to complete his scope of work and then come to us and then is when we sign the documents so up until we sign the document we have not announced it or we have not made it public. We are expecting that to be completed by January or February, so when they hand over the facility to us it takes us typically eight to nine months to complete our scope of work.

Alankar Garude: Okay, broadly assuming the January and February timeline is met, so the second hospital also should be operationalized around the end of FY2025?

Abhinay Bollineni : Correct.

Alankar Garude: Understood and secondly. If you look at Thane can you help elaborate on the legal issues out here because this is just an O&M. The facility is ready. We were planning to sign the MOU few months back but I think Dr Bhaskar said we will sign it in a short span of time so can you help understand what is the current situation out there?

Abhinay Bollineni : The legal issues on the title of the land and building are very clear. The first one and a half years is the time that we spent on clearing out those issues. As of now on the property the work is actually happening so if one visits the property, the work on site is progressing. We are doing our interior works and services works to fulfill our timeline to open it in the next financial year but there has been some delay in the closure of the full transaction because we have then brought in an investor to complete the transaction and unfortunately there has been a delay in executing that from the investor. He is confident that he will close it by this

month and if it does not get closed by him this month then we will go ahead and do it on
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Page 13 of 21 ourselves because we have sufficient capital available to complete the transaction. There are no more pending legal issues on the title and the land and building; those were cleared four to five months ago. We started work on site two months back and in the next year, we should complete all the work on site.

Bhaskar Rao Bollineni: The investor said that he will be able to close by June end. Because of his issues of money to procure is getting delayed and two days back he said he will be able to do it in the next one week's time. Because it has been postponed for the last three months, we simultaneously arranging it from our side but for any cost end of this month, if it is not going to happen by the investors, KIMS will be able to do it on its own.

Alankar Garude: What is the breakup right now assuming the investor money comes in what would be the breakup of capital allocation between us and the investor?

Abhinay Bollineni : If the investor comes then our capital requirement for this project is Rs.120 Crores.

Alankar Garude: If it does not come?

Abhinay Bollineni : It should be around Rs.430 Crores.

Alankar Garude: Understood. The followup question here is with the revised timelines, how should we look at the EBITDA drag from these expansion plans when it comes to FY2025 and if possible can you throw some light on FY2026 as well?

Abhinay Bollineni : On FY2025 because all of these will be operational except for Nashik. The others, Mumbai and Bengaluru will be operational only for a quarter and like I had always indicated in the past on a revenue base of close to Rs.2600 Crores to Rs.3000 Crores the impact that these three facilities will have will be less than 1% to 1.5% on the overall revenue.

Alankar Garude: Okay the other question is on Telangana so we have been around this 30% EBITDA margin mark for a few quarters now. Given that we are already at 70% plus occupancies is there any scope for margin expansion in this cluster?

Ab

Abhinay Bollineni : Yes, actually in this quarter there would have been a margin expansion but we had undertaken some one-time expenses towards renovation of the hospital which if it did not incur that then there would be a margin expansion in Q2, so 31% is a steady state margin that we should expect in the Telangana cluster.

Alankar Garude: One final question if I may what is the EBITDA margin share of minority right now with all the recent acquisitions?

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Page 14 of 21 Sachin Salvi : I will give you this number Alankar. I do not have this number handy.

Alankar Garude: Sure okay that is from my side. Thank you.

Moderator : The next question is from the line of Abdulkader Puranwala from ICICI Securities. Please go ahead.

Abdulkader Puranwala: Good morning and thank you for the opportunity. My first question is on the Nagpur facility so if I go as per your investor presentation in Nagpur you are already clocking an occupancy of around 80 odd but on the margin side at 80% occupancy the margin still seems to be quite lower as compared to what we are doing at a company level so how should we see margins and overall revenue trajectory in Nagpur going ahead?

Abhinay Bollineni : Right, Nagpur if you actually look at the operational beds it is still 250 beds against a bed capacity of 334 so that incremental 80 beds that are available is where a lot of the growth will come, and when that 80 beds get occupied a lot of the margin expansion will happen plus there has been a lot of focus on volume growth and revenue growth which is what we were trying to achieve and stabilizing the unit for the last one year. The integration and cost optimization has not happened significantly like we would want them to happen and a lot of these things have to do with attrition playing its role because of the high cost of nurses and a few other initiatives that we had to take so the margin will get aligned to what we are seeing at Sunshine and eventually it will get aligned to what we are seeing in Telangana and Andhra but I think our primary focus was to stabilize and increase the volume which we have now achieved successfully for Q2 and October and November also we are seeing similar trends on the volume growth and revenue growth. We will now focus on optimizing cost and also as the growth in revenue happens you will start seeing more margin expansion.

Abdulkader Puranwala: Okay. Just to clarify if I would have understood this correctly that though we have 250 beds operationalized currently but in terms of the medical staff and the doctors what we have is properly aligned to the actual bed capacity which is 334 so it is a matter of where you operationalize the remaining but you will be able to scale up the margins here is that the correct understanding?

Abhinay Bollineni : Correct.

Abdulkader Puranwala: Okay sure and secondly we are talking about going solo in Thane and then acquiring some minority stake for hospitals where we do not have a 100% shareholding, how should we look at this from a long-term perspective in terms of the funding so I understand you would
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Page 15 of 21 be generating good amount of cash flows, but are you also trying to raise debt to achieve this aspirations or the funding would happen in such a way or the stake buyer would happen in such a way that it is only restricted to the internal accruals only?

Abhinay Bollineni : We will definitely leverage on debt, but we will keep it very tight. We will keep it 1:1.5 times or 2 times debt to EBITDA ratio and even if the Thane investor thing does not happen, I think we are confident of doing it on our own since we have sufficient cash flows available and also we are buying minority share only as and when we have sufficient cash flow available and if we have to defer that for a while we will do that and take that decision.

Abdulkader Puranwala: Understood. Thank you

u so much and wish you all the best.

Moderator : The next question is from the line of Rahul Jeewani from IIFL Securities. Please go ahead.

Rahul Jeewani: On both Sunshine and Nagpur, we have seen a very good improvement in occupancies on a quarter-on-quarter basis, so how much of this occupancy improvement in Sunshine and Nagpur was owing to the seasonal business and what kind of a trend do you expect on occupancy that these two hospitals going forward?

Abhinay Bollineni : Most of the growth that has come in is in Sunshine Gachibowli and not in Begumpet so that is a clear indicator that there is definitely some seasonal growth in this. There is no denying about it but there has been a lot more stabilization and a lot of doctors who have got added in the recent few quarters; we started seeing a lot of traction in their volume in Gachibowli so if you actually look at the breakup which we did not provide here but a lot of the growth actually happened from Gachibowli versus Begumpet. As far as Nagpur is concerned, again there is a seasonal impact. Part of it is seasonal impact but like I said we had on boarded a significant number of doctors in the last few quarters and now we are starting to see results from those doctors. We are hopeful in the next two to three months we will on board more doctors and the revenue should stabilize at the numbers that we had indicated with us.

Rahul Jeewani: Sure and in terms of doctor addition plans both for Sunshine and Nagpur, how much of that is completed. Can you indicate in terms of let us say a percentage basis in terms of let say an X number of doctors which you wanted to hire at Sunshine in Nagpur, how many of those doctors have already been hired?

Abhinay Bollineni : As far as Gachibowli is concerned, we are almost through. There is one doctor who is going to join in December and with that, we do not need any more incremental doctors except for one or two and that to not immediately, and that was sorted. It is just that we need to start seeing the volumes go up like it has been happening in the last three to four months. As far
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Page 16 of 21 as Nagpur, we are still at 50% of what we want to add in our total capacity and this 50% was important because they going to give confidence for the rest of the people to start coming on board.

Rahul Jeewani: Sure and Begumpet earlier we were indicating that the impact which we will have on account of running two facilities would be to the tune of Rs.20 Crores but this time around you have indicated the combined numbers will be around 7 to 8 Crores?

Abhinay Bollineni : We optimized the cost. Earlier, we had a two to three-month plan to change over. Now in November itself because the season is a little low and the occupancies are a little low we are trying to optimize and see how the shift can happen sooner.

Rahul Jeewani: Okay, so that has led to savings as in terms of these cost coming down from Rs.20 Crores to a Rs.7 to Rs 8 Crores kind of number?

Abhinay Bollineni : Correct and some of the contracts with the medical equipment manufacturers to shift a lot of these equipment free of cost versus having an additional cost also led to this.

Rahul Jeewani: Sure and on the Sunshine Hospital we are already clocking 23% EBITDA margin and with this rental savings which we will start accruing from Q4 as we shift to this new campus so do you think that on Sunshine we can inch up to a margin profile of 20% to 30% in FY2025 on a full year basis?

Abhinay Bollineni : Yes, we were at 23% for H1. The rental cost savings for a quarter will be close to Rs.4.5 Crores so Rs.28 Crores will be around Rs.32 Crores yes I think it should be not 30% but closer.

Rahul Jeewani: Okay sure so Rs.4.5 Crores is the rental cost impact for the quarter?

Abhinay Bollineni : Yes Rs.4.5 Crores to Rs.5 Crores.

Rahul Jeewani: Okay sure thanks. I will join back in the queue.

Moderator : The next

question is from the line of Omkar Kamtekar from Bonanza Portfolio. Please go ahead.

Omkar Kamtekar: Thank you for the follow up. I just needed an understanding of whenever we expand so it is mostly in a cluster in a specific state? I want to understand the philosophy behind it and what are the next states so we already are going into Bengaluru and Chennai and other states, so I want to understand the philosophy and how do we mushroom, so we looking at a KIMS Hospitals
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Page 17 of 21 mushrooming growth? I wanted to understand the philosophy behind it and how does that work?

Abhinay Bollineni : Our focus is to just pick fewer states and do fewer hospitals, but large format hospitals and we believe just being present in fewer geographies will save a lot on bandwidth. We will have a lot of optimization on the cost, a lot of benefits on doctor recruitment and talent acquisitions in the clinical community, and also from a patient point of view, the brand is much more recognized in one geography. The ability to attract better pricing and market share from insurance companies. These are the benefits that we have seen in the Andhra and Telangana clusters. When there is an opportunity in one or two markets to do a lot more number of hospitals why to go to newer markets and try to reinvent the wheel is our thought process. For now, Maharashtra and Karnataka are where we see a big opportunity and a big vacuum in the number of beds required versus what is available today and we believe if we execute that story right that itself can be bigger than what KIMS is today so that is the reason we said we will just stay focused on these two markets two to three markets and be able to scale it up very well.

Omkar Kamtekar: Okay and in a correlate to that the payer mix has very slightly changed so in for H1 FY2024 the corporate and peer mix has increased by 2% vis-à-vis the FY2023 cash has come down so are we actively getting partnerships with corporates or are they for approaching us so how is that working and is this going to increase significantly over say two, three and four years and the cash proportion comes down and the insurance also increases?

Abhinay Bollineni : You are referring to cash and insurance or you are referring to corporates.

Omkar Kamtekar: Corporate has increased by 2% in the six months FY2024 from FY2023, so are we actively getting more participation from or we are seeking that?

Abhinay Bollineni : There is no active empanelment. We partner with all the companies so sometimes during the variations there is more demand from a few of these organizations at the moment. I do not think it will change. The cash and insurance will remain at 75% to 80% and there will not be a decline moving forward. In fact, it will only increase because the markets that we are now getting into like Nashik, Mumbai, and Bengaluru are heavily cash and insurance-driven markets.

Omkar Kamtekar: Correct, so the underlying thing was because we are going to enter into these markets that are heavily insured or have this insurance backing, will the margin trajectory change because you generally have different revenue structure or pricing for these negotiated KIMS Hospitals
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Page 18 of 21 contracts with corporates or insurance is there a material impact that could happen on that front?

Abhinay Bollineni : For the following year.

Omkar Kamtekar: For maybe say two to three years would that happen because generally these contracts are long-term contracts and they are negotiated at different prices from say non-insured person or a corporate person or an insurance person so can there be a material impact say maybe upward or downward is the question on the margin?

Abhinay Bollineni : There will not be a downward trend, but there will be an upward trend. That happens once in three years for private insurance and wherever government-sponsored schemes are there it is

a little longer five to six years.

Omkar Kamtekar: Okay thank you very much.

Moderator : The next question is from the line of Harsh from Bandhan AMC. Please go ahead.

Harsh: Good morning. The last quarter you made a comment that 1500 beds would be added in Andhra Pradesh and Telangana, so from the PPT we have 700 beds Anantapur and Kondapur that is in FY2026 and FY2027, how do we bridge the other 800 beds from the next three to four year perspective in those base markets apparently, which is Andhra Pradesh and Telangana anything say Srikakulam, Anantapur and Ongole. Could help us understand for the remaining 800 beds?

Abhinay Bollineni : Sure. There is bed expansion in Anantapur that we have mentioned. The Ongole, there is a

bed expansion. It is a small bed expansion that is why we have not mentioned it which is close to around 50 to 75 beds. Srikakulam there is a bed expansion and Vizag there is a bed expansion of 75 beds which we again did not mention because it was something that the trust is doing and not us. The two big additions are Nellore and Rajahmundry. The reason we have not mentioned it is we are just waiting for certain timelines and clearances from the government and once we get those we will be in a better position to publish those timelines so between those two assets alone we will be adding close to 400 odd beds incremental capacity.

Harsh: These two 400 bed project?

Abhinay Bollineni : 400 plus 200 is 600 and then 700 breakup is already available so around 1200 to 1300 beds there was another micro market that we wanted to open up in L B Nagar so that also we

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Page 19 of 21 have nothing certain at this point in time and when there is something certain we will bring it to the board and take necessary approval.

Harsh: Sure that is helpful, but it would be a fair assumption that a large part of this base market expansion the beds that we are discussing in micro markets would be before the end of Q4 FY2025?

Abhinay Bollineni : Whatever has been published in the PPT will be done before Q4. Nellore and Rajahmundry we are still yet to get approvals for. Once we do get them, it will take us 24 months to build that hospital and that will be an incremental bed capacity of 400 beds.

Harsh: Sure. Thank you.

Moderator : The next question is from the line of Namit Arora from Indgrowth Capital. Please go ahead.

Namit Arora: Thank you for the opportunity. My question was around insurance. Are you working on any internal operational projects because there has been some discussion not specifically your company, but generally across hospitals about issues about customer processing on insurance, the time it takes and

Moderator: Management, we have lost her so we will take the next question. The next question is from the line of Abhay Marda from Yashwi Securities. Please go ahead.

Abhay Marda: Good morning. I just wanted you to throw some light on the Bengaluru expansion that we are doing. What is the number of beds and how many beds will be operational initially and what is the ARPOB as we are looking forward to?

Abhinay Bollineni : We had indicated in the past we are hoping we will target to get around 700 beds in that micro market. We have one project that will come into operation in the next financial year which is in a micro market called Mahadevapura. Of the 400 odd beds, we will operationalize around 30 beds in year one. Obviously, our aspiration is to operationalize both but based on doctor traction we will take a call and the ARPOB currently in that market it is around Rs.60,000 per bed per day. We may start off a little lower maybe at around Rs.50,000 and then as the hospital matures it will scale up.

Abhay Marda: Okay thank you so much.

Moderator : The next question is from the line of Namit Arora from Indgrowth Capital. Please go ahead.

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Page 20 of 21 Namit Arora: Thank you for the

opportunity again. My question was around insurance, are you working on any internal project or engaging with insurance companies because there has been some discussion about the time it takes to process insurance claims, etc., and some misuse so I am just trying to see how are you focusing on enhancing customer service and customer excellence given the issues around insurance. Thank you?

Abhinay Bollineni : There are a couple of third-party applications that are working to solve this problem. There is a significant delay in communication between the hospital and the insurance provider. We are not directly working with these companies because it will consume a lot of our bandwidth, but there are four to five players in the market who are trying to solve this problem and we have partnered with one or two to see if we can help them also solve this problem.

Namit Arora: Got it. That is very helpful. Thank you very much and all the best to the entire team.

Moderator : The next question is from the line of Omkar Kamtekar from Bonanza Portfolio. Please go ahead.

Omkar Kamtekar: Thank you for the follow up. With respect to the pledging so there is a small pledge of the promoter state of 16.2%, what is the plan so are we going to reduce it or it is going to stay stable just a clarification on that?

Bhaskar Rao Bollineni : As I mentioned before also that my brothers have been pledged for their company and they are also planning to release it as early as possible. They are keeping some assets for sale which is going to happen. The moment it happens at least 50% to 60% of the shares will be

de-pledged.

Omkar Kamtekar: The (inaudible 56:19) the half what you say?

Bhaskar Rao Bollineni : Yes.

Omkar Kamtekar: Any approximate timeline maybe a quarter or two or maybe a year?

Bhaskar Rao Bollineni : One or two quarters. End of this financial year.

Omkar Kamtekar: Okay, so end of FY2024, we can say this pledging would go down under 10%?

Bhaskar Rao Bollineni : Yes.

Omkar Kamtekar: Thank you. That is it.

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Page 21 of 21 Moderator : As there are no further questions from the participants, I now hand the conference over to the KIMS management team for closing comments.

Bhaskar Rao Bollineni : Thank you very much for asking all the questions and we are very happy that your company is growing as we expected during our IPO show time. Whatever we promised we could able to deliver and there are definitely good plans in the next two to three years we are tied and then we are very, very cautious about our financial burden on the company. Mostly, we will be doing internal accruals, and as well as the debt EBITDA ratios we will not cross. We are very, very disciplined in that aspect and at the same time we will try to do what is good for the company whether it is a light asset model in the long run or short term we are planning on that, so we will achieve what we are having in our mind. Thank you very much.

Moderator: On behalf of IIFL Securities Limited which concludes this conference. Thank you for joining us and you may now disconnect your lines.

Call: Feb 2024

08th February, 2024

The General Manager,
Department of Corporate services
Bombay Stock Exchange Ltd (BSE)
Phiroze Jheejheebhoy Towers,
Dalal Street,
Mumbai - 400 001.
Scrip Code - 543308
ISIN: INE967H01017 The Manager,
Listing Department
National Stock Exchange of India Limited,
Exchange Plaza, 5th Floor,
Plot No.C/1, 'G' Block
Bandra - Kurla Complex
Mumbai - 400 051.
Symbol - KIMS
ISIN: INE967H01017

Dear Sir/ Madam,

Sub: Transcript of Earnings Conference Call with Analyst / Investors.

In Continuation to our letter dated 30th January 2024, the Company organized a conference call with the Investors/ Analysts on Monday, 5th February 2024 at 9:30 AM (IST). A copy of the Transcript of the conference call held with the Investors/ Analysts is enclosed herewith and the same has also been uploaded to the Company's Website at <https://www.kimshospitals.com/investors> > Disclosures under Regulation 46 of SEBI (LODR) Regulations, 2015 > Analysts Calls schedule, PPT & Transcripts > Transcripts, Audio & Video Recordings > FY 23-24.

Kindly take the same on record and display the same on the website of your exchange.

Thanking you,
Yours truly

For Krishna Institute of Medical Sciences Limited

Uma Shankar Mantha
Company Secretary & Compliance Officer
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"KIMS Hospital Limited
Q3 FY '24 Earnings Conference Call"
February 05, 2024

MODERATOR : MR. RAHUL JEEWANI – IIFL SECURITIES LIMITED
MANAGEMENT : DR. BHASKARA RAO BOLLINENI – FOUNDER AND
MANAGING DIRECTOR – KIMS HOSPITAL LIMITED
D R. ABHINAY BOLLINENI – EXECUTIVE DIRECTOR
AND CHIEF EXECUTIVE OFFICER – KIMS HOSPITAL

LIMITED
M R. SACHIN SALVI – CHIEF FINANCIAL OFFICER –
KIMS HOSPITALS

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Moderator: Ladies and gentlemen, good day and welcome to Kim's Hospital Q3 FY24 Earning Conference Call hosted by IIFL Securities Limited. As a reminder, all participants' lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star, then zero on a touch-tone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Rahul Jeewani from IIFL Securities. Thank you and over to you, sir.

Rahul Jeewani: Yes, thank you. Good morning, everyone. I'm Rahul from IIFL Institutional Equities. I welcome you all to the Q3 Earnings Conference Call of Kim's Hospitals. From Kim's, we have with us Dr. Bhaskara Rao Bollineni, Founder and Managing Director. Dr. Abhinay Bollineni, Executive Director and CEO, and Mr. Sachin Salvi, CFO. Over to you, sir, for your opening comments.

Bhaskara Rao Bollineni: Good morning to all of you. I welcome to the first meeting of investors in the new year. I take this opportunity to wish all of you a happy, healthy, and a prosperous new year ahead.

Now I will proceed to appraise of you the financials and operational results and also other developments. To start with the financial results, the consolidated revenue for the nine months ended December 2023 stood at INR1,874 crores, showing an increase of 14% year on year. EBITDA for the nine months ended December 2023 INR491 crores, registering an increase of 6.3% year on year.

The gross revenue is INR609 crores for the Q3 FY24, recording a growth of 7.1% on year on year, and a decline of 7.1% on a quarter-on-quarter basis. The EBITDA is 150 crores for the Q3 FY24, a decline of 4.6% year on year, and a 16.5% quarter on quarter basis. The EBITDA margin is 24.7% in Q3 2023, and 27.5% in Q2 FY24.

That is an INR77 crores in Q3 FY24 against INR82 crores and INR101 crores in Q3 FY23 and Q2 FY24 respectively. There will be a little bit of this marginal this one, but I will go into the operational issues. Both inpatient and outpatient volumes recorded a growth of 3.6% and a 2.7% on the year-on-year basis, and a decline of 7.1% and a 7.3% on a quarter-on-quarter basis. The total 3.92 lakh of OPD concerns were done during the quarter. The debt occupancy in Q3 was 72.2%.

The average revenue per operative debt increased by 3.1% year on year basis and declined by 1.3% on a quarter-on-quarter basis. The average revenue per patient increased by 4.8% on year-on-year basis and marginally declined by 0.4% quarter on quarter basis. Q3 is a traditionally lean period on account of festivals. Further the festivals, the elections to state assembly in Telangana and the cyclone in Andhra Pradesh had their impact. That's why the decline in Q3 was happening.

The other developments, Kim Sunshine is shifted to our own spacious premises on the main road of Begumpet. The location and the size will lead to increased patient flow. This is also another reason where during the shifting, there will be a little bit of decline.

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Our Nasik unit is expected to be opened in the first quarter of 2025, most probably in May and June. The most important things for our organization is the academic events apart from the financial results. The academic activities provide necessary impetus for the hospital in adopting new skills and technologies, enhancing patient comfort and clinical outcomes.

Kim's Accord's top priority to this aspect and our doctors are encouraging to take active part in academic activities across the world to presentation of papers, contribution to national and international journals and participation in workshops and academics. I will mention a few of the such highlights. Dr. Sarathchandra Mouli Veraval

li, a clinical director, Department of

Rheumatology and Clinical Immunology of Kim's has presented a case at the global meet.

The experts, the rheumatoid arthritis, he went during the American College of Rheumatology Convergence 2023 in San Diego in November 23. It is a wonderful opportunity and his contribution was well received and appreciated in the global community. The Indian Rheumatology Association also honored Dr. Veravalli in November 2023 with the IREA Colcon Oration Award in recognition of his outstanding contribution to field of rheumatology teaching,

research and professional services.

Dr. Naga Lakshmi, the senior consultant pain physician of Kim's, presented her work on a stellate ganglion block and T1 sympathectomy for refractory ventricular tachycardia at the Southeast Asia TCT cardiology conference held at Mumbai. Such a procedure was first of its kind in the world and the case series were published jointly by Dr. I. K. Rao, the senior consultant and cardiologist and electrophysiologist and Dr. K. Naga Lakshmi in the prestigious Europease Journal. Kim's vascular trainees have participated in the program conducted by Vascular Society of India at Mumbai and we bagged the second prize as the best training vascular institute across India.

The diagnosis of whether a child is able to hear or not can be done right at birth if the defect is detected early and rectified. The permanent damage can be rectified. Many parents are ignorant of this aspect and the child suffers a lifetime of silence to create awareness on the subject and to highlight the latest technologies available.

Kim's Kingsway Hospital at Nagpur conducted a deafness awareness program inaugurated by the RSS chief, Dr. Mohan Bhagwat and attended by various experts, NGOs, social activists and others. The other one is, a three-day workshop on urogynecology was held at Kim's in association with the UPIA called Sprinter Summit 2023 with participation of international experts in the field.

This event was inaugurated by Dr. Pramil Shai, governor of Telangana state. In association with the Obstetrics and Gynecological Society of Hyderabad, Kim's conducted a fertility conference with participation from experts in the field to highlight the latest developments. Pain is difficult for anyone to bear and it makes the sickness horrifying. Kim's conducted a two-day workshop to update the healthcare professionals on the techniques and modalities available for pain management.

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Several doctors from within and outside the state participated in the mega-workshop. Arthritis is a common problem nowadays affecting all classes of people. On the occasion of the World Arthritis Day on October 8th, Kim's Hospital conducted an Arthritis Awakeness Walk in association with the Indian Rheumatology Association.

The event evoked a good response with great participation. Breast Cancer Awareness Month was conducted in October 2024 with a good publicity illuminating public places, including Governor's Raj Bhawan in pink colour, besides having various seminars and meetings arousing public interest. Pfizer India has joined hands with Kim's Hospital to establish a centre of excellence for adult vaccination.

Given the rising case of respiratory illness, the centre will reinforce immunization against vaccine-preventable diseases such as pneumococcal disease, hepatitis A and B, human papilloma virus and influenza. Encouraged by the overwhelming response to Kim's cuddles, Kondapur, an exclusive mother and child centre, Kim's cuddles wing is set up at Secunderabad also to provide such exclusive care to mother and child in new initiative in Secunderabad. As I mentioned before, being a healthcare provider, we not only provide, we also need to create a sort of an awareness into the public where they can able to approach the right person for the right doctor so

that they can able to unnecessarily spend a lot of money.

That's why we create this arthritis awareness program, the breast cancer program and the vaccination. Prevention is better than cure. That is the reason we also take part in a lot of awareness programs every time, even including the papilloma, carcinoma, and even the urogynec department, which most of the people are not aware of it.

Kim's hosted the first SIGN conference to be held in India in January 2024. Society for Image-Guided Neurointerventions, in short SIGN, is an international body comprising of scientists, engineers and clinicians exploring technological advances in various fields, including interventional neuroradiology, MRI, PET, neuropharmacology, and artificial intelligence. The conference brought 125 neuroscientists experts from 12 nations to deliberate cutting-edge advances in the field.

It is the first SIGN conference to be held in India and Kim's is the proud host for that. So these are all our achievements of prevention and creating awareness of the health diseases. Now what we have been achieved having been diseases being occurred, how in the remote places where we can able to treat, how Kim's is contributing to the society.

First ever case in the world done at Kim's, Secunderabad. A three-day-old neonate was diagnosed with a combination of two very major lesions in the heart called transmission of great arteries and a total anomalous pulmonary venous connection. Both these problems are surgical emergencies and the baby would not have survived if the surgery is not done.

A few such cases have been reported in the literature. However, they are all above one week or

even a few months of age, but not on a three-day-old baby. Dr. Anil Kumar, Senior Consultant Pediatric Cardiac Surgeon, performed a day-long surgery. The review is done after two months
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and the baby is doing well. It all depends upon how we can able to improvise our health care in a year-on-year basis to handle the more complex and more advanced techniques.

In Kim's, Kondapur, the doctors and staff at Kim's, Kondapur went out of their way to learn sign language to be able to communicate with the deaf, mute parents of a child. The couple brought their prematurely born daughter to the hospital after they lost her twin. The baby was hospitalized for 76 days because of multiple complications. Over 10 days, three doctors, four junior doctors and four nurses learned the sign language to be able to communicate with the mom and explain her the condition and progress of the child and measure her.

These staff taught the sign language to other members of the team also. This is called walking the extra mile. Now those are parents are relieved, years of age. The only hope is liver transplantation. Responding to the family's dire need, the hospital stepped forward and sponsored the procedure almost entirely. The child weighing a mere 6.4 kg underwent liver transplant procedure for a gruelling 10 hours by a multidisciplinary team.

This is the effect of resources that are available across the KIM'S group can be best utilized and provide a proper care in the remote places too. A 38-year-old young man was without a heartbeat for over an hour when he suffered a cardiac arrest. After remaining in ICU for 45 days, he was discharged and the case is considered a medical miracle.

As per American Heart Association, CPR is stopped after 40 minutes. There is no return of heartbeat. In this case, doctors choose to overshoot the threshold limit considering the age of the patient and other conditions and continued the CPR with defibrillation shocks till the heart function was restored. It is a great achievement, a miracle in the medical field itself.

KIMS-Kingsway Hospitals, Nagpur successfully used pressurized intraperitoneal aerosol chemotherapy generally used in abdominal malignancy to tr

eat a 72-year-old woman with

ovarian cancer. Why it is mentioned here is because this is the first time such a procedure has been done in Nagpur in central India for advanced cancer along with peritoneal spread.

KIMS-Saveera Hospital, Anantapur, myasthenia gravis crisis, a rare complication of myasthenia gravis, was successfully treated at KIMS-Saveera Hospital, Anantapur. This disease is very rare and serious with incidence of two to five cases per 1 lakh population and a mortality rate up to 30%.

The named disease became well known about 30 years back when Amitabh Bachchan was diagnosed with this and news attracted the attention of whole nation. Back then, such treatments were available only in centres like Mumbai. Today, we are able to treat them even in remote areas like Anantapur. The patient was treated successfully and discharged.

In another case, KIMS-Saveera doctors saved the life of a 68-year-old patient suffering with breathing issues due to a blood clot in his lung blood vessels. Due to patient's many comorbidities, the doctors opted for a mechanical thrombectomy using advanced Penumbra Indigo Aspiration System to use it for the first time in Rayalaseema region.

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In another first in the region, KIMS-Saveera doctors performed a retrograde intrarenal surgery on a 3-year-old baby who had a big stone in his kidney measuring 21 millimeters, which is a rare condition. This is the first of its kind surgery on such a young kid in the whole of Rayalaseema region.

KIMS Kurnool, doctors at KIMS Hospital at Kurnool successfully performed a complex surgery to remove a 9-centimeter stone from the urinary tract of a 6-year-old boy. It is first occurrence of such a large stone among children in Rayalaseema region. The entire surgery was conducted free of cost under Arogyasri scheme.

KIMS-ICON, Vizag, a remarkable story of medical expertise and resilience took place. KIMS Cuddles, Vizag. When a pair of twins arrived at just 27 weeks gestation, both infants required ventilation support at birth, battling hyperglycemia, anemia, and feed intolerance. Through personalized treatment plans and round-the-clock care, the twins began to thrive. Most importantly, their crucial stay in the neonatal intensive care unit lasted for a very short time of 61 days.

It shows the immense potential of premature babies and the critical role of the specialized care. Some of the famous personalities who were prematurely born were Winston Churchill, who

became the Prime Minister of the UK, and the famous scientist Albert Einstein, and Sir Isaac Newton.

A 6-year-old girl who battled multiple organ failure was treated and saved at KIMS Cuddles, Vizag. The girl with a rapidly deteriorating health condition was admitted and put on mechanical ventilation. She developed a severe blood infection, sepsis, caused by consumption of street food. In a tough battle spanning of 20 days, she encountered various complications including kidney injury, anemia, low platelets, and electrolyte imbalances.

The advanced approach coupled with meticulous adjustments and monitoring eventually saw a returning to normalcy. All these cases received wide coverage in the media, being rare and complex cases treated at various centres of KIMS that include urban centres and remote areas, so that many of the people aware of these types of diseases come in time to the hospitals and get cured. All these cases received, I would conclude now, thanking you for your continuous trust and support.

I'm confident that we will be able to post good results for March 2024, and we are very happy that we are on the same track that we promised and will continue to deliver good results in the coming years. I will keep you apprised of any key developments. Thank you for your patient hearing. Sorry for anything because of my sore throat, I could not be able to convey properly.

Thank
you.

Moderator: Thank you very much. We will now begin the question-and-answer session. And the first question is from the line of Dhara Patwa from SMIFS Limited. Please go ahead.

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Dhara Patwa: Thank you for the opportunity and good morning all. So, my first question is related to Telangana unit. So, we have seen margins have slipped significantly in the quarter to 28.8% and this has been lowest since nine quarters. Although nothing has changed on our per bed occupancy front, so what is the reason for that? Are we taking any additional cost over here? And will this continue? Is this a new base or there was some one-off in the quarter? Yes, that's my first question.

Abhinav Bollineni: So, as far as Telangana and Andhra both are concerned, there has been some one-time expenses. If you look at the revenue growth in both the markets have been healthy as far as the year-on-year growth is concerned. But on a quarter-on-quarter there has been a decline in revenue both in Telangana and Andhra. This has led to multiple factors. One, because of strong elections in Telangana, there is a dip in the administrative work. And a couple of doctors during the last quarter, senior doctors were also on leave.

Plus, there has been a dip in the transplant revenue in the December month. So, all of these factors have led to a decline in revenue in Telangana teams as well as a slight decline in revenue in AP teams because of the cyclone effect in Vizag. The other villages have fairly done well, but there has been a significant dip in revenue from the Vizag effect.

And we have invested a lot of costs in doctors because the doctor cost has gone up. And because of the decline in revenue, operating leverage has played out. And that is why you see a dip in the EBITDA margin. And there has also been a one-time expense towards renovating and launching. Renovating some of the KIMS Hospitals as well as launching the Cuddles brand at KIMS at Secunderabad.

These are one-time non-recurring expenditures towards a tune of almost INR5.5 crores to INR6 crores in just the KIMS cluster, which is in Telangana and AP. We will continue to have one more such one-time expenditure in Q4. And then from Q1 onwards, it will normalize. These are largely expenditures that we have incurred towards launching KIMS Cuddles as a brand, plus renovating some of our existing facilities just to bring them up to speed to the current market and to add some additional spaces.

Dhara Patwa: Okay, sir. That was quite helpful and a detailed answer. Sir, second thing was, what could be the ARPOB of Cuddles that we have started in Secunderabad? So, will the ARPOB be similar to other children's hospitals? Any colour on that, if you could provide?

Abhinav Bollineni: Yes, it should be very similar to other children's hospitals.

Dhara Patwa: Okay, so in the range of around 40,000-45,000, right?

Abhinav Bollineni: Correct.

Dhara Patwa: Okay. So, last question is on Nagpur unit. So, we are near to a target of 17%-18% EBITDA margin. So, from here on, what kind of growth and margin expansion are we looking for Nagpur unit in the next two-three years? And do you think the same success story can be replicated to Nasik unit as well? Because even that is in Central India. And now, since you have expanded

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outside South India, this was the first hospital we were seeing. So, do you think the same success story could be replicated in the Central India for Nasik unit as well?

Abhinay Bollineni: Yes, I think we are very confident that we will be able to do the same, if not better. But, yes, the turnaround that has been quite encouraging. And I think in the next couple of quarters, we should also be able to hit 20%-25% EBITDA margin in the Nagpur facility. And then on, it will probably stabilize

anywhere between 25% to 26%.

Dhara Patwa: Okay, sir. So, that's it from my side. Thank you.

Moderator: Thank you. A reminder to all participants, you may press star and 1 to ask questions. The next question is from the line of Lakshminarayanan from Tunga Investment. Please go ahead.

Lakshminarayanan: Yes, thank you. So, a couple of things. One is that, you know, just wanted to understand what is the split between the healthcare revenues and the pharma revenues for the nine months?

Abhinay Bollineni: Between healthcare revenue and pharma revenue?

Lakshminarayanan: Yes, Yes. Because you actually split in terms of the revenues you get from the pharmacy and how much you get from the healthcare.

Abhinay Bollineni: I don't have that number off hand, but we'll share that with you separately after the call.

Lakshminarayanan: Got it, got it. So, then in terms of the, if you look at the RPOP, what are the levers you are thinking to increase the RPOP from the current level of around 31,000 to a group level?

Abhinay Bollineni: I think, the continuation to keep adding more and more specialties, like for example, in Andhra, all of Andhra hospitals do not have oncology today. So, in the next couple of years, we are going to add oncology, which will increase the RPOP. So, the oncology RPOP will be around 25,000-26,000 to begin with.

So, that will overall pull up the RPOP in the Andhra hospitals. Likewise, we start to introduce more tertiary care work in Andhra or in the other facilities of KIMS. All of this, along with price rise, will continue to increase the RPOP.

Lakshminarayanan: Got it. So, in Andhra Pradesh, our utilization is around 79%. I mean, the occupancy is around 79%, and it's almost 84% when, you know, for the last, sorry, we are almost at 78%, and last quarter it was around 84%.

In general, what is the maximum occupancy you can actually increase to before it would be like, the top would be around 90% or what is the...

Abhinay Bollineni: 80% is an indication that we need to add more bed capacity. So, that's one of the reasons why in Andhra and Telangana, we are adding more bed capacity to the existing facilities.

Lakshminarayanan: Got it. And what is the mix of the payer mix in the Andhra hospitals, in the Andhra cluster, to be specific?

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Abhinay Bollineni: So, around 60% of it is cash and insurance, and the remaining 30%-40% will be state government and central government schemes.

Lakshminarayanan: Got it. Got it. Thank you, sir.

Moderator: Thank you. The next question is from the line of Nikhil Mathur from HDFC, Mutual Fund. Please go ahead.

Nikhil Mathur: Hi all. Good morning. Sir, I have a few sets of questions.

My first question is on the mix in AP in Telangana. You did mention that the transplant surgeries were a bit less this particular quarter, but historically what we have seen is that ARPOPs tend to be better in 3Q while there is an occupancy dip, but the mix tends to be better. So, what exactly has happened on the ARPOP side in the third quarter in both AP and Telangana, why there is a dip on a sequential basis apart from the dip in transplants?

Abhinay Bollineni: I think transplants have been one of the key factors, Nikhil, and also, we have seen in Q3 a lot of the surgical work got postponed. The ARPOPs are very high. In November, because of festival and then the state election, we have seen a significant delay or postponement of the elective work.

And also in December, there was a strong flu because of which a lot of these admissions were again having to be postponed because of infection. And all of them started happening and then the whole rearing started. And that's why we have seen a decline in the surgical work that is happening. Plus ARPOPs, which are the both, these two are both strong contributors to higher ARPOPs.

Nikhil Mathur: So, do you see that a lot of thi

s is lost revenues or do you see that a lot of this work is coming back in the more near term?

Abhinay Bollineni: Yes, I think we have started seeing things improving. So, usually, I mean, it's a trend after

December, going back to Jan 15, there is a decline. But from post Jan 15, we have seen an uptick in work across all our hospitals.

And if you see actually in Andhra, the revenues have been more or less stable except for the big impact that we had in the coastal belt, which three of our hospitals got impacted [inaudible 30:49.0]. But otherwise, the other hospitals have fairly performed well. Telangana, largely because of the election and a lot of these factors, there has been a decline. But January has been quite good in line with our expectations on the revenue side.

Nikhil Mathur: Okay, got it. And sticking to both these clusters further, what kind of ARPOB increase should one build, let's say, in a two-to-three-year time frame? Is it going to be more pricing led or do you think that the mix can change in the coming two, three years? So, any thoughts on ARPOB improvement in the coming two, three years?

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Abhinay Bollineni: So, the case mix in Andhra is definitely going to change. There's no doubt about it because we're adding a lot of focus on cancer in mother and child in Andhra. So, in that way, the case mix will get skewed a lot towards oncology and a little bit towards mother and child. So that will improve the ARPOB. I think the health will be coupled with a price hike and insurance price hike. So, we should look at a good 5% to 7% ARPOB growth other than the volume growth.

Nikhil Mathur: Okay, got it. And on the occupancy, I'm looking at occupancy and operationalized beds.

Obviously, this quarter is subdued at 61% in Telangana and 70%-odd ...

Abhinay Bollineni: Nitin, could you come again, please? I couldn't hear you.

Nikhil Mathur: Is this better?

Abhinay Bollineni: Yes.

Nikhil Mathur: So, I'm talking about occupancy on operating beds. This quarter, obviously, is quite subdued at 61% in Telangana and 70-odd percent in Andhra Pradesh. But what are the peak occupancy levels that you can deliver in both these clusters?

Abhinay Bollineni: I think in Andhra, we're comfortable up to 80%, 85%. And in Telangana also, we're comfortable at 80%. And in both these markets, we are adding beds. So, in the next one, two years, we will be adding a lot of beds that will lease out a lot of pressure on occupancy.

Nikhil Mathur: Okay, got it. On Mumbai, can you share some update on what's happening there? I don't think I've come across the nature of financing of that project in the investor presentation or in a recent release. So, can you just clear some air on what's happening there? What's the total capex? How much will be funded by KIMS? How much will be by the partner?

Abhinay Bollineni: As far as the timeline of the project is concerned, there has been no impact on the timeline. The work on ground has started, like we had mentioned in our previous call. And we are on track to operationalize the facility by the end of the year, as we have committed, in Q4 of 2025.

So, the work on ground is fine, but there has been a delay in stitching up the funding plan. So, by the end of this month, if the alternative funding plan that has not happened, does not meet the needs, I think we will be known and go ahead and complete the project by ourselves.

Nikhil Mathur: So, by Feb end, if there's no deal stitched, then it....

Abhinay Bollineni: In April

Nikhil Mathur: In April it will be funded by you?

Abhinay Bollineni: Right.

Bhaskara Rao Bollineni: I think we are very confident that little bit on -- we are very confident that deal can go through, maybe in the next 10 days. By 15th of February itself, we are through. And we'll take a decision and then we'll come back and explain to all of you in the

next 15 days.

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Nikhil Mathur: Got it, sir. Thank you so much.

Moderator: Thank you. A reminder to all participants, you may press star and 1 to ask questions. The next question is from the line of Abhay Marda from Yashwi Securities. Please go ahead.

Abhay Marda: Hello, good morning. Am I audible?

Bhaskara Rao Bollineni: Yes.

Abhay Marda: So, I just wanted to get an idea about what is the debt guidance and what is the current debt levels. Like previously you mentioned that your debt levels will be around 1 to 1.5 times of EBITDA. So, what is it at currently?

Sachin Salvi: So, the current debt outstanding around it is INR705 crores. As far as guidance on the debt will remain the same, we will be raising some debts for funding our expansion projects.

Abhay Marda: And this Malad facility that is coming up in Mumbai. So, in case if the deal does not go through,

then debt will be required to fund that, right? Or will, mostly it will be from internal debtors.

Abhinay Bollineni: Are you referring to Malad? Yes, which -- Did you say Malad or did I hear it wrong?

Abhay Marda: No, no, I am saying the Mumbai. The Mumbai capacity. Yes, the Thane facility.

Bhaskara Rao Bollineni: Thane facility, we are very confident. We are very confident in the next 10 days we will be able to let you know. And as we are talking about, as we promised before, the debt EBITDA may not be able to cross 1 to 1.5.

Abhay Marda: Okay, okay. Thank you so much.

Sachin Salvi: Apart from this, just to answer the earlier question which some investors have asked about income from pharmacy. It is about 30% of the total income which we are having. As of 31st -- For the quarter end of 31st December, we have reported INR690 crores of which INR185 crores is from income from pharmacy. Just to answer the earlier question.

Moderator: Thank you. A reminder to all participants, you may press star and 1 to ask questions. The next question is on the line of Harit Ahmed from Avendus Spark. Please go ahead.

Harit Ahmed: Good morning. Thanks for the opportunity. So, my first question is on Sunshine. There seems to be a quarter-on-quarter as well as Y-o-Y declines in ARPOB Sunshine. So, is this an impact of the relocation that you have done? But when I look at the occupancy, it seems to be in line with the previous quarter. So, just trying to understand what's happening there.

Abhinay Bollineni: So, on Sunshine, Harit the Gachibowli facility has done fairly well. I mean, there has been sequential growth on the Gachibowli facility. We have seen a significant impact on the development facility during the shifting. So you could imagine, the revenue have dipped by almost 15% in that facility. We had to run both the facilities for a while. So, there is a little bit of confusion among patients.

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And unfortunately, Dr. Gurava Reddy was on leave because of the demise of his father for almost 15-20 days. So, that led to a decline in volume of work at the Sunshine facility.

Harit Ahmed: And Dr. Abhinay, now that we are getting closer to the commissioning of the Nashik's hospital, can you talk a bit about our pre-operational activities in terms of hiring of doctors? And if you can also give some colour on the kind of losses that we should think of from this facility for FY25?

Abhinay Bollineni: I think before meeting with doctors has been quite encouraging. We have been meeting with them for the last one-two months. There is a good traction among doctors who want to come as shareholders and doctors who want to come as non-shareholders but come and meet the department.

We are looking at operationalizing it in the next two-three months or three-four months. And by the end of March, we should kind of finalize a lot of the doctor contracts. So, I think in the next investor call

I, we'll probably be able to give you a better update on the tie-up with doctors and how the trajectory of growth could potentially look like.

Harit Ahmed: Okay. And last one on Bangalore, the 415 beds that you have guided for commissioning towards FY25.

Abhinay Bollineni: Could you speak a little louder? I didn't hear the question.

Harit Ahmed: Yes, it's on Bangalore where we have guided for commissioning around 400 beds towards the end of FY '25. And I understand there are 2 hospitals that we are targeting there. So, if you can give some break-up in terms of the timelines for each of those and also the expense for both the assets.

Abhinay Bollineni: So, at this point in time, we have Sol Space which is Bangalore-J&A [inaudible 40:34.6], which is 415 beds. We are on track to have that also operational by Q4 of FY '25 along with the SPARMA facility. This will be around 424-430 beds.

The other facility, the work on ground is happening but we have not been able to sign the contract yet. That is more an O&M contract with a local provider. So, maybe in the next -- and they have been occupied with some permissions that they are getting to the hospital and medical college by April-May, we should be able to sign that contract and then we will make a decision.

Harit Ahmed: So, these 415 beds is for the first hospital in Bangalore?

Abhinay Bollineni: Yes, this is the hospital.

Harit Ahmed: Alright. Thanks Dr. Abhinay.

Moderator: Thank you. The next question is from the line of Dinesh from INS and INC. Please go ahead.

Dinesh: This is Dinesh.

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Moderator: You are unmuted. Please go for your question.

Dinesh: Yes, good morning. Thanks for the opportunity. Am I audible?

Abhinay Bollineni: Yes. Yes, you are.

Dinesh: Yes. So, sir, actually in previous quarters, the management has said that the main growth in Andhra would be coming from mother and care segment as well as oncology segment. So, how is the traction in mother and care segment right now currently? Can we see any positive traction? And also, can you please comment on Vizag gastro unit which was commented previously? The management has said that it would be negative for the complete year. So, can we see any positive traction in the Vizag gastro unit?

Abhinay Bollineni: Vizag gastro unit is starting to do well. I think this quarter, the losses have come down to as low as INR30, INR40 lakhs. I think in the next one, two quarter, it should be fully turned around and become EBITDA positive. The revenue growth has been quite healthy. But again, because of the gastro unit in Vizag, there has been some impact in the last quarter.

As far as mother and child in Andhra is concerned, we are fully operational in two of our facilities, one in Kurnool and one in Vizag. Like we said, we are adding brown seed expansion in each of our hospitals. So, over the next 24 to 36 months, we will see 50 to 60 beds across each of our hospitals go up and, in some facilities, much bigger. That's when we will start seeing more growth coming in from oncology and mother and child.

Dinesh: Okay, sir. Got it, sir. And my second question is on Sunshine Hospital. As we move to the new Begumpet building, so can we see any operational decline in the current quarter or in the first half of FY'25?

Abhinay Bollineni: In this, I think there is some continued lag that will be there in Q4. But after that, I think it should stabilize because by then, we would have completed the full transition. Patients would also be aware about this and things would have settled down in the new facility.

Dinesh: Okay. So, actually, the management has stated previously that Sunshine would reach a EBITDA maximum of 30%. So, currently, looking at the performance of Sunshine, I think it is right on track to meet that guidance, it is true, sir?

Abhinay Bollineni: Could you come again, please? I didn

't follow your question?

Dinesh: Yes. Actually, previously, the management has guided that Sunshine's EBITDA would reach 30%. So, looking at the current performance of Sunshine, it was actually doing...

Abhinay Bollineni: Yes. In the long term, we are pretty confident that we will be able to get a 30% EBITDA margin as mentioned.

Dinesh: So, at least in the next financial year, sir? By the next financial year?

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Abhinay Bollineni: Yes. We should be able to get to those kinds of numbers very soon.

Dinesh: Got it, sir. Thank you, sir. That's all from me, sir.

Moderator: Thank you. The next question is from the line of Nikhil Mathur from HDSC. Nikhil, please go ahead.

Nikhil Mathur: Thank you so much for the follow up. On Nashik, am I getting my numbers right? So, we are looking for a bed capacity of 280 beds, right, in FY'25?

Abhinay Bollineni: Correct. That's the full capacity.

Nikhil Mathur: Okay. And this would be available for the full year or you expect some start-up time and it might take a few months?

Abhinay Bollineni: We are looking at May, Nikhil. We are looking to operationalize it in May or towards the end of May. So, I think from the second quarter, we will have it.

Nikhil Mathur: Understood. And I think one of the participants asked you on the doctor recruitment process in Nashik. So, just wanted to understand where would we be getting the talent in Nashik? Sorry, I am not that aware of the existing hospital landscape of that city. So, if you can just help us understand a bit, what is the talent pool available, where you will be getting it, what are the organized competition that you see in that market from other major players?

Abhinay Bollineni: So, the organized competition is largely from Medicovert Hospitals and Sahyadri Hospitals. These are the two large facilities there. Both of them are 300 beds each. And there is a good supply of talent in the Nashik market. Some of the strong established clinics have shown a lot of interest. A lot of doctors are willing to shut down their nursing homes and move to a larger hospital, such as KIMS, and be able to lead the department and take things forward.

So, we are not seeing any challenge. There is a challenge for the initial few specialties that we are going to launch. But as we keep improving the kind of work that we are going to start doing there [inaudible 47:13], we will start looking at doctors of Nashik or in the neighbourhood of Nashik who are practicing in different markets. We will try to start attracting them back to their...

Nikhil Mathur: Okay, got it.

Abhinay Bollineni: To begin with, like I said, the basic work that is happening in the tertiary care world, there are

enough doctors available in Nashik practicing either in their hospitals or in competition hospitals.

Nikhil Mathur: Okay, understood. And sir, we have seen of late in the North, especially let's say in Lucknow, new facilities -- greenfield facilities that have been set up, they have bugged the past trend of five-six years of gestation. Rather, they have kind of reaching an optimum margin level in a two to three-year time period itself.

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What are your thoughts on whether that is the supply-demand mismatch in a city like Nashik or some other cities that you are looking to enter as well? So, do you think that it could be a wider country trend of supply-demand imbalance due to which the gestations of greenfield setups can be faster than what we have seen in the past?

Bhaskara Rao Bollineni: See, the five-six years of a greenfield project was there about a decade back. The reasons were, at that time the talent was not there, the technology was not there, people's awareness in healthcare was not there. Now today, there are talents that are available.

It all depends upon the number of

specialties that you start in a greenfield project. We shortened the breakeven point. Now anywhere -- wherever you start a greenfield of unknown place, it may even come down to one or two years. Places like Nashik, even still further come down, I believe it will be less than a year.

Nikhil Mathur: Am I getting it right that you are suggesting that EBITDA breakeven for Nashik can be within a year as well?

Bhaskara Rao Bollineni: I think so.

Nikhil Mathur: Understood. Got it, sir. Thank you so much.

Moderator: Thank you. The next question is from the line of Lakshminarayanan from Tunga Investments. Please go ahead.

Lakshminarayanan: Yes, thanks again. One question related to your employee costs. I see that employee costs are actually inched up on a 9-month basis, also on a quarter-on-quarter or annual or a sequential basis. So, can you just help me understand what is the band -- what is the percentage of revenue you like to keep the employee costs at?

Abhinay Bollineni: So, there were two reasons that the cost went up. One, we had invested in King's Cuddle, which we had to recruit more number of people as we launched the hospital. And, number two, because we were also shutting down the old functions, the CT and CT of our department there.

So, we had to run operations in both the hospitals. So, we had to recruit more number of people to be able to run the operation. Plus, given the new facilities in Bombay and Bangalore and Nasihk are getting operational soon, we have picked up some staff for each of those hospitals in our current payroll, which will then be moved to those hospitals where those hospitals are operational. This is to bring them up to speed with the system and process.

Lakshminarayanan: Got it. So, this employee cost will be fairly fixed in nature, right? There is no variable component of it?

Abhinay Bollineni: No, no, no. It is all fixed.

Lakshminarayanan: And, what is the band you like to keep it at? See, right now it is around 16.8% on a nine-month basis. So, what is the band you like to operate at?

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Abhinay Bollineni: We continue to remain the same way, like it has been for the last few years.

Lakshminarayanan: Okay. And, in terms of the consultancy fees, which comes in the other expenses, approximately what has been that number you keep in mind as a percentage of revenues?

Abhinay Bollineni: Given that we have a lot of new facilities in the country and in Nasihk where the doctor cost is slightly higher, I think around 20%-22% is where we think we will be able to complete the delivery of the doctor cost.

Lakshminarayanan: Got it. Got it. The second question is that, what is the mix of OP revenues and IP revenues for us on a nine-month basis?

Abhinay Bollineni: The ballpark number should be around 75% of IP revenues, 75%-80% and 25%-20% of IP revenues.

Lakshminarayanan: Got it. Got it. And, how has it trended in the last nine months? Has it changed a bit when compared to last year?

Abhinay Bollineni: No, it has remained the same. It has not changed.

Lakshminarayanan: Good. And, the last question is related to the pharmacy revenues. What is the kind of gross profit margin you make on pharmacy revenues?

Abhinay Bollineni: Hello?

Lakshminarayanan: Yes, in terms of your pharmacy revenues, which you gave a number that is around 30% of your revenues, what kind of gross margins you make on pharmacy revenues?

Abhinay Bollineni: We will look at that number and come back to you on this.

Lakshminarayanan: Okay. Okay. And, one last question. In terms of, you are entering into markets like Bangalore, which is highly competitive and also there is a higher critical care bed per population. How are you thinking about getting into these new markets where there is already a strong competition like Manipal or Apollo or Fortis in these places?

Abhinay Bollineni: Sir, as I mentioned earlier in our

call, Bangalore, we are entering those micro-markets that are still underserved. So, it is not Apollo, Manipal, they are all present in Bangalore. But Bangalore is a very large city with multiple cash materials and multiple micro-markets.

So, we believe there are a few unserved and untapped markets and the city is growing. So, while we enter the market, we continue to expand in the micro-market over a period of time. And, that is why we call it the South Indian City.

Lakshminarayanan: And, how do you stand out yourself in these markets in terms of attracting both talent as well as customers? How do you scale up in these markets?

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Abhinay Bollineni: Yes, I think it is the same thing. You come up with a good hospital with good infra and good technology. Even if you look at the technology that is available today in Bangalore, it is still much lesser than some of the other South Indian cities.

Or, the number of 500 bedded hospitals that are available in Bangalore today are still less. So, if you are able to get to the right micro-market, you will be able to attract a lot of doctors who are practicing in smaller hospitals and not being able to do full justice. You will also be able to reach larger hospitals because you get better support from a multidisciplinary approach. You get more support on the technology front. This will help improve clinical outcomes for them as well. And, that is how we look at attracting doctors.

Moderator: Thank you. The next question is from the line of Rahul Jeewani from IIFL Securities. Please go ahead, sir.

Rahul Jeewani: Yes. Hi, Dr. Abhinay. Given that we have relocated the Sunshine Hospital beside Begumpet, what happens to the old Paradise Campus? Any plans for that? Yes.

Abhinay Bollineni: So, Rahul, in Secunderabad, we have three blocks. So, we are demolishing Block 1 and Block 2 to add another tower there, which will give us an incremental bed. We haven't finalized on how many more beds we want to add there. But, because we are demolishing those two buildings, a lot of the services that were being rendered there are now being moved to the old Sunshine Hospital.

And, plus, we are also starting a new service on the rehab front, which is 100 beds dedicated for rehab services. Part of that cost is also getting incurred in this quarter's report for refurbishing the Sunshine Hospital to launch new rehab services at that hospital. And when the new facility for exchange is ready in the next 3-4 years, we will then shut down that hospital completely and move everything back to the KIM'S Hospital.

Rahul Jeewani: Okay. Sure. And on Sunshine, you spoke about 30% EBITDA margin. Now, part of this margin benefit was expected to accrue from the rental savings which you would have seen because of this relocation. But, given the fact that now you are retaining the Paradise campus as well, are you still confident of margins accruing to that particular kind of a number?

Abhinay Bollineni: Yes. So, I think this is in respect to the bed. In respect to the bed, we are pretty confident that Sunshine will be able to get a 30% margin.

Rahul Jeewani: Despite this campus still being retained by Sunshine?

Abhinay Bollineni: Correct. Yes. And this is a temporary thing. This is only for the next 3 years or 2-3 years. One chance, certainly, is where we will again surrender this and move back.

Rahul Jeewani: Sure. And just one request to Sachin. Sachin, in terms of segmental EBITDA disclosures, we have provided the EBITDA on the post-Ind AS and including other income. Whereas, earlier we used to provide the same number on a pre-Ind AS basis and extra for other income. So, if you can provide that as well, that will be helpful?

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Sachin Salvi: Sure, sure. We will do that also.

Rahul Jeewani: Su

re, sure, sure. Thank you. That's it from my side.

Moderator: Thank you. The next question is from the line of Harsh from Bandhan AMC. Please go ahead.

Harsh: Yes. Hi, Abhinay. Good morning. Just one question from my side. In terms of your views, in particular, for this cashless everywhere news that is floating around. So, there are certain guidelines that GIC has come about with. So, just from your understanding for the sector, for the hospitals, obviously for the patients, it is beneficial. But just your thoughts on this cashless everywhere part?

Abhinay Bollineni: For the patients, it is definitely beneficial. For the hospitals also, it is beneficial where a few of the insurance companies are acting very stringent on the empowerment norms. But we still have to figure out how this happens. We have been talking to a few of the insurance companies. But we are yet to see full clarity on how this will be executed, at what pricing will it be done and so on and so forth.

Harsh: So, the on-the-ground work is still yet to be done. There is a lot more to be done before it gets implemented.

Abhinay Bollineni: Correct, correct. There is a lot more clarity to be achieved and a lot more work to be done.

Harsh: Okay. Thank you. Thank you.

Moderator: The next question is from the line of Dheeresh Pathak from WhiteOak. Please go ahead.

Dheeresh Pathak: Yes. Thank you for the opportunity, sir. So, this is slide 28 on capex. For Nasik, INR250 crores and Thane, INR250 crores. Now, this is all in capex, right? Including the land and the building, irrespective of the financing arrangement. This is the total budget cost, right?

Abhinay Bollineni: Dheeresh is it you. Please repeat questions again.

Dheeresh Pathak: So, Nasik, the slide 28 mentions 300 beds, INR250 crores of capex, right? So, this INR250 crores is including the value of the land and the building which the partner brought in or this is excluding that?

Abhinay Bollineni: Include it's the total project cost.

Dheeresh Pathak: Total project cost, right? And similarly, the Thane project also, INR250 crores is the total project cost, right? Assuming the land and the building is owned by, right?

Abhinay Bollineni: No, in Thane, it is not the total project cost. Sorry, Nasik is concerned that it is the total project cost.

Dheeresh Pathak: So, Thane, how much will it go up by if you have to spend on the land and the building?

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Abhinay Bollineni: It will go up by another INR200 crores.

Dheeresh Pathak: Another 200 crores, right? Yes, and that is what we are doing?

Abhinay Bollineni: We are doing it only by ourselves.

Dheeresh Pathak: Understood. And maybe I missed this earlier. I think the audio from my side was not that good.

You were giving some explanation. If you don't mind, can you just explain like in, let's say slide 20, right? The Telangana, the main cluster, the revenues are up year-over-year for the quarter.

So, for example, we did almost INR20 crores more revenues in this quarter in Telangana cluster versus what we did in the same quarter last year. But EBITDA in absolute term is down. So, there is some cost. I think you were giving an explanation. I didn't fully understand. Similarly, in AP cluster also, the revenues are up, but the absolute EBITDA is down. So, there is some cost which, can you just explain again, what are the one-time costs that are there?

Abhinay Bollineni: Are you asking me what are the one-time costs that we incurred? I couldn't hear you.

Dheeresh Pathak: Yes. So, if you see slide 20. For revenue growth, we have been adding a lot of new specialties and a lot of new doctors across all our hospitals. You know, some of the significant ones are the mother and child launch at the KIMS United Committee. So, all of these are the sequential. In Q1 and Q2 also, the doctor cost has gone up. But correspondingly, the revenue flew through

.

But in Q3, the doctor cost continued to remain the same as Q2.

But given that there is a dip in revenue because of the factors that I had mentioned earlier, we couldn't play out the operating leverage. So, though the revenue grew up, the EBITDA margins are dipped because we have invested in these doctors and revenues have not flown through because of multiple other reasons. So, even in the AP cluster, you have added new doctors with goes of I thought the mature clusters and, we are not adding new doctors?

Abhinay Bollineni: No, there are mature clusters from an age point of view, but there is a lot of growth that is still left in Andhra. When we are adding oncology, mother and child, there are many more specialties that we still have not been able to fully mature in Andhra facilities. So, it's a continuous thing on year-on-year basis we keep on continuing to add more and more doctors.

Dheeresh Pathak: Okay, and one last question. So, in this slide, let's say 20, you show bed capacity, operational beds. So, you are already running at very high occupancy. Why don't we convert more of the capacity into operational? There is still, 125 beds that can be added. Same in the Andhra cluster. Another 250-260 beds can be added from capacity to operational. So, what is the constraint from converting them from capacity to operational?

Abhinay Bollineni: Nothing, I think we will operationalize them soon. In the next two quarters, we will operationalize them.

Dheeresh Pathak: Okay, how many will move from capacity to operational in the next two quarters in these two clusters?

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Abhinay Bollineni: We should bring in another 150 new beds.

Dheeresh Pathak: Okay, thank you so much.

Moderator: Thank you. That was the last question. I now hand the conference over to management for closing comments.

Bhaskara Rao Bollineni: Thank you very much for all the stakeholders who have given good views and their concerns.

And we have a very, very pretty well understood the healthcare in a better way. And as we move on to a new area, it is not that in the last one quarter or two quarters we will put efforts on that.

Before entering into the new areas like Bangalore, Thane, Nashik, Nagpur, and many other places, we do a lot of homework, identify the talent is available or not, the specialties that is lacking in the geography and all those things.

Few of those things we are not able to convey properly now or because of various reasons. When we meet in the coming couple of months, we will come personally and meet all of you. We will explain how we can able to turn around. Even if you take about two decades back when Hyderabad was started, many people have the idea that it is already super saturated at that time. It was a big place like so many other chains were there at that time we started.

Now we evolved as a number one player in Hyderabad. And that we have our own plans for making this new geography also very well. We give a lot more confidence on you people and we are very happy the way it is moving. We are very, very happy that we can able to deliver what we thought. Thank you very much for your patience and concerns. We will definitely be with you.

Moderator: Thank you. On behalf of IIFL Securities Limited, that concludes this conference. Thank you for joining us and you may now disconnect your line.

Call: May 2024

23rd May, 2024

To,
The General Manager,
Department of Corporate services
Bombay Stock Exchange Ltd (BSE)
Phiroze Jheejheebhoy Towers,
Dalal Street,
Mumbai - 400 001.
Scrip Code - 543308
ISIN: INE967H01017 To,
The Manager,
Listing Department
National Stock Exchange of India Limited,
Exchange Plaza, 5th Floor,
Plot No.C/1, 'G' Block
Bandra - Kurla Complex
Mumbai - 400 051.
Symbol - KIMS
ISIN: INE967H01017

Dear Sir/ Madam,

Sub: Transcript of earnings conference call with Analyst / Investors.

In Continuation to our letter dated 11th May 2024, the Company organized a conference call with the Investors/ Analysts on Friday, 17th May 2024 at 9:30 AM (IST). A copy of the transcript of the conference call held with the Investors/ Analysts is enclosed herewith and the same has also been uploaded to the Company's Website at <https://www.kimshospitals.com/investors> > Disclosures under Regulation 46 of SEBI (LODR) Regulations, 2015 > Analysts Calls schedule, PPT & Transcripts > Transcripts, Audio & Video Recordings > FY 23-24.

Kindly take the same on record and display the same on the website of your exchange.

Thanking you,
Yours truly

For Krishna Institute of Medical Sciences Limited

Uma Shankar Mantha
Company Secretary & Compliance Officer
MANTHA
SHANKER
UMADigitally signed by
MANTHA SHANKER UMA
Date: 2024.05.23
14:13:27 +05'30'
Page 1 of 19

"KIMS Hospital
Q4 FY '24 Earnings Conference Call"
May 17, 2024

ANALYST: M R. RAHUL JEEWANI – IIFL SECURITIES LIMITED

MANAGEMENT : DR. BHASKARA RAO BOLLINENI – FOUNDER AND
MANAGING DIRECTOR – KIMS HOSPITAL
DR. ABHINAY BOLLINENI - EXECUTIVE DIRECTOR AND
CHIEF EXECUTIVE OFFICER – KIMS HOSPITAL
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Moderator: Ladies and gentlemen, good day, and welcome to KIMS Hospital's Q4 FY '24 Earnings Conference Call hosted by IIFL Securities Limited. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Rahul Jeewani from IIFL Securities Limited. Thank you, and over to you, sir.

Rahul Jeewani: Thank you. Good morning, everyone. I'm Rahul from IIFL Institutional Equities. On behalf of IIFL, I welcome you all to the quarter 4 Earnings Conference Call of KIMS Hospitals. From KIMS, we have with us Dr. Bhaskara Rao Bollineni, Founder and Managing Director, Dr. Abhinay Bollineni, Executive Director and CEO; and Mr. Sachin Salvi, CFO. Over to you, sir, for your opening comments.

Bhaskara Rao Bollineni: Good morning, all of you. A hearty welcome to all of you. I'm happy to share with you that this is a landmark year in the growing trajectory of KIMS. As we have completed a purposeful and a successful journey of 20 years, though we entered into public domain in 2021 there lies a good spectrum of passion, perseverance and hard work of two decades behind what we are today. Investors are our growth engines and the patients are our driving force. This is not only an occasion for celebration, but also a time for reflection to work with a redoubled commitment and renewed vigor in serving the cause of our patients and protecting interest of our investors. Now I would not keep you waiting, first straightaway proceed to present the financial data. Gross revenue at INR6,376 million an increase of 9.8% year-on-year and 4.7% on quarter-on-quarter basis. EBITDA of INR1,628 million, a decline of 3.2% on year-on-year and a growth by 8.3% on a quarter-on-quarter basis. The EBITDA margin Q4 FY '24 stood at 25.5% versus 28.9% in Q4 FY '23 and 24.7% in Q3 FY '24. EBITDA excluding other income margin stands at 25.1%, showing a decline of 3.2% on year-on-year and an increase of 0.8% on a quarter-on-quarter basis. PAT at INR716 million in Q4 FY '24 against INR987 million and INR767 million in Q4 FY '23 and Q3 '24, respectively.

Operational highlights. Both IP and OP volumes recorded a growth of 2.9% and 7.9% on a year-on-year basis. On a quarter-on-quarter basis, OP volume showed a growth of 4.9%, while IP volumes declined marginally by 2.3%. The average revenue per operated debt increased by 12.1% on a year-on-year basis and at 11.5% on a quarter-on-quarter basis. The average revenue per patient increased by 7.5% on a year-on-year basis and 7% on a quarter-on-quarter basis. Conducted 4 lakh OPD consultations in FY '24 grown by 7.9% year on year and 4.9% on quarter-on-quarter basis.

KIMS -- I'm pleased that the quarter gone by is marked with important developments. KIMS entered into a definitive agreement for taking land on a 99-year lease and purchase of building for setting up a new hospital in Thane, Mumbai, it will be multi-specialty with a state-of-the-art tertiary care facilities with a capacity of 290 beds, which will be commissioned by quarter 4 of KIMS Hospital
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'25. This will be the first hospital of KIMS in Mumbai and third in Maharashtra following its presence in Nagpur and Nashik is expected to be functional by next year. The other expansion plans are moving as per schedule. There is an year there is muted growth in Telangana. The reasons for that is in Kondapur we don't have the bed capacity, which we are adding another 600 beds, which will be commissioned in the third or fourth quarter of this year. Secunderabad, the doctors engagement hiring was a bit delayed because they are committed, but the other parent unit people didn't allow them to leave, but they are joining now that we'll be able to increase the growth in the coming year. EBITDA has decreased. We have incurred a

INR10 crores financial onetime thing. That's why the EBITDA has gone -- from 32% has gone down because of this INR10 crores has been spent.

The good thing is Sunshine is increasing as we expected and shows a good track. There are good consultants have joined. Last quarter, these onetime are double incurrence of INR3 crores, which also has brought down that we'll be able to hold over in the coming years. The second important thing, which we want to add a few more departments in the old Sunshine building with a reduced rental, we have continued the premises. Here in Secunderabad we are also building another 6 lakh square feet in the next 2 to 3 years.

So we demolished the number of -- the existing buildings, so that there will be some shortfall of the beds as well as the other facilities that we are hiring in Paradise. That's why we continue this another 2 years until the new building is coming in Secunderabad facility. And in spite of all those things, the good growth has come from Sunshine. We are very, very confident we'll do more than what we expected in Sunshine. As you've seen that Nagpur is also tracking a good growth as we promised.

This year, we are able to achieve -- this month, we are able to achieve INR18 crores of revenue and 20%, as we expected, the EBITDA growth is there. Moving forward in this year, we will definitely achieve 22% -- 24% of EBITDA growth with revenue also increasing. After that, we also plan to add few more beds in the Nagpur, which will be pipeline once this one -- after this year is over.

Nashik, we have already planning to start in the first week of first quarter and it is online. And we have seen a good track of consultants are coming forward after seeing our performance in Nagpur most of the consultants are going to board in the next month or the second quarter of the financial year.

And with these consultants are already given an appointment order, they have confirmed that they're going to join. With this, I don't think we find any loss funding is required for this. Most probably within 1 year of the operations will be positive EBITDA, which initially, we thought it may take a little longer time and also may require a loss funding, which is a positive note that we are seeing. This all will be definitely helpful how the Nagpur is turnaround.

We faced a lot of problems but we overcome that. We understood now in the entire Maharashtra, the KIMS brand is also putting up and also the announcement of the Thane group. These are all the 2 important features coming things. As we also know that the Bangalore project is also being KIMS Hospital

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commissioned in Q4. And apart from this, I will also, as usually, I'll put some academic events, I will give you a snapshot of the academic activities that is a regular phenomena with KIMS. These seminars and workshops create awareness enabled sharing of knowledge, poster expertise and help in grooming young talent. I'm not going to focus all the achievements, but I'm highlighting 1 or 2, and then I will close this one.

There is a 127 neuroscience experts from 12 nations participated in the conference of SIGN, Society for Image-Guided Neuro interventions, to deliberate on cutting-edge advancements in the field of neurosciences including artificial intelligence and mini robots for brain surgeries. It is the first conference held in India by the prestigious body and KIMS had co-hosted this international event in January '24 that was able to encourage us to bring a new therapy which will be launched in the next 3 quarters.

With this, the KIMS Hospital successfully performed 16 brain surgeries using a cutting-edge device developed by Korean forum, SKIA, AR combined with artificial intelligence by D

r.

Manas Panigrahi, Head of the Neurosurgery, this marks the first in India and globally of utilizing the technology for brain surgery.

Not only this clinical knowledge, this should also needs to be required as technical skills alone may not be enough to ensure best outcomes of surgery. Poor communication is a common act much ignored factor that accounts for errors made during surgery. The first nontechnical course for surgeons that is NOTSS Masterclass by the Royal College of Surgeons of Edinburgh, oldest surgical college in the world, in partnership with the Association of Surgeons of India hosted by KIMS Hospitals in Hyderabad.

So I will just give a brief of the clinical achievements. And as the time is not there, as we mentioned, I cut down all the clinical achievements and have done the academic courses. As a health care professional, I always end up speaking about health whatever the occasion. Today is World Hypertension Day, hypertension is commonly referred as high blood pressure.

The international theme for the year 2024 on the occasion is measure your blood pressure accurately, control it, live longer. The objective is to promote awareness of high BP and aiming to control noncommunicable diseases caused by hypertension. So please follow the norms and take care of your health. I thank you for your trust and support. We have done well in the year

gone by and we are very confident of doing still better in the current year. Thank you.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Dhara Patwa from SMIFS Limited.

Dhara Patwa: Congratulations on good set of numbers. Just a few questions. Sir, can you share more details on the INR10 crores expense, which led to decline in EBITDA? What was that related to?

Sachin Salvi: So these were largely expenses that we incurred at KIMS Secunderabad and KIMS Kondapur. This is largely towards renovating the current facility to complete the KIMS Cuddles expansion in KIMS Secunderabad. And also, we are starting a large rehab program in the old Sunshine hospital. And also, this is a lot of renovation work that's happening in that hospital to start the program, which is slated to start from June 15.

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So these were some onetime nonrecurring expenditure that we had incurred in Q3 and Q4. We don't see any expenditure in -- that we will incur in Q1 and moving forward.

Dhara Patwa: Okay, sir. Sir, depreciation cost in this quarter has seen a sudden pike from INR35 crores to INR46 crores. So is it the new run rate we should assume going forward? Or this been increase as we operationalize more beds in coming future?

Abhinay Bollineni: Dhara, this will sustain. The depreciation cost for the Q4 has increased because the Begumpet hospital has commenced operations in the month of December 2023. Since we have capitalized that in December 2023, the depreciation for that premises as well as the medical equipment has started in Q4. So this will remain at the same level in the coming quarter unless we add any other facility, which will lead to increase in the depreciation cost.

Dhara Patwa: Sure, sir. Sir, one last question on update on the O&M contract for Bangalore like we were expecting that it will be completed in -- somewhere in April or May. So any color on that?

Abhinay Bollineni: Yes. So the contracts are ready. We should be announcing it mid next week with the final signed SPA. Final signed lease agreement, we should be announcing in next week. We should make a formal announcement next week.

Moderator: The next question is from the line of Nikhil Mathur from HDFC Mutual Fund.

Nikhil Mathur: Sir, I first wanted to check on Sunshine. I believe there were some rental cost savings that were to commensurate after the new facility opening up. So just wanted to understand, is 4Q FY '24 fully incorporating the benefits of those rents

I cost saving or there are more -- yet to come through in FY '25?

Sachin Salvi: So on the Q4 number, Nikhil, we have incurred close to INR3 crores of cost at Sunshine which is nonrecurring in nature. So that would have actually put the EBITDA number at 26.5% against the 24.3% number that has been reported. So the INR29 crores EBITDA number would have been normalized to around INR32.5 crores at Sunshine. This is nonrecurring, largely towards rental expenditure and onetime expenditure because of movement from the old facility to the new facility. Maybe -- a part of it maybe to the tune of INR1 crores we will still have in Q1. But after that, everything is normalized. So the normalized EBITDA for Sunshine on Q4 would have been INR32.5 crores.

Nikhil Mathur: So the margin going forward has to be looked at from a base level margin of 26%, right? Is that the right understanding?

Sachin Salvi: Correct. So incremental revenue from here on significant portion will flow-through to the EBITDA.

Nikhil Mathur: Understood. Sir, for the new facilities like Nagpur and Thane, there is no P&L charge yet, right, either on the opex front or on the depreciation front, not yet, right?

Sachin Salvi: There is no P&L charge, Nikhil.

Nikhil Mathur: Okay.

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Abhinay Bollineni: There is some costs that we are incurring because we have hired resources for Nashik and Thane currently, which is what is getting charged to the P&L.

Nikhil Mathur: Okay. If I were to look at the fixed cost buildup in these facilities in the next, let's say, 1 year, 2 years, obviously, there will be some revenue set off, but I'm just looking at the cost structure for now. How different will it be from your corporate cost structure today? I mean, a very simple way of doing this is that I take your fixed cost below gross margin, above EBITDA divide by 3,500 beds. I get a INR38 lakh kind of a number, a very ballpark number per bed, which is the cost structure today, which includes employee, doctors and other expenses.

So for these 2 facilities, would it be materially different from what we're incurring at the

corporate level today? And these are slightly on the metro and Tier 1 cities, right? So can it be on the higher side?

Abhinay Bollineni: Are you talking about Thane and Nashik Nikhil?

Nikhil Mathur: Yes, I'm talking about these 2 facilities.

Abhinay Bollineni: So I think Nashik, we have 300 beds and INR38 lakhs will translate to INR9 crores of fixed cost per month.

Nikhil Mathur: Okay.

Abhinay Bollineni: Is that what you're referring to?

Nikhil Mathur: Yes, yes.

Abhinay Bollineni: Is that including the consultant and consumable costs?

Nikhil Mathur: This includes employee costs, doctors and other expenses?

Sachin Salvi: Yes, around that number, maybe a little lower, may not be as high as INR9 crores, may be around INR7 crores, INR7.5 crores is what we are anticipating as fixed costs, including doctors, employees, other fixed costs and the consumable cost for revenue of INR8 crores, INR9 crores.

Nikhil Mathur: Okay. And Mumbai, maybe Thane basically.

Abhinay Bollineni: Thane will be slightly higher by 20%, 30%.

Nikhil Mathur: Okay. Got it. And sir, one final question. AP Telangana, what's happening on the occupancy side. I thought that there was likely to be some sharp rebound post 3Q because of the elections in Telangana, but that hasn't happened. Is there something specific to the AP-Telangana cluster that's driving down in patients or there's some competitive intensity at play here. So if you can just talk about inpatient volume growth and occupancies in these 2 clusters.

Sachin Salvi: So I think if you look on a year-on-year basis, on a full year basis, both AP and Telangana together as a cluster have delivered a 10% growth in revenue. Telangana has been slightly on the lower side. AP has been on the higher side. But as a cluster together, we've been able to KIMS

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deliver 10% growth. And that's what we have been indicating towards -- for these mature hospitals. There has been some delay in hiring in the Telangana cluster because a lot of these hirings are towards senior doctors and they've taken time to be onboarded.

But we don't -- we are not seeing any -- or anticipating any tough time in the next few quarters as far as onboarding and growth in Telangana in AP are concerned. And more so in AP, given that a lot of the capacity addition will come into play by Q3 and by Q4 of this financial year, we will see a strong rebound in revenue growth, especially from cancer and mother and child after this capacity addition...

Nikhil Mathur: So AP inpatient volume growth this year is around 6% is that a steady growth rate of inpatient volumes for both these clusters. Will that be a reasonable assumption?

Abhinay Bollineni: I think Telangana and AP together consistently delivering the 10% revenue growth is something that we have been doing in the past. This year also, we have delivered 10%. We'll continue to be able to deliver...

Nikhil Mathur: Okay. And just one final question, if I may squeeze in. Your AP ARPOB growth has been quite strong, both Q-on-Q and sequentially. Is it a bit related to pricing? Or is it more case mix? And what is the outlook for ARPOB growth in the next couple of years for AP as a cluster?

Abhinay Bollineni: I think it's a function of the ALOS coming down and the ARPOB going up Nikhil. So, I think, consistently given the high occupancy in a few of the hospitals in AP, we are working towards reducing the ALOS especially till we get this additional bed capacity addition that's happening there. So the average of FY '24 is a good ballpark to look at the growth year-on-year.

Moderator: The next question is from the line of Ameya from JM Financials.

Ameya: Yes. So the first question I have is specific to the Kondapur region. I expect like we have announced that we will be expanding some beds for the unit because it is operating at very high occupancy. At the same time, there is 1 more hospital chain, which has also opened up their new for the last 1, 1.5 years and planning to expand it to more than 1,000 beds there.

Currently, they might be operating around 400 beds, overall, it will have 2 large bedded hospitals in that particular macro market. So how do you see this thing playing out specific to the Kondapur region, just you can comment on that.

Abhinay Bollineni: So bed capacity in Kondapur has happening. In that micro market a lot of bed capacity addition happened in the last 3, 4 years. If you look at -- or close to 3,000 beds got added in the last 4 years, 5 years. But I don't -- there has not been any doctor attrition in KIMS Kondapur. We just lost 1 doctor among the 100 doctors that we have there. The revenue growth year-on-year has been quite strong in spite of limited bed capacity available there. We have not been able to add a lot of specialties. We've not been able to bring a lot more talent to the current facility because of the size constraint.

Once we move to the new facility, given the strong base of revenue that we already have, we are

able to add more specialties like transplant, cancer and few other subspecialties that we have not been able to do today. So I don't see the current competition as any reason for us to have delayed growth in Kondapur as far as the new facility comes.

Ameya: Going ahead do you expect that vision to develop or something which is in your mind why you are adding up more capacity there, like considering the competition is still coming in like how that region is performing in terms of the population growth, etcetera, like what's your stance on that?

Abhinay Bollineni: I think now that's become the new ci

ty, and it's important that KIMS has a very strong presence.

And we are positioning the new Kondapur Hospital as our new flagship facility for Hyderabad, even by bed size, total bed size will be around 800 beds. We are bringing the best technology in the Hyderabad market, in the Kondapur hospital in terms of onco care, in terms of neurology, neurosurgery and stuff like that. So we are pretty confident that the scale-up will be in line with how we have seen things happen at KIMS Secunderabad in the past in spite of the current competition there.

Ameya: Sure. The second question is on the Bangalore macro market where we are opening up new units. If you can give us some color about that region as well?

Abhinay Bollineni: So Bangalore we're opening two hospitals. One is in Marathahalli which is a 450-bedded hospital, which is slated to commence operations by Q4 as presented. That is a multi-specialty hospital with the focus on all specialties, and we're starting to operate mother and child and cancer on day 1, unlike our other hospitals in the past. So part of the building is on lease and part of the building is owned by the company.

The cost per bed is close to INR1 crores per bed. And the current ARPOB in that micro market is close to INR1.8 crores per bed. So our ability to be able to deliver that project at a significant low cost when compared to the current ARPOB which also help us break even faster. And the micro market is pretty underserved. If you look at a 5, 7-kilometer radius, we don't see any hospital of a significant size and scale.

The closest hospital is probably 7, 8 kilometers from the hospital that we are building. And the other thing -- the other hospital is in South Bangalore, which is again a 300-bedded hospital. This is on a revenue share format. The cost per bed, again will be around INR70 to INR80 lakhs, and we are getting in 300 beds in Phase 1, there is ability to scale up by an incremental 100, 150 beds over a period of time as and when required. Again, competition in this micro market, we have Narayana Hrudayalaya, the flagship hospital which is around 6, 7 kilometers from there. And the remaining hospitals are beyond 10 kilometers from this micro market.

Again, very underserved in the Electronic City, the highly denser micro market but pretty underserved. So we're pretty confident about both these hospitals being able to turnaround in ARPOB very soon.

Ameya: Sure. The last question I have on the way we went for the Thane hospital, which was kind of -- we moved away from our investment strategy and preferring asset light investments. So what -- going ahead, how do we see on that front? Like you would continue to prefer to have investor

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on more while opening up a unit or we could see a similar kind of investment like we saw in Thane as well.

Abhinay Bollineni: So I think Thane, in the current shape and form with 300 beds and the total investment of INR500 crores looks a little expensive at INR1.7 crores or INR1.8 crores per bed. One thing is the current ARPOB in that micro market is also in the similar line. So that is a good sign. We are not spending more than what is the current ARPOB. In 5 years down the line when the hospital matures, then the ARPOB will be much higher. That is -- so from our investment point of view, it still will make good return on capital employed, number one.

Number two, the hospital also has additional FSI, once we start building and utilizing that additional FSI, we will be getting an incremental bed of 150 beds at a cost of 150 to 200 beds at a cost of INR80 crores to INR100 crores so when that does happen, then the cost per bed will come down further, which will further improve the return on capital employed.

But we don't plan to do it now. We'll take it up in Phase 2. That's when we reach a 60%, 70% occupancy on the current beds.

Ameya: Right. So Phase 1, it will be 300, the

n you are saying the 150 beds will be added.

Abhinay Bollineni: We could potentially go up to 450 to 500 beds. And that's when the cost per bed also will drop down significantly.

Moderator: The next question is from the line of Abdulkader Puranwala from ICICI Securities.

Abdulkader Puranwala: Sir, just on the brownfield bed addition in Telangana and Andhra region. So could you just highlight the number of beds what you would be adding in the next 2 to 3 years in each of the core clusters?

Abhinay Bollineni: So in Telangana, we are adding Kondapur which part of it will be commissioned early next year. And part of it will be commissioned towards the end of the year which is an incremental bed capacity of 550 beds as far as Telangana is concerned. We are also -- we're building a new facility in KIMS Secunderabad but that will take 3 years.

The net bed addition may not be more than 200 beds in KIMS Secunderabad which is Telangana.

This is other than the to the Sunshine Hospital. As far as Andhra is concerned, we are adding close to 50 to 100 beds on an average in each of our facilities. And a lot of this will be commissioned in the next 24 months. And this is largely towards cancer and mother and child expansion.

Abdulkader Puranwala: Got it. Then on the capex front, so now that Thane is going to be entirely funded and Nashik is coming up plus Bangalore and Kondapur as well. I mean so how do you see the debt profile of the company and this capex program would be entirely funded through debt? Or there will be substantial amount of approvals as well.

Bhaskara Rao Bollineni: See, as of now, with this, the capex is we have only about INR900 crores is the debt even moving forward quarter-on-quarter our equity is around nearly INR2,100 crores by March '24. And

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moving forward, the equity will go up to INR2,400 crores or INR2,500 crores. So we have a leverage of even if we put it about 0.5% of the equity and debt, so we will be in compliant with that and then we will be very, very disciplined, should not cross maximum of maybe 1 quarter or 2, it costs us 2.6. I think a very, very comfortable with the arrangements of the loan and as well as the internal accruals.

Abdulkader Puranwala: Sure, sir. Got it. Sir, lastly, on the Thane project again. So like in Nashik, you mentioned that operational breakeven or profitability can be achieved in the first year itself. How do you see the Thane cost profile and ARPOB and the other operational metrics. So would it be a first year breakeven possible here as well? Or there might be some investments which would be in terms of the doctor cost, which might lead to a delay breakeven here?

Abhinay Bollineni: I think it's too early to comment on Thane, but we are as planned always been that we will breakeven towards the end of the first year of operation. And even if there is some loss coming, it won't be significant, we have modeled around INR10 crores towards the funding of Thane.

But Nashik because we have closed on 90% of the doctor hiring, we are pretty confident that 12 months of operations, we should become EBITDA positive. And the last 3 months, we will be delivering EBITDA margins given the doctor sign-ups that we have.

I think in Thane, it's a little too early maybe by end of next quarter, we should be in a better position to talk about the doctor in this moment on going over.

Moderator: The next question is from the line of Harsh from Bandhan AMC.

Harsh: Just 2 quick questions. One is on Andhra Pradesh. You mentioned that you'll be adding 50 to 100 beds across those micro markets potentially with these new specialties, onco, mother and care, gastro and maybe some other units. If I recall correctly, you had also made a comment on the Andhra Pradesh margin expansion potential expanding by another 300, 400 beds over the next 2-to-3-year period.

So with these additions, there will be incremental hiring as

well. So should we still factor in that

EBITDA margin potential just from the base market perspective? Or you would feel that the incremental additions were sort of weigh on that margin expansion potential? Just from the Andhra Pradesh market.

Abhinay Bollineni: No, I think we can easily assume that margin expansion in Andhra Pradesh in spite of adding new doctors because the base EBITDA earlier we used to give a bifurcation of mature and nonmature. The mature facilities still do a higher EBITDA margin when compared to a nonmature EBIT facilities. So one, there is scope to improve the nonmature facilities. And on top of that, we're talking about more growth. So, I think that consistent expansion of 300 basis points is still doable in spite of doctor addition and expansion.

Harsh: Okay. And again, on the base market to pick up from the earlier question, you mentioned a 10% revenue growth expectation for AP-Telangana on a combined cluster basis. So if you want to-let's say, break this up into 2 or 3 levers volume, price and mix. Volume is additive, we are expecting a mid-single-digit volume growth on an overall basis.

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Pricing might not recur every year, but you will still have a 2% to 4% pricing benefit and the other is mix. So when you do this calculation, do you feel that the 10% expectation is slightly on the conservative side because your incremental business from these new specialties over a period of time will be highly ARPOB accretive in the sense ARPB or ARPOB accretive. So I'm just trying to figure out whether that 10% expectation is on the conservative side or...

Abhinay Bollineni: If it pans out to be higher than 10%, then it's a good upside. But I think given both the clusters are mature and we're adding specialties and it takes time to add these specialties create bed capacity addition.

The guidance of this 10% was from a -- when you look at it from a 3-, 4-year perspective, the year-on-year growth would be 10%, 10%, but there could be years in between where we grow at a higher growth rate, especially when these bed capacity additions come there will be a 20%, 25% growth in different clusters. But when you take a long-term view, I think the sustainable growth rate in this cluster will be 10%. There will be years where there will be -- the growth rate will be higher.

For example, this year, the growth rate in Telangana has been only around 6%. But when these new doctors onboard, I think the growth rate will be at around 15%, 20%. So it gets -- the average will be around 10%.

Harsh: Sure. That makes sense. And just on one last bit. You had made a comment a few quarters back that the cumulative EBITDA hit from these facilities, the new facilities, Bangalore, Thane and Nashik would be in the range of somewhere around INR25 crores to INR45 crores in their first year of operations. again, this was maybe 2, 3 quarters back, but obviously things have changed.

Abhinay Bollineni: Wait, wait. INR10 crores per facility, if one had to model an EBITDA loss, we have said we are assuming not more than INR10 crores per facility. Now given almost commissioning state of Nashik, we don't anticipate that loss funding required given the doctor sign up and doctor onboarding the way it's happening. As we get closer to the other 3, I think we are still pretty confident about that number not being more higher than INR10 crores per unit.

Harsh: This INR10 crores per unit is building in, let's say, a 30% occupancy -- average occupancy for the entire year?

Abhinay Bollineni: Correct. Correct. Right.

Moderator: The next question is from the line of Dhruv Shah from Dalal & Broacha Stock Broking.

Dhruv Shah: My first question is with respect to your debt outlook. So there has been about INR500 crores of increase in debt. So just wanted to know is this towards -- entirely towards Thane Hospital?

Sachin Salvi: No. Part of it is towards Thane hospi

tal. So about INR250-odd crores is towards Thane.

Bangalore is about INR140 crores. Then we have Nashik and Nagpur also we have taken debts.

So it is not only towards Thane. Thane is only INR250-odd crores.

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Dhruv Shah: Okay. And what is your outlook on that? I mean, when can we expect some debt reduction going forward is at any time soon?

Sachin Salvi: So at least in next 1 or 2 years, we are having Bangalore, Nashik and Thane project, which is going on. The debt repayment will start after -- generally start after 12 months because we generally have a moratorium of 12 months. So the repayment starts after 12 months, so you can expect after FY 26, unless we take up any other new projects.

Dhruv Shah: Sure, sir. And what will be your target debt to EBITDA? What are you expecting? Is it 1.2, 1.3 or higher than that?

Sachin Salvi: So, the mandate which we have received from our Finance and Investment Committee as well as our Audit Committee debt-equity of 0.7 and debt-EBITDA of 1.5. So we will control ourselves within that one.

Dhruv Shah: And just about the Thane and Nashik Hospital, can you just give a highlight on what kind of margin profile or ARPOB profile can we expect over there?

Abhinay Bollineni: Nashik and Nagpur, the margin profile will be similar around -- I think all the 3 hospitals, the margin profile will be similar. Nagpur we're on track. Last quarter, we closed at 18.5%. But then there is still operating -- optimization that can be done and we are pretty clear on how it can scale up to our 25% EBITDA margin at Nagpur. I think if we do that in Nagpur, we're pretty confident to replicate the same thing in Nashik as well as in Thane.

As far as the ARPOB profile, I think Nagpur, Nashik will be similar the ARPOB. Thane, the current competition in that micro market, the ARPOB is around INR55,000, INR56,000. So I think that's whereabout is where KIMS also we'll aspire to do. And the current ARPOB in

Nagpur is around INR33,000, INR34,000 -- INR35,000. In Nashik also we're seeing similar potential.

Dhruv Shah: And last question is occupancy. So on Y-o-Y and Q-o-Q basis, we are seeing some difference in occupancy. So can you highlight what is the reason? I mean there has been some increasing incidence of viral and infections. So why are we seeing this drop?

Abhinay Bollineni: I think there's efficiency on the ALOS. The ALOS has come down. If you look at the IP volume and OP volume, there has not been a dip -- a significant dip. I think that some of our mature facilities were like Kondapur, Rajahmundry where we're running out of capacity, we are trying to optimize on the ALOS a little, and that's where you're seeing the overall dip in ALOS because of which the occupancies have fallen. So IP volume is a good indicator, and we are not seeing any strong -- any decline in that.

Dhruv Shah: And if I can just squeeze one last question. When are you expecting breakeven at Thane Hospital and at Nashik?

Abhinay Bollineni: Sorry, could you come again?

Dhruv Shah: Breakeven in Nashik and Thane?

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Abhinay Bollineni: So Nashik, I think -- both hospitals, I think the first year of operations is where -- and the last quarter of the 10th, 11th, 12th month is where we're looking at breaking even -- operationally breakeven.

Dhruv Shah: Okay, sir. And just last, what was the total capex at the Thane Hospital?

Sachin Salvi: The total capex as of 31st March '24 is INR265 crores. The total capex anticipated for the Thane facility is about INR515-odd crores.

Dhruv Shah: INR515, right?

Sachin Salvi: INR515 crores,

Moderator: The next question is from the line of Bino Pati from Elara Capital.

Bino Pati: Most questions answered. Just looking for some color on how the pricing has changed over the last 12 months especially for insured patients. So what kind of negotiations had happened with insurance compa

nies. When did this happen? And what sort of price increases have you got?

Abhinay Bollineni: So last -- most of our contracts get renewed this financial year, we have had some price hikes from a few contracts, but those are not the strong volume drivers. A lot of that is going to happen in this financial year. And whatever has happened, we have seen a healthy price hike that we usually get, which is close to 10%, 12% from these private insurance company. GIPSA was supposed to happen last year, which is bulk of our insurance volume. It was actually slated to happen in November last year but there is no sign of that happening yet. So we are also eagerly waiting for that price hike to happen. And if that does happen, that is a good 20%, 30% jump in insurance pricing.

Bino Pati: When was the last time the GIPSA revision happened?

Abhinay Bollineni: So GIPSA usually is a cycle of 4 to 5 years.

Bino Pati: So the last revision was maybe 5 years back.

Abhinay Bollineni: Yes, correct. Correct.

Bino Pati: Understood. And for the other insurance companies, we expect some time this year it should happen. So when interest rate benefit of that will come in the 2Q, 3Q, 4Q.

Abhinay Bollineni: Correct. Correct. There are different time lines for each of these contracts and a good chunk of them happened in this financial year.

Bino Pati: Understood. And these -- if I understand correctly, these negotiations happen once in 2 years.

Abhinay Bollineni: Once in 2 years. But by the time it comes to execution, there is a 6-, 9-month delay in the business bilateral negotiation.

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Bino Pati: Understood. And just one last color. Last time, you said you got again 10%, 12% increase. Do you expect similar increase this year also?

Abhinay Bollineni: Yes. Among the renewals, yes, we should anticipate that.

Moderator: The next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: Sir, can you help us understand the volume growth in the Secunderabad and Kondapur Hospital is a bit better. I mean, from what we understand, while we may be a bit constrained in Kondapur on the capacity side, there definitely seems to be more scope on occupancies in Secunderabad, We get that point one, the new doctor additions and they will start contributing.

Just trying to understand what are the dynamics here in terms of competitive pressures? I mean

wasn't the doctor issue relevant in the past? I mean, why do we need more doctor additions to kind of take care of the volume growth, especially in Secunderabad. If you can just help us understand these things would be really helpful.

Abhinay Bollineni: So, Alankar, thank you for asking this question. The way we've been always projecting growth in all our hospitals, not just at Secunderabad is there will always be a point where volumes will stop growing in each specialty. So the only way to continue to grow the hospital is if you add more subspecialties and add new clinical therapies, that's when the volume growth happens. So even historically, the growth in KIMS Secunderabad or any of our hospitals has not been because of growth in the current specialty. It is -- there is a marginal growth in the current specialty, but a significant growth driver has always been new clinical specialties getting added, new clinical therapies getting or addition of doctors in the current clinical specialty.

So Telangana, KIMS Secunderabad being a mature facility, having catering to a lot of the specialties, we are now onboarding a lot of new therapies like a new doctor that's joining us is a doctor who specialized in arthroscopy work. A new department that we're starting is rehab care because we have a strong neuro and orthopedic program.

So when you launch new specialties, that is what will drive more volume into the hospital. And in KIMS, Secunderabad, though there is space and capacity, onboarding of these docto

rs and

starting the program is where we are seeing some delays. Like we started Cuddles in KIMS Secunderabad. The ramp-up has been quite healthy. We started it in November. The growth in volume, both in neonatology care and pediatric care has been quite healthy. But now we're waiting for obstetric care to be -- to start obstetric and gynec care in a bigger way in KIMS Secunderabad. So when all of these additions keep happening, the volume growth happens, it's not that the current doctors alone will be able to drive that growth.

Alankar Garude: Understood. And any view on the competitive dynamics in that micro market.

Abhinay Bollineni: So in the last 1 decade in KIMS Secunderabad, the micro market in which we operate here, there has not been any bed addition other than the new KIMS Sunshine hospital that we've added in the last 1 decade in this -- if you take a 10-kilometer radius, we have not seen any bed addition that has happened other than what KIMS has added and KIM Sunshine has added.

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Alankar Garude: Understood. Okay. The second question, Abhinay, is, I mean, on Sunshine, you mentioned about good traction with the consultants joining. Can you help us understand, I mean, more details on how has been the pickup of the new facility. You made that point on margins as well, but just wanted to understand the time lines on -- I mean, when exactly should we be able to reach those peak margins on Sunshine.

Abhinay Bollineni: So I think Sunshine, we moved in December. And in spite of moving in and all the instability there, we have seen the quarter 3 and quarter 4 revenues improved. So we have done much better in quarter 4, in spite of it being a dull season, especially for orthopedics, given the seasonality impact there. And because of movement, we still have seen a good revenue growth. That is a positive sign of the new hospital.

But we have already started engaging with a lot of the doctors that we want to onboard for KIMS Sunshine. The initial response has been positive. And given that these are all senior doctors from other hospitals in this micro market, we anticipate that the onboarding process will be a little slow. And like we had earlier communicated, I think Q3, Q4 is when we're looking to fully complete the onboarding process for -- or at least 80% of the onboarding process for Sunshine. And from then on, maybe another 1, 2 quarters is when the ramp-up of the doctors that got onboarded will happen.

But having said that, Q1, the run rate also has been quite encouraging for Sunshine after moving into the new facility without much doctor addition. So it just speaks that the new facility with location, the presence and the branding itself has been able to attract a little more volume in spite of not adding a lot of new clinical therapies. Though we've added a few, but I'm saying the response has been quite encouraging so far.

Alankar Garude: Understood. So basically, second half of FY '26 is when should we -- by when we should expect margins to meaningfully implement possibly full year impact in FY '27 when we look at Sunshine.

Abhinay Bollineni: However, having said that, Q4 the normalized -- if you remove the nonrecurring is still at 26.4% in Q4. I'm sure Q1 will be a little better given the higher revenue growth versus Q4. But you're right, as '26 is when we should see a full stability in Sunshine EBITDA margin.

Alankar Garude: Sure. And one last question. So we have earlier mentioned about getting into other cities of Maharashtra as well, Pune, Aurangabad, Kolhapur, maybe Mumbai as well. So given that we already have quite a lot on our plate, are you still looking out for expansion opportunities in these markets?

Abhinay Bollineni: So at this point in time, we have slowed down a little on our capital deployment because I think we've reached a healthy number on the debt-to-EBITDA and debt to equity. But what we are still e

xploring and we are seeing positive traction is on the O&M contracts with the commitment to buyback over a period of time. And we are seeing that a positive traction of such assets, both in Maharashtra as well as in Telangana and AP, but nothing that we've been able to execute yet. Maybe over the next 2 quarters, we should be able to bring more such O&M contracts on board,
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where we have no capital commitment today, but we have full control of the asset with a commitment to be able to buy it over a period of time.

Alankar Garude: And directionally there, Mr. Abhinay, will we be still open for any partnership, equity partnerships like we have done in the past? Or incrementally expect the contribution from such partnerships to keep on coming down?

Abhinay Bollineni: No, I think there is a lot of traction on partnership. A lot of these O&M contracts are also because of the willingness to partner with KIMS even in Maharashtra and obviously in AP and Telangana. But given that our ability to be able to deploy and do more capital infusion is limited at this point in time, we are trying to do a completely asset-light model, which is no capital commitment and purely run the hospital operations and take responsibility for ramp-up.

Moderator: Next question is from the line of Gagan Thareja from ASK Investment Managers.

Gagan Thareja: My question -- first question is around the operating margin profile. How should we think of the evolution of operating margin from the current levels over the next 3 years? And if you could give some phase-by-phase assessment '25, '26 and '27, it would be helpful because new hospitals are coming up and the old ones, as you said, can see some improvement. So on balance, how does it work out?

Abhinay Bollineni: I think a tough answer to give you at a consol level, but if you can look at it on a cluster-by-cluster level, I think both Sunshine and Nagpur, a lot of the fix on ground has been done, at least 80% to 85%. All the incremental revenue that flows in, I think, it's safe to assume that 35% of that will flow -- 35%, 40% of that will flow through to the EBITDA. Likewise, in KIMS Telangana and AP, a lot of the incremental revenue, also around 30%, 35% of that will flow-through to the EBITDA.

Gagan Thareja: But are we in a position to maintain margins? Or is there a positive or negative bias to the margins going ahead at least for the next couple of years? And I'm just looking at the direction of the margins. I'm not asking...

Abhinay Bollineni: No, I think we don't see anything strong coming for us to be able to sustain and maintain these margins. There could be some one-off expenses like we had incurred in the last -- in Q4 and Q3, which is towards repair, maintenance and establishing new projects. But otherwise, we don't see any outliers cost that will hit the P&L and impact the EBITDA margins for these 3 clusters.

Gagan Thareja: And what should be the impact of the commissioning of these clusters on your top line in the next 2, 3 years?

Abhinay Bollineni: To the top line?

Gagan Thareja: Yes, I mean on the revenue line?

Abhinay Bollineni: Of the new facilities that are -- the 4 new hospitals that are getting added?

Gagan Thareja: Yes, yes.

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Abhinay Bollineni: So like we said, each customer Bangalore the current ARPOB is around INR1.8 crores per bed, but we anticipate it to be a little lower given, one, our pricing strategy, plus initially the case mix could also be a little different. And we're confident of getting to a 25%, 30% kind of an occupancy in end of the first year, like is the case with Thane, where the ARPOB is around INR50,000, INR55,000. We are pretty confident around 25%, 30% occupancy we should get there. And the same is the case with Nashik. Nashik, I think the occupancy will be slightly higher, given the traction that we have fro

m all the doctors there, time it will take for them to onboard.

Gagan Thareja: I mean the second year after commissioning what sort of occupancy levels do you think can be a reasonable assumption to work with?

Abhinay Bollineni: I think by the third year of operations, we should get to a 50%, 55% kind of an occupancy by end of the third year.

Gagan Thareja: And what sort of increase in the depreciation and finance cost? Are we -- can we budget for the next year and the year after that the commissioning of.

Sachin Salvi: So in the -- like in this year and financial year '25 most of our assets like Bangalore, Thane are getting capitalized. So there will not be much impact on account of these additions, at least in financial year '25. Some impact will be there since it will be getting capitalized, February '25. But in financial year '26 financial year '27 these will be utilized for the full year.

There will be some impact on account of depreciation, on account of these 2 assets. As far as Nashik is concerned, we are saying we will be capitalizing or we will be running it -- we'll be fully running this hospital somewhere in the month of June. So there will be impact of some 7 or 8 months in this financial year.

Gagan Thareja: And what was the capex incurred for '24? And what will be the capex budget for '25 and '26.

Sachin Salvi: So in financial year '24, the total capex, which we have incurred is to the tune of about INR650-odd crores. This includes the land and the building, which we have purchased at Thane. In financial year '25, we are anticipating it to be in addition, about some INR400 crores to INR600 crores.

Moderator: The next question is from the line of Aman from PhillipCapital.

Aman: I think one question. So on the Thane cluster -- on the Thane acquisition that you have made, right? So from what I understand the Thane, I think in a 5, 6-kilometer radius, we also have Jupiter flagship hospital in Thane where you guys have acquired the new Hiranandani Hospital, right? So now the question is what is our strategy in Thane, do we still expect to maintain the similar levels of ARPOB? And I mean how are we going about those situations in Thane is what I'm trying to understand.

Abhinay Bollineni: I think there is opportunity for Jupiter and KIMS to coexist. I think there's opportunity for 1 or 2 more hospitals to come and coexist there. The opportunity -- the catchment is quite a big catchment, very untapped. There's only 1 hospital of such a great repute which is Jupiter of that KIMS Hospital

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scale -- size and scale. If you look at more mature markets like in South, you will see the bed density that of population like Thane would require would be much, much higher.

So I think there is enough opportunity. There are enough number of doctors that are there practicing in that micro market or outside who are willing to come back and play a full-time role and be a part of a hospital to help build the whole thing. I don't think there will be any impact on pricing or any such things like that. The ARPOBs will continue to be healthy and continue to grow as the case mix improves in that micro market.

Aman: Are we doing anything on the diagnostics front with the Thane hospital? Because from what I understand also that Jupiter has a significant amount -- significant number of diagnostic services in the Thane hospital. So are we planning to do something along those lines with our hospital too?

Abhinay Bollineni: I don't think we are changing any of our core strategy for Thane. We're just sticking to hospital services here.

Moderator: The next question is from the line of Rahul Jeewani from IIFL Securities.

Rahul Jeewani: Yes. Sir, I had a question on our operating margins. Now last year, first half of the year, if you look at our operating margins, excluding other income, we were around 26%. And in the second half of the year, that number has declined to

24% because of the one-off costs, which you incurred. So third quarter, there was a one-off of INR14 crores and fourth quarter there was a one-off of around INR10 crores, so maybe INR25 crores one-off impact. So for our modeling purposes going into next year, should we take 26% as the base margins to be working with? Or do you think some of these one-off costs would continue going into next year as well?

Abhinay Bollineni: I think there will be a very small part, if at all, in Q1, Rahul. But I think the Telangana cluster will stabilize at a 32% EBITDA margin net of these one-off costs in Q1 and moving forward.

Andhra continues to remain the way it is. There could be some margin expansion in the revenue and ramp up. And Sunshine, like I said, there has been onetime cost. It will be at around 26.5% as the base, and then it will move on further beyond that. Nagpur, the base that has been established is around 18.5% for Q4, and that will continue to grow as the ramp-up happens and a little bit of operational efficiency.

Rahul Jeewani: Sure, sure, sure. And Dr. Abhinay, just some clarification on Kondapur expansion. The presentation talks about these 500 beds getting commissioned in first quarter of FY '27. But I think Dr. Bhaskara or you pointed that some part of this capacity could come online next year as well. So can please clarify on that?

Abhinay Bollineni: Yes. So Kondapur given that it's right addition to our current facility. And since it's a very large facility instead of waiting for the full hospital to be commissioned, we have divided that into 2 phases. So part of it will be commissioned in FY '26, where we move a lot of the outpatient

services and we run both the facilities for some time, and we get some incremental 100 beds in Phase 1. By the time the full facility is operational, which is all the incremental 500 -- 550 beds. We are anticipating it to be done by quarter 1 of FY '27.
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Rahul Jeewani: Okay. So next year, beginning of the year, we would see these 100 incremental beds getting commissioned in the existing block.

Abhinay Bollineni: Correct. In the new block -- in the new facility. We have divided the hospital into 2 phases. So in Phase 1, next financial year -- Q1 of next financial year, we will move partly into that facility where we'll get an incremental 100 beds over and above our current facility. So we can move few specialties and start few specialties.

Moderator: That was the last question for today. I now hand the conference over to the management for closing comments. Over to you, sir.

Bhaskara Rao Bollineni: Thank you very much all the questions which are pertaining to the growth and all, we also got some knowledge out of it. And we will be very, very happy to say that definitely, we are in the growth engine. We will be able to deliver what we promised. And before closing, make sure that today is being a Hypertension Day, keep all your blood pressures, measure accurately, control it and live longer.

So this is the one important health disorder which is not related to anything except whatever the stress, especially if all of you are in the financial front. So don't worry about that. financials will keep growing and keep your blood pressure intact. Thank you. Thank you very much.

Moderator: Thank you. On behalf of IIFL Securities Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines. Thank you.

Call: Aug 2024

14th August, 2024

To,
The General Manager,
Department of Corporate services
Bombay Stock Exchange Ltd (BSE)
Phiroze Jheejheebhoy Towers,
Dalal Street,
Mumbai - 400 001.
Scrip Code - 543308
ISIN: INE967H01017 To,
The Manager,
Listing Department
National Stock Exchange of India Limited,
Exchange Plaza, 5th Floor,
Plot No.C/1, 'G' Block
Bandra - Kurla Complex
Mumbai - 400 051.
Symbol - KIMS
ISIN: INE967H01017

Dear Sir/ Madam,

Sub: Transcript of earnings conference call with Analyst / Investors.

In Continuation to our letter dated 02nd August 2024, the Company organized a conference call with the Investors/ Analysts on Thursday, 08th August 2024 at 9:30 AM (IST). A copy of the transcript of the conference call held with the Investors/ Analysts is enclosed herewith and the same has also been uploaded to the Company's Website at <https://www.kimshospitals.com/investors> > Disclosures under Regulation 46 of SEBI (LODR) Regulations, 2015 > Analysts Calls schedule, PPT & Transcripts > Transcripts, Audio & Video Recordings > FY 23-24.

Kindly take the same on record and display the same on the website of your exchange.

Thanking you,
Yours truly

For Krishna Institute of Medical Sciences Limited

Uma Shankar Mantha
Company Secretary & Compliance Officer
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"KIMS Hospital
Q1 FY '25 Earnings Conference Call"
August 08, 2024

MANAGEMENT : DR. BHASKAR RAO BOLLINENI – FOUNDER AND
MANAGING DIRECTOR – KIMS HOSPITAL
D R. ABHINAY BOLLINENI – EXECUTIVE DIRECTOR
AND CHIEF EXECUTIVE OFFICER – KIMS HOSPITAL
M R. SACHIN SALVI – CHIEF FINANCIAL OFFICER –
KIMS HOSPITAL

MODERATOR : MR. RAHUL JEEWANI – IIFL SECURITIES LIMITED

Moderator: Ladies and gentlemen, good day, and welcome to KIMS Hospital Q1 FY '25 Earnings Conference Call hosted by IIFL Securities Limited. As a reminder, all participant lines will be in a listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing the star, then zero on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Rahul Jeewani from IIFL Securities Limited. Thank you, and over to you, sir.

Rahul Jeewani: Yes. Thanks. Good morning, everyone. This is Rahul from IIFL Institutional Equities. I welcome you all to the first quarter earnings conference call of KIMS Hospitals. From KIMS, we have with us Dr. Bhaskara Bollineni, Founder and Managing Director; Dr. Abhinay Bollineni, Executive Director and CEO; and Mr. Sachin Salvi, Chief Financial Officer. Over to you, sir, for your opening comments.

Bhaskara Rao Bollineni: [inaudible 0:01:06] Let us start on a happy note. Today 8th August, is a very important day. It happens to be a National Happiness Happens Day. So I would like to present the financial and operational details for the Q1 FY '25. I'm sure the results will make you happy.

The gross revenue of INR693 crores, a growth of 13.8% year-on-year and 8.7% on a quarter-on-quarter basis. EBITDA of INR184 crores, a growth of 14.9% on year-on-year and a 13% on a quarter-on-quarter basis. EBITDA margin at 26.6% versus 26.3% in quarter 1 FY '24 and -- FY '25 -- and 25.5% in quarter 4, '24. EBITDA margin, excluding other income, stands at 26.1%, a growth of 0.1% on year-on-year and 1% on a quarter-on-quarter basis.

PAT at INR95.1 crores in quarter 1, '25 against INR86.7 crores and INR71.6 crores in quarter 1, FY '24 and quarter 4, FY '24, respectively. Consolidated EPS of INR10.8, a growth of 7% on a year-on-year and 32.1% in a quarter-on-quarter basis.

The average revenue per operating bed grew by 21.3% and 12.2% on year-on-year and a quarter-on-quarter basis, respectively. Average revenue per patient grew by 6.1% and 1.4% on year-on-year and a quarter-on-quarter basis, respectively. IP volumes grew by 7.5% and 7.1% on year-on-year and a quarter-on-quarter basis, respectively. Conducted 4.2 lakh OPD consults in quarter 1, FY '25, which grew by 10.2% on year-on-year and 2.6% on a quarter-on-quarter basis.

On the last quarter, I apprised you about our setting up a 290-bed hospital in Thane, Mumbai. Again, today I would like to inform you about yet another new acquisition at Vizag. Queen's NRI Hospital, a 200-bed hospital at prime location in Vizag. It was established in 1995. It's a built-up area of around 1,50,000 square feet by a couple of doctors. The hospital is in a densely populated locality with a good potential. KIMS already have a multi-specialty hospital and gastro unit at Vizag and this addition will help it garner a larger share in the market.

This hospital is situated in a prime locality, which makes it an attractive proposition. KIMS entered into an agreement with the owners of QNRI to purchase 100% holding in the company.

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The addition of new hospital will make the chain of our hospitals longer and stronger. Our Nashik Hospital is due for inauguration in this quarter.

All other projects are progressing well according to the time lines. You all know that we are making a clinical achievement as I keep mentioning in all these investors presentation, in the district headquarters hospitals, all are doing very well so that the patients need not go to the higher centers that is one of the prime motto of KIMS that we need to be able to cater more care towards the patients at the district level itself.

We got a few awards. KIMS has made a

name in organ transplantation having done about 190

lung transplants in 3 years, which is an all India record. Last year, the entire -- all the transplants put together, we did 259, which is the highest number in Telangana state.

I am happy to inform that KIMS Secunderabad is awarded best Brain Stem Death Certifying Team for South region by National Organ and Tissue Transplant Organization. Government of India and a function held on 3rd August at New Delhi, which was presided over the Honorable Union Minister of Health and Family Planning, Shri J. P. Naddaji. This is an important recognition of our commitment and excellence in the field of organ transplantation.

It is not only the transplantation, we are trying to create an awareness national level. We are the leader in the lung transplant. That's why we need to improve the awareness among the hospitals about the importance of the organ donation. So we thank the government for this honor.

You are aware that 18% GST is levied on life and medical insurance premiums, recently, the

Union Minister of Road Transport and Highways, Shri Nitin Gadkari has urged the Finance Minister to consider the withdrawing of the above tax.

It is a commendable move and it will help in making health care more accessible and affordable. It will reduce the cost of insurance premiums, paving way for increased reinsurance penetration that can lead to more inpatient and outpatient coverage and service at hospitals and more and more patients will come into the purview of the insurance so that the affordability will improve. More and more people will utilize the hospitals for the diseases. It is hoped that government will take favorable decision that will have impact on providing wider insurance coverage and helped benefits to people.

We are in the middle of Olympic Games. Our congratulations to the players from all over the world participating in Olympics and especially to our Indian players. Special congratulations to Manu Bhaker, Vinesh Phogat and Neeraj Chopra for their outstanding performance. Let us congratulate all the winners and hope that our players will bring more medals and glory to the nation.

Who will ever imagine that mere 100 grams can make a world of difference. It is unfortunate that barely 100 grams of extra weight had deprived our Vinesh Phogat of a possible gold medal at Olympics. In any case, she made history without a medal and the entire nation stands by her for a valiant fight. We are just 1 week away from our 78th Independence Day, I take this

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opportunity to convey Independence Day greetings to all you in advance. Thank you very much. Opening for the questions and answers.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Amrya from JM Financial.

Amrya: it's Amrya from JM. So congratulations to the management on the good set of numbers and the spikes we have taken on the transplant side. So just first question I have on the ARPOB. So our ARPOB for the quarter has grown substantially at around more than 20%. It seems to be led by the Telangana and Andhra region, but here, the occupancy has actually come down. So can -- is it possible to explain dichotomy here, like what has changed here in the hospitals during the quarter?

Abhinay Bollineni: So Amrya, thank you for the question. See, actually, if you actually look at the IP volumes, there has been consistent growth in IP volume. ALOS is something that we -- because of high occupancies in Andhra in some of our hospitals, we've been constantly working and trying to bring down the ALOS. And if you look at quarter-on-quarter, we have been trying to reduce the results. In this quarter also, we put consistent efforts to bring it down. So -- and that is the reason why the ARPOB has gone up, but I think we should pay more attention to

IP volume growth
and ARPP. That will be a true indicator of this.

Amrya: So is it a change in case mix like higher volumes on the oncology etc, which would have led to this change or something related to case which would be driven...

Abhinay Bollineni: Nothing very significant change in the case mix. There is some marginal improvement in the payer mix, the incremental revenue that came in -- came from a cash and insurance payer where the ALOS is less. So that is one contributing factor. Our constant endeavour to reduce the length of stay for scheme patients, especially corporates and our registry is one function.

So a result of these 2, let the stay has come down. But like I said, again, we should pay more attention towards ARPP and IP volume. That's honestly a measure of what the growth is on a quarter-on-quarter basis.

Amrya: Sure. The second question I have is that you have changed the way we used to report occupancy. Now we are showing it on a capacity basis. So it has -- overall also, it has come down. At a hospital level, it is also very substantially low. So is there any indication you can give what optimal capacity utilization or occupancy would be for our match of hospitals going ahead?

Abhinay Bollineni: I think based on the feedback that we got from investors, we had removed 1 layer of reporting there and we are just now reporting occupancy and total bed capacity. But a healthy occupancy is around 70% -- 65%, 70% is a healthy occupancy.

Amrya: Sure. And that would be on the new formula?

Abhinay Bollineni: Yes, yes, on the bed capacity.

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Moderator: The next question is from the line of Abdulkader from ICICI Securities.

Abdulkader: Sir, just to understand the ALOS reduction better. So taking from the previous participant, I

mean, so what are the factors actually which has contributed to this reduction in ALOS and could we, in the near term, built on the kind of a reduction? Or this was more specific to this particular quarter?

Abhinay Bollineni: I think we can sustain this number but further reduction, we are putting constant effort to see how we can further reduce. So there are 2, 3 factors that drive ALOS. One is that the incremental revenue that came in, came from a payer mix, which is cash and insurance that is large, where the ALOS is predominantly low. Number two, the case mix within the quarter could be one contributing factor. And we are also putting effort, especially in Andhra, where the ALOS has been high because of a significant volume of patients coming in from the government-sponsored scheme. We are putting constant effort to see how we can reduce that length of stay because of a lot of documentation process that is involved. We're trying to make things more efficient there.

Abdulkader: Got it. And secondly, on the margins from Telangana cluster, though there is a sequential bounce back in terms of the margins, but it still continues to be a little lower than where we were. So any colour here where the peak margins can touch or was there's an absolute increase in EBITDA margins possible from where we are in Telangana?

Abhinay Bollineni: Yes, there is going to be an expansion of EBITDA margin. But at this point in time, we are investing on a lot of new programs. So if you look at, we started a new program called -- for rehab services in KIMS Secunderabad. We have onboarded a lot of doctors recently in the last 2 quarters. So, there is a drag of almost INR 1 crores, INR 1.5 crores per month from the new doctors and from the rehab program. So once the growth in revenue happens and there's some assurity there, things should bounce back to a 30-plus kind of an EBITDA margin. And also, there is some corporate hiring and corporate expansion that will continue to remain the same. But as revenue grows up, the margin expansion will definitely happen at Secunderabad.

Moderator: The next

question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: My first question is, can you update us on some of your upcoming hospitals. So Thane, Nashik and maybe update on the Bengaluru facilities about the progress? And are you on track to launch these as per the time line indicated earlier?

Abhinay Bollineni: Yes. I think all Bombay and Bengaluru, we are pretty confident we'll be able to launch them by the fourth quarter of the financial year. So far, we don't anticipate any delay in either of the projects. Both the projects in Bengaluru and in Bombay, we don't anticipate any delay at this point in time.

Damayanti Kerai: And Nashik, you mentioned you are -- you'll be shortly inaugurating it this quarter?

Abhinay Bollineni: In Nashik, we are fully commissioned and fully ready. We've onboarded a few doctors. We're just waiting for the final OC, occupancy certificate from the local authorities. We should expect that post August 15, and then we are ready to start.

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Damayanti Kerai: And how many beds you will start with, in the Nashik?

Abhinay Bollineni: Nashik is around 75, 80 beds, we will operationalize 75, 80 beds but the capital for all the 3 and 250-odd beds is fully incurred or is in the process of being fully incurred. What we mean by operationalize is we will have manpower only for 70, 80 beds. As the occupancy goes up, we will keep adding more.

Damayanti Kerai: Sure. My second question is on your efforts to improve mix, especially in the AP region. So I guess you earlier were focusing on oncology and whether a child care to really improve the margins or mix, etc. So again, like some update like have you implemented it across the hospitals in AP region?

Abhinay Bollineni: So like in the investor presentation, we have 3 units that will add cancer and mother and child by end of this financial year. We have acquired Queen's NRI, which is a hospital that also has cancer. So the Vizag geography that will take care of the needs for cancer care and the remaining geographies out of the 6 other geographies, 3 of them, we will operationalize cancer in this financial year.

Damayanti Kerai: Okay. And my last question is on your funding for the capex, which you have planned. So I understand, again, you're relying a lot on your internal accruals to fund more sort of projects. So is that the case? And in case like you have to borrow, what kind of cost of borrowings, etc. you are seeing?

Sachin Ashok Salvi: Right. So as far as the cost of borrowing is concerned, as of now, our average cost of borrowing is about 8.5%. We anticipate that the cadence in the same range. We are trying to fund as many projects as possible from our internal resources. As we have said in earlier discussion also that we intend to keep our debt-to-equity ratio within the range of 0.75 to 1. So that remains.

Moderator: The next question is from the line of Naman Bagrecha from IIFL Securities.

Naman Bagrecha: Congratulations on a strong set of numbers. Sir, just wanted more dope on the acquisition, the Vizag acquisition in terms of what kind of current revenues are and over the next 3 to 4 years,

what kind of scale-up do you see? And also in terms of margins and current ARPOBs, I mean you've mentioned in the slide that the potential ARPOB will be around 38,000, 42,000. So wanted to know more in terms of what is the mix currently? And also, as you mentioned that this was started by doctors. So what has led them to sell this hospital to us?

Abhinay Bollineni: Yes. So I think this was started by a family 20 years ago. And I think the second generation was not very keen on taking things forward and they were also settled outside of the country then they decided to divest the hospital and go back to U.K. with their children.

But coming to the hospital, it's currently doing a revenue of around INR65 crores, INR70 crores.

At a poi

nt in time, it used to deliver a revenue of INR120 crores, INR150 crores. But given that competition came in and a lot of doctors left the hospital because they didn't see growth, we feel that the potential for this hospital, given the excellent location, is close to getting, scaling up to KIMS Hospital

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almost INR250 crores of revenue over a period of 3 to 4 years' time with a very healthy EBITDA margin.

We plan to do just cash and insurance in this hospital. We don't plan to focus on any scheme patients in this facility. We have another hospital, which is just 10 kilometers away from this facility. We'd like to continue to do all scheme work in that hospital and continue to focus on building the cash and insurance there. And that's why we believe that the ARPOB could be in the range of 40,000, 45,000. And the idea is to also make it a center of excellence for cancer and transplant services in Andhra Pradesh.

Naman Bagrecha: So currently, the ARPOB would be around 38,000 and we would like to see...

Abhinay Bollineni: The ARPOB is around 20,000. Given that 30% of their income currently is from scheme patients and that's why the ARPOB. But if you look at the ARPOB of our Vizag unit, which does only - - which does cash and insurance, the cash and insurance ARPOB is around 35,000, 38,000 per bed per day. We are anticipating it to be slightly higher given the geographical presence and given the kind of work that we want to do in cancer and transplant.

Naman Bagrecha: Got it. So, what kind of, let's say, additional or incremental capex would be required to add these facilities like an MNC, oncology, etc., that we're making?

Abhinay Bollineni: All the technology is already there. It's a running hospital with cancer services. So there is nothing on the medical equipment that we'll have to do immediately over the next 2 years, 3 years. But after 2, 3 years, we might have to do a refresh on the entire medical equipment. But at this point in time, we'll have to renovate the facility to bring it to the standards of what our KIMS Hospital would typically look like, which could incur a capex of INR20 crores, INR25 crores. But on the medical front, I don't see a major capex happening over the next 24 months.

Naman Bagrecha: So also just -- I mean, on this one, what could be the current margins and the scale up you're seeing?

Abhinay Bollineni: The margin currently is around 8% on a INR65 crores kind of revenue. And we believe over the next 3 to 4 years, we should be able to get to a INR250 crores kind of revenue in this hospital.

Naman Bagrecha: Okay. So my next question is more -- is regarding to the previous participant's question. There has been a sharp decline in ALOS. So I mean, this is -- is my understanding correct, is that the scheme patients which were spending or staying for a longer period, that has now reduced or something else has happened this quarter -- sir, just wanted to understand on the ALOS rate, this quarter, we see a sharp decline in ALOS. I missed your opening comments to a participant where you highlighted the -- that it was because of scheme patients now having a lower ALOS. Is this correct? Or my understanding is completely wrong?

Abhinay Bollineni: Yes, that is correct. There are multiple factors for that to happen. But one of the strong reasons is because we are focusing on reducing the length of stay for scheme patients. Both Aarogyasri and public sector companies. But the other reason could be because of the change in case mix - KIMS Hospital

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- is because of the change in case mix and payer mix. Incremental revenue that came in, came in from a payer mix where the ALOS is substantially lower than the average.

Naman Bagrecha: So we believe that this would be sustainable for the balance 3 quarters of this year? Or we could... Abhinay B

ollineni: It should be sustainable.

Naman Bagrecha: Okay. And just the last one before I get back to queue. You mentioned that 65%, 70% kind of occupancy on total bed capacity is what we'll be looking at. And currently, Telangana and

Andhra Pradesh would be closer to let's say, around 50%. So what kind of -- I mean, in terms of, by when can we achieve the 65%, 70% kind of number on a full year basis?

Abhinay Bollineni: I think -- so, in Andhra, there are 2 -- 1 facility where we have almost only a 30% occupancy. It is because we don't do any scheme there, it's only focused on cash and insurance. That facility may not go up to a 70% ever. It's just that even at the 30%, 35% occupancy, we've been able to do healthy numbers. We are -- there are some hospitals that are at 90%, 95% kind of an occupancy. Right now, what you're seeing is at 52%, 53%. But over the next 3 years, 3 to 4 years, we should be -- and the current capacity, we should be able to get to that kind of an occupancy. 70% to 75%.

Moderator: The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services.

Tushar Manudhane: Okay. Sir, just on this IP volume for Sunshine, there has been significant increase both year-over-year as well as quarter-on-quarter. If you could shed some light on this aspect?

Abhinay Bollineni: So Sunshine -- like we had always said, once the new facility comes in, we stabilize, we'll be able to show growth in revenue. And we moved into the new facility in Jan, Feb -- sorry, in December, Jan. And the first quarter -- the last quarter, we needed time to stabilize that hospital in the new facility. And after we moved there, onboarding of doctors, growth from the existing specialties led to the growth. In fact, I think Q2 will be even more promising in terms of how things are shaping up at Sunshine.

Tushar Manudhane: And so subsequently, sir, so what do you think the ARPOB will settle at as we ramp up in terms of patient volume?

Abhinay Bollineni: I think Sunshine is a cash and insurance-based facility, the current ARPOB level should be able to -- will sustain.

Tushar Manudhane: Okay. But then can that be scope to the higher ARPOB.

Abhinay Bollineni: Because we'll also be adding a lot more specialty where the ARPOB could be because you're still currently heavily dependent on Ortho where the ARPOBs are higher. So since we add -- we'll add a lot more specialties, it could be at a much lower ARPOB. So assume that we will sustain ARPOB is the reason.

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Tushar Manudhane: Understood. So on Telangana front, both at least IP volume has been a very stable. We've not seen much of increase -- occupancy is also sort of dipping. And both in -- even in Andhra Pradesh as well, the IP volumes has been little stable. Any particular reason to call out?

Abhinay Bollineni: I think, once we add -- in Andhra, particularly, once we add more capacity, we add these specialties like cancer and mother and child. We will pursue an uptick in growth, both in volume and revenue. As far as Andhra is concerned, again -- sorry, as far as Telangana is concerned, we have taken measures to add new doctors. Subsequent quarters, there has been good growth in revenue, healthy growth in revenue. The subsequent quarters, we will see this growth as well.

Tushar Manudhane: Understood. And with this 48% occupancy here like 30% EBITDA margin for Telangana. So, with the addition of new specialties or the ramp-up of occupancy. So what is the scope of improvement in the profitability for this year?

Abhinay Bollineni: For every incremental revenue in Telangana and Andhra, we should assume 35% of that will flow through to EBITDA .35%, 40% of the incremental revenue will flow through to it.

Moderator: The next question is from the line of Bino from Elara Capital.

Bino: One thing, is any of our specialist Doctors has moved from the Secunderabad f

acility to the new
Sunshine facility?

Abhinay Bollineni: No, no, nothing. No. No one.

Bino: Okay. Sunshine is our fresh recruitment?

Abhinay Bollineni: Yes. No one from KIMS.

Bino: Okay. And the shift out of the old facility has already happened fully?

Abhinay Bollineni: It's fully done. It was done in January. It's fully done in January. This needed some time before all patients got aware that the facility has now moved. So after that happened, we've seen good uptick in revenue.

Bino: Okay. And what's the plan with that old facility?

Abhinay Bollineni: So we have started our -- we had demolished Block 1 and Block 2 in KIMS Secunderabad. We have now shifted some services to that facility. We have started a rehab program. And after 3 years when the new facility in KIMS Secunderabad is ready, we will then shut that and hand it over because it's a lease...

Bino: Okay. Okay. And so that old facility revenue is also now reported under Sunshine or under Secunderabad?

Abhinay Bollineni: KIMS Secunderabad.

Bino: Okay. The old Sunshine facility revenues under Secunderabad?

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Abhinay Bollineni: There's very little revenue coming in from there because we only moved some back in specialties. But yes, whatever is coming is coming in KIMS Secunderabad.

Bino: Okay. And what's the update on Nagpur, is it now stabilized? Can you expect a steady performance in the level for what we are seeing right now?

Abhinay Bollineni: Yes. I think Nagpur has stabilized, there will be margin expansion further. In fact, there have been some onetime costs in this quarter. If not, it would have been INR11 crores kind of an EBITDA on a INR53 crores revenue. On a INR52 crores revenue, we would have done INR11 crores EBITDA. I think things are now quite stable there. We are actually planning to do a further capex in Nagpur. We will come out with a plan soon where we expand the total number of beds, and we also had cancer as a service.

Moderator: The next question is from the line of Rahul Jeewani from IIFL Securities.

Rahul Jeewani: Yes. Dr. Abhinay, can you talk about these incremental hirings which we are doing on the corporate side. So the ex CFO of Aster has joined you. So what are your broader plans in terms of, let's say, some of the senior corporate hires?

Abhinay Bollineni: So like we've always been indicating in the past, there's a very strong team that's coming to build out the Karnataka story. I think Sreenath is officially onboard. We should have the rest of the leadership join us. We have Dr. Nitish will come on board as the Managing Director and the CEO of that cluster. He's been -- he was with Narayana Hrudayalaya for 20 years, with Aster for 10 years.

So we are expecting a very strong traction from the local doctors and his leadership further intact Karnataka story. And that's why we've always been very confident about building that story, and we have gone ahead and invested on 2 hospitals. We plan to do a few more in Bengaluru once these two hospitals stabilize.

Rahul Jeewani: And Dr. Nitish and Sreenath, have they invested equity into the Karnataka cluster? And in terms of, let's say, our plans for doctor hiring for the 2 Bengaluru hospitals, how have those been progressing?

Abhinay Bollineni: So far the doctor hiring, the traction is quite good. I think given their roots in Karnataka for the last 25, 30 years, and they connect, Nitish himself is a doctor, and they connect with the local doctors, has been quite helpful and with the KIMS model and the equity model, things look very promising. But again, since we're -- 6 to 8 months away from commissioning the hospital, it will be too early to comment. But I think overall, there is a lot of positivity in -- for both the hospitals and the kind of facilities that we're building, the kind of specialties that we are coming up with in that.

Rahul Jeewani: Sure, Dr. Abhin

ay. And in terms of equity, are these guys investing equity into the Karnataka cluster?

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Abhinay Bollineni: Yes. So the way it's structured is they hold a parcel of land in a place called Sarjapura, where we intend to build a large health city, but over a period of time after we stabilize these 2. So that has been evaluated at a fair market value and that will be brought on board as equity. And that will be their share of contribution. And as and when more capital is required for the cluster, all parties would get diluted.

Rahul Jeewani: Sure. And by when do you expect to start, let's say, building out this new hospital in Sarjapur?

Abhinay Bollineni: No, I think the first plan is to stabilize these 2, show investors attraction and gain confidence. And then we announce further expansion in Karnataka.

Rahul Jeewani: Sure, Dr. Abhinay. One last question from my end. In terms of overall occupancies, obviously, the dip which we have seen this quarter has primarily been driven by, let's say, the lower ALOS.

Now in terms of this optimal capacity utilization, which you indicated would be INR65 crores, INR70 crores kind of a number in 3 to 4 years. What kind of an ALOS number should we be working with for this 60% to 70% capacity utilization?

Abhinay Bollineni: I think the current ALOS 3.9 has not been decided -- 3.62 is what has been reported, anywhere between 3.6 to 3.8 is what is definitely sustainable ARPOB sorry, ALOS.

Moderator: The next question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: I just have 1 more additional question. So just want to understand what is plan to reduce promoter pledge like right now, there is notable pledging of promoter stake. So what are plans to unwind this?

Bhaskara Rao Bollineni: So, what's happening is promoter pledge, what we are looking for, now is approved by the Board, we're going to the AGM that the main promoters who are there, who are having a different

business that is infra, then their shares have been pledged. So we are going to make them public so that the main promoters and KIMS will not have any pledge maybe by end of this quarter, next quarter.

Damayanti Kerai: Sir, is the reclassification of the -- yes, some of the promoters. Okay. So this will happen by end of this quarter -- second quarter?

Bhaskara Rao Bollineni: We approved already by the Board and SEBI has been informed, as the AGM, which is in 29th of this month. After that, maybe it takes about a couple of weeks.

Moderator: The next question is from the line of Gagan Thareja from ASK Investment Managers.

Gagan Thareja: Okay. My first question is on the ARPOB, which is up almost 21% year-on-year and 11.5% quarter-on-quarter. If the case mix has not changed much, is it reasonable to infer that most of the ARPOB increase is coming from an increase in tariffs and increase in procedure rates?

Abhinay Bollineni: The ARPP remains to be constant. It is a function of 2, 3 things. Like I said, one significant driver for it is to reduce the length of stay for scheme patients. Number two is that the

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incremental revenue came in from payers, which is cash and insurance and from procedures where the length of stay has been lower.

So these two things have been contributing to this reduction in ALOS. I think moving forward, a good base would be around 3.6% to 3.8%. That range would be a good ALOS to assume. We'll not take -- we are not taken a price hike in the first quarter. We have done some price hike in July but not in the first month.

Gagan Thareja: So what was the price hike in July? One. And secondly, your reinsurance rates were also due for revision. Is that also contributed?

Abhinay Bollineni: For renewal, we're expecting that to happen in August itself. But again, this has been delayed for quite some

time where unless it happens, they are not too sure about this.

Gagan Thareja: Okay. Because it's a very substantial jump and very difficult to sort of fathom how that sort of works out.

Abhinay Bollineni: We've got multiple time lines from the association, and we are yet to see that happen.

Gagan Thareja: Second one, sir, you will be commissioning 4 hospitals. I think all put together in the last quarter of the year or perhaps first quarter of next year. Cumulatively, what will be the addition to the fixed cost and gross block because of these?

Sachin Ashok Salvi: So as far as the gross block is concerned, the total addition on account of Bengaluru and Bengaluru, Nashik and Thane would be about some INR700 crores to INR800 crores plus.

Gagan Thareja: And the fixed cost addition?

Abhinay Bollineni: Fixed cost per center other than doctors and consumables, the cost per center will be around INR6 crores, which is your cost per HR and your other expenditure per center. So we have 3 centers, Nashik into -- sorry, Thane and 2 Bengaluru. Between these 3, we'll have an INR18 crores -- INR18 crores to INR20 crores kind of a fixed cost. This is other than the cost of doctors and consumables.

Gagan Thareja: And what would be the absolute increase in debt over the year as you sort of invest into the facilities? What's the current gross debt? How does it move?

Sachin Ashok Salvi: The current gross rate is about INR1,150-odd crores as on 30th of June 2024. By the year-end, when we are commissioning all these hospitals, we will be having a debt of about some INR1,600 crores to INR1,700 crores.

Moderator: The next question is from the line of Amrya from JM Financial.

Amrya: I have one question on the positioning of our hospital chain. So a few years back, we were considered as an affordable care of category brand in Telangana and Andhra. However, over a

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period, we have seen this gap between the premium hospitals and affordable being reduced a bit as we have increased our case wings it also in terms of hospital services provided, etc.

So is there a concerted effort towards improving our brand and moving towards a bit of premium brand? And also if you can guide us on the positioning of this? And also when you move to the different regions, will you try to keep these brands? Would this be similar in these regions or you would go with the needs of those particular locality?

Abhinay Bollineni: I think we are continuing to stick to our ethos, which is to be affordable. We are trying to be affordable across all the services that we offer. So we are a quaternary tertiary care provider. We will offer all these services. As we offer, the ARPOB will continue to go up. I don't think we believe in anything such as a premium priced hospital or an affordable price -- premium sized

hospital. We're very happy with the way we are positioned, the kind of case mix, the kind of ARPOB that we deliver. And we continue to focus on that space.

Amrya: Sir, but the regions like the HITEC City, Kondapur etc, which are getting populated with high income residency, etc. So even in these regions, you would keep the hospital as an affordable care hospital or there you would go with the needs of the region?

Abhinay Bollineni: No, I think even -- so in Kondapur, we have been present in that geography for the last 10 years. For us, the pricing in Kondapur or the pricing in Secunderabad remains absolutely the same.

Moderator: The next question is from the line of Harith Ahamed from Avendus Spark.

Harith Ahamed: So for the new hospitals that's coming up at Nashik, Thane and Bengaluru. Could you give a sense of the kind of ARPOBs that we are expecting there? Will it be in line with our current level of around 37,000, 38,000. Could it be higher? Because I understand that there will be a higher share of scheme patients when you start off. So any colour

r would be helpful.

Abhinay Bollineni: So I think Thane and Bengaluru, the ARPOB will be similar to what we are delivering at Sunshine and at the Telangana cluster, which is around 60,000. Both -- we believe that will be the case, both in the case of Thane and in Bengaluru. In Nashik, what we have projected or what we're seeing is to be similar to what we are seeing in Nagpur, which is around 35,000 kind of ARPOB.

Harith Ahamed: Okay. And given Nashik commissioning is scheduled to happen in this quarter and then there'll be some losses that's coming from -- that's going to come from there. Will you expect to maintain the kind of margins that we've delivered in the first quarter for the entire FY '25?

Sachin Salvi: Yes, I think the margin -- the losses that will come from Nagpur will be -- sorry, from Nashik will be marginal. It should get offset with the growth that is coming from Queen's NRI, which is a new acquisition. Growth that will come from Sunshine and Nagpur and Telangana cluster. The margin profile will continue to remain as same.

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Harith Ahamed: Okay. And then last one. So apart from Mr. Sreenath Reddy, who is joining as Director of Business Strategy and M&A. You also mentioned Dr. Nitish Shetty also joining us. In what exact capacity is he coming on board?

Abhinay Bollineni: He is going to be the CEO of the Karnataka cluster.

Harith Ahamed: Okay. And the equity that they will own, will that be at the overall Karnataka region level? Or will it be related to the new Sarjapur Hospital, which you're planning to?

Abhinay Bollineni: It will be for the entire Karnataka cluster.

Moderator: The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services.

Tushar Manudhane: Just on Andhra cluster, if you could just help understand let's say the picking order of the profitability of different units.

Abhinay Bollineni: So you have 3 units in Andhra that delivered a 30-plus kind of a margin. And then you have another 4, 5 units that are around 18% to 20% kind of a margin. So as they mature and they grow, we believe that the entire Andhra cluster could expand to 27%, 28% kind of an EBITDA margin. This could happen over the next 24, 36 months.

Tushar Manudhane: Which are the ones which are pulling down the margins?

Abhinay Bollineni: Right now, 1 unit in -- they're all in their growth phase, I wouldn't say pulling down the margins. The unit in Kurnool, the unit in Vizag and Srikakulam are where they're still in the growth phase because of which the margins are still on the lower side.

Tushar Manudhane: Understood. And similarly on the Telangana cluster, sir, if you could also explain there?

Abhinay Bollineni: So Telangana, I think both the hospitals are mature. We're continuously investing in new project -- new areas of treatment and new doctors. And also a lot of the corporate cost expansion has happened in Telangana for the group. That is reflected in Telangana for the group. That is why you see a dip in the margin. But there's a drag of almost INR1.5 crores from doctors who have been recruited but yet to see maturity in those. Or yet to even break even.

Moderator: The next question is from the line of Charul Agrawal from Bank of America.

Charul Agrawal: Congratulations on a good set of numbers. My first question is regarding the Nagpur facility. So we have seen a new competitor entered the market. Do we see any kind of competitive pressure in terms of doctor hiring? And you also announced that you were -- you might be looking to expand in Nagpur region. So any sense what is happening from a competition perspective on that locality?

Abhinay Bollineni: I think our doctors have been approached by competition, but the good part is, we have not lost anyone. And in spite of competition coming, I think our growth has been -- it's been the best

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growth over the last few quarters. So we don't see -- I think there is space for all of us. We continue to grow and I hope our competition also continues to grow.

Charul Agrawal: Sir, my next question is on the insurance that you did mention about insurance being a significant growth driver in your opening comments with some regulation changes. I just wanted to understand that if we think of it from a metro city versus a Tier 2 and beyond approach, I think in the past, you have highlighted that insurance is more of a Tier 1 and metro city phenomenon. But if you put it in terms of your growth drivers, do you see insurance of significant growth drivers specifically for the Tier 2 and beyond?

Abhinay Bollineni: Not in Tier 2, Tier 3, it's still growing very slow there. There is a significant growth in metros and Tier 1 only.

Charul Agrawal: Okay. And how do the recent government steps that you mentioned, how do they change outlook for KIMS? Is it going to be significant?

Abhinay Bollineni: Your voice is not very clear. Could you please. There's a lot of disturbance.

Charul Agrawal: Is it better now?

Abhinay Bollineni: Yes. Better.

Charul Agrawal: I was just asking that since you were mentioning the recent changes in the insurance norms, in your opening remarks. Could you help us understand if that changes anything significantly for KIMS in terms of growth drivers?

Sachin Ashok Salvi: So, I think it is mentioned in the opening remark that only that Union Minister has proposed that the GST, which is there on the insurance premium can be reduced. It is not yet implemented.

Bhaskara Rao Bollineni: So the proposal by the Union Minister to the Finance Minister, removal of the GST, which is 18% on insurance premium. Suppose if they remove that 18%, there are more and more people will come into the insurance bracket. So that then affordability will become a good challenge for them.

Charul Agrawal: Got it. And sir, one last question, if I may. So you have announced quite a bit of expansion plans and you're also building upon it. So I wanted to understand how much more headroom do you see in terms of adding more capacity, what would be the metrics basis which you could decide that now from an expansion mode, we might want to consolidate a bit. So how should we look at it from a medium- to long-term perspective?

Sachin Ashok Salvi: So Charul, I was answering your question. We are currently managing 4,000 beds. We can expand the number of beds, but the limited -- the limit which we are having is the capital constraint. We have said we don't want to exceed a debt equity of 0.75:1, debt-to-EBITDA of 1.75:1. As far as we are within that range, I think we can -- there is no other limitation and it is already there. The only limitation, as I've said, is the capital constraint. At that point in time, we will reconsider our decisions if any expansions are in pipeline.

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Moderator: The next question is from the line of Parvati Rai from Equentis Wealth Advisory.

Parvati Rai: A quick on the margin. So while we did say that it's sustainable for this year and the current quarter level margins of around 26.5% despite when you are -- I mean, the Nashik coming in, where the loss would be opposite. For next year, given Q4 and Q1 would see a couple of other hospitals coming online. Any colour if we can have there? What would be the sustainable EBITDA margin with these new hospitals coming in?

Abhinay Bollineni: I think as soon as this financial year, we have Queen's NRI coming in. And we also have -- which is already an EBITDA-generating asset. We have Nashik that could potentially have some drag for the first 2 quarters. And that's why we said, given that both of them have set off, we would the margin of the other units will continue to sustain.

Parvati Rai: Yes. So any colour

possibly with the other units. So I mean from a next year perspective as well should we see this level sustaining? Or will there be a slight change with the new hospitals coming in?

Abhinay Bollineni: So the next year, I think the growth will continue to sustain for the existing hospitals. There will be incremental growth coming in from Nashik and Queen's NRI that has been acquired. But there will be drag on Thane, Bengaluru, both the facilities in Bengaluru. We were expecting -- or we have modelled a drag of not more than INR30 crores to INR40 crores across these 4 facilities -- across these 3 facilities.

Parvati Rai: Okay. That's good. And with respect to the average revenue per operating bed while there have been questions and you did kind of give your views. So for the full year, while this quarter has seen a massive jump and we understand the change that you spoke of and the driving factors, will this kind of level sustain? Or again -- so what is the sustainable ARPOB for the whole year that you're looking at largely, given the mix change or the scheme or the changes that we saw,

the insurance or the incremental revenue picking up?

Abhinay Bollineni: I think the numbers are sustainable at the same range, 38,000 -- is -- 36,000 to 38,000 which is a sustainable number.

Moderator: The next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: Sir, first question for Nashik, Thane and 2 Bengaluru hospitals. So from where are we hiring the doctors? Is it largely some other corporate hospitals or relatively smaller hospitals?

Abhinay Bollineni: A mix of both. They are largely -- in Thane -- it's a mix of 50-50 from closest competitor and smaller hospitals are willing to consolidate and move to bigger hospitals. But in a place like Bengaluru, it will largely be from similar competition.

Alankar Garude: Understood. And Abhinay, any thoughts on the seniority profile of the doctors? And how would you compare the seniority profile to, say, the Secunderabad hospital?

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Abhinay Bollineni: See, Secunderabad is a 20-year-old hospital. So it's very unlikely to have that level of seniority across these facilities. But you will -- we will find a similar skill set that we have at KIMS Secunderabad in both Bengaluru and in Thane.

Alankar Garude: Understood. The second question is on Secunderabad. You had mentioned earlier some time back that there is also a possibility of doing brownfield in that hospital, if the need be. Now given the lower ALOS as well as lower occupancy, would it be fair to assume that there will not be any expansion in Secunderabad over the foreseeable future?

Abhinay Bollineni: So Alankar, in our previous call, we had mentioned that of the 1,000 beds that we have in Secunderabad, 200 beds are present in Block 1 and Block 2, the current situation -- and that's a 20-year old facility. Though we have those beds available, they are not in a state that can be properly utilized for growth.

Hence, we have demolished Block 1, Block 2, we're building a new tower there, which can accommodate an incremental 350, 400 beds, but we will operationalize them or -- we will incur that capital over a period of time as and when required.

Alankar Garude: So precisely on -- yes, sorry. Go ahead.

Abhinay Bollineni: We are building the structure to have an incremental 300 beds. But as and when required, we will start utilizing that.

Alankar Garude: Understood. Sir, the question essentially was from an operationalization standpoint, do the plans get delayed a little bit, given the lower ALOS and lower occupancy now?

Abhinay Bollineni: There are no plans to delay it. There is a lot of services that we want to start and we are constrained with space. It could not be big, but there's a lot of outpatient daycare services that we want to start, but we are constrained with space. S

o we will continue to go ahead.

Alankar Garude: Fair enough. And one final question. As on date, at a corporate level, how many doctors would be under the equity model? And if you could bifurcate those between the listed company and the subsidiaries? And maybe the second one there would be what was the minority share of EBITDA in the first quarter?

Abhinay Bollineni: Minority share is around 10%, 11%. But on the share of doctors, we'll come back to you.

Moderator: Ladies and gentlemen, this will be the last question. It's from the line of Aditya from Sowilo Investment Managers.

Aditya: I don't know if this was asked before, but my question was more on the lines of -- I mean there was this government order that they try to cap the prices of private hospitals. So anything on that?

Bhaskara Rao Bollineni: Nothing, I think. See basically, now the Supreme Court has requested, the central government are asked to look into that and go over. And then later on, all these hospitals have been contested

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and then they are looking at it. I think it looks like it is a standstill and a few governments are working and the state government if you able to work and negotiate with the prices, which even before we said that it is a very tough job to do all these things. It's a very complex way of dealing these things. We cannot be able to fix the pricing. I think that may not be any risk in the health care as of now in India.

Moderator: Sorry to interrupt, sir. The current participant has been disconnected. As there are no further questions from the participants, I would now hand the conference over to the management for their closing comments.

Bhaskara Rao Bollineni: Let me summarize all the questions that have been put forward and auditable questions and thanks for asking all these questions. Initially, when we were talking about the affordability,

accessible, quality, a couple of decades back, there was not many players in the healthcare who are doing all these variety of patients to take care of the remote places, that's the way we are putting.

Now many healthcare players are coming, small, small hospitals and all these scheme patients and lower patients, lower socioeconomic group people are getting treated in those areas. It has been well strengthened in all the entire countries, thanks to all the Ayushman Bharat or the AarogyaSri.

So now insurance penetration also is increasing. So once the insurance penetration is increasing, then again, the affordability increases and the ARPOBs and average revenue per bed to keep on increasing. It is not we really, particularly to improve these things, automatically shift is happening in the Indian healthcare. And as we all mentioned that before, whenever we look for any acquired new geography and new acquisitions, we'll thoroughly investigate and put forward and how much is the growth we are going to anticipate by those particulars.

But even then we keep telling, and then we have been proved time and again. Onetime '18, '19, we acquired about 3 hospitals, the Ongole, Kurnool and Anantapur. We could be able to turn around and continue to grow. And the last time, again, we took about 3 hospitals in Nagpur, Sunshine and 2 Sunshine hospitals in Hyderabad. Even though Sunshine Hospital was a single branch dominated one, people will have some reservations so we are going to change and we thoroughly done a lot of due diligence and then investigations here in Nagpur.

So that's why we could be able to turn around now and that gives a lot more confidence and comfort towards our investors. And the other important thing now we are going -- having been shown turnaround in Nagpur, Maharashtra. Now we are going to move forward with Nashik and Thane. The way which we have been anticipated in Nagpur, we also should be able to anticipate. Now the Karnataka cluster, we are going and then we g

ot a good management skills with our efforts and brand, which is in Karnataka also, KIMS has a lot of Telugu-speaking people are there. So these both combinations and we can turn around the Karnataka cluster. And the more important whenever we go, we always -- from the beginning, we want to partner a few of the doctors and management, so that they will also have a skin in the game, at least 15%, 20% of KIMS Hospital
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the entire cluster. And that way, we can be able to move on. As we're promising in the mother and child and oncology in Andhra units, we are doing that.

Whenever we get an opportunity, some of the things we will be able to slow down, take over the opportunity because of the cash cost mismatch that right now, we've got the Vizag unit, which is a good one, what we feel. So these things we're able to look and it is very promising. And then whenever based on the opportunities, the existing units that we need to be able to look, we'll be able to channelize the capex so that it will be able to move forward.

And the other important thing when we keep on improving and the absolute number of bottom line may not keep -- continuously keep growing because in the beginning, the new unit comes, the percentage may come down. But the absolute numbers will keep going. And the LRs and other things are different seasons in different quarters. There are a few important things. One of the elective surgeries, emergencies and the complex procedures, in that quarter, if you have any -- more of emergency and complex procedures, the ALOS may a little bit grow.

And mostly on elective cases, the ALOS will come down and then we are adding the new therapies there. Some of these things may not be able to take a longer time. That's why the ALOS will also come down. These are all a few important things. And as we mentioned that the capex and the EBITDA, which we were not able to be very, very cautious about it. Maybe in 2, 3 quarters, we may cross that limit what we have, but that we'll be able to separate after that. So, we don't need -- because the opportunities are there, either I have two options, either I should be able to raise the IPO or I should be able to take a little bit leverage of the debt and then expand and come back a number.

These are all the few that whatever is the good for permission for the company [inaudible 1:06:54] we want to do with our own internal accruals and debt. So that the growth is -- pertaining to growth and the opportunities will also look forward. Thank you very much for patient hearing and I'm sure that we satisfied most of your questions.

Moderator: On behalf of IIFL Securities Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines. Thank you.

Call: Nov 2024

15th November, 2024

To, To,
The General Manager, The Manager,
Department of Corporate services Listing Department
Bombay Stock Exchange Ltd (BSE) National Stock Exchange of India Limited,
Phiroze Jheejheebhoy Towers, Exchange Plaza, 5th Floor,
Dalal Street, Plot No.C/1, 'G' Block
Mumbai - 400 001. Bandra - Kurla Complex
Scrip Code - 543308 Mumbai - 400 051.
ISIN: INE967H01025 Symbol - KIMS
ISIN: INE967H01025

Dear Sir/ Madam,

Sub: Transcript of earnings conference call with Analyst / Investors.

In continuation to our letter of intimation dated 04th November 2024 with reference to the conference call with the analysts/investors , we would like to inform that the Company had convened the conference call with the investors /analysts on Monday, 11th November 2024 at 10:00 AM (IST).

A copy of the transcript of the conference call held with the Investors/ Analysts is enclosed herewith and the same has also been uploaded to the Company's Website at [https:// www.kimshospitals.com/investors](https://www.kimshospitals.com/investors) > Disclosures under Regulation 46 of SEBI (LODR) Regulations, 2015 > Analysts Calls schedule, PPT & Transcripts > Transcripts, Audio & Video Recordings > FY 24 -25.

Kindly take the same on record .

Thanking you,

Yours truly

For Krishna Institute of Medical Sciences Limited

Uma Shankar Mantha
Company Secretary & Compliance Officer

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"KIMS Hospital
Q2 FY25 Earnings Conference Call"
November 11, 2024

MANAGEMENT : DR. BHASKAR RAO BOLLINENI – FOUNDER AND
MANAGING DIRECTOR – KIMS HOSPITAL
DR. ABHINAY BOLLINENI – EXECUTIVE DIRECTOR
AND CHIEF EXECUTIVE OFFICER – KIMS HOSPITAL
MR. SACHIN ASHOK SALVI – CHIEF FINANCIAL
OFFICER – KIMS HOSPITAL
DR. NITISH SHETTY – CHIEF EXECUTIVE OFFICER –
KIMS BANGALORE CLUSTER
MR. SREENATH REDDY – DIRECTOR BUSINESS
STRATEGY – M&A OF KERALA AND KARNATAKA
CLUSTERS

MODERATOR : MR. RAHUL JEEWANI– IIFL SECURITIES LIMITED

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Moderator: Ladies and gentlemen, good day and welcome to KIMS Hospital Q2 FY '25 Earnings Conference Call hosted by IIFL Securities. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference call is being recorded. I now hand the conference over to Mr. Rahul Jeewani from IIFL Securities. Thank you, and over to you, sir.

Rahul Jeewani: Yes. Thanks. Good morning, everyone. This is Rahul from IIFL Institutional Equities. I welcome you all to the Second Quarter Earnings Conference call of KIMS Hospitals. From KIMS, we have with us Dr. Bhaskar Rao Bollineni, Founder and Managing Director; Dr. Abhinay Bollineni, Executive Director and CEO; Mr. Sachin Salvi, CFO; Dr. Nitish Shetty, CEO for KIMS Bangalore Cluster; and Mr. Sreenath Reddy Director, Business Strategy and M&A. Over to you, sir, for your opening comments. .

Bhaskar Rao Bollineni: Yes. Good morning, and a warm welcome to all of you. It is about 1 month back, that is on 9th October, we lost a crown jewel of Indian corporate world in the death of Shri Ratan Tata. He was universally respected because he was the conscience keeper of Indian industry. He's known for his values, ethics and standards. My heartfelt tribute to this towering legend. We are witnessing the theory of rains and floods in Telangana, Andhra Pradesh and Kerala during August and September months, causing havoc and heavy damage.

As a part of our corporate responsibility, KIMS donated INR1 crores to each of these three states for aiding the victims. Let me now announce our financial and operational details of quarter 2, '24-'25. It can be observed that there has been a growth on each and every parameter both in quarter-on-quarter and year-on-year basis, reflecting the good work done by KIMS. Gross revenue of INR782 crores, a growth of 19.4% on year-on-year and a 12.9% on a quarter-on-quarter basis. EBITDA of INR223 crores, a growth of 23.8% on a year-on-year and a 21.2% on a quarter-on-quarter basis.

EBITDA margin at 28.5% versus 27.5% in quarter 2 FY '24 and 26.6% in quarter 1 FY '25. EBITDA margin excluding other income stands at 28.1% versus 27.2% in quarter 2 FY '24, 26.1% in quarter 1 '25. PAT INR120 crores in quarter 2 FY '25 against INR101 crores and INR95 crores in quarter 2 FY '24 and quarter 1 FY '25, respectively. Operational results. Average revenue per operated bed grew by 22.9% and marginally declined by 0.5% on year-on-year and quarter-on-quarter basis, respectively.

Average revenue per patient grew by 9.7% and 0.7% on year-on-year and quarter-on-quarter basis respectively. IPO volume 55,741 grew by 9.1% and 12.2% year-on-year and quarter-on-quarter basis respectively. 4Q volume INR4,73,989 grew by 12.2% and 12.5% on year-on-year and quarter-on-quarter basis respectively. The other developments that happened in KIMS. In the last quarter, I appraised you about acquisition of Queen's NRI Hospital in Vizag.

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I'm pleased to inform that we have taken total control of the said hospital, and we will see good results soon. I'm happy to inform that we are making our foray into Kerala also. We have taken over Sreechand Hospital in Kannur and entering the O& M contact with Westfort Hi-tech at Thrissur. The expansion blueprint includes setting up a fully equipped Oncology and transplant centers in Kannur and 350-bed hospital in Thrissur, dedicated to advanced transplant services.

These initiatives are intended to make high-quality h

ealthcare available at Tier 2 and Tier 3

towns. Our launch in Kerala is preceded by a lot of immaculate preparatory work, particularly in identifying and recruiting best managerial and clinical talent, focusing on 3C character, caliber and commitment. Accordingly, we have inducted a band of highly experienced and accomplished persons who are familiar with the terrain of Kerala and Karnataka. The rich expertise and long experience is expected to yield good results. We had a press meet in Kerala, which was well attended and grew huge positive response.

I'm sure it will translate into good growth in the coming months. We are also exploring further opportunities in Kerala and Karnataka, so as to exploit the full potential of this new team. Let me give a brief introduction of these new executives joining KIMS family. Mr. Sreenath Reddy joins us as the Group Director of Finance Strategy, M&A of the Kerala and Karnataka clusters. He is a Chartered Accountant and a Lawyer who held top positions in leading hospital groups

and pharmacy chains for the past 25 years.

Dr. Nitish Shetty joins as the CEO of KIMS Bangalore Cluster . He brings with him 25 years of leadership experience in reputed healthcare groups. He had matured health institutes, grow with his acumen and passion. Mr. Farhan Yasin joined as the CEO of KIMS Kerala Cluster. He has two decades of experience in the fields of marketing and management in healthcare sector and has a thorough knowledge of Kerala healthcare landscape.

Mr. Arjun Vijaykumar joins as the Head of Finance and Accounts for the Kerala Cluster. He is a chartered accountant with 16 year's experience have worked in healthcare and the financial sectors with the global firms like Deloitte. Thus, these four professionals come with a collective experience of 85 years between them and all are well familiar with regions. In all our expansion plans, we want to nourish the local talent flavor as they have a better understanding of the local needs and expectations.

We are confident that the new stream of executives will strengthen our brands adds further in our growth trajectory in general in Kerala and Karnataka states in particular. I'm happy to add that now KIMS has a presence in Guntur of Andhra Pradesh, which is one of the important and densely populated towns and a quite near to the upcoming new capital Amaravathi. We have entered into O&M agreement with Insignia Healthcare Private Limited of a brand name of Secura hospitals, a multi -specialty.

We have also entered into MOU with Wipro GE Healthcare Private Limited Secura Hospitals and exclusive oncology center. Already, we have a strong presence in Andhra Pradesh and these initiatives will make our impact stronger. We are confident of good results. A good portion of these expressions will be met by internal approvals and also a prudent debt to the extend

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essential. Over the next two or three quarters, the debt to EBITDA will be, as we promised, around 2 in the wake of these expansions.

KIMS always encouraged the doctor's equity likewise. Here also the doctors and key personnel will be investing in the project, resulting in a sense of ownership and increase the responsibility on their part besides impact in cost of funds. Our hospital like Nashik and Maharashtra operations are poised for a good growth. All other projects are running as scheduled. The acquisition of Vizag unit and the commencement of operation at Nashik resulting adding up 625 additional beds, post shifting of Sunshine to the new premises, there is a marked improvement in its performance.

Few of the academic accomplishments. The annual conference of Neuro spine Surgeons Association with India was held in Hyderabad

with the participation of 450 spine surgeons from all over the world. On this occasion a special workshop was organized by KIMS Hospitals that received a lot of attention and appreciation. Serbian Neurosurgery Society along with internal pediatric neurosurgery and Southeast Europe Neurological Societies recently held a conference at Belgrade, with a participation of 37 countries, including India. Four of our doctors, Dr. Manas, Dr. Vasantha Reddy, Dr. Sravan and Dr. Sandhya presented their work done at KIMS to the forum and it was widely appreciated and awarded.

KIMS Hospital signed MOU with Intuitive a global leader in minimal invasive care and robotic -assisted surgery. To this collaboration, KIMS Hospitals seek to enhance the availability of advanced surgical technology across geographies where access has been limited . It helps to include the patient at in Tier 2 and Tier 3 cities. In addition, we plan to offer robotic -assisted surgical training program to doctors in our associated colleges and institutes further contributing to future of health care in this region.

KIMS Hospitals and associated Arrhythmia Research and a Training Center conducted an education symposium with the participation of 300 cardiologists and electrophysiology from Telangana, Andhra Pradesh, Chennai, Kolkata, Bangalore and Delhi. The fact included distinguished electrophysiology from all over the country. The main objective was to highlight the importance of ECG in clinical decision -making of various cardiac disorders.

We are pleased to convey that as many as 14 submissions from the Department of Rheumatology, KIMS Secunderabad associated for American College of Rheumatology

convergence in 2024. At the Annual Meeting of International Urogynecology Association held in Singapore, our URO Gynec Department the privilege of presenting 9 papers on a lot of appreciation.

KIMS Foundation Research Center KFRC in collaboration with a Deaf Enabled Foundation has started an exclusive telemedicine center with idea of interpretation on deaf. This initiative will make health care more accessible to the people. Dr. Raghuram our breast cancer expert, was bestowed an honorary fellowship of International Surgical Society is the first and only surgeon for millions of continent in 120 years history of this prestigious organization to receive this highest recognition.

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Two important clinical achievements I should mention here Dr. Madhu Devarasetty surgical oncologist and robotic surgeon and his team completed 100 Robotic Whipple for pancreatic and periampullary carcinoma. He is one of the largest [inaudible 0:12:13] in India probably the highest. KIMS Sunshine Hospital achieved a significant milestone by successfully performed the first -ever Episealer Knee Implant surgery in Telangana and Andhra Pradesh states. This revolutionary procedure marks a new era in personalized joint care, offering hope who suffering from deliberating knee cartilage injuries.

KIMS Kondapur Hospital recently reached the landmark of successfully done 300 robotic joint replacement surgeries, which in a short period of time. I come now assuring that KIMS will continue to make its efforts to make healthcare more accessible and affordable with the quality care.

Thank you very much. over to Rahul.

Moderator: Sir, should we begin with the Q&A?

Abhinay Bollineni: Please Yes, we can.

Moderator: Thank you. We will now begin the question -and-answer session. First question is from the line of Ameya Chalke from JM Financial. Please go ahead.

Ameya Chalke: Congrats to the management with a good set of numbers. The first question I have is on the Telangana part of the business, where the occupied beds have come down during the quarter. However, the

revenues have grown sharply from INR309 crores to INR370 crore. Is there anything to allow in this? Is it risk which have changed so dramatically or from year -on-year basis? And is it structural in nature?

Abhinay Bollineni: No, I don't think there's any structural changes. It could -- it will only be a case mix change or an ALOS debt. If you look at the H1 last year and H1 this year is the ALOS has significantly come down.

Ameya Chalke: Okay. So got it. So we should see a partial seasonal impact in this basically?

Abhinay Bollineni: Correct.

Ameya Chalke: Got it. And the second question I have is how many months of operations for Nashik and Vizag have been taken into this quarter?

Abhinay Bollineni: Nashik, nothing, in fact investing INR10 lakhs. Vizag is 1 month revenue.

Ameya Chalke: Sure. And just one query. If we add the four revenues of four different segments of Telangana, Andhra and Sunshine and Nagpur, it comes a bit higher than the overall group. So is there any intersegment revenues which is canceling it or if there is some mismatch?

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Sachin Ashok Salvi: I mean you're right. These are on account of eliminations, which has to be adjusted in the Telangana cluster in revenue as well as EBITDA.

Ameya Chalke: Okay. And Vizag revenue will not be significant, what you mean to say during the quarter?

Sachin Ashok Salvi: Yes.

Ameya Chalke: Okay. And the last question I have is you have announced a 3,000 -bed ambition plan for Kerala cluster. Is it possible to give time line to achieve this target? How much of this will be achieved by organic or inorganic? And there are some established chain in this region in Kerala region? How do you see competitive intensive there? And is there sufficient scope for your hospital chain to come?

Sreenath Reddy: So, we are looking at the addition of bed of the 3,000 numbers over a period of time. It could be in the next 5 to 6 years. And where we are looking at it is, it's a mix of both asset -light model as well as Greenfield. Initially, it will be mostly asset light. The two new facilities that we have talked about, which Chairman had talked about both Kannuras well as Thrissur is asset -light.

Our focus is going to be because many of them are approaching us on asset -light models in Kerala. Mostly, it could be asset -light. But having said that, in the big cities that could be in Kochi and Calicut, we would like to go with Greenfield projects, but that is it. I think your second question was a lot of other groups are present. Yes, definitely, there are groups. But if you look it, these are all local groups. We don't have the international players. So we'll be one player, which will have a national presence.

And Kerala is one important state for us being the sole and because the management team and the doctors and others whom we have identified, they're all part of this earlier growth, and we are pretty confident that we will be able to get good results both from the growth as well as in terms of the EBITDA from this geography.

Ameya Chalke: Sure. So engines basically in Tier 2, Tier 3 cities, they will try to acquire or do the O&M kind of agreement with the existing hospitals. So that will not add new beds basically in those cities. Is that the right understanding?

Sreenath Reddy: Yes, that is right. In Tier 2, Tier 3, if you look at the smaller cities, it's mostly asset light, and the capex for that will be very, very minimal.

Abhinay Bollineni: And also this city will also not see new bed capacity addition because we are already physically present, and we are only changing the operator.

Amey Chalke: Okay. And overall capex plan, does it change because of this ambition? Or is it still the same?

Abhinay Bollineni:

No, I think we'll still continue to maintain the debt levels that we spoke about earlier. And at least the 2 projects that we spoke about are asset -light O&M contracts. So it really doesn't have much impact and doesn't change much of the capex plan.

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Amey Chalke: Thank you so much. I will join back the queue.

Moderator: Thank you. The next question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: My first question is on operating costs. So as like multiple new units will be coming into your network in next couple of quarters. how should we look at operating costs building up from here? And very broadly, what will be the broad impact on the operating margins? And extending it, since you have a big plan to extend into Kerala market where we understand operating costs are generally higher than, say, Bengaluru market, etcetera. So, over a long period of term, how should we look at the margins from current levels?

Abhinay Bollineni: Yes. I think one, the three new assets that came into play this year are Vizag, Kannur, Guntur and Nashik. And except for Nashik, the other three are running hospitals with some revenue. So, there will not be much EBITDA loss from these companies. In fact, the EBITDA loss would be as low as INR1 crores, INR2 crores because they're already on the verge of breakeven.

And as far as Nashik, we are still pretty confident within 12 months, we should be able to breakeven. And for the first 12 months, we could have an EBITDA loss of at best INR5 crores to INR10 crores. As far as the other hospitals that are coming up next financial year, which is

Bangalore and like you had earlier indicated INR10 crores, INR15 crores loss per asset is what we're looking at. So cumulatively, a INR30 crores , INR40 crores drag on an EBITDA of the current scale and size, I don't think will impact the margin significantly.

Damayanti Kerai: So from these assets, which you discussed annual drag of say INR30 crores to INR50 crores, not more than that cumulatively?

Abhinay Bollineni: The new ones that have only come up next year. The ones that have come up this year, we will not see any drag except for Nashik, which would be to the tune of INR10 crores.

Damayanti Kerai: Okay. And a slightly longer -term basis when you increase your presence in Kerala in that way like how should we look at the margins?

Abhinay Bollineni: So first of all, Kerala, we are pretty confident we should still be able to get to a 25% kind of a margin and that is what some of the best hospitals in that geography have been doing. So right now, we have these two facilities, we'll focus on these two and we're pretty constant these two will scale up to 25% margin in the course.

Damayanti Kerai: Okay. Good to hear that. And then I have a question on the promoter pledge. So earlier, I guess, you mentioned that there will be some reclassification of some promoter entities to public. So that should be bringing down the fledge part. But I guess September number doesn't reflect that. So how do you see that moving?

Bhaskar Rao Bollineni: Actually, we have been applied to that. So we asked some queries. We are answering those things definitely maybe next 1 or 2 months that could be done. And after doing that, personally from my side, there will not be any pledge.

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Damayanti Kerai: Okay. So yours will be completely taken off, right, once that ...?

Bhaskar Rao Bollineni: Correct.

Damayanti Kerai: Okay. And my last question is on margins in the AP cluster. I think we have seen notable improvement there. So obviously I guess, we've talked about case mix, etcetera. And this quarter, I understand there is seasonal benefit also. So where do you see

AP cluster margins settling in
very broadly?

Abhinay Bollineni: as we had always indicated AP, there are still actually 2 assets that are still in the less than 3% margin. But over a period of time, once those two and the remaining hospitals scale up because of the total beds in AP, 40% of the beds are still less than 5 years old. So, as they scale up, we will be able to get to a 27%, 28% kind of a margin there.

Damayanti Kerai: Ok that's helpful. Thank you very much.

Moderator: The next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: Good morning, everyone. Bhaskar sir, Abhinay, just wanted to understand your thought process in selecting Kerala as a focused market, especially given the plan to add 3,000 beds. Kerala is also a slightly different market with maybe higher cost, etcetera. So, I just wanted to understand whether this entry was it slightly opportunistic considering that we got a great team, your insights will be really helpful. And maybe I have a follow -up there, but maybe your response here and then I can ask that other question.

Bhaskar Rao Bollineni: So if you take the health care in India, Kerala is the one, which is matured, more literally and understanding the health care and the insurance penetration is more, and there will be a lot of procedures and every small type to also do very well. And there is a lot more gaps that are available in Kerala, there is a need. So, the team that has been totally understand the geography much better than us. So that's why we are mentioning mostly we are going with the asset -light model. And that's why we have selected that. Anything you want to add, Nitish on this ?

Nitish Shetty: Yes. Thank you, sir. I have vast experience in Kerala. You know my previous organization has a dominant presence in Kerala. What we have seen is in the last 10 years, Kerala previously has been a decent market as it was dominated by the government hospitals and charitable institutions.

Last 10 years and especially post COVID, there is a general acceptance of a large private hospital. And when I say large private hospitals at much more among the organized players.

My previous employer has leveraged on that opportunity and has grown well. But I see there is an opportunity for much more players to come in because there is a need in terms of the non-communicable disease, payment is very high. The requirement in Kerala is densely populated. The whole state is organized and then most important thing is the clinical talent available is very healthy.

Even in the Tier 2, Tier 3 cities, including that three super specialist in all specialty. Considering this opportunities, lack of reorganized players, large players in Kerala and the incidence of KIMS Hospital
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disease and also the acceptance of the general population for moving away from the government hospitals and trust hospitals to private hospitals, especially for tertiary care work.

Earlier patients from Kerala used to go out of Kerala state for tertiary care work. When I say tertiary care high-end work like neuro, cardiac, organ transplant, but they have accepted that now, so that can be delivered in the state itself. So KIMS comes with a vast experience in organ transplant, cardiac, neuro and all the cutting edging work and there was a very good high-end infrastructure models in other states, something seamless can be eliminated and add to that, KIMS has a unique model of partnering with the local talent, especially with doctors.

That model can further the growth of KIMS in Kerala. In a nutshell, I see a huge opportunity in Kerala. When it comes to other states within the metros and Tier 2, Tier 1, but in Kerala it is in Tier 2, Tier 3 and more and also in large cities as well.

Alankar Garude: Appreciate the response, sir. Maybe the context to ask this question was also, I mean we had till about 2 years, 3 years back, zeroed in on Maharashtra is the next AP, Telangana, equivalent for the company with 5 years, 10 years view. Now with I mean the focus on Kerala as well as, I mean, the additions coming up in Bangalore, have we curtailed our expansion plans for Maharashtra beyond plans which you've already announced?

Abhinay Bollineni: So, Alankar, I don't think the plans for Maharashtra will slow down because of this. We are -- whatever we have announced, we are anyways committed to. We will also bring a few more opportunities on board very well as far as Maharashtra is concerned. It is just said Kerala was a very unique opportunity. We have the right people, who understand the market totally. There were a lot of opportunities to do asset-light hospitals. It means the large state, where multiple asset-light opportunities in that state.

All we have to do is one or two Greenfield over a period of time. So it made very strategic sense as far as to consolidate our position in South with Andhra, Telangana, foray into Karnataka, Kerala, it makes us very dominant and strong South Indian operator, but that doesn't derail any plans that we have for Maharashtra or what you -- how you wanted to expand in Maharashtra where capital required for Kerala is significantly low.

Alankar Garude: Understood. So basically, Maharashtra that earlier plan of adding as many beds maybe more ask what you've announced in Kerala stays over the next say 5 years to 10 years?

Abhinay Bollineni: Correct. That continues. It just that Kerala we have formulized it with a [inaudible 29:20] we didn't do in any other states.

Alankar Garude: Okay. Fair enough. The second question is can you please take us through the progress in Sunshine and Nagpur? Sunshine was quite impressive this quarter. Nagpur saw healthy growth, but margins came up sequentially, if you can help explain the dynamics in both these markets?

Abhinay Bollineni: So as far as Sunshine, we're always pretty confident that it will do better than KIMS Telangana cluster in terms of its margin. And we were all -- it took a year, 1.5 years for us to show turnaround because we have to move from the current facility to the new facility. And right after KIMS Hospital
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it moved the revenue significantly improved, we were able to onboard good doctor talent. In fact, the new hospital is almost still it's a very high occupancy now you'll have to look at expanding that facility as well.

So I think now Sunshine has an aspiration to reach 30 -plus EBITDA margins. From now on, whatever incremental revenue, 40%, 50% will flow through to EBITDA. So we're pretty happy as far as that turnaround is concerned. As far as Nagpur again the revenues have stabilized. EBITDA margins are also healthy at 25%, between 25% to 30%. It's just that this quarter, we had a lot of renovation and onetime expenditure work.

So if you actually normalize the EBITDA, it would have been in the range of INR15 crores, INR16 crores which is around 25%, 26%. So we are, again, pretty confident Nagpur will continue to deliver a 30% EBITDA margin as the revenue ramp -up happens.

Alankar Garude: Understood. And maybe one final one clarification. When you said INR10 crores, INR15 crores EBITDA loss in Bangalore, does this include the second project as well?

Abhinay Bollineni: So I said per center we will incur a INR15 crores EBITDA loss for the first 12 months of operations.

Alankar Garude: Okay. So then, I mean, you also said INR30 crores to INR40 crores drag from the newer assets. So that doesn't include Nashik then. So two hospitals in Bangalore and one in Thane would...

Id...

Abhinay Bollineni: Yes, correct.

Alankar Garude: Okay. Fair enough. Thank you.

Moderator: Thank you. The next question is from the line of Bino Pathiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil: Good morning and congratulations for a great set of numbers. Most of my questions are asked. Hence just a couple of bookkeeping questions. In your presentation, the total number of beds for the -- at the end of the quarter is 4,610, but if I add up your four cluster beds, it comes to only 4,275. There is a difference of 335 of which I guess Kerala is 200. Could you explain the remaining difference, please?

Abhinay Bollineni: Sachin, Rudra, could you?

Rudra: So in the -- if you see in the group, if you add these four clusters, it won't total up to the group because the Nashik beds have been added which we have mentioned in the down group and 10 beds of Dr. Meda, the Meda Institute of Podiatry that we have taken that has been added into the group. So from the -- what we thought is from the next quarter, because Nashik didn't have the significant revenue from the next quarter, we will add up into the Maharashtra cluster, we will create a cluster, and we will show those things.

Bino Pathiparampil: Understood. So is the 200 bed in Kerala part of this 4,610?
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Rudra: No, not in this quarter because it started from October.

Bino Pathiparampil: Okay. That will also get added next quarter. And for Guntur and Kerala which are like O&M, what would be the accounting treatment? Would you be fully consolidating the numbers or would you recognize only the 5% income -- revenue share income as income?

Sachin Salvi: We'll be recognizing the...

Abhinay Bollineni: Go ahead Sachin. Sachin, go ahead.

Sachin Salvi: We'll be recognizing on the 5% revenue share since we are not having any equity in that company. So we'll not be doing any consolidation.

Bino Pathiparampil: Okay. So the revenue numbers which you -- the segment cluster -wise revenue numbers you report will not have their revenue?

Sachin Salvi: Yes.

Bino Pathiparampil: Okay. Understood. And that is for Guntur as well as for Kurnool?

Sachin Salvi: Wherever we have.

Abhinay Bollineni: Guntur and Thrissur. Kurnool, we report the 100% revenue and EBITDA. There's a lease rental that goes out from that P&L for Kurnool. For Thrissur and Guntur, we only report 5% as the operating income.

Bino Pathiparampil: Understood. Thank you. And one last....

Sreenath Reddy: I would like to clarify, I think there was one question earlier, I think from JM or some, I'm not able to recollect. The thing is that -- see when we are talking about asset light, at least in Kerala we are talking about all the bids coming into our control and the revenues, profit or loss is going to be to our account. So that is the asset -light and management model that we are talking about in Kerala. So just a clarification.

Bino Pathiparampil: Okay. And can I just get an estimate of the full year capex for this year?

Abhinay Bollineni: We don't have the numbers offhand, but can we share that -- it's part of the presentation and we can share that with you separately.

Bino Pathiparampil: Sure. Okay. Thank you.

Moderator: Thank you. The next question is from the line of Ankush Mahajan from Axis Securities. Please go ahead.

Ankush Mahajan: Congrats for a good set of numbers. Sir, my first question is, if we see from the last four quarters, earlier, we have a -- on a consolidated basis, we have an ARPOB in the range of INR30,000, KIMS Hospital
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INR31,000. Now it is in the range of INR38,000, INR40,000. So what are the reasons behind it that the ARPOB has increased?

First one. And the second one, if you see across the industry for second quarter, the major growth is driven by the increase in occupancies, but in our cases the growth is majorly increased by the ARPOB growth and occupancies has come down. So would you throw some light on it?

Abhinay Bollineni: That's something we should be happy about. It's not a seasonal impact, and it's an impact that it's going to sustain for the coming quarters as well. But as far as the ARPOB growth, I think as you indicated earlier, there has been some impact on price revision from GIPSA and some insurance renewals. And most of it is also because the ramp -up in Sunshine, ramp -up in Nagpur is coming largely from quaternary and tertiary care. So the ARPOB contribution there is also a little higher.

Ankush Mahajan: And sir, last one is what is the margin guidance for the full year, EBITDA margins?

Sachin Salvi: I think what we have delivered for H1 will continue to...

Abhinay Bollineni: Something that we'll deliver for H2 as well 27% to 28%.

Ankush Mahajan: Thanks sir.

Moderator: Thank you. The next question is from the line of Rahul Jeewani from IIFL Securities. Please go ahead.

Rahul Jeewani: So sir, on these two assets, Thrissur and Guntur where we would be getting just 5% revenue share, can you talk about your thought process in terms of taking on these assets? Because typically, in O&M model at least for some of the other O&M models, which we have been having we give out 5% rental revenue share, whereas for these two assets because the economics would just be 5% revenue share for us, do you think that given the economics, it made sense for us to take on these assets?

Abhinay Bollineni: So, Rahul, the plan is given the current debt situation in the company and we didn't want to

exceed the debt, net debt to EBITDA is 1 is to 2 and we found some opportunities and we were able to structure some of these opportunities in such a way that the promoters were okay that we take full control of running the hospital and scaling up and ramping up the hospital.

But eventually, maybe in a 4 year, 5 year time line or a 3 year to 5 year timeline, we can bring these assets back into KIMS, either through a merger or through infusing 51% primary and getting to 51%, 76%.

Rahul Jeewani: Sure, Dr. Abhinay. So would we have first right of refusal for acquiring these assets?

Abhinay Bollineni: Correct. In fact, we'll have an option where we can exercise it at the end of -- at our discretion, at the discretion of KIMS between the third to fifth year or even later, where we can bring this
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under KIMS. So when the debt situation is better and we're able to buy out these assets, we'll start buying off these assets.

Rahul Jeewani: Sure, Dr. Abhinay. And have we had any, let's say, agreement in terms of the multiples which we would be paying for these assets going forward once we try to acquire them?

Abhinay Bollineni: It will be on par with -- in any Tier 2, Tier 3 markets, it will be less than a single -- I mean less than sub -10 kind of a number.

Rahul Jeewani: On EBITDA?

Abhinay Bollineni: On EBITDA.

Rahul Jeewani: Sure. And the second question which I had Dr. Abhinay was on this expansion plan. Now the expansion plan which the PPT talks about, there our spend is going to be around INR1,600 crores. but out of that INR1,600 crores Thane and the two Bangalore assets were there which would now soon get commissioned in fourth quarter. So of the INR1,600 crores capex, how much have we already incurred so far? And then with, let's say, the expansion which we are targeting in Kerala and the existing units what kind of a capex spend do you expect in '2

6 and
'27?

Abhinay Bollineni: So Kerala, as far as Kannur is concerned we have already spent whatever you're seeing in the current debt, it also includes the debt for Kannur. So that is already taken care of. As far as Thrissur is concerned, the spend will not be more than INR30 crores, INR40 from the KIMS side for its scope of work. As far as the expansion plan of 1,600 beds, I'm not sure if I have the right number, but I think to the tune of INR800 crores, INR900 crores spend has already been done and is already factored in currently, but we can check the exact numbers and come back to you, Rahul.

Rahul Jeewani: Sure. So of the INR1,600 crores you are saying INR800 crores, INR900 crores has already been incurred?

Abhinay Bollineni: Yes, or maybe a little less than that, Rudra do you have the numbers off hand?

Sachin Salvi: Yes. So Rahul, we have already incurred to the tune of INR800 -odd crores on these projects, Bangalore and Thane which are in pipeline. Nashik is already done and that stood at on 30th September 2024.

Rahul Jeewani: Okay. So the remaining spend would essentially be for our existing units which is, let's say, Kondapur, Anantapur, Srikakulam and Ongole?

Sachin Salvi: Yes.

Rahul Jeewani: Sure, sir. And sir, can you update us in terms of the Kondapur expansion, how that is progressing? So Kondapur, we were going to add 500 beds and 100 to 150 beds were supposed
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to get commissioned in -- or will get commissioned in first quarter of FY '27. So how has the progress been there?

Abhinay Bollineni: It's on track, Rahul. We may commission it maybe a quarter early as -- might be able to commission it a quarter early. But for now, things are on track for us to be able to commission in FY '21 -- quarter 1 FY '27.

Rahul Jeewani: Sure, Dr. Abhinay. And how many beds would you be commissioning in Phase 1 100 to 150?

Abhinay Bollineni: So given the traction and given how things are moving, I think an incremental 200, 250 beds is what we will commission. So we'll move the current 250 and commission an additional 250, so 500, 600 beds.

Rahul Jeewani: Okay. Sure. And that you are saying can get commissioned maybe a quarter earlier as well?

Abhinay Bollineni: Correct.

Rahul Jeewani: Sure sir. Thank you. I will join back the queue.

Moderator: Thank you. The next question is from the line of Anubhav Sahu from McPro Research. Please go ahead.

Anubhav Sahu: So I want to understand, since we have been exploring various types of hospital business models for the last few years, whether it's O&M or lease or doctor partnership. So for the inorganic initiatives, could you summarize what are the key parameters we are looking at while finalizing the target? And what payback period, peak debt equity, etc we are looking at? And if you can also define or redefine the catchment area we are looking at?

Abhinay Bollineni: Sorry, your first part of the question was not clear. Your voice is not very audible.

Anubhav Sahu: So I want to understand what are the key parameters we are looking at while finalizing a target for inorganic this thing. So I mean, maybe in terms of payback period we are looking at for the target and what peak debt to equity we are comfortable at the group level when we are looking at this kind of acquisition over the year?

Abhinay Bollineni: Yes. I think like we had mentioned in the past, I don't think that changes anything. We will continue to look at 350, 400 beds as the minimum size for us to set up a hospital. Anything that we don't see a visibility of INR175 crores, INR10 crores kind of an EBITDA potential we may not continue to look at. And even to begin with is just 150, 200 beds as long as there's oppo

rtunity

to scale up to 400, 500 beds or 350, 400 beds depending on which market we're operating in.

I think we are pretty comfortable as long as we have that line of visibility to scale up. As far as net debt to EBITDA, I think we are very comfortable to maintaining it at a net debt-to-EBITDA of 1 is to 2 at a console group level. And we should -- except for one or two quarters in between, we should be able to stick to that through the year.

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Anubhav Sahu: Okay. And in terms of ROI or payback period we're looking at while investing in those assets any timelines?

Abhinay Bollineni: 5 years, 7 years is what we typically look at, but because these are now larger format hospitals, we should take a much longer -term view on these hospitals because we're acquiring land for building hospital over the next 10 years' time. So we're building Phase 1 over it and then you build Phase 2. So you'll have to take a little longer view. As long as we are able to invest efficiently and the capital per bed is on par with the ARPOB per bed and the revenue growth is happening and EBITDA margins are happening. ROI should not be such a concerning factor.

Anubhav Sahu: Okay. And you may have mentioned before. Could you just remind me please how much spent we have already done this year and how much is left or how much is done for the rest of the year's and next year?

Sachin Salvi: So Anubhav I will try to answer this question. So as far as Bangalore is concerned, up to September '24, we have spent almost like INR260 -odd crores. For another Bangalore project,

we have spent about INR10 -odd crores. For Thane, we have spent about INR350 -odd crores. So Nashik, we have already spent about INR155 crores. So this is the total spend, which we have done up to September '24.

For the remaining part of the year for Bangalore both the assets since most of the medical equipment have to be deployed, we may have to spend up to about INR100 crores to INR125 crores for each asset. For Thane, we will need about INR125 crores for medical equipment. So this medical equipment spending will be only spending. Otherwise, civil spending, most of the things have been done. And in fact, for the medical equipment spending also we have paid some advances. So this is what the position as on 30th September '24.

Anubhav Sahu: And so what's lined up for next year in terms of...

Sachin Salvi: Sorry, your voice is.

Anubhav Sahu: Sorry for that. So in terms of capex, what is lined up for next year, next fiscal year?

Sachin Salvi: So next fiscal year, I think, as Abhinay has also said, most of the assets which we are doing are on a asset-light model. Most of the spending, which is required for the existing Bangalore and Thane assets would be done by the end of this financial year. So next year, about some INR500 crores or INR600 crores, Abhinay, correct me if I'm wrong.

Abhinay Bollineni: Yes, that's it. .

Anubhav Sahu: Okay. That's it. Thanks a lot. It was very helpful.

Moderator: Thank you. The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

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Tushar Manudhane: Sir, just on this Andhra Pradesh cluster, the occupancy sort of is lower year-over-year or even if I compare it with FY '24 number, while the beds are largely stable. So if you could just throw some light, maybe I'm not sure if you have already responded to this, but just to understand, how do we think of improving the occupancy in first place at the existing sites? And then subsequently, we've been going to invest at Anantapur and other centers within Andhra Pradesh. So if you could give some idea in terms of at least the places where we are a

adding significant
beds where the occupancy stands at those sites?

Abhinay Bollineni: So as far as Andhra Pradesh is concerned, there are some hospitals where the occupancy is still low and there are some hospitals, where the occupancy is above 90%. So Rajahmundry, Anantapur and such hospitals, the occupancy is very, very high. And that's why we are adding more bed capacity there. A place a Nellore, for example, Ongole, for example, where the occupancy there is still opportunity it is only at 30%, 40% -- 40%, 50%, we can still add -- the growth can happen.

But when we acquired these hospitals, they were a very large government hospital. And we knew it will take that much time for it to scale up. So there was -- and we're okay with it. So -- and also a part of the occupancy dip is because of acquiring Queen's NRI which came in, that could be one of the reasons why there is a dip.

Tushar Manudhane: Got it. And this INR90 crores to INR110 crores investment for Anantapur for 250 beds. So this is like lower than, let's say, typical run rate of INR1 crores, INR1.5 crores or INR2 crores per bed investment. So if you could just share why the investment per bed number looks lower as far as this investment is concerned?

Abhinay Bollineni: I think in that market, it's difficult for us to get beyond that. The ARPOB in that market is only around INR70 lakhs, INR80 lakhs per bed. So we'll have to play around within that ARPOB range.

Tushar Manudhane: Just on -- conceptually since you highlight both ARPOB as well as ARPP in the presentation. At least this quarter, it seems that ARPOB growth is quite 22%, while ARPP growth is just 9% to 10%. So I mean while...

Abhinay Bollineni: There's a dip in ALOS so that is also want to reflect there.

Tushar Manudhane: Understood. And lastly, for Thane project like and the other where the opex of -- as you highlighted in the earlier remarks of the opex drag of INR30 crores to INR40 crores, this is with the assumption of what kind of occupancy will be sort of end in 12 months of operation?

Abhinay Bollineni: Sorry, could you repeat the question, please?

Tushar Manudhane: Sir, in the earlier comment you highlighted like opex drag of INR30 crores to INR40 crores that would happen on account of addition of new hospitals including that of Thane. So those new centers like probably what kind of occupancy assumptions are we going by, let's say, first year of operation?

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Abhinay Bollineni: We're assuming a 30% kind of an occupancy on the total bed capacity 25% to 30%.

Tushar Manudhane: Understood. Got it. Thank you sir. That's it from my side.

Moderator: Thank you. The next question is from the line of Ameya Chalake from JM Financial. Please go ahead.

Ameya Chalake: Thank you for giving me an opportunity to ask follow up question. So first question I have is on the KIMS Cuddle. Abhinay is it possible to tell like how many hospitals currently have KIMS Cuddle and what are the plans in other hospitals when are we going to add that? And also, if we could give some color on the ARPOBs and the profitability of this KIMS cuddle unit?

Abhinay Bollineni: So, Ameya, we don't track that separately. It's part of the adult hospitals. It's just one more specialty within the hospital. So there is -- we don't look at the profitability of that department separately. But right now, it is operational in 7 hospitals. And as and when, like in Anantapur, we're adding extra -- add initial infrastructure to start mother and child. As and when the infrastructure gaps are fulfilled, we will keep adding mother and child in all specialties. As far as Bangalore, Thane and any other new project that's coming up, they're adding mother and child on day one.

Ameya Cha

lake: Okay. So -- but in terms of ARPOB and profitably, will it be more than the company average or how is it -- how should we look at it? Is it going to improve our ARPOB?

Abhinay Bollineni: ARPOB will be similar. ARPOB will be similar. Again, at a gross marg in level, it is because the consumable cost is significantly low, the gross margin is better than the adult specialty, but because we don't look at it beyond gross margin, we don't track it at an EBITDA level.

Ameya Chalake: Okay. Sure. The second question I have is on the Wipro agreement which we have done recently, INR700 crores agreement. How is it to help us operationally and how -- net-net, how much spend we have to do annually for this kind of agreement?

Abhinay Bollineni: So this is a contract that is spread over the next 3 years to 5 years. This takes into account the replacement of the current hospitals because most of them are 10 -year-plus old and a new hospital. So it just gives us leverage to negotiate better because you're doing a group order, but there is no -- there is no time pressure on this. We can execute this over the next 3 years to 5 years.

Ameya Chalake: Okay. But will it have any P&L impact or it's not going to change materially the existing lease, whatever we have agreement which we have?

Abhinay Bollineni: Correct. It has no impact on P&L. All of this is captured in the overall capital numbers that we're talking about and the debt situation that we've already spoken about.

Ameya Chalake: Okay sure. Thank you so much. I will join back. Thank you.

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Moderator: Thank you. The next question is from the line of Rahul Jeewani from IIFL Securities. Please go

ahead.

Rahul Jeewani: Sir, on the Kerala cluster, while we are targeting to add, let's say, 3,000 beds over the next 5 year to 6 year period and that would be dependent on M&A. But realistically speaking, let's say, over the next 3-year period, what kind of a bed capacity can be factored in for the Kerala cluster? So would it be closer to 1,000 or 1,500 beds over the next 3 years?

Abhinay Bollineni: At this point, Rahul we have two assets, one in Kannur and one in Thrissur. So these are certain projects and cumulatively will have 400 beds in Kannur over a period of time and 350 beds in Thrissur, so 750 beds. So nothing else we signed definitely -- definitely. We are exploring multiple opportunities in an asset -light model. And if it does happen, then we could potentially look at adding another 500 beds next year in an asset -light model. But again, like I said, it's still in discussion. There is no definitive agreements there.

Rahul Jeewani: Sure, Dr. Abhinay, let's say, for the, let's say, Kannur, Thrissur and another 500 beds next year. So we get to around 1,300, 1,400 beds. So for the 1,300, 1,400 beds, what would be your actual capex outflow would be very limited given the asset light O&M model?

Abhinay Bollineni: Correct. So for Kannur it was only INR40 crores, INR50 crores. And for Thrissur, it will be around INR20 crores, INR30 crores.

Rahul Jeewani: Okay. Sure. Thank you. That's it from my side.

Moderator: Thank you. As there are no further questions, I would now like to hand the conference over to the management for closing comments.

Bhaskar Rao Bollineni: If there is no more questions, then we will close.

Moderator: Sir, do you have any closing comments?

Bhaskar Rao Bollineni: Okay. See, what is happening now with all the question that you people been asked and you have some good concerns. Our motto to go whatever we have been identified in Maharashtra, we are not coming we are going to exclude the entire Maharashtra. And as well as these O&M contracts what happened there are

a few doctors who built and not able to run it. So they were looking for it. Then we have some constraint on our balance sheet, then usually we thought we'll take it on a O& M contract, so that we are able to understand the culture, build the human resources, legal teams and everything.

Whenever there is an opportunity, it is in our discussion we can be able to acquire those things. And as a vacation points out and Kerala is a good clinical talent. Mostly, there was looking mainly of the primary, secondary and early tertiary care and we are very expert in a quaternary and high-end tertiary care with a lot of oncology, more and more complex cases that are coming. That is the purpose we want to explore these opportunities without putting a lot more funds identifying it and discussion will be at our end to whether we should be able to acquire over a period of time.

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The second important thing wh at we have been negotiated with the [inaudible 58:58] where when we're able to give it to one company, having been selected many of the companies where we've been able to bring down the capex nearly INR150 crores to INR200 crores to financial cost. Otherwise, there will be another INR200 crores you need to incur when you want to independently look into the different, different companies.

Similarly by puttin g all these [inaudible 59:27] over a period of time and good training that we're able to give a lot more operational efficiency and at the same time, more comfort towards the patients. Today, when the -- if you're able to look into that next 2 years to 3 y ears, with the robotic -assisted surgeries and operational efficiencies here loss may still come down drastically. That's what the world is looking in healthcare. We are in the in that.

And we do being in health care, we do a lot of evidence based and a lot of research oriented. We like all these things and then we see that -- it will be good for the organization to grow very steadily without causing any burden on the balance sheet. And most of these O&M what we are talking about, there are some of the lo cal doctors and other people who want to practice. They

are going to build the land and building an entire thing. Our capex will be very, very minimal.

Over a period of time, our balance sheet strengthens. And if there is an excess cash, these are discussions that we've been able to keep increasing our share in those keeping acquisitions. Thank you very much for your patient present hearing and concerns and raised a good question that was able to enable the management to keep thinking and expanding the company. Thank you very much.

Moderator: Thank you. On behalf of IIFL Securities Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Call: Feb 2025

13th February, 2025

To,
The General Manager,
Department of Corporate services
Bombay Stock Exchange Ltd (BSE)
Phiroze Jheejheebhoy Towers,
Dalal Street,
Mumbai - 400 001.
Scrip Code - 543308
ISIN: INE967H01025 To,
The Manager,
Listing Department
National Stock Exchange of India Limited,
Exchange Plaza, 5th Floor,
Plot No.C/1, 'G' Block
Bandra - Kurla Complex
Mumbai - 400 051.
Symbol - KIMS
ISIN: INE967H01025

Dear Sir/ Madam,

Sub: Transcript of earnings conference call with Analyst / Investors.

In Continuation to our letter dated 31st January 2025, the Company organized a conference call with the Investors/ Analysts on Friday, 07th February 2025 at 10:30 AM (IST). A copy of the transcript of the conference call held with the Investors/ Analysts is enclosed herewith and the same has also been uploaded to the Company's Website at <https://www.kimshospitals.com/investors> > Disclosures under Regulation 46 of SEBI (LODR) Regulations, 2015 > Analysts Calls schedule, PPT & Transcripts > Transcripts, Audio & Video Recordings > FY 24-25.

Kindly take the same on record and display the same on the website of your exchange.

Thanking you,
Yours truly

For Krishna Institute of Medical Sciences Limited

Nagajayanthi J. R
Company Secretary & Compliance Officer

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"Krishna Institute of Medical Sciences Limited Q3
Earnings Conference Call"

February 07, 2025

MANAGEMENT : DR. BHASKAR RAO BOLLINENI – FOUNDER AND
MANAGING DIRECTOR – KIMS HOSPITAL
DR. ABHINAY BOLLINENI – EXECUTIVE DIRECTOR

AND CHIEF EXECUTIVE OFFICER - KIMS HOSPITAL
MR. SACHIN ASHOK SALVI – CHIEF FINANCIAL
OFFICER- KIMS HOSPITAL
DR. NITISH SHETTY – CHIEF EXECUTIVE OFFICER,
KIMS BANGALORE CLUSTER
MR. SREENATH REDDY, DIRECTOR – BUSINESS
STRATEGY AND M&A, OF KERALA AND
KARNATAKA CLUSTERS
MODERATOR : MR. RAHUL JEEWANI – IIFL CAPITAL

Krishna Institute of Medical Sciences Limited
February 07, 2025

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Moderator: Ladies and gentlemen, good day, and welcome to the KIMS Hospitals' Q3 FY '25 Earnings Conference Call, hosted by IIFL Capital.

As a reminder, all participant' lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Rahul Jeewani from IIFL Capital. Thank you, and over to you, sir.

Rahul Jeewani : Hi. Good morning, everyone. This is Rahul from IIFL. I welcome you all to the 3rd Quarter Earnings Conference Call of KIMS Hospitals being hosted by IIFL Capital. From KIMS we have with us today, Dr. Bhaskar Rao Bollineni – Founder and Managing Director; Dr. Abhinay Bollineni – Executive Director and CEO; Mr. Sachin Salvi – CFO; Dr. Nitish Shetty – CEO for KIMS Bangalore cluster; and Mr. Sreenath Reddy – Director, Business Strategy and M&A.

Over to you, sir, for your "Opening Comments".

Bhaskara Rao Bollineni : Good morning. I extend a warm welcome to all for the First Investors Meet of this year. Gross revenue of INR 790 crores, a growth of 29.7% year-on-year and 1% on quarter-on-quarter basis. EBITDA of INR 205 crores, a growth of 36.4% on year-on-year and a decline of 8.1% on quarter-on-quarter basis. EBITDA margin is at 25.9% versus 24.7% in Q3 FY '24, and 28.5% in Q2 FY '25.

PAT at INR 93 crores in Q3 FY '25 against INR 77 crore and INR 121 crore in Q3 FY '24 and Q2 FY '25 respectively. Consolidated EPS for 9 months of INR 7.1, face value of shares at INR 2, a growth of 15.6% adjusted for share split on year-on-year basis. Cash and cash equivalents include cash, bank balance, deposits with maturity less than 12 months, and investment in mutual funds at INR 112 crores as on Q3 FY '25.

Financial Highlights consolidated:

Consolidated revenue from operations grew by 27.5% on year-on-year, and it declined by 0.6% on quarter-on-quarter basis, INR 772 crores. Consolidated EBITDA pre IndAS grew by 34.7% on a year-on-year and declined by 10% on quarter-on-quarter basis to INR 198 crore.

Consolidated EBITDA pre-IndAS and excluding other income to total revenue margin stands at 23.3% growth of 25.4% year-on-year basis, and a decline of 16.2% quarter-on-quarter basis.

Operational highlights consolidated, average revenue per operating bed grew by 25.2% and 0.5% on year-on-year and quarter-on-quarter basis respectively. Average revenue per patient grew by 12.2% and 2.8% on year-on-year and quarter-on-quarter basis respectively. IP volume grew by 13.8% and declined by 3.1% on year-on-year and a quarter-on-quarter basis respectively.

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The other developments, we are happy to announce the opening of 200-bedded multispeciality hospital at Guntur on a O&M basis called KIMS-SIKHARA at Guntur, a very important town in Andhra Pradesh near the capital city of Amravati. Honorable Chief Minister of Andhra Pradesh, Sri Chandrababu Naidu Gara has kindly consented to inaugurate this hospital on 12th February of this month.

KIMS entered into an agreement with VIMS, Valiyath Institute of Medical Sciences Hospital in Karunagappally of Kollam district. It has total 300 beds. This is our second hospital in Kerala. The other 1 is at Kannur, opened recently, and is doing well.

Within 3 months, the Kannur Hospital has been breakeven, which is a landmark in Kerala. As you may be aware, Kerala occupies the first place in the percentage of people seeking hospitalization.

KIMS Hospital has now set a benchmark in neuro care. It now features South Asia's first MRI-guided focused ultrasound with Hero 3T MRI. It is proven to reduce the tremor-dominant Parki

nson's and essential tumors in patients. It gives benefits of without anesthesia, reduce the tremor totally, safe and effective, admission is not required, single session treatment. The first patient was a young maths teacher from rural parts of Maharashtra. He was suffering from hand tremor.

Moderator: Yes, sir. Please go ahead.

Bhaskara Rao Bollineni : On hearing about this, he came to us and underwent a noninvasive surgery, free procedure, and walked back tremor-free immediately. KIMS Foundation and Research Center has developed AI-based smart glasses for the visually impaired that will enhance their quality of life. The product was launched in a function based by Honorable Governor of Telangana, Sri Jishnu Dev Varma.

Recently, KIMS Secunderabad was awarded a "Nonclinical Operations Excellence Award" by Associations of Healthcare Providers of India. KIMS Kondapur also got the award for "Excellence in Nursing Practices" from AHPI. KIMS Cuddles, Secunderabad hosted a 2-day conference of Indian Association of Pediatric Surgeons of both Telugu states which had a record number of participants, was able to speak about the awareness -- Clinically, KIMS achieved another distinction by becoming the first private hospital in the country to have performed 100 robotic-assisted Whipple surgeries, a significant achievement in treatment of pancreatic cancer. You all know that we have been ordered about 25 robots, out of that 5 has been placed in the different institutions, and all of them are doing well. That is a good move. And it is going to improve the quality of the treatment.

At KIMS-Kingsway Nagpur robotic-assisted CABG was successfully done, which is first in Central India. This minimal invasive approach ensures faster recovery, reduced postoperative pain, shorter hospital stay and minimal scarring. KIMS Manavata Hospital, Nashik performed

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the first finger joint replacement, this is the first of such operation in Maharashtra and third in India. KIMS Secunderabad has done within no time about 200 robotic knee replacements. I have intentionally kept good and important news in the end. I'm glad to inform that a distinguished personality has joined our Board. He is Sri Suresh Patel, who held many prime positions in his illustrious career. He was CMD of erstwhile Andhra Bank, presently Union Bank of India. Later occupied 1 of the top positions in the country as CVC, that is Central Vigilance Commissioner. Our Board will be greatly enriched with his contributions.

And all the projects, new projects are going in time. The only thing is that the entire civil structures have been finished in Quarter 4. As we know that the problems what we faced in Nashik, after the civil works are finished to get the required permissions to start the hospital it takes a longer time, that may be we may be the end of the first quarter or the beginning of the second quarter we can able to commence with all these permissions, so that we can able to bring down the loss funding.

Usually, when we finish the civil structure, we will try to recruit all the doctors, and then it causes a loss running heavily. So, after getting all the permissions if we start and then start recruiting the doctors, we can able to minimize the loss running. So, that is the reason that we may able to start full-fledged in quarter 2 all the 3 projects in Thane and Bangalore, 2 hospitals, quarter 2 of '26, maybe early in Quarter 1.

Okay. Over to the audience for any questions, clarifications.

Moderator: Thank you very much. We will now begin the question-and-answer session. Anyone who wishes to ask a question may press "*" and 1 on the touchstone telephone. If you wish to remove yourself from the question queue, you may press "*" and 2. Participants are requested to use handsets while asking a question.

Ladies and gentlemen, we will wait for a moment while the

question queue assembles. Ladies

and gentlemen, you may press "*" and 1 to ask a question. A reminder to all participants, you may press "*" and 1 to ask a question.

The first question comes from the line of Anshul Agrawal from Emkay. Please go ahead.

Anshul Agrawal : Hi, thank you for the opportunity. So, my first question is on the Maharashtra cluster. Is there any one-off there? I see a sharp decline in the EBITDA margins here. And what could be the EBITDA loss in the Nashik unit?

Abhinay Bollineni : So, Maharashtra cluster now reports both Nashik and Nagpur. In Nagpur, there has been a decline in revenue owing to a seasonal impact. And hence, there has been a decline in EBITDA and some one-off, write-offs, some general dues and PBDD provisions. As far as Nashik is concerned, we have incurred a loss of INR 5 crores for the quarter. And that's why Maharashtra

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as a cluster, the EBITDA margins have significantly declined. But that should get rectified by

Q2, Q3, given that Nashik will break even by then.

Anshul Agrawal : So, Nashik is on course to breakeven within 1 year of commencement. Would that be correct?

Abhinay Bollineni : Correct.

Anshul Agrawal : Thank you. That's it from my end.

Moderator: Thank you. Ladies and gentlemen, to ask a question, please press "*" and 1 on a touchstone telephone. The next question comes from the line of Abdulkader Puranwala from ICICI Securities. Please go ahead.

Abdulkader Puranwala : Hi, sir. Thank you for the opportunity. Sir, my first question is with relation to your corporate overheads. So, if we see for the current quarter because of the fresh hiring, I think the corporate overheads have gone up. So, could you please provide some colour as to how do we see this cost? Is there any one-off element also included in this? And how should that run for the next coming years?

Abhinay Bollineni : Sorry, could you repeat the question again, please? It's not clear.

Abdulkader Puranwala : Sure, sir. Sir, my question was pertaining to the corporate overheads. I think the corporate overheads in this particular quarter as a percentage of your revenue has gone up. Partly my understanding is because of the recruitments what we have done in terms of your various clusters. So, if you could provide us some colour as to how this would pan out in the next couple of years it would be helpful.

Abhinay Bollineni : So, I don't think we have classified corporate overheads in any part of the presentation. There has been a decline in EBITDA margin, because of the decline in revenue from mature units owing to seasonal impact from Q2 versus Q3. But Q3 and Q3, there has been significant growth both in revenue and EBITDA. And there has been a one-time expenditure towards the tune of INR 8 crores to INR 9 crores, largely due because of provisions of PBDD and general dues write-off. Other than this, there has not been any expansion in corporate expenditure. And I don't know where from this has been picked up, but it's not mentioned anywhere in the presentation.

Sachin Ashok Salvi : And as far as corporate overheads on account of new hiring, in particular in Bangalore cluster, as such we have not started yet the Bangalore cluster. So, it is not coming to a profit and loss account directly. It's a preoperative kind of a thing. So, there is no surge in corporate overheads as such. Understand, if we start the newer cluster, I think it would not be there.

Abdulkader Puranwala : No, I was referring to, if I look at your operating margins across the cluster, I think it is close to 26.5% as per the presentation. While on a reported basis, the EBITDA margin was close to 24%. Second is on the Bangalore project. So, any reason why the time line for project 2 has shifted to, say, from Q4 of fiscal '25 to Q2 FY '26?

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Abhina

y Bollineni : The second project in Bangalore, there has been some delay in execution, and hence the delay.

Abdulkader Puranwala : Okay, sir. More questions but I will get back in the queue.

Moderator: Thank you. A reminder to all participants, you may press "*" and 1 to ask a question. The next question comes from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Mohammed : Good morning. Thanks for the opportunity. So, when I look at Telangana and AP clusters, not just for this quarter but for the entire FY '25, there's been a strong ARPOB growth, and you called out ALOS reduction as the primary reason for that. So, if I look ahead, maybe into FY '26 and '27, how should we think about ARPOB growth in these 2 regions for us?

Abhinay Bollineni : So, Harit, both in Telangana and in Andhra for the first 9 months, there has been a significant growth in both volume, IP and OP volume as well as ARPP in absolute number. So, I think that will continue to do. That will continue to grow at the same growth rate. And there will be more efforts in trying to reduce the ALOS from the current 3.5 to probably 3.3, 3.4. And that impact on the ARPOB will reflect.

Harith Mohammed : Okay. And the other income that you reported for this quarter, it's a bit higher than the usual run rate. What exactly is leading to this?

Abhinay Bollineni : So, there is the income from sale of land at Chennai of about INR 12.5 crores, which is sitting in other income in this quarter. Otherwise, it is normal.

Harith Mohammed : Okay. Last one, this time we have clubbed Sunshine with our Telangana cluster. So, if you can comment a bit about the performance there, how the quarter-on-quarter revenue ramp-up has been, as well as some colour on the margin profile there?

Abhinay Bollineni : So, Sunshine ramp-up is in line with the rest of Telangana, Harit. There has been a decline in revenue of INR 8 crores, and proportionately the EBITDA decline of around INR 6.5 crores in Sunshine. The occupancies are holding up. The Begumpet facility is doing quite well. So, we are pretty happy with the progress there and how things are ramping up. So, the reported number for Sunshine this quarter on revenue has been INR 150 crores versus INR 158 crores of Q2. And EBITDA has been INR 40 crores versus INR 48 crores of Q2.

Harith Mohammed : That's very helpful. I'll get back in the queue. Thanks.

Moderator: Thank you. A reminder to all participants, you may press "*" and 1 to ask a question. Ladies and

gentlemen, you may press “*” and 1 to ask a question.

The next question comes from the line of Arunachalam, who is a freelancer.

Arunachalam V : Very, very good morning. In fact, I'm very happy with the organic growth, both in the revenues as well as in the bottom line. Sir, what is actually our flagship? Like what would be the
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percentage of, in all the specialties, like whether it's cardiac or neuro or gastro or what, or onco, or it's equal?

Abhinay Bollineni: So, it's a fragmented split, the specialties. Cardiac is around 17%, 18%. Rest of the specialties like renal, neuro, orthopedics, oncology, gastro are around 9% to 10%. So, it's quite fragmented across all specialties.

Arunachalam V : What I wanted to know is how does somebody perceive KIMS? Do they see you as a multi-specialty or do they see you as a cardiac or a neuro or a onco? That's why I wanted to know.

Particularly you're also into medical tourism, that's why, like international patients.

Abhinay Bollineni: Given that the specialty mix is quite fragmented, I think the perception is that we are a multi-specialty hospital.

Arunachalam V : Thank you very much.

Moderator: The next question comes from the line of Amey Chalke from JM Financial. Please go ahead.

Amey Chalke: Yes. Thanks for taking my questions. Congrats to the management on a good set of numbers. The first question I have

is on Kerala geography. So, far we have announced 3 units. 1, we have already added, Kannur. There are other 2 Kollam and Thrissur. So, what would be the time line for these Kerala units? And if you can also provide some colour on how would the mix, like would these be more secondary care kind of hospitals or there would be tertiary care as well? And what could be the rough ARPOB range for these units?

Abhinay Bollineni: So, right now, we have Kannur, which is fully operational. We are planning to add another 50 beds by end of this financial year in the same campus. And maybe another 2 years from now, we should be able to add another 100, 150 beds in the Kannur facility, taking up to 350, 400 beds. The intent for Kerala as a cluster is to have all specialties and all of them will be doing tertiary, quaternary care work. So, in Kannur, we will soon start kidney transplant. With the expansion for 100, 150 beds, we are factoring in oncology also with LINAC and all the other services that are required. So, all the hospitals in Kerala will be focused on tertiary and quaternary care. Kollam, which should commence operations from April onwards.

Moderator: Sorry to interrupt. Ladies and gentlemen, please stay connected while we reconnect the management line. Ladies and gentlemen, we have the management line reconnected. Sir, please go ahead.

Abhinay Bollineni: So, I spoke about Kannur. As far as Kollam is concerned, we are renovating the facility, we rebranded by the first week of April. That again will be a hospital that will service tertiary and quaternary care. We don't have oncology at the moment, but we may eventually look at adding oncology once there's an initial ramp-up. Thrissur should take another 12 to 18 months -- 12 to 15 months before we start operation. In Phase 1, we don't intend to have oncology there, but we intend to do transplant as a service, both liver and kidney. And as the hospital ramps up, we will
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add oncology as a service there. But overall, the idea is to have oncology and all the other specialties in all the hospitals in Kerala. We might just stagger it and do it in phases.

Amey Chalke: Sure. And the second question I have is on the Queen's NRI. Since the acquisition, how has this unit fared? And if you can give some colour on the same on the operational front of this unit? Whatever improvements have happened so far? And what would be now the profitability? Has it improved since we acquired?

Abhinay Bollineni: So, Queen's NRI is a very old facility. It's a 20-year-old facility. So, a lot of the renovation works are happening at this point in time. And usually, Northern Andhra, December, Jan is a very weak season, even among our other hospitals in North Andhra. And most of the doctors have agreed to join only post-Sankranti. So, as we speak, a lot of the onboarding is happening. By the end of Feb, a lot of the doctors will be fully onboarded. And part of the renovation works also should be completed. So, right now, we are not losing any money. It is EBITDA-neutral. But there will be a good ramp-up in FY '26.

Amey Chalke: What would be the occupancy for this unit currently?

Abhinay Bollineni: It will be around 35%, 40%.

Amey Chalke: Okay. Got it. Thank you so much.

Moderator: Thank you. A reminder to all participants, you may press “*” and 1 to ask a question. The next question comes from the line of Rahul Jeewani from IIFL Capital. Please go ahead.

Rahul Jeewani: Sir, on the Nashik Hospital you talked about EBITDA breakeven in 2Q, 3Q of next year. But can you also talk about how has the doctor onboarding progressed in that hospital? And do we have the requisite doctor talent to allow us to break even in, let's say, 2Q, 3Q of next year?

Abhinay Bollineni: Yes. So, I think all the doctors have been identified, Rahul, and joining letters have been issued to all of them. In the month

of Jan, the occupancy has been around 25 beds, so that's around 10% occupancy and revenue of INR 3.5 crores to INR 4 crores in the month of Jan. We are yet to onboard 50% of the doctors. And the dates given are mostly Feb and March. So, hopeful that by March end, 70%, 80% of the doctor onboarding would have been completed, hence 2 quarters from there.

Rahul Jeewani: Sure, sir. And in terms of specialties, are you offering all the specialties at Nashik right now?

Abhinay Bollineni: Yes, except for oncology, we are offering all specialty.

Rahul Jeewani: Okay. Sure, sir. And sir, you called out this INR 8 crores to INR 9 crores of onetime, let's say, write-off on advances. So, this was across clusters or was this in the Telangana and AP cluster?

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Abhinay Bollineni: It was almost INR 3 crores in Nagpur alone, INR 3.5 crores in Nagpur alone, and the remaining is in Telangana and Andhra.

Rahul Jeewani: Sure, sir. And we had these write-offs in Nagpur last quarter as well. So, are we done with these write-offs? Or do you expect this to continue, let's say, going forward?

Sachin Ashok Salvi: So, in Nagpur, before we took over, there were a lot of advances which were given and we haven't obtained any services or the vendors have not provided any goods against those particular advances. So, we have taken a write-off against these advances in the last quarter and this quarter. Most part of it is done. In Q4 of this financial year, you will see about some INR 50 lakh, INR 1 crore kind of write-off. But over and above that, I think most part of it is done in Nagpur.

Rahul Jeewani: Okay. So, you mean that another INR 2 crores to INR 3 crores of write-off might come in the next quarter as well?

Abhinay Bollineni: Not INR 2 crores to INR 3 crores. I'm saying INR 50 lakhs to INR 1 crore.

Rahul Jeewani: Okay. Sorry, I missed that number. Sure. And 1 question on the Thane micro market. Now one of our peers is already in that market, and we would be coming up with a hospital 1Q, 2Q of next year, while 1 another larger peer has also announced their plans to enter the Thane market. So, how do you see the competitive intensity in that market shaping out to be, let's say, over the course of the next 4 to 5 years? Do you think that the competitive, let's say, intensity in that market would worsen? Or you think that there is enough demand for, let's say, 3 or 4 large players to coexist in that micro market?

Abhinay Bollineni: It just reinforces our thought process that there is a lot of opportunity in Thane. And given that Jupiter announced its second hospital in Thane is even more reassuring. I think there is an opportunity for all of us to coexist. And the good part is there is a good 4, 5-year window before these hospitals get commissioned. So, it gives us a good runway to get ourselves established in those micro markets.

Rahul Jeewani: And sir, in terms of doctor, let's say, ability to attract doctors in Thane, that I think should not be much of a problem. Is that correct?

Abhinay Bollineni: Correct.

Rahul Jeewani: Okay, sir. And sir, last question from my end. So, Nashik is expected to break even next year, but what kind of loss funding you are expecting from Thane and the 2 Bangalore hospitals in '26 and '27? Thank you.

Abhinay Bollineni: So, I don't think we should have loss funding for '27. But for '26 for each of these facilities, we have factored around INR 10 crores to INR 15 crores.

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Rahul Jeewani: On each. So, the total could be around INR 30 crores to INR 45 crores.

Abhinay Bollineni: Correct.

Rahul Jeewani: Okay. That's it from my end.

Moderator: A reminder to all participants, you may press "*" and 1 to ask a question. The next question comes from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Ahame

d: So, on the Kollam unit, which we have acquired, can you talk a bit about this hospital. So, my question was on Kollam Hospital. Just looking for some colour on the history of this hospital, whether it's already operational, how long has been, how many years this has been operational? And if you can share something about the unit level economic shares in terms of revenues and

whether it's profitable at this point?

Abhinay Bollineni: Harith, are you referring to Kollam?

Harith Ahamed: Yes, yes, Kollam, yes.

Abhinay Bollineni: Yes. So, Kollam is a running hospital. It's been operational for the last 4 to 5 years. It's very similar to when we acquired Kannur, around INR 4 crores to INR 5 crores of revenue. I think here it is around INR 3 crores to INR 4 crores of revenue. I think on the operational breakeven, it should be similar to what we have experienced in Kannur at around INR 8 crores, INR 8.5 crores, we should be EBITDA neutral.

Harith Ahamed: Okay. Okay. Got it. And in Kerala and Bangalore, 2 markets where you've stepped up your focus, can you talk a bit about the bed addition plans, what's the pipeline looking like? And which are the cities that you'll probably be targeting? And then specifically, if you can comment on your plans for Kochi, which is a large market in the state of Kerala, if you have any plans to enter that market?

Abhinay Bollineni: Yes. I think like we have mentioned in the past, both Karnataka and Kerala are very, very important for us. In Karnataka the plan should be that we scale up to at least 2,000 beds in Bangalore over a period of time. At this point, we have 2 hospitals that will get commissioned in this financial year. And then we have a land for a third facility, which we will start work maybe in the next financial year, and then it might take 3 years from there.

But as far as Kerala is concerned, the idea is to again scale up to around 2,500 to 3,000 beds over a period of time. The cities, obviously the focus is to be in the larger cities like Kochi, Calicut, Trivandrum. But at this point in time, we have no visibility. There are a good number of transactions that we are looking at, both acquisition in greenfield and semi Brownfield. But at this point in time, we have 3 assets which will add up to 1,000 beds, which is Kollam, Thrissur and Kannur. As and when we zero down on more assets in Kochi and Calicut, we will have

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more.... But the intent is to do a large facility in Kochi, close to around 1,000 beds over a period of time.

Harith Ahamed: And on Karnataka, if you can talk a bit about your plans there?

Abhinay Bollineni: Like we said, Karnataka, we already will commission 2 this year. We will also start a hospital in Sarjapur soon. Maybe we should start construction from early next year. And then 3 years from there, that should get commissioned. We are looking at 2 more opportunities in the northern part of Karnataka. Nothing concrete at this point in time. But as and when we do, we can potentially add around 750 -1000 beds in the northern part of Bangalore. We are largely present today in the South part of Bangalore. We are looking to add around 750-1000 beds in North Bangalore. But that will take time. It will take at least 3 to 4 years before we start it.

Harith Ahamed: Okay. That's helpful. And last 1, when I look at the Maharashtra cluster ARPOB for this quarter, there's been a decline quarter-on-quarter, and I assume it's coming from the Nashik Hospital.

But if I think of, let's say, at maturity, what kind of ARPOB should we expect for Nashik? Will it be in line with Nagpur or slightly higher? If you can give some color, that will be helpful.

Abhinay Bollineni: It will be in line with Nagpur, around INR 32,000, INR 33,000. Right now, it's around INR 24,000, INR 25,000. Nagpur continues to be at INR 34,000, INR 35,000. So, a blended number came out to be a

round INR 28,000, INR 29,000. But I think once it matures or towards the end of the first 12 months, we should get to INR 30,000, INR 33,000 kind of ARPOB.

Harith Ahamed: Okay. That's helpful. Thank you very much.

Moderator: Ladies and gentlemen, you may press "*" and 1 to ask a question. The next question comes from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane: Sir, firstly, on this ARPP with respect to Andhra Pradesh, seen significant scale up. So, is it to do with the case mix or payer mix?

Abhinay Bollineni: It's a mix of both, case mix and payer mix.

Tushar Manudhane: In the sense which particular case as in therapy has been driving this? And how sustainable that is?

Abhinay Bollineni: There is a price hike because of insurance renewals. There has been a case mix change. And also, Queen's NRI is largely driven by cash and insurance as a payer and very little scheme work.

So, all these 3 factors pulled the ARPP up.

Tushar Manudhane: Understood, sir. And as far as the expansion plan goes, while we've been expanding in, let's say, Bangalore as well as the other areas. But like in Telangana itself, which has been the profitable territory for us, is there any scope for further expansion out there?

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Abhinay Bollineni: Yes. Even in Telangana, I think we have a lot of opportunity still. I think Sunshine is still only

at 60% occupancy. There is an opportunity for us to ramp up beds there. Ramp up occupancy there and increase, expand margin. So, I think eventually Sunshine's margin expansion will be in the tune of 30%, 33%. So, there is 1 opportunity there. Kondapur, we are adding incremental 500 beds, which should be ready by the next financial year. So, that will be a big pull for us in terms of revenue and EBITDA. And yes, these 4 assets for now and then we are doing O&M hospital in a place called Kompally, which is a 250-bedded hospital, which also should get commissioned by the end of this financial year.

Tushar Manudhane: Got it. And I just missed this, the overall operational cost that would come up because of these new hospitals which are expected to commence in FY '26, if you could just repeat that number?

Abhinay Bollineni: Overall operational, could you repeat the last part?

Tushar Manudhane: Overall addition in the operational cost because of commencement of new hospitals.

Abhinay Bollineni: Yes. So, since we are reporting them on a cluster-on-cluster basis, Telangana, Andhra, we are not seeing much incremental costs happening because we are all mature. Maharashtra, we said Nashik will be around INR 10 crores to INR 15 crores of operating losses. Kerala, fortunately, we have broken even. And Thane as well as Bangalore, we should see INR 10 crores to INR 15 crore drag this financial year for each of the assets.

Sachin Ashok Salvi : And for the Kompally asset, as such we are doing an O&M model. So, operational cost comes on our book.... Sir, as far as Kompally asset is concerned, since it is an O&M model, by the structure of the transaction itself, no operational cost comes on our books. We are only entitled to some percentage of revenue that would be accounted in the income. So, there would not be any material operational cost on account of that in our books.

Tushar Manudhane: That's it from my side. Thank you.

Moderator: Thank you. A reminder to all participants, you may press "*" and 1 to ask a question. The next question comes from the line of Arunachalam, who is a freelancer. Please go ahead.

Arunachalam: Good morning, again. You plan massive expansions in Kerala. So, just 1 question is, the brand name, does it not cause any sort of confusion amongst the public, because there's 1 more hospital there? Or how does it work, sir?

Abhinay Bollineni : Are you referring to the other KIMS in Kerala?

Arunachalam: Yes, yes, yes

, correct, sir. Correct, sir. Correct, sir.

Bhaskara Rao Bollineni : See, basically it is Kims Health City, isn't it?

Arunachalam: Correct. Correct, sir. Correct.

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Bhaskara Rao Bollineni : And we are putting a co-branding as of now, KIMS Sreechand and KIMS Valiyath. And then Thrissur also. By the time it is coming to operational, then our brand also will be able to come to the notice of the public.

Arunachalam: Okay. Okay. That's I just wanted to know if there is any plan for rebranding it or something like that? That's the question.

Bhaskara Rao Bollineni : Already keep adding the existing hospitals. And when we do standalone, we will look into that, whether we should be able to rebrand or we will continue by the time if brand has been penetrated well in the public.

Arunachalam: Thank you, sir. I wish you all the best for all the markets. Thank you.

Moderator: Thank you. Ladies and gentlemen, you may press "*" and 1 to ask a question. The next question comes from the line of Parvati Rai from Equentis Wealth Advisory. Please go ahead.

Parvati Rai : So, I wanted to understand, given these expansion plans and while last quarter you did mention that debt to EBITDA would be around 2x in the next couple of years as we take the expansion.

Sir, interest expense was quite high again this quarter. So, going forward, as we move into FY '26, what is the kind of debt and the interest that 1 should factor in or assume, because for the last 2 quarters, it's inching up. And also, on the CAPEX for the next year, is it still at around INR 500 crores or INR 600 crores guidance or given the expansion plans there's a change that we are looking at?

Sachin Ashok Salvi : So, as far as outstanding debts are concerned, as on 31st December 2024 it is roughly INR 1,550 crores. Why interest expense, which has been debited to the profit and loss account in this quarter increased mainly because Nashik assets we have capitalized and all the borrowing cost on that asset gets factored into interest expenses from this quarter. As far as the guidance as to the CAPEX, nothing changes. It remains the same. Already, we have announced, and we are on track.

And we have already got a sanction for most of our facilities, and the expenses which we are incurring are in line with our budgets, which we have estimated for these projects. I think in couple of calls earlier also we have mentioned the outstanding debts would be somewhere in the range of INR 1,750 crores kind of a range, which will be still in the range of below 1:1 debt equity and below 2:1 in terms of debt to EBITDA. That will continue. Certainly, once all these hospitals get matured, this debt equity and debt EBITDA percentage will come down in the

subsequent financial year.

Parvati Rai : Okay. That's it from my side. Thank you.

Moderator: Thank you. The next question comes from the line of Alankar Garude from Kotak Institutional Equities. Please go ahead.

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Alankar Garude : Thank you for the opportunity. First question, what was the minority share of EBITDA in the 3rd Quarter? And maybe if you can give the number for 9 months as well?

Sachin Ashok Salvi: So, it is 9.9% in Q3. And if you're asking about 9 months, it would be somewhere in the range of 10% to 11% in the same range it would be, not more than that.

Alankar Garude : And any outlook, sir, which you can share, given so many new hospitals coming up in FY '26. Any outlook as far as FY '26 is concerned?

Sachin Ashok Salvi: Outlook as to minority interest, you are saying?

Alankar Garude : Yes, yes. How many of these hospitals which are coming up have a doctor equity structure? And accordingly, how do you expect the minority share to play out, because some of these hospitals will also have losses. So, do you expect a major change versus the

10% number or it will be

broadly in the same range?

Abhinay Bollineni: The only incremental thing will be the Karnataka cluster, where we have 20% minority towards the Management team. But otherwise, Thane is 100% owned by the company. Kondapur is anyways part of the current hospital. So, we don't see any other minority adjustments.

Alankar Garude : Got it. And maybe the other question is related to the expansion. We have been announcing new deals every few months. Now while we have been adding beds over the last 2 decades, the pace of additions as well as the expansions being announced is currently much higher than what we have seen in the past. So, just trying to understand what are the key learnings so far? And on the operational side, which are the areas you believe we need to invest in to manage the execution smoothly?

Abhinay Bollineni: Yes. Obviously, the size of the Company has also increased. So, that allows us to do more transactions. That is number 1. As far as where we would like to strengthen, I think on the project delivery, there is scope for us to improve further. We'd like to strengthen that, and we are trying to build teams to strengthen that because we want to do a lot more assets in the future. So, we need to have a consistent team that we should be able to achieve on that.

And also, 1 of the big reasons why we are doing this bed addition is if you see, there is a lot more opportunity in AP and Telangana. We have a very strong team. We are on ground. So, it makes sense to keep investing more and more to expand this opportunity.

And thanks to Sreenath and Nitish in Karnataka, Farhaan and team in Kerala, given their bandwidth, given their experience, it makes sense for us to grab as many opportunities as we get and be able to scale up as quickly as possible in these clusters. Given their 20, 25 years of experience in running such large format clusters.

Alankar Garude : Understood. Maybe, Abhinay, just 1 question pertaining to what you mentioned for some of these newer hospitals in terms of ramp-up, for Kollam, for Thrissur as well as I think for Nashik

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as well you mentioned that except oncology, you will start with all the specialties. On the other hand, for our AP hospitals, we have been adding oncology over the last few years. So, just wanted to understand basic question, why keep oncology for later? Any specific reason?

Abhinay Bollineni: No, I think all our greenfield projects, all the large format hospitals, we have oncology from day 1. So, Bangalore, in both the hospitals, we have oncology from day 1. Thane, we have oncology. Nashik, contractually, we are obligated to not do oncology, and that's why oncology is out of the equation there.

As far as Kerala is concerned, the way the current hospitals are designed is it does not have space for oncology. So, when we ramp up the current hospitals and then do the next phase of expansion, that's why we are bringing oncology. But like say, if we are doing a greenfield project in Kerala or a Brownfield, we will focus on oncology from day 1.

Alankar Garude : Yes. That's it from my side. Thank you.

Moderator: Thank you. The next question comes from the line of Rahul Jeewani from IIFL Capital. Please go ahead.

Rahul Jeewani: Hi, sir. So, sir, on the AP cluster, our margin profile is still lower than what we do in the Telangana cluster, largely because of our presence in some of these Tier 2, Tier 3 markets in AP.

But with the addition of, let's say, oncology and mother and child care, which you have been planning across your hospitals, when do you think that the AP cluster margins can catch up to Telangana cluster margins in terms over what time period can we get to Telangana cluster

margins in AP.

Abhinay Bollineni: Yes. Rahul, there will always be a gap of 300, 400 basis points from AP and Telangana, given the ARPOB and given the posi

tioning there. But I think if you look at Q2, we have reached 26%

margin in AP. And that's in spite of adding Queen's NRI. With Oncology and Mother and Child coming in most hospitals in this financial, in FY '26 and FY '27, maybe 3 years from there we should get to a 30% kind of a margin.

Rahul Jeewani: Okay, sure. So, AP cluster margins with 9 months are around 24%, you expect it to inch up to 30% in 2 years?

Abhinay Bollineni: Yes, in 2 to 3 years after the oncology facilities are operational.

Rahul Jeewani : And you said that most of these Onco, let's say, capacities will get commissioned in '26?

Abhinay Bollineni: '26-'27.

Rahul Jeewani : Okay. Sure sir. So, yes, that's it from my side.

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Moderator: Thank you. A reminder to all participants, you may press "*" and 1 to ask a question. The next question comes from the line of Aman from PhillipCapital. Please go ahead.

Aman: Thank you for the opportunity, and taking my question. So, I just had a question around your Maharashtra cluster. So, we have seen a fair amount of bed addition. So, what has been the occupancy trend in the Q4 so far that we have seen?

Abhinay Bollineni: Sorry, in Q4?

Aman: Yes. So, what has been the trend so far, we are almost halfway there in Q4, right? So, what has been the trend in occupancy?

Abhinay Bollineni: So, in Nagpur, the occupancy is around 160, 170. In Nashik, it's around 30, 35 in the month of January.

Aman: So, that was the only question from my end. Thank you.

Moderator: Thank you. Ladies and gentlemen, you may press "*" and 1 to ask a question. The next question comes from the line of Jay from Finnovate. Please go ahead.

Jay: Good morning. My question pertains to medical tourism. Could you provide an estimate of the percentage of international patients that KIMS currently caters to? Additionally, which hospitals in your portfolio contribute the most to international patient revenue? And what are your views on the broader medical tourism trend in India? And how do you see KIMS positioning itself to capitalize on this segment? Thank you.

Abhinay Bollineni : Hyderabad as a location is not very well recognized for international tourism. But however, we have seen significant improvement from last financial year to this financial year. This year, we should do around INR 40 crores to INR 50 crores of revenue from international patients. We are hoping that it will go up to INR 100 crores, INR 150 crores over a period of time, just from the Telangana cluster. But I think given Bangalore and Thane, both the locations are very well poised for international patients. We are hopeful that the contribution for international from these 2 hospitals will be significantly higher than what it is in Telangana.

Jay: Okay. And just if KIMS as the hospital is tracking this metric, so how much EBITDA % the Company is earning from these international patients?

Abhinay Bollineni: Right now, since it's in growth phase, there is hardly any EBITDA. We are still at INR 40 crores, INR 50 crores of revenue. But once we scale up, there will be more EBITDA coming. But it won't be higher or on par with what we are already getting from domestic.

Jay: Sure, sir. That's it from my side. Thank you and all the best.

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Moderator: Thank you. A reminder to all participants, you may press "*" and 1 to ask a question. Ladies and gentlemen, you may press "*" and 1 to ask a question. As there are no further questions from the participants, I now hand the conference over to the management for closing comments.

Bhaskara Rao Bollineni : Thanks for all the questions and enlightening us with the knowledge. And we are very happy the way which we are running in terms of bed expansions, and as well as the specialties we are adding. And we have been planned in such a way that it wi

Il not able to go beyond our EBITDA

debt more than 2 at any given time, and also equity to debt will be less than 1, maybe 0.8, 0.9.

And opportunities are coming, and the doctors are showing the good response with all the new facilities and even the existing facilities where we are adding new branches. And that way I think we are very happy the way which we are performing, and we will still continue to perform the same thing. Thank you very much.

Moderator: Thank you. On behalf of IIFL Capital, that concludes this conference. Thank you for joining us,

and you may now disconnect your lines.

Call: May 2025

(no extractable text — likely a scanned image PDF)

Call: Aug 2025

13th August, 2025

To,
The General Manager,
Department of Corporate services
Bombay Stock Exchange Ltd (BSE)
Phiroze Jheejheebhoy Towers,
Dalal Street,
Mumbai - 400 001.
Scrip Code - 543308
ISIN: INE967H01025 To,
The Manager,
Listing Department
National Stock Exchange of India Limited,
Exchange Plaza, 5th Floor,
Plot No.C/1, 'G' Block
Bandra - Kurla Complex
Mumbai - 400 051.
Symbol - KIMS
ISIN: INE967H01025

Dear Sir/ Madam,

Sub: Transcript of earnings conference call with Analyst / Investors.

In Continuation to our letter dated 30th July 2025, the Company organized a conference call with the Investors/ Analysts on Thursday, 07th August 2025 at 10:00 AM (IST). A copy of the transcript of the conference call held with the Investors/ Analysts is enclosed herewith and the same has also been uploaded to the Company's Website at <https://www.kimshospitals.com/investors> > Disclosures under Regulation 46 of SEBI (LODR) Regulations, 2015 > Analysts Calls schedule, PPT & Transcripts > Transcripts, Audio & Video Recordings > FY 25-26.

Kindly take the same on record.

Thanking you,
Yours truly

For Krishna Institute of Medical Sciences Limited

Dr. Nagajayanthi J. R
Company Secretary & Compliance Officer
NAGAJAYANTHI
JUTTUR
RAGHAVENDRA
DASDigitally signed by
NAGAJAYANTHI JUTTUR
RAGHAVENDRA DAS Date: 2025.08.13 17:50:53 +05'30'
Page 1 of 18

"KIMS Hospital
Q1 FY '26 Earnings Conference Call"
August 07, 2025

MANAGEMENT : DR. BHASKAR RAO BOLLINENI – FOUNDER AND
MANAGING DIRECTOR – KIMS HOSPITAL
D R. ABHINAY BOLLINENI – EXECUTIVE DIRECTOR
AND CHIEF EXECUTIVE OFFICER – KIMS HOSPITAL
M R. SACHIN SALVI – CHIEF FINANCIAL OFFICER –
KIMS HOSPITAL

MODERATOR : MR. RAHUL JEEWANI – IIFL CAPITAL SERVICES
LIMITED

KIMS Hospital
August 07, 2025

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Moderator: Ladies and gentlemen, good day, and welcome to the KIMS Hospital Q1 FY '26 Earnings Conference Call hosted by IIFL Capital Services Limited. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Rahul Jeewani from IIFL Capital Services Limited. Thank you, and over to you, sir.

Rahul Jeewani: Thank you, Shruti. Hi. Good morning, everyone. This is Rahul from IIFL Capital. I welcome you all to the first quarter earnings conference call of KIMS Hospitals. From KIMS, we have with us today Dr. Bhaskar Rao Bollineni, Founder and Managing Director; Dr. Abhinay Bollineni, Executive Director and CEO; and Mr. Sachin Salvi, CFO. Over to you, sir, for your opening comments.

Bhaskara Rao Bollineni: Greetings, and a hearty welcome to you all. Next week, we are going to celebrate 78th Independence Day. The air is full of Independence Day and other ensuing festivals. You will be pleased to know that this is our Silver Jubilee Year. We started our journey in 2000 with our first hospital in my native town of Nellore in Andhra Pradesh with our objective of providing affordable and accessible health care.

Encouraged by the overwhelming response, we expanded to other towns and opened our flagship hospital at Secunderabad in 2004. Today, we have 25 centers across 5 states with a bed capacity of 8,000 and a strong workforce of 20,000 plus and all set to grow further in the near future. All this is made possible due to the trust and confidence of patients arising out of our clinical excellence, outcomes, and latest technology and of course, support of you, investors.

Let me now rush to the present the financial and operational highlights. Total revenue of INR879 crores, a growth of 26.8% on year-on-year and 9.6% on quarter-on-quarter basis. EBITDA of INR200 Cr, a growth of 8.5% on year-on-year and a degrowth of 1.4% on quarter-on-quarter basis.

EBITDA margin at 22.7% vs. 26.6% in Q1 financial year '25 and 25.3% in Q4 FY '25. PAT at INR85 Cr in Q1 FY '26 against INR95 crores and INR106 crores in Q1 FY '25 and Q4 FY '25, respectively. Consolidated EPS for Q1 FY '26 of INR1.96, a degrowth of 9.2% on year-on-year basis. Cash-and-cash Equivalents includes cash, Bank balance, Deposits with maturity less than 12 months and Investment in Mutual funds at INR70 crores as on Q1 FY '26.

Q1 FY '26 financial highlights, the consolidated one. The consolidated revenue from operations of INR872 crores, a growth of 26.6% on year-on-year and 9.4% on quarter-on-quarter basis. Consolidated EBITDA pre-Ind AS of INR194 crores, a growth of 9.8% on year-on-year and degrowth of 1.5% on a quarter-on-quarter basis.

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Consolidated EBITDA pre-Ind AS, excluding other income of INR187 crores, a growth of 8.6% and a degrowth of 2.9% on year-on-year and quarter-on-quarter basis, respectively. The new units, the quarter was packed with action. We launched our Thane unit, and it has made a promising beginning.

We also launched our 2 units at Bangalore, done the Puja in Mahadevpur and Electronic City with a bed capacity of nearly 800 beds, both the units put together and may be operationalized in the quarter 2 and almost all the licenses and everything has come. So we are in a position to

start very soon.

And our new units in Kannur and Kollam in Kerala receiving encouraging response. We also opened our new units in Seethammadhara at Visakhapatnam, along with fertility center, Cuddles, Child Care, and a Specialty Unit. We also opened 100 beds in Srikakulam, the new facility. Maybe in th

e next couple of quarters, we may be opening in oncology in Anantapur and ongole. So these are all our new things which we planned are going in a proper manner. Maybe 1 or 2 months getting delayed because of the licenses to get from the organization in the government. And we also have a very good clinical achievements we achieved in this last quarter, a lot more are there with all these clinical achievements, and we are growing much faster starting from a 1 year baby to 85-year-old male.

And with all these clinical achievements, there is a proper growth which are going to come up. And these stories make the achievements that much. Interaction with international experts is a regular phenomenon at our hospital.

Last week, Professor Kelly Hunt, Prof, Department of Best Surgical Oncology, the University of Texas MD Anderson Cancer Center, hosted vis teams and delivered a lure on advancing surgery through collaboration and science. There was a lively academic interaction. We should greatly appreciate the expertise and latest infrastructure.

So, I conclude now assuring you of our continued efforts to expand our reach and pursue excellence for even better performance in future. Thank you.

Moderator: Thank you. We will now begin the question and answer session. The first question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: My first question is on your new units related cost. So it appears that the losses which you have booked from these new units in 1Q is high. So how should we see like especially Nashik, Thane is something which I understand contributed towards the losses? So if you can share how do you see operations scaling up at these units? And what kind of losses you are expecting, say, for this year and FY '27?

Abhinay Bollineni: So as far as -- so we have 5 hospitals which are new in the system and which are contributing to small margin or losses. So coming to Maharashtra cluster, we have Nashik and Thane. The ramp-up at Nashik has been a little slow because of tie-up from insurance -- delaying tie-up from insurance and corporate.

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But otherwise, if you look at the cash business ramp-up, it is quite decent, and we are pretty happy with the response so far. But there are some losses from the Nashik unit, which we feel will neutralize by the end of Q2.

As far as Thane is concerned, the ramp-up has been significantly good. The month of July, we closed a revenue of INR9 crores from the Thane facility, which is much better than what we had expected. We see a very positive trend as far as Thane is concerned. So by end of Q3, we should neutralize the losses from Thane as well and it will be EBITDA neutral.

In the Kerala cluster, we have 2 units. One is Kannur, which is EBITDA positive. We have Kollam, which is -- there was an EBITDA drag in Q1, which also is because we are doing a lot of renovation work. But Q2, we should neutralize the EBITDA losses from the Kollam facility, which is commissioned in Kannur. And from Q3 onwards, it will start contributing to EBITDA.

Damayanti Kerai: Okay. So all these put together and plus 2 new units in Bangalore, which are going to start in next few days or so. What kind of losses we should bake in for this year from the new unit?

Abhinay Bollineni: As far as Q2 is concerned, I think from both Nashik and Thane, we should have a loss of around INR8 to INR10 crores in Q2. And as far as Kerala is concerned, we should have it neutralized, so there won't be much losses, maybe INR1 crores at best. And as far as Bangalore is concerned because we'll start commissioning the hospital now. We got our Pollution Control Board license just yesterday. In another week, we should get our KPME license a week to 10 days. And once we get it by -- towards the end of August, we should commission both the hospitals. So that will be another INR10 crore drag for the month of September. INR10

to INR15 crores.

Damayanti Kerai: INR10 crores to INR15 crores. And these Bangaluru units, should we assume cost breakeven within, say, 2 to 3 quarters of operation or it could take slightly longer?

Abhinay Bollineni: We should assume September is the first full -- September month is the full month of operations, given that obviously will take time to get on these licenses. So from there, 12 months, we should positively breakeven in both the facilities in Bangalore. We should at least become EBITDA neutral.

Damayanti Kerai: Okay. So within 12 months, say, you are going fully operation from September, so 12 months from there onwards.

Abhinay Bollineni: Correct.

Damayanti Kerai: Okay. My second question is on your mature cluster of Telangana. So there, we have seen improvement in ARPOB over last, I think, a few years. However, my question is that the IP volume and occupancy looks more range bound. So how should we look at these 2 KPIs? Because I understand ARPOB...

Abhinay Bollineni: What did you say about occupancy? I couldn't follow that.

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Damayanti Kerai: Yes. So I'm saying we have seen improvement in ARPOB in Telangana cluster hospitals, but occupancy and IP volumes, if we look at, I think, last few quarters' number, they look more range bound. So how should we look at these 2 metrics? That was my question.

Abhinay Bollineni: Telangana, like we said, is a very mature cluster. So we're still growing at a 5%, 6% volume growth on a year-on-year basis, so which is in line with what we predict. And with the Kondapur hospital getting commissioned next year and some expansion happening in the Gachibowli hospital, we should see -- next year, we should see good volume growth in the Telangana cluster. But since it's a mature cluster, we are not anticipating growth beyond the 5%, 6% kind of volume growth.

Damayanti Kerai: Okay. So mid-single-digit growth for a mature cluster is something which we should assume

Abhinay Bollineni: By volume. We are running out of capacity in most of these facilities. When Kondapur comes in and when the new Secunderabad comes in, we will then get more beds operation.

Moderator: Our next question is from the line of Eknath Shinde, an Individual HNI Investor.

Eknath Shinde: I just want to understand a telescopic view. My question is, as KIMS expand its footprint across Tier 2 and Tier 3 cities, how are you leveraging the digital health platforms with AI or remote care models to enhance the stability, the scalability, and patient outcomes? And could these tech-led innovations become a structural moat for over the next 3 to 5 years?

Abhinay Bollineni: Yes. I think we're using AI and technology to help improve patient outcome and patient care monitoring within the hospital. I think as a company, we are very focused in delivering tertiary care and quaternary care -- and we do not want to digress from -- we will use technology, embrace technology as much as we can within the hospital.

But the key focus is to get the Tier 2, Tier 3 build capacity because there's a lot of unmet capacity demand and focus on being able to slice both tertiary and coronary care, but embrace as much technology as we can. We have always invested a very high on technology, both on AI-related stuff as well as equipment, new equipment that have come in the system.

Moderator: Our next question is from the line of Rahul Jeewani from IIFL Capital Services.

Rahul Jeewani: So the EBITDA losses which we indicated from these new hospitals in 2Q, that number, I think, would be closer to INR20 crores to INR25 crores for 2Q. So can you also quantify the combined losses which we had from these new hospitals in 1Q, just so that we better understand in terms of how these losses are moving from 1Q to 2Q?

Abhinay Bollineni: The combined losses in Q1 is INR21 crores.

Rahul Jeewani: Okay. So this INR21 crore hit you have already seen, so the incrementa

I hit in 2Q then would

be around, let's say, INR5 crores kind of a number?

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Abhinay Bollineni: INR5 crores to INR10 crores because there will be some improvement in the Maharashtra cluster, Kerala cluster, there will also be a drag from the Telangana cluster

Rahul Jeewani: Dr. Abhinay, you said that Nashik ramp-up obviously has been slow, so because of insurance delays. So when do you expect these insurance contracts to get formed up in Nashik? Because I think Nashik, we commissioned last year in September. So yes, apparently...

Abhinay Bollineni: The real commissioning started only from January, Rahul. So last month, we have ramped up to almost INR7 crores of revenue. So we are pretty happy with the progress made as far as the cash on -- as a player. But with insurance and CGHS, which both account for typically 60% of the volumes in most hospitals in that cluster, we are expecting both empanelment to be done over the next 3 to 6 months' time.

Rahul Jeewani: Okay. So the INR7 crores revenue for Nashik is just from the cash business. So with these incremental insurance contract, the business should further ramp up?

Abhinay Bollineni: Correct.

Rahul Jeewani: Sure. And on Sangli also, did we book any losses for the quarter or Sangli, we did not have much of losses?

Abhinay Bollineni: Sangli is an O & M contract, and we just get the revenue -- we get a revenue share. But when

you look at Sangli independently, we had no losses in Sangli for the full quarter.

Rahul Jeewani: Okay. And what was this quantum of revenue which you booked from Sangli this quarter?

Abhinay Bollineni: INR4.3 crores.

Rahul Jeewani: Okay. So Dr. Abhinay, can you just guide us in terms of how we -- because we have some of these O&M contracts. So I think in the core Telangana cluster -- Telangana AP cluster as well, I think Guntur is again an O&M contract. So for these O&M contracts for which the capacity doesn't get added, but we will be seeing this revenue accretion. So across all these O&M hospitals, what kind of a revenue number should we be factoring in, let's say, from a next 12- to 24-month perspective?

Abhinay Bollineni: So we'll start sharing that detail separately, Rahul, in the presentation from next time onwards. We got that feedback from some investors. We'll put O&M revenue separately.

Moderator: Our next question is from the line of Karan from GE Shipping Family Office.

Karan: Congratulations on a good set of numbers. Sir, I just wanted to double click on the Kerala cluster.

As we are aware, there's a brand already present in that cluster. So I just wanted to understand what our strategy will be going forward attracting better volume growth in that particular geography? So that's my first question.

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Abhinay Bollineni: I think the state as such is a very large state. We have seen good volume growth in both our facilities. And competition is not present in all the districts of Kerala. And if you look at where all we are commissioning hospitals, like in Kollam, we don't see -- sorry, in Karnataka, we have no competition from any large corporate trisure that we're setting up, we have no competition. So there is enough space for 2, 3 players to coexist and be able to scale significantly.

Karan: And sir, from that perspective, since we are a relatively new brand in that state, if you can just talk about the initiatives that have been taken to reach out to the residents in the geography over there so as to make a brand presence?

Abhinay Bollineni: Just like we are always focusing on those micro markets where there is unmet demand. And when we go there with the right infrastructure, right technology, and the right model to attract talent, the talent themselves attract a lot of patients and they are old patients, new patients and seeing the infrastructure. There's a

lot of unmet demand in those micro markets. So a lot of these detailed activities lead to increase in footfall in these hospitals.

Karan: And sir, just one last question on the Bangalore, Karnataka cluster. We're commissioning 800 beds there. And you mentioned there will be some EBITDA losses that will kick in from Q2 onwards. So sir, at what occupancy level do we feel that in 12 months, we can see EBITDA margins kind of neutralize there and help us in the consol EBITDA profile?

Abhinay Bollineni: Yes, I think around a good 30%, 40% occupancy, we should become EBITDA neutral -- it depends on the initial doctor cost and the time line in which the doctors are joining.

Karan: And from the perspective of talent hiring, etcetera, we are already, I'm assuming on track for that so that...

Abhinay Bollineni: Talent hiring for Phase 1 of the growth, we are pretty solid. We just -- once the licenses are in place, those doctors will have to start resigning from their current organization and then come on board. So that onboarding will take 2, 3 months or 1, 2 months. So that's where we are foreseeing some losses.

And because also Bangalore is a heavily insurance-dependent market, so there could be some initial delay in ramp up because we have to get the empanelment and that's a time-consuming process. But overall, the kind of talent we've been able to attract, I think we're pretty confident within 12 months, we should neutralize EBITDA losses from both the hospitals and Bangalore.

Karan: That's great to hear, sir. One last question, if I can squeeze in. From the 800 beds, are we looking at opening them in a phased manner? And if so, what would be the time line of that?

Abhinay Bollineni: Sorry, in which manner.

Karan: In Bangalore?

Abhinay Bollineni: We will only commission 50, 75 beds in each hospital to begin with. And as we ramp up those occupancies, we'll add more beds.

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Moderator: Our next question is from the line of Nancy Yadav from Allegro.

Nancy Yadav: Just two short questions. Firstly, I wanted to reconfirm the net debt number. I think I missed that part?

Sachin Ashok Salvi: So the net debt as on 30 June 2025 is INR2,020 crores.

Nancy Yadav: And I also wanted to ask that like you mentioned the EBITDA losses are INR21 crores. So could you give an approximate of how much of this is coming from Thane is there any other asset adding to this?

Abhinay Bollineni: So we have Thane, Nashik and Kollam adding to this. Thane would be around INR11 crores because we had only one month of revenue, but we commissioned the hospital from May. Nashik will be around INR7 crores. And then you have Kannur INR3.5 crores.

Karan: Alright sure. Thank you so much.

Moderator: Our next question is from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampil: A couple of questions. One, Srikakulam 120 beds, have we commissioned it because I haven't seen any addition in the cluster.

Abhinay Bollineni: Yes, we commissioned it in Q2.

Bino Pathiparampil: What are the plans of expansion beyond FY '27 because right now, all the announced projects are ending Q4 FY '2027...?

Abhinay Bollineni: Sorry your voice is not very audible? Would you speak up a little louder.

Bino Pathiparampil: Hello, is it better now? I was wondering about expansion plans beyond FY '27. All our announced plans are now getting commissioned by end of FY '2027. So have you started working on more greenfield projects? And what sort of areas are you looking at?

Abhinay Bollineni: We are exploring more opportunities, but the focus area and the clusters remain the same, more work in Karnataka, Kerala, Maharashtra and obviously, a lot of greenfield -- brownfield addition in Telangana and Andhra. But our intent is to first stabilize a lot of the greenfield hospitals that got commissioned this year or will get commissioned this year. So towards the end of the year, when we

are more stable from all of these operations, we will then start closing on a lot of the other greenfield hospitals, which will anyway take 3, 4 years, and that's a good enough time for us to stabilize operations.

Bino Pathiparampil: Got it. And one last question. In these new hospitals that are coming up, say Thane, Bangalore, etc, when you go for insurance empanelment, is there a rate negotiation that happens? or is there a standard slab which applies to that bed hospital and it just gets fixed?

Abhinay Bollineni: No, there is a negotiation that happens.

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Moderator: Our next question is a follow-up question, and it's from Damayanti Kerai from HSBC.

Damayanti Kerai: Given your experience in Nashik for some delays in insurance empanelment, etc, what are your expectations for Bangalore to get the intended coverage which you're looking for?

Abhinay Bollineni: Thane, Bangalore, Nashik will all take similar time. It will take us anywhere between 9 to 12 months for full empanelments to be completed. That's the typical timeline that we're seeing even in Thane. 9 to 12 months, we should safely assume for full empanelment.

Damayanti Kerai: So say, in Bangaluru, you are starting in September. So initially, we should assume it's more cash patient, you might be capturing. And then as you build up on the insurance empanelment part, that will come gradually.

Abhinay Bollineni: Correct.

Damayanti Kerai: Okay. That's helpful. My second question is on Andhra Pradesh cluster. So in last few quarters, again, because of your focus towards improving mix, etc, it has gone up from the historical level.

But when we look at its ARPOB, it's still -- there is a meaningful gap between Andhra Pradesh cluster and the network level ARPOBs. So do you think there is more room to cover up this gap? And what could help you to achieve that?

Abhinay Bollineni: No, I don't think we will -- that gap will ever get bridged. These are two independent micro markets and the being potential of the patients in that cluster is only so much. We will never be able to get to Telangana or Maharashtra kind of ARPOBs. Even within Maharashtra, the ARPOBs in Thane and Nashik are very different. So I think we'll have to look at it from a micro market point of view.

Damayanti Kerai: Sure. But see, right now, I think you are at 24,000 or so 24,000, 25,000 ARPOBs. So if we look AP cluster on its own, we can assume some more growth at December, 25,000 ARPOBs?

Abhinay Bollineni: Yes, there's definitely growth -- potential growth in the current -- from where it is currently, but it may not get to 69,000 like we see in Telangana because case mix is also going to change.

Oncology is going to become a significant part. We are focusing on trying to reduce A-laws.

Economy in that each of these districts are also improving, gain potential is improving. We will definitely improve, but there will always be that gap that will exist between Telangana and Andhra.

Damayanti Kerai: Sure. And I think oncology is one segment which you are focusing a lot to improve mix in the AP cluster. So has it been launched across all the units there? Or you have some room to improve them?

Abhinay Bollineni: We have only commissioned oncology in one of our hospitals. The others are yet to get

commissioned.

Damayanti Kerai: Okay. So that is yet to come. Okay, thank you for your response, I will get back in the queue.

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Moderator: Our next question is from the line of Abdulkader Puranwala from ICICI Securities.

Abdulkader Puranwala: First question is with regards to your AP cluster. So if you look at the margins for this particular quarter, there has been some bit of a softening. So can you help us understand why has this happened this quarter?

Abhinay Bollineni: So we have had the preoperative expenses on some of the Srikakulam hospitals, which go

t

commissioned. So we had hired a lot of doctors in Q1. Though the hospital got commissioned on July 1, we were onboarding these doctors for Srikakulam. So the doctor cost went up there.

And QNRI is a hospital that we acquired over there, we had seen some incremental doctor onboarding and a lot of renovation work is happening.

So there's been some delay in being able to complete the renovation work as a result, because of which the ramp-up is also getting slightly delayed, but we should complete renovation soon and then ramp up in QNRI will happen. -- but it is largely because of these two assets, onboarding of talent.

Abdulkader Puranwala: Got it. And sir, when you talk about your EBITDA losses in Q2, we see a cumulative range of INR30 crores, INR35 crores, that even factors the new hospitals at Andhra issue.

Abhinay Bollineni: There are no losses in Andhra. There are no losses in Andhra. There -- there are no hospitals in negative EBITDA. It's just that there has been some EBITDA drag because of this incremental cost.

Abdulkader Puranwala: Okay. Understood. And sir, in terms of your guidance, how should we look at your EBITDA margins to plan out for the current fiscal? Any color on that front?

Abhinay Bollineni: I think anywhere in the 22% to 25% range is what we are looking at.

Moderator: Our next question is from the line of Harish Bihani from Kotak AMC.

Harish Bihani: A couple of questions from my side. First is on the insurance part for the new unit. If I understood correctly, we were also trying to tie up with some intermediary who can help solve this issue at least for the foreseeable future. So anything on that, that you would like to highlight?

Abhinay Bollineni: Yes. So that is happening, Harish. We are using that vendor -- a couple of vendors to actually help do a reimbursement for all the insurance patients. But still a lot of patients are -- we are losing a lot of patients because they don't want to go through this cumbersome process. They just want complete cashless like even in Thane, which is fully commissioned. -- we are losing good number of patients in spite of which we've been able to ramp up, but they don't want to go through this whole cumbersome process. They just want to do either cashless or go to a different hospital. So that leakage also will get plugged once we have full empanelment of all these.

Harish Bihani: Sure. And this issue takes time largely because of challenges with regard to a new hospital and the insurance company is taking a bit longer. So why does this happen essentially?

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Abhinay Bollineni: One, there are some bilateral negotiations that happen on tariff. And two, the process itself is quite cumbersome. And now a lot of changes happening at the insurance level also. It's just become a little more cumbersome than before. Even traditionally, it has always been very cumbersome to get empanelment to new hospitals.

Harish Bihani: Sure. But for a hospital like us, which we are there in multicity, is it possible that some of this can be fast tracked? Or you are giving an outer limit of, say, 9 to 12 months and it could be fast tracked a bit earlier?

Abhinay Bollineni: It could be, but it's safe to assume a longer timeline given there's a lot of bilateral negotiation on what the tariff should be, what the potential of the micro markets are because though we operate in -- we have multiple hospitals and with these companies, they all focus on each of these micro markets very differently.

Harish Bihani: Sure. And you don't want to give up on a particular number simply because that becomes the base for any future negotiation Understood. And in terms of the ramp-up of some of the existing units where we are setting up incremental beds like Ongole, Anantapur, Kondapur let's presume, say, you stated in the presentation that will come in Q2, Q4, Q1 FY'2026 and FY'2027. So we should assume t

hat it starts, say, a quarter later? And how soon will these ramp up? Will it take,

say, 6 months year, 1.5 year, obviously, depending upon the number of beds, too, but just some

flavor on that?

Abhinay Bollineni: These -- Ongole is a very small addition. That should ramp up in 3, 4 quarters. But Anantapur, Kondapur, Rajamundry, we are building really large capacity. So full occupancy, 70% of that, - 70%, 75% occupancy of those beds will take a good 4 to 5 years' time before we get there. We get to 70% kind of an occupancy.

Harish Bihani: We're waiting for the incremental beds. Incremental beds.

Abhinay Bollineni: We are almost doubling capacity in each of these locations.

Harish Bihani: Right. Sure. Understood. And on the losses for the new units, so let's just assume that if it's around, say, INR80 crores this year, how will this look like, say, next year and a year after that ballpark in terms of the absolute size changing from 80 2x number, 2x number, some positive number in FY'28?

Abhinay Bollineni: See I think beyond Q1 of next year, we will have some drag from Bangalore, even that should be neutralized by Q2 of next financial year. But beyond Q2, we have no greenfield assets that are getting commissioned. So we don't see these losses continuing beyond that up until FY'2028, FY'2029 when we look at more greenfield hospitals that come.

Harish Bihani: So somewhere like INR20 crores for the full year or...

Abhinay Bollineni: For FY '2027 I think INR20 crores is a good number, INR20 crores, INR30 crores.

Moderator: Our next question is from the line of Bhavya Gandhi from Dalal & Broacha Stock Broking.

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Bhavya Gandhi: Sir, just wanted to understand what would be the peak revenue at realistic level of occupancy because different capacities are coming at different point in time. If you can provide a cutoff date when everything will be operational? And what sort of revenue can we generate and EBITDA that we can generate at that point in time once all the facilities come on stream? Maybe even if it's FY'2028, FY'2029, some ballpark number if you can provide?

Abhinay Bollineni: It's a very difficult answer to give because we keep adding capacity. I don't think that will -- capacity addition will continuously happen.

Bhavya Gandhi: At least for the existing capacity, when can we expect everything to come on stream? And what could be the peak revenue at realistic occupancy?

Abhinay Bollineni: We'll have to look at that. We'll come back to you on that.

Bhavya Gandhi: Sure. And just wanted to understand at least 2, 3 years, how would the ARPOB look like, some number if you can provide?

Abhinay Bollineni: I think a healthy 4%, 5% growth on ARPOB is what we should factor in.

Bhavya Gandhi: Okay. And even for the -- I mean, for the upcoming facilities, what kind of ROCE are we targeting at least once the ramp-up happens?

Abhinay Bollineni: 20%-25% -- kind of ROCE.

Bhavya Gandhi: So on a full -- if you include the matured as well as the upcoming facilities, what is the steady-state ROCE one should work out with?

Abhinay Bollineni: We should look at it cluster by cluster. As the hospitals mature because hospitals are coming in different time lines. It should look at each hospital, each cluster by cluster. Each cluster we should be able to deliver a 20%-25% kind of EBITDA.

Bhavya Gandhi: Fair enough. And sir, for the upcoming facilities, have we faced any -- I mean, competitive intensity in any of the geographies? I understand a few geographies, we don't have competition, but at least for the mature markets where there is healthy competition -- or are we seeing differentiated approach or any something of that.

Abhinay Bollineni: I think where we see a lot of competition is in our Telangana cluster. But fortunately, we've been able to improve both revenue and volume in the Telangana cluster.

Bhavya Gandhi: For the newer cluster

s, sir, actually, I was looking out for newer clusters.

Abhinay Bollineni: New clusters, we don't see much -- at least the hospitals that have gotten commissioned. We are not seeing any competition press.

Moderator: Our next question is from the line of Nikhil Mathur from HDFC Mutual Fund.

Nikhil Mathur: Sir, my first question is...

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Moderator: Nikhil sir, sorry to interrupt Nikhil sir. Your voice is sounding very low.

Nikhil Mathur: Sir, my question is on the relationship between ALOS and occupancy in your Telangana cluster. So assuming that 3.4% is the ALOS that remains -- I mean, it remains at that level for the foreseeable future. What kind of occupancy -- peak occupancy can you achieve on this sort of an ALOS?

Abhinay Bollineni: 65%, 70% occupancy at Telangana cluster is what we can achieve.

Nikhil Mathur: Okay. So 50% theoretically can go to 65%, 70% that we can expect?

Abhinay Bollineni: Correct.

Nikhil Mathur: Got it. So would that mean that the margins in the Telangana continue to trend up? I mean, Y-o-Y basis, they are up, but if that occupancy were to be achieved at some point in time, the margins or EBITDA per bed can continue to trend up in this cluster?

Abhinay Bollineni: Yes, it should potentially trend up.

Nikhil Mathur: Okay. Despite the competitive intensity like doctor poaching or anything of that, so that might happen?

Abhinay Bollineni: Yes, we have -- whatever competition capacity had to come in has come in, in the last 2, 3 years, and we have not seen any loss of doctors from our network of hospitals.

Nikhil Mathur: Okay. Got it. The second question is on the Congo mix for the company. I mean my understanding might be wrong, but what we hear is that Congo is a mix that most of the corporate chains are kind of focused on and increasing that part in the overall mix. So the first question is, if in the entire Congo mix, what is the ARPOB difference versus corporate level?

Let's say, if you are in Telangana, the ARPOB reported this quarter is some INR69,000. What can be the ARPOB of the Congo specialties? I'm a very ballpark number. I'm not looking at an individual specialty level ARPOB at a very ballpark level?

Abhinay Bollineni: Nikhil, sorry, we don't have that information off hand with us, but we'll share it with you.

Nikhil Mathur: But some broad sense, I mean, 2x, 2.5x, 3x depending on some sense you have on the average billing of a patient over a 3.5 days kind of a time period.

Abhinay Bollineni: It should be -- it will definitely be 20%, 30% higher than the blended ARPOB and there will be a drag on the other broad specialties, which account anyways for 30% of the revenue.

Nikhil Mathur: Okay. And at a company level, your Congo share is somewhere around 58%. I would imagine that Telangana would be much on the higher side and all the other clusters would be much, much lower. So this 58% can settle at 65%, 70% in the coming years?

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Abhinay Bollineni: Yes, correct. But most of our hospitals, whether new or old, the Congo specialty is around 60%, 65%.

Nikhil Mathur: So even in Maharashtra, Andhra Pradesh as well where, let's say, Maharashtra, empanelment are still missing in certain...

Abhinay Bollineni: Yes, yes. Still the revenue from these specialties are broadly the same. That's not going to change. That's not going to impact.

Nikhil Mathur: But then how will the -- I mean, you're guiding to a 5% kind of ARPOB increase at an overall level. So would it be more specialty mix driven then or it will be more payer mix driven then?

Abhinay Bollineni: It will be a blend of both. It will be a blend of both. I said 5% on ARPOB is pricing, but once the stability in specialties payer happen, it will take a while for the ARPOB to change. At a mature cluster, I'm saying it will grow at a healthy 4%, 5%. I'm not talking about a company level

I. At a company level, there will be significant change because almost 2,000, 3,000 beds have gotten added in different micro markets. So by the time it stabilizes, it will take some while. And the ARPOB in both Bangalore and Thane are significantly higher than what our company ARPOB is today.

Nikhil Mathur: Okay. Understood. And then what is the mix between surgical and medical today? I mean -- and how will that change in coming years?

Abhinay Bollineni: I think it will remain constant. I don't see that changing.

Moderator: Our next question is from the line of Rahul Jeewani from IIFL Capital Services Limited.

Rahul Jeewani: So sir, you indicated that the Telangana cluster would see, let's say, a 5%, 6% kind of an IP volume growth, which would accelerate with, let's say, Kondapur and Gachibowli next year. So can you similarly talk about the Andhra cluster, so what kind of IP volume growth you see in Andhra given that we have been adding these onco and mother and child care specialties as well in some of these IP hospitals?

Abhinay Bollineni: A similar growth profile, Rahul, because these are both mature clusters for us though there are some new greenfield opportunities in Andhra. But since they are all brownfield capacity addition currently, we should assume the similar growth profile in both these clusters.

Rahul Jeewani: Sure, Dr. Abhinay and Dr. Abhinay while, let's say, within both these clusters, there would be a few hospitals which would be operating at peak occupancies, but there would be hospitals within these clusters, which are at a lower occupancy. So let's say, Sunshine and Telangana or some of the other Tier 2, Tier 3 hospitals. So if we look at occupancies for both these clusters, it is around 55%, 60% only. So given that we are only at 55%, 60%, don't you think that the IP volume growth could be slightly better than what you are indicating?

Abhinay Bollineni: In Secunderabad, we have some limitation on the beds because some of the beds are being rebuilt

-- so we don't have that bed capacity though on the beds that we have, we are at a higher
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occupancy. But unless the new beds get commissioned, though it's part of the overall capacity, unless the new beds get commissioned, it will be difficult for us to ramp up in this hospital. And both in Gachibowli and Begumpet, which is the Sunshine hospital, we are adding new technology like cancer and some other specialties for more growth to happen in the future.

Rahul Jeewani: Okay. Sure, sure. And on this ARPOB growth or this quarter at a company level, we saw a 11%-12% kind of an ARPOB growth. And you indicated that the mature markets or the cluster should see a 5% ARPOB growth. But because we are entering into these markets where inherently the ARPOB is higher than the company average, if you had to put out a number to overall ARPOB growth at a company level, what would that number be, let's say, over the next 2 to 3-year period?

Abhinay Bollineni: We haven't really put that number through Rahul, but I think you should safely assume INR50,000, INR55,000.

Rahul Jeewani: Okay. So this company level ARPOB, which is right now at around INR43,000 that you think can improve to INR50,000, INR55,000 kind of an ARPOB?

Abhinay Bollineni: Yes, that will happen once ramp-up in both Bangalore and Thane happen.

Moderator: Our next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: So out of the 300 beds at Thane, how many are operational? Is it just 100 beds?

Abhinay Bollineni: Yes, 100 beds.

Alankar Garude: And Dr. Abhinay, when do you expect to increase the operational beds to, say, 250, 300 beds? I mean, typically, it takes a year or less than that. What's the initial sense?

Abhinay Bollineni: You see, right now on 100 beds, we are at around 50%-55% occupancy for the month of July. So as we reach 70% occupancy and 100 beds, we'll

I open another 50 beds. But I think 3 to 4 years for a full 300 beds to be fully commissioned and to reach 70% occupancy is what we are looking.

Alankar Garude: Got it. The other question, again on Thane and even on Nashik, has the doctor recruitment in both the hospitals largely being done?

Abhinay Bollineni: In Thane, it's largely done. Just some onboarding is left, which will happen in August, September. At least for Phase 1 of the growth, we are fully sorted as far as Thane is concerned.

For Nashik, we have 2, 3 specialties that are still pending, which will potentially happen once we have the insurance company is onboard.

Alankar Garude: Got it. And maybe it's very, very early to ask this question, but just taking a chance on ARPOB at Thane, how is the ARPOB looking like initially at this point of time comparable with the other peer in the market, higher, lower? Any sense on that?

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Abhinay Bollineni: It's very early because the specialty mix is yet to stabilize. But it will be similar. It will be very similar to what the other peer in the market.

Alankar Garude: Fair enough. The second question is, see, given you have opened hospitals in quite a few new markets in Maharashtra and Kerala over the past few quarters, and you'll be opening more as well, especially the Bangalore one -- can you take us through some of the initial learnings you spoke about insurance empanelment. But apart from that, any learnings you would like to share, especially if you compare these new hospitals with your existing AP, Telangana facilities?

Abhinay Bollineni: So like we have always indicated, Maharashtra has been always slightly more difficult than the rest of South in doctor onboarding. But that's because culturally, the things are very different in both these micro markets. South is more an institutional practice. In Maharashtra, this is a new system that's evolving.

So that we have anticipated that it will take that much time when we set up the first few hospitals. But in Thane, it has been quite good. The response has been very impressive. We've got good traction from a lot of doctors - leading doctors in those micro markets. So we are pretty positive about the long-term growth in Maharashtra and in also the other micro markets, Karnataka, and Kerala.

Alankar Garude: Got it. And one final question. When you talk about Sunshine and Nagpur, one, in terms of occupancies and resultant in terms of margin expansion, is there any further scope for expansion at both Sunshine as well as the Nagpur facility?

Abhinay Bollineni: Yes. Sunshine and Nagpur both have scope for expansion. As incremental revenue comes in, 40% of that.

Alankar Garude: Where are we in the journey on Sunshine when it comes to changing the specialty mix?

Abhinay Bollineni: now We've reached a healthy number -- when we first commented we anticipate ortho to grow as well at similar growth rate, but that has continued to grow. But whatever we wanted to achieve in most of our other specialties we have done. We need to create more space in the new Begumpet Hospital at Sunshine. This is to add more specialties such as oncology and pulmonology. As far as Gachibowli is concerned, we want to expand to a slightly -- we want to add more beds again to add more oncology. So with that expansion, I think what we had earlier envisage for Sunshine will be fully done.

Moderator: Our next question is from the line of Harith Ahamed from Avendus Spark.

Harith Ahamed: Dr. Abhinay, can you talk a bit specifically about our O&M units? We've signed a few arrangements under the O&M model. So which among the 3 or 4 are currently operational? Was there any contribution for the quarter? And if you can also talk a bit about what exactly our involvement is in these units? Do we have our own people at what levels and the kind of capex commitment from our side?

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Abhinay Bollineni: So there's no capex commitment from our side, Harith, on these assets. We have 2 hospitals that are operational. One is Sangli and one is Guntur. There is one more hospital in Hyderabad that will get operationalized soon. But from both Guntur and Sangli, the response has been quite good. We've been able to break even in both the hospitals. And both the hospitals today are doing a top line of around INR17 crores, INR18 crores per month, and we see the potential for that to double over the next 3 to 4 years. So that's the contribution from both these hospitals. And they both have broken even now.

Harith Ahamed: And what exactly is the arrangement we have of people at what level?

Abhinay Bollineni: So we have full control over the P&L, the hospital operations, the clinical talent hiring. All of that is under the scope of KIMS. And we manage the entire hospital for the promoters, and we take a certain share of the revenue.

Harith Ahamed: Okay. And are we looking to add more units under this model? I'm asking because from an operational bandwidth standpoint, would there be a constraint given we have a ramp-up hospitals at Nashik, Thane, and the upcoming hospital at Bangalore?

Abhinay Bollineni: We're adding one in Telangana. But I don't see anything at least for the next 3 quarters, we don't see any more O&M contracts happening.

Moderator: Our next question is a follow-up question from Nancy Yadav from Allegro.

Nancy Yadav: Like we spoke about the EBITDA tax from Thane, Nashik, and Kollam, I also wanted to get the revenue numbers for the 3 facilities?

Sachin Ashok Salvi: As far as the top line number is concerned for Thane we did INR5.5 crores. For Nashik, we did about INR15.5 crores. Kollam, we did INR9 crores.

Nancy Yadav: In Kollam you did INR9 crores?

Sachin Ashok Salvi: Yes.

Moderator: Ladies and gentlemen, this was the last question for today. I now hand the conference over to the management for closing comments. Over to you, sir.

Bhaskara Rao Bollineni: Very good interaction and then the updates what you have given the questions was able to think through. And what we are trying to achieve, as I mentioned many times that we believe that we can able to achieve the -- what we have been achieved the CAGR of top line and bottom line in the last decade. We are also planning to achieve that. That's why we are doing the executive models. And Andhra units are basically purposefully we put, there is a lot of scheme patients and the ARPOBs and the capex what we invest in per bed is also less when compared to the rest of the Maharashtra and Telangana and Karnataka.

And the other important things, the medical and surgical branches when you see there is nothing like only Congo that able to do more, even some of the non-Congo specialties when the

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complexity of the case, it will also give more of value. And the ARPOBs growth also will be there from 5% to 10% growth will come because of the new things which have been added. As we anticipated, there will be a lot of tough time for us in Maharashtra and Karnataka to align with the doctors. But after seeing that, what the learnings we learned, they are also very happy the way culture what we built in KIMS.

They're also moving towards this side that is a very good positive note on us so that we have more encouragement to develop more and more in Karnataka, Kerala and Maharashtra because they are also aligned and what we have planned, what we are doing with our things in the South, Andhra and Telangana, that is very, very positive so that it will be easy for us to move forward

a little further.

And we also look for some strategy of O&Ms where -- because we have a lot of expansion plans with a lot of debt is sitting on the books and the O&M model is a better model so that we can also build and have a control. At th

e same time, we are getting some money for the EBITDA,

and we are looking around in a few years that there may be INR100 crores top line per month.

So nearly INR9 crores will come from there up to 9%. With all those things what we looked at, there is a very, very positive look from our side, the plan, and the growth very meticulously, very cautiously and consciously deploying the capex.

And also the operational leverage where without causing inconvenience to the patients, without going back on our affordability, accessibility, quality, these are all there. I think we are very, very pretty a good spot as far as health care is concerned. KIMS is in a good position.

Once again, thank you very much for all your questions and the knowledge that you have been given in the form of questions. Thank you.

Moderator: Thank you. On behalf of IIFL Capital Services Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Nov 2025

14th November, 2025

To,
The General Manager,
Department of Corporate services
Bombay Stock Exchange Ltd (BSE)
Phiroze Jheejheebhoy Towers,
Dalal Street,
Mumbai - 400 001.
Scrip Code - 543308
ISIN: INE967H01025 To,
The Manager,
Listing Department
National Stock Exchange of India Limited,
Exchange Plaza, 5th Floor,
Plot No.C/1, 'G' Block
Bandra - Kurla Complex
Mumbai - 400 051.
Symbol - KIMS
ISIN: INE967H01025

Dear Sir/ Madam,

Sub: Transcript of earnings conference call with Analyst / Investors.

In Continuation to our letter dated 04th November 2025, the Company organized a conference call with the Investors/ Analysts on Monday, 10th November 2025 at 09:00 AM (IST). A copy of the transcript of the conference call held with the Investors/ Analysts is enclosed herewith and the same has also been uploaded to the Company's Website at <https://www.kimshospitals.com/investors> > Disclosures under Regulation 46 of SEBI (LODR) Regulations, 2015 > Analysts Calls schedule, PPT & Transcripts > Transcripts, Audio & Video Recordings > FY 25-26.

Kindly take the same on record.

Thanking you,
Yours truly

For Krishna Institute of Medical Sciences Limited

Nagajayanthi J. R
Company Secretary & Compliance Officer

Encl: As above

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"KIMS Hospital
Q2 FY '26 Earnings Conference Call"
November 10, 2025

MANAGEMENT : DR. BHASKARA RAO BOLLINENI – FOUNDER AND
MANAGING DIRECTOR – KIMS HOSPITAL
D R. ABHINAY BOLLINENI – EXECUTIVE DIRECTOR
AND CHIEF EXECUTIVE OFFICER – KIMS HOSPITAL
M R. SACHIN ASHOK SALVI – CHIEF FINANCIAL
OFFICER – KIMS HOSPITAL
M R. SREENATH REDDY – CLUSTER HEAD –
KARNATAKA CLUSTER, KIMS HOSPITAL

MODERATOR : MR. RAHUL JEEWANI – IIFL CAPITAL SERVICES
LIMITED

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Moderator: Ladies and gentlemen, good day, and welcome to KIMS Hospital Q2 and H1 FY '26 Earnings Conference Call hosted by IIFL Capital Services Limited. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star, then zero on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Rahul Jeewani from IIFL Capital Services Limited. Thank you, and over to you, sir.

Rahul Jeewani: Hi. Good morning, everyone. This is Rahul from IIFL Capital. I welcome you all to the second quarter earnings conference call of KIMS Hospitals. From KIMS, we have with us Dr. Bhaskara Rao Bollineni, Founder and Managing Director; Dr. Abhinay Bollineni, Executive Director and CEO; Mr. Sachin Ashok Salvi, CFO; and Mr. Sreenath Reddy, the Cluster Head for the Karnataka cluster. So over to you, sir, for your opening comments.

Bhaskara Rao Bollineni: Good morning to everybody. I extend a warm welcome to all of you for the investors meet. Today is a World Science Day for Peace and Development, celebrated annually to emphasize science's contribution to society and development.

Today also happens to be World Immunization Day to promote the use of vaccines to protect people of all ages against preventable diseases and highlight the importance of immunization in global health. Both occasions are closely connected to healthcare.

Let me first take you to the financial and operational results of quarter 2 2025-2026. I am pleased to state that results are good in spite of the fact that the second quarter of the year is usually dull. Financial year quarter 2 updates, total revenue of INR965 crores, a growth of 23.3% on year-on-year and 9.8% on a quarter-on-quarter basis.

EBITDA INR208 crores, a decline of 6.7% on year-on-year and a growth of 4.3% on quarter-on- basis. EBITDA margin at 21.6% versus 28.5% in quarter 2 FY '25 and 22.7% in quarter 1 FY '26. PAT at INR72 crores in quarter 2 financial year '26 against INR121 crores and INR85 crores in quarter 2 FY '25 and quarter 1 FY '26, respectively.

Consolidated EPS for quarter 2 FY '26 of INR1.67, a decline of 37.7% on a year-on-year basis and 14.7% on quarter-on-quarter basis, respectively. Cash and cash equivalents includes cash, bank balances, deposits with maturity less than 12 months and investments in mutual funds at INR120 crores as on 30th September 2025.

Quarter 2 financial '26 financial highlights consolidated. Consolidated revenue from operations of INR961 crores, a growth of 23.6% on year-on-year and 10.2% on a quarter-on-quarter basis. Consolidated EBITDA pre-Ind AS is INR201 crores, a decline of 8.6% on year-on-year and a growth of 3.6% on quarter-on-quarter basis. Consolidated EBITDA pre-Ind AS, excluding other income of INR197 crores, a decline of 8.5% and a growth of 5.3% on a year-on-year and a quarter-on-quarter basis, respectively.

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Operational highlights. Consolidated average revenue per operating bed grew by 9.8% and a decline of 2.3% on year-on-year and a quarter-on-quarter basis, respectively. Average revenue per patient grew by 7.3% and a decline of 1.6% year-on-year and a quarter-on-quarter basis, respectively. IP volume 64,288 grew by 12.2% on a quarter-on-quarter basis and 15.3% on a year-on-year basis. OP volume, 5,92,725 grew by 18% on quarter-on-quarter and 25.1% on a year-on-year basis.

The academic accomplishments

Historic win and provide movement for Indian rheumatology and KIMS.

This is t

he first time India had won the Knowledge Bowl Trophy since its inception more than 40 years ago. KIMS team headed by Dr. Sarath Chandramouli, our Clinical Director and Head Department of Rheumatology and Clinical Immunology has won the first place, that is gold medal, at the American College of Rheumatology ACR 2025 Knowledge Bowl Chicago, competing with teams from renowned international centres such as the University of Minnesota, Yale School of Medicine, Mayo Clinic, University of Toronto, University of Chicago, University of California and many others. Our team also received the Team Spirit Award. The accomplishment is a matter of pride for KIMS as it's for the nation.

Dr. Manas Panigrahi, Head of the Department and a senior consultant neurosurgeon made a presentation at the Congress of Neurological Surgeons at Los Angeles, that received wide acclaim.

Dr. Aswini Dutt, Department of Nephrology won the Dr. APJ Abdul Kalam Award being adjudged as Best Nephrologist in Hyderabad.

Our recently opened Thane, Mumbai centre organized an open house to let people have first-hand knowledge about its facilities resulting in a huge and encouraging response. The unit conducted more than 1,000 surgeries in less than 6 months, a testimony to the growing goodwill. At KIMS Kondapur, we introduced a Medtronic major robotic guidance system. This is the first in Telangana and Andhra Pradesh states, opening a new era in spine surgery.

We have launched Asia's first XR experience lab at our Gachibowli unit. The state-of-the-art facility enables clients to explore innovative applications in augmented reality (AR) and virtual reality (VR) across four key verticals: screening, diagnosed, therapeutic and skill building. The pilot project of 6 months revealed positive outcomes in reducing anxiety, improving mobility, decreasing analgesia usage and enhancing patient satisfaction.

KIMS Manavata Hospitals, Nashik launched its comprehensive cardiac unit, and it is the only one centre in entire North Maharashtra to have EP studies and ECMO facilities.

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An exclusive IVF centre with the latest facilities opened by KIMS in Banjara Hills attached to KIMS Sunshine Hospital.

KIMS Secunderabad, cuddles and dietetic department conducted a conference, on nutrition for first 1,000 days of life revolving around mother and child to a packed audience.

KIMS Hospitals Kannur has achieved a series of firsts to its credit and rendering services that were hitherto not available there. In fact, it brought out a booklet highlighting such 32 first-in-kind accomplishments in first 32 weeks.

I will not burden you with unending list of complex surgeries and great treatments taking place at our various units.

KIMS Shikhara Hospitals of Guntur performed its first multi-organ transplant procedure. The unit is yet to complete its first year and such transplants augur well for the progress in saving lives in urban centres and the surrounding areas.

KIMS Mahadevapura performed the multi-organ transplant program in Bangalore within the first quarter of the inception of the organization.

I would conclude now by mentioning about an important thing, which every Indian must know.

Our National song, Vande Mataram was penned by Bankim Chandra Chatterjee 150 years ago, and just three days back on 7th November, the nation celebrated its 150th glorious anniversary.

The song aroused patriotic fervour among the masses and played an important role in freedom struggle. This 150th anniversary will be celebrated for the whole year.

Thank you very much. Open for the

discussion.

Moderator: The first question is from the line of Piyush Kumar from Magnus Hathaway Investments.

Piyush Kumar: Sir, my first question is regarding even though there is a revenue increase of 23.3% this quarter, the EBITDA and PAT degrew by minus 6.7% and 40.3%. Can you just shed some actual light why this is happening?

Abhinay Bollineni: We have commissioned almost 4, 5 new hospitals, which are yet to break even and because of which the revenue ramp-up in those hospitals are happening, but haven't reached EBITDA breakeven. So that is why there is a drag on the asset.

Piyush Kumar: Okay. Sir, and my next question is, a bit, currently regarding we are driving around INR42,016 in average revenue per occupied bed. So can we just expect in the next few quarters or maybe

next 8 quarters, can this number touch INR50,000 per bed?

Abhinay Bollineni: Yes. As Maharashtra especially Thane and Bangalore scale up, there is -- and because the current ARPOB in both these markets are around INR50,000 and the potential for it to scale up will be around INR65,000, INR70,000 like in the case of Hyderabad. There is in all fairness, it can go up from the current INR40,000 to INR43,000 to almost INR50,000 over the next 8 quarters.

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Piyush Kumar: All right, sir. Sir, just one last question. Currently, we are driving around 17% revenue from cardiac sciences, 14% from ortho and 11% from neuro. Sir, can you just shed some light which one out of all these is more profitable for you as a hospital?

Abhinay Bollineni: Very difficult to look at it at a specialty wise, at a hospital level is what we track. We don't look at it at a specialty level.

Moderator: The next question is from the line of Sucrit D. Patil from Eyesight Fintrade Private Limited.

Sucrit D. Patil: Good morning to the team, I have two questions. First is for Mr. Rao. Rao, as more hospital chains are expanding across India, what is KIMS doing to build a strong edge not just through the bed count or regional dominance, but something deeper like a way of working or thinking that grows over the time and makes your competitors hard to copy. I just want to understand the forward-looking view.

Bhaskara Rao Bollineni: Basically, what we are looking is, you know, healthcare is a very, very important one. We should not be able to compare like a business. Even the previous person whoever asked on an individual base department-wise, which is good and all. Our entire aim is in healthcare, there are 4, 5 important things are there for a human being.

Like today, most of us are doing only the curative care. There is a lot more we need to insist on the prevention, preventive actions, how we can be able to shape so that whether we can be able to create an awareness or a further checking and creating a diet and other things.

Second important thing is after the discharge; there is one more important thing to improve the quality of the patient is we need to be able to look into the rehabilitation program. And basically, now today, oncology is growing, the need of oncology is expanding and growing need. We need to also see whether the palliative care and end-of-life care should be able to do that.

And to improve the quality care for the patients and the employees and everything, we need to move on to a different platform of what we are doing like AI generated and even the digital platform where the patients should be able to be eased out so that they should be able to come back. And now the patients are also expecting a good infrastructure and a good technology. We are always on the forefront to provide a good technology as well as the good infrastructure. Good infrastructure will keep the patients very happy as long as they stay in the hospital and supported by good services, as I mentioned about AI and the digital thing.

And the good technology, whichever is available, will be able to help the patients to see t

hat

they will recover much faster, not to be able to stay longer period in the hospital. And at the top we can attract the good doctors also. Like if you see in the last 1 or 2 quarters, we were able to acquire Gamma Knife, magnetic resonance focused ultrasound and TULSA-PRO and these are all the good equipment which we are able to, even 25 robotic machines of DaVinci. So that each unit, whether it's a Tier 1, Tier 2, Tier 3, so that is our main aim of healthcare that we should be able to sustain.

Sucrit D. Patil: Okay. My second question is to Mr. Salvi, sir. I believe he's on the call today.

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Sachin Ashok Salvi: Yes, I'm there.

Sucrit D. Patil: I just want to understand about the margins and cost savings that you will be doing in the future.

As operating costs and margin transitions keep affecting the profits, how are you planning to protect the margins?

And are there any smart internal methods that you will be implementing like tech platforms or clinical protocols that will help you keep the service quality high, but not putting pressure on the existing. And even if they don't show the numbers on your balance sheet. So I just want to understand how you want to balance cost pressures with keeping the margins strong over the time.

Sachin Ashok Salvi: As far as the margins are concerned, we are not compromising on the quality. That is the first thing, which is very clear to us. If you see our business and if you typically divide our business into different clusters, different clusters are following different margin trajectory.

As far as the core clusters are concerned, like Telangana and Andhra Pradesh, they are into

something like 25%, 30% EBITDA margin, which is sustainable EBITDA margin. And in fact, we are seeing for the last couple of quarters, the margins are improving. We are ensuring that this margin trajectory remains there.

As far as the new hospitals are concerned and mainly into Maharashtra and Kerala and Karnataka, these are very large formatted hospitals. It takes some time for them to ramp up. Typically, we assume that from the day of start it takes at least 12 months to ramp up. And within this time, typically, any hospital, any newer hospital makes an EBITDA loss somewhere ranging from INR20 crores to INR30 crores. As far as we are covering that loss is EBITDA erosion in our overall consolidated balance sheet, I think we are through.

As Dr. Rao has also mentioned in the initial comment, already Thane has started seeing a good ramp-up. If the same ramp-up continues for the Bangalore and other clusters, I think as a consolidated financial statement, which you are seeing, we will continue to do about 27%, 30% kind of EBITDA margin over the next at least say, 2 or 3 years.

Moderator: The next question is from the line of Amey Chalke from JM Financial.

Amey Chalke: So, first question I have, I joined the call late, but have we given adjusted EBITDA margins for the new hospitals like absolute EBITDA and the EBITDA margins and how it has performed year-on-year?

Abhinay Bollineni: Sorry, Amey, we lost you. Could you repeat the question again, please?

Amey Chalke: Have we given the adjusted EBITDA margin for the existing hospital, adjusting for the new hospitals, the year-on-year performance, how it has done?

Abhinay Bollineni: Yes, the presentation has cluster by cluster. And within that also, we have given details of each asset by asset, the newer assets, how they are ramping up.

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Amey Chalke: Sure. So the second question I have is on the Telangana cluster, where the growth seems to be around single digit in terms of revenue. The occupancy is also in the range of 50%. So what optimized occupancy we should assume here for the Telangana clusters and how we can improve performance here?

Abhinay Bollineni: So Telangana, like we've always been historically guiding it will conti

nue to grow at a high

single-digit number only, like we have mentioned in the past. The reason you have seen the bed capacity at 50% is because in Secunderabad, where the flagship hospital is, we are rehabilitating almost 300 beds, which are not completely functional today, which were operational before.

So once the rehabilitation of those beds are completed and the new Kondapur hospital is commissioned, which is expected to happen in Q1 next financial year, we can expect a little better growth in the Telangana cluster. But since it's a very mature cluster with a large-format hospital, I think we are only guiding towards a high single-digit kind of a growth.

Amey Chalke: Sure. And another question I have on the Andhra. So here, the revenue growth has been good, largely strong double digit, but it is largely led by the ARPOBs. However, the occupied beds have fallen by almost 100 beds. So how we should think about the Andhra region going ahead?

Abhinay Bollineni: In Andhra region, there is still a lot of opportunity for growth. We are adding capacity in almost all our current hospitals. Either we are adding oncology, we're adding mother and child, and we're adding other specialties that don't exist. So, I think a healthy growth rate in high teens -- mid-teens are what we are looking at Andhra that we consistently continue to grow in the same format.

Amey Chalke: But the occupied beds which have fallen, that what you would attribute to? Like is it like the...

Abhinay Bollineni: Bed capacity has increased, ALOS has reduced. I think the more -- the better indicator is to look at the IP volume and OP volume, which is consistently growing.

Amey Chalke: Sure, sure. And is it possible to give the occupancy we are currently operating the Thane hospital at?

Abhinay Bollineni: Thane currently is hovering around 80 beds being occupied, 80, 90 beds. I'm talking about the month of October.

Moderator: The next question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: Good morning, my first question is on EBITDA loss at the new facilities. So, I think last quarter, you mentioned by June next year, all the new units should be cost neutral. So, do you hold on to that? Or do you think that time can shift a bit? Any comment on like the margin trajectory, please?

Abhinay Bollineni: We are still sticking to that. We don't see any hospital that can slip beyond except for one hospital in Bangalore, which is still not commissioned. So, what we had always committed earlier is within the first 12 months, we are pretty confident that we can breakeven and looking at the growth trajectory of all the hospitals that are commissioned, we are very confident that all

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hospitals within the first 12 months will become EBITDA neutral. One of the Bangalore facility will get commissioned by end of this month. So, we are assuming a 10-month breakeven time line. So that one facility can slip beyond the first quarter.

Damayanti Kerai: Okay. And meanwhile, the margins will likely stay in like early 20s, the way we have seen for this quarter or with say, Thane picking up, we continue to see pickup over that time as well?

Abhinay Bollineni: I think maybe the next 2 quarters, it will be similar. But from Q1 because most hospitals would become EBITDA positive, things should start getting better. The margin expansion should happen.

Damayanti Kerai: Sure. My second question is a clarification. In your presentation for the Maharashtra cluster, census beds came down to 442 compared to 645 last quarter. And on the other hand, the non-census bed grew to 195 from 85. So, what has happened, if you can help us understand that?

Abhinay Bollineni: Sorry, one second.

Damayanti Kerai: Yes. So, for the Maharashtra cluster. There has been some bed relocation.

Abhinay Bollineni: No, no. I think there's been a typo there.

Damayanti Kerai: Okay. So can you like...

Abhinay Bollineni:

ni: It continues to remain the same as what it is in Q1.

Damayanti Kerai: Like both the census and non-census bed, it's similar?

Abhinay Bollineni: Correct. We'll again reverify this data point and come back to you after this call, but there is no change on that number from Q1 to Q2.

Damayanti Kerai: Okay. And my last question is on the margin profile for Andhra cluster. So very strong set of numbers. And I think when I look at how your drivers have moved, so except the OP volume, I think which has seen very strong sequential growth, your ARPOB just grew at 2% or so. And then I think your ARPP also remained flat. So what explains this sizable jump in margins, in case of 22% margin going to almost 28% in September quarter?

Abhinay Bollineni: I think one Srikakulam hospital, which we commissioned a new facility, the ramp-up there is happening quite well. And the margin erosion that is happening in the new acquisition of Vizag, that has come to almost negligible. So, both of these have contributed to the margin expansion in Andhra. But in Quens NRI, there was a drag in last quarter, which now is EBITDA positive in Q2, which is the hospital that we acquired last year. And Srikakulam, there has been an additional 150 bed capacity addition there.

Damayanti Kerai: Okay. So, Vizag and Srikakulam, okay. So, for the cluster, AP cluster, what should be the sustainable margins? Somewhere in mid-20s, will that be the fair assumption?

Abhinay Bollineni: Yes, I think anywhere between 25% to 28% margin is what we are looking at.

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Damayanti Kerai: Sorry, can you please repeat? I just missed that number, 25% to...

Abhinay Bollineni: 28% EBITDA margin in Andhra Pradesh is what we can sustain.

Moderator: The next question is from the line of Abhishek Maheshwari from SkyRidge Fund Managers.

Abhishek Maheshwari: Sir, just one question regarding -- since you've commissioned new hospitals of late, the gap between your operational bed and actual bed capacity has increased quite substantially. So can you share some perspective on how you plan to close this gap and if there is any time line to it?

Abhinay Bollineni: No, we've added the significant bed capacity, over the last 1 year. We've added almost 3,000, 4,000 beds. And it will take a good time before we ramp up those beds. And this is a capacity that we've built for the next 4, 5 years.

Abhishek Maheshwari: So just for understanding, so when you open a new hospital, let's say, a hypothetical 100-bed hospital, so is it that only 20, 30 beds are operational at the beginning? And as they get occupied, you add another 20, 30 beds. Can you just explain the process further?

Abhinay Bollineni: Yes. If you take the example of Thane, you have the 300 beds that are fully ready. The capacity of those hospitals, but we only commission around 100, 150 beds, census, non-census all together. As we reach 70%, 80% occupancy within this, we then add -- all we have to do operationalize the other beds is add more manpower. Rest everything is fully done up in the hospital. The capex required is fully incurred.

Abhishek Maheshwari: So that should close as and when occupancies improve?

Abhinay Bollineni: Correct.

Moderator: The next question is from the line of Anshul Agrawal from Emkay Global.

Anshul Agrawal: The first question is on Thane. Why would the losses increase on a Q-on-Q basis despite the spurt in revenues? Have we opened new beds here because you mentioned that there's no change in Maharashtra cluster beds?

Abhinay Bollineni: No, no. So Thane in Q1 was only commissioned for 45 days. So it's not the full quarter.

Anshul Agrawal: Okay. In that sense, if I add up the losses in the revenue of the Thane cluster in Q2, would that give some sense about the fixed cost for this hospital?

Abhinay Bollineni: Fixed cost is around INR12 crores a month, INR12 crores, INR12.5 crores a month.

Anshul Agrawal: Okay. So if we haven't

increased the number of beds. This loss should sort of certainly move down in Q3?

Abhinay Bollineni: Correct.

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Anshul Agrawal: Got it. Any guidance for the Bengaluru units in H2 on the EBITDA losses or the trajectory of EBITDA losses, given the fact that it's a INR25 crore loss. A possible clarification, are there any costs of PES hospitals included in the current quarter?

Abhinay Bollineni: Yes, there is 30% of the loss that we're showing is from the PES hospital. There's been a delay in getting permission. We are hopeful that this month, the permission should come and we should commission the hospital. So doctors have been onboarded. Some of the doctors have been onboarded. Staff has been onboarded. As a result, we are having these operational losses. But once we get the license this month, we should start commissioning the hospital from next month. But as far as Mahadevapura is concerned, the hospital is now 2 months old. The performance of the hospital is quite good. It's better than what we had expected it to be. And we had also completed the first liver transplant within the first 45 days. So we're pretty confident about the ramp-up at Mahadevapura and that it will break even within the 12 months of commissioning the hospital.

Anshul Agrawal: Any indicative numbers that you would want to suggest for H2 for these 2 units with regards to EBITDA losses?

Abhinay Bollineni: Difficult to predict that. The ramp-up has been quite good at Mahadevapura. And if we commission beds in December, we're hopeful that, that also will ramp up quite well. So I think the next 2 quarters are crucial for us to stabilize both these hospitals, and then we'll be able to give forward guidance on how things will stabilize.

Anshul Agrawal: Sure. Any update on insurance empanelment for the new hospitals? Would you like to share any colour on this?

Abhinay Bollineni: Yes. I think the top five companies, we've started interacting with them and building a deeper relationship. We are hopeful that at least the top 5 companies by Q4, we should be able to complete all empanelment.

Anshul Agrawal: This would be for all new hospitals, right, Thane, Nashik and Bangalore?

Abhinay Bollineni: Correct Nashik, Bangalore and Thane, but for only the top 5 companies.

Anshul Agrawal: Got it. Just one last question from my end. Would we be able to sort of quantify the CGHS price hike benefit for our sort of specialty mix?

Abhinay Bollineni: Should be around 20% on the CGHS business.

Anshul Agrawal: And is there any way that we can attract more CGHS patients during the ramp-up phase for these new hospitals?

Abhinay Bollineni: we've always done a good volume of work from CGHS across Andhra and Telangana. And I think among all the hospitals here, we've done -- we do the most volume of work as far as CGHS and related players are concerned. So, I don't think we put any special additional focus to drive more patients, but we'll continue to treat whatever comes by.

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Moderator: The next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: In the presentation, you have mentioned about achieving breakeven in the next 2 months at Thane as well as the Mahadevpura hospitals. In this, what is the bed count you are building in? And also, in terms of cost, you mentioned about INR12 crores, INR12.5 crores in Thane. Is this the cost which you are looking at when you talk about breakeven?

Abhinay Bollineni: Yes. So as far as Thane is concerned, Alankar, we will break even in the next 2 months based on how things are going in the last few months. As far as Bangalore is concerned, we are talking about 12 months from the time we commissioned the Mahadevapura. There's a typo error on the presentation, which we will update right after this meeting. As far as Thane is concerned, we're talking about INR

12.5 crores of fixed cost, plus you add consumable costs based on consumption. So typical breakeven, we are seeing it will be around INR15 crores. And if you look at the month of October, we have closed around INR12.5 crores, INR13 crores of revenue.

Alankar Garude: Understood. And this is on what bed count, Abhinay?

Abhinay Bollineni: This will be around 130 beds operational.

Alankar Garude: 130 operational beds. Okay. And I mean, with more beds expected to be operational soon, do you expect the cost number to increase? And could that push out the breakeven time lines or irrespective of that, we should be at breakeven in the next 2 months?

Abhinay Bollineni: I think we should break even in the next 2 months. But the margins may not be very strong because we'll keep operationalizing more and more beds. So a large part of the doctor count is already factored in, in the current cost structure today. But as you add more manpower, yes, there could be 1 or 2 months that will slip and go back.

Alankar Garude: Got it. The second question is on this -- so we have seen a lot of news flow around the discussions with insurance companies and the hospital industry. Now given for us, particularly, we have opened a bunch of new hospitals and we are going to open a few more new hospitals, we need a faster insurance empanelment.

We've already seen some delay, in Nashik you spoke about completing the empanelment with the top 5 by the end of this fiscal in Nashik, Thane and Bangalore. But just directionally, if you can share your thoughts, can any delay in insurance empanelment be a risk for us, especially given that we are in an expansion phase at the moment?

Abhinay Bollineni: So Nashik now just last week, we completed 2 out of the top 5. The remaining 3 also we should complete soon. Things are now moving quicker. I think there's a lot of ambiguity on GIC for quite some time. I think that ambiguity is now settling down and things are moving. We are also trying to express our thoughts around how we can empanel things faster for newer hospitals, and we are working with these insurance companies on that.

Alankar Garude: Okay. And any update on Thane, what's the current status?

Abhinay Bollineni: On the empanelment status?

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Alankar Garude: Yes.

Abhinay Bollineni: I think again, the top 5 companies, we should be able to complete by Q4.

Alankar Garude: Sorry, I meant the current status, what would be the status as on date?

Abhinay Bollineni: On the top 5, we have just one empanelled in Thane.

Alankar Garude: Got it. Got it. And one final question. See, broadly, if I look at our network, Abhinay, out of the 24, 25 facilities, we have a few hospitals which are loss-making. You spoke about breaking even in the currently operational ones by the first quarter. So in terms of the overall margin profile going into FY '27 and beyond that as well, would it be safe to say that we should expect a significant delta as far as profitability is concerned, given many of these newer hospitals will start going towards the corporate EBITDA margin?

Abhinay Bollineni: Correct. But it will take time before we get there, Alankar. Most of these hospitals got commissioned in the last 10 months. And the first year typically goes in kind of reaching a breakeven point, and then it will take time for them to reach a double-digit margin. So definitely, the delta is there, but it will take a good number of quarters before we're able to bring it to a corporate level EBITDA margin.

Moderator: The next question is from the line of Deepthi Rajulapati from Axis AMC.

Deepthi Rajulapati I hope, I'm audible? So my question is regarding CGHS exposure that you have. You mentioned 20%. I believe that is AP and Telangana, particularly. But if we see overall mix, how much would it be? And we shouldn't include Aarogya Sri, I believe, because it's a state scheme. What is it particularly CGHS share

of it?

Abhinay Bollineni: So, we refer to 15% to 20% will be the price hike impact. But as a percentage of revenue, it will be for these 3 clusters for where Andhra, Telangana and Nagpur, this is where we do most of the CGHS work currently. That number will be around 10% of the revenue.

Deepthi Rajulapati Okay. And in overall revenue?

Abhinay Bollineni: Overall revenue -- for these three clusters, it will be 10% of the business will come from CGHS and related companies, excluding Aarogya Sri.

Moderator: The next question is from the line of Rahul Jeewani from IIFL Capital Services Limited.

Rahul Jeewani: Yes. Sir, in Nashik, there has been a significant delay in terms of ramp-up. So it has been almost now 1.5, 2 years since we commissioned that hospital, and we are still incurring losses there. So where do you think that ramp-up in Nashik has been, let's say, slower versus our expectations versus what we have been able to achieve in Thane or even the first Bangalore hospital?

Abhinay Bollineni: Rahul, Nashik got commissioned in December, Jan last year, not 1.5 years. So we got the OC in January. So the doctor onboarding started from mid-Jan, February. So we're 10 months now into commissioning the hospital. I think there is some confusion around that.

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Number two, as a market, if you look at Nashik, unlike what is happening in Thane and in Bangalore, 35%, 40% of the hospital business in these hospitals in this cluster comes from CGHS and related parties. Now for empanelment of CGHS and related parties, you need something called a 17 (2)(ii)b license, which typically takes time.

Even historically in Andhra and Telangana, it has taken us at least 9 months to 1 year to get that after the hospital gets commissioned. So we should get that license income tax certificate over the next -- we're hopeful we should get that in the next 2 months. If we get that, then we start applying for ECHS, ESI, CGHS and all the related parties. And since those accounts for almost 35%, 40% of the business, that is a significant contributor.

Now what you're seeing the revenue is all cash revenue. Even insurance accounts for 30%. So between these 2 payers, it is 70% and 30% is only cash. And because 70% is credit, we are finding it difficult for all the doctors who have committed to come and join KIMS to join before this empanelment is done because they fear that their business could get impacted. Unlike in larger cities like Thane and in Bangalore, doctors are confident that even if insurance is not there, their patients are willing to pay cash and go for reimbursement later. And the contribution from CGHS and such companies is very, very less.

Rahul Jeewani: Sure, Dr. Abhinay. So if we get the CGHS license in Nashik, then obviously, with the revised rates on the CGHS business, post that, Nashik should see a good ramp-up?

Abhinay Bollineni: Good ramp-up post insurance and corporate empanelment. So what you're seeing today is only 30% -- whatever revenue is coming is all the cash revenue, which typically accounts only for 30% of any hospital in Nashik, whether it is Medicovert, Sahyadri, 70% is from insurance and CGHS-related businesses.

Rahul Jeewani: Sure, Dr. Abhinay. And in terms of insurance empanelment for the Nashik market, how many have we empanelled as of now? And how many are you targeting, let's say, by the end of the year?

Abhinay Bollineni: So, 2 of the companies have been empanelled, 2 of the top 5, where at least before the end of the financial year, we're looking at getting 3 more, the remaining 3.

Rahul Jeewani: Sure. And let's say, generally, in terms of the pushback, which has been there between the hospital and the insurance companies and the fact that we have commissioned several hospitals across newer markets. Now in terms of when you are negotiating with these insurance companies, do they squeeze you to lower pricing or provide them

a better pricing in the core

markets as well in return for coverage in the newer markets? So does that happen or that's not the case that the negotiation happens on a market-by-market basis?

Abhinay Bollineni: Fortunately, that has not happened so far, Rahul, but it's not that they're trying to squeeze just as we're trying to arrive at an optimal pricing because a lot of the hospitals in these markets are 10 years old, and we are asking for better pricing given our infra is new, given the kind of technology investment is quite new. So that's why it's taking a little time between having these bilateral negotiations to get the right pricing for the hospital. But it has no bearing on Andhra and Telangana or Nagpur.

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Rahul Jeewani: Okay. Sure, Dr. Abhinay. So last question from my side. You said 15%, 20% price increase benefit on the CGHS business in the 3 clusters, which is around 10% of the revenue for those clusters. So when do we expect the margin benefit from the CGHS price increase to start flowing into our numbers? And would that help us to offset some of the margin pressures, which we would see over the next 2 quarters because of losses from the new hospitals?

Abhinay Bollineni: Yes. I think Q4 is when we should see the full impact of CGHS.

Rahul Jeewani: Sorry, please go ahead.

Abhinay Bollineni: I'm saying it will definitely offset some losses that we are seeing from the newer hospitals.

Rahul Jeewani: Have we quantified the margin benefit, which we could see at an overall company level from the CGHS price increase benefit?

Abhinay Bollineni: It will be around 60% of the incremental revenue, Rahul.

Rahul Jeewani: Dr. Abhinay, I was asking in terms of, let's say, these CGHS is around 10% of the revenue for those 3 clusters, maybe at an overall company level, the contribution could be 5%, 6%. Now on 5% of the top line, if we assume a 20% kind of a price increase, that could potentially add around 100 basis points to your overall margin. So is that math broadly correct or the number which you might have worked internally is different than that?

Abhinay Bollineni: So we are expecting INR1.5 crore growth in revenue per month and 60% of that is around INR90 lakhs, so around INR10 crores, INR12 crores of EBITDA growth for the next financial year.

Moderator: The next question is from the line of Shivam Dave from MIV Investment.

Shivam Dave: I just had one question for the Bengaluru hospitals that are coming up. There are a lot of competitors, Narayana, Aster DM and Apollo as well. So what is the competitive dynamics of that city? And for 2 regions where you're coming up, one is Mahadevapura and the other one is Electronic City. How is the competitive intensity in those geographies?

Abhinay Bollineni: So we have the management team of Bangalore also on the call. I'll just allow them to answer this.

Shivam Dave: Yes.

Sreenath Reddy Yes, Sreenath here. So yes, any market for that matter, there's always competition. So no doubt Bangalore has got all the big players. Having said that, if we are able to identify the right location, which we have already identified and starting these hospitals and also the right talent in terms of the clinical talent and the management team, which we have at this stage all with us. And therefore, we feel we are very confident that we should be able to do well in this particular geography in spite of the competition. And there are certain micro markets, right, which are still untapped. So some of these micro markets, also there are potential even for future expansion also in Bangalore, not that every micro market is covered in Bangalore.

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Shivam Dave: Okay. Got it. So in some specific markets where you're coming up, you believe that the intensity is a bit lesser?

Sreenath Reddy That's correct.

Moderator: The next question is from the line of Dhananjai Bagr

odia from Alchemy.

Dhananjai Bagrodia: Most of my questions are answered. Very strong set of numbers. Actually, my question is for Abhinay. Abhinay, predominantly right now in all these new markets we're entering, we've done a good job in the past markets. What is our right to win? And are we going to be positioning ourselves as most premium, a little cheaper than the competitors? How are we thinking about these new micro markets in terms of positioning?

Abhinay Bollineni: I think the intent is to keep it as affordable as we can. And in most markets, we have gone, we have always tried to price ourselves a little lower than competition to keep it more accessible and affordable. We've been able to maintain the same positioning in both Thane and Bangalore. We will continue to do the same.

Dhananjai Bagrodia: Okay. And -- but in terms of, see, that could be versus the A+ players. But let's say, today, a government hospital or a little smaller specialty hospital, how would we look to be positioning versus them? I'm just trying to wonder, are we, today when a customer thinks of us in these new areas when there's no prior history, who is coming to us? What kind of customer is thinking of coming to us?

Abhinay Bollineni: I think those are not the right comparisons. We should look at hospitals that are comparable in that micro market. So like it will be a Manipal or a Narayana in a state with Jupiter.

Dhananjai Bagrodia: Okay. So, we'll keep our pricing similar or relatively affordable compared to them?

Abhinay Bollineni: Affordable.

Dhananjai Bagrodia: Okay. So we are not playing -- we don't want to just keep going ARPOB, which in one end. We wanted to improve our specialty mix and along those lines, right?

Abhinay Bollineni: We're more focused on building volume and that's why we're building large format hospitals. We believe that there is a lot of volume in each of these micro markets. And if we keep the pricing more affordable, we can attract a larger audience.

Dhananjai Bagrodia: Okay. Fair enough. Congratulations, you've done a fantastic job.

Moderator: The next question is from the line of Kunal Randeria from Axis Capital.

Kunal Randeria: Sir, firstly, on O&M. So your Sangli and Guntur hospitals, I believe you are getting a revenue share, right? So EBITDA loss or profit really that wouldn't matter to you from an accounting perspective, right?

Abhinay Bollineni: Correct. It does not matter. It's just that we are trying to show that in Maharashtra, Sangli is doing quite well on the EBITDA side as well.

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Kunal Randeria: Fair enough. Fair enough. Sir, second question, I have a couple of questions around Kondapur unit. Now your bed expansion will go up to 550 next year.

Abhinay Bollineni: Incremental.

Kunal Randeria: Yes, I mean, total will be 550, I believe.

Abhinay Bollineni: No, no, 550 will be incremental. So total will be around 770 beds.

Kunal Randeria: All right. All right. Now sir, you already run a 220-bed hospital. It's a fairly profitable hospital with 30% plus margin and revenue of like INR350 crores. So I'm just wondering now with your bed count going up, let's say, 1.5x extra I mean higher, how is the fixed opex move, number one? Number two, this average of making INR1.5 crore, INR1.6 crore a bed revenue, is that possible in future also? And thirdly, since Gachibowli Hospital is just a few kilometers away, will there be kind of cannibalization also?

Abhinay Bollineni: No, I think Kondapur is seeing very strong demand. In fact, we are losing a lot of our patients because of bed capacity constraint. We are pretty confident that this new 750, 800 bed hospital will ramp up very well, better than our expectations over the next 3 to 4 years since it's a large format hospital. I don't think both Gachibowli, Kondapur have very different catchments. Though they are only 4 kilometers apart, the catchments are very different for e

ach of these

hospitals. I don't see any cannibalization.

Kunal Randeria: Right, sir. And the cost, sir, how would that be moving up?

Abhinay Bollineni: Yes, it depends on all the new specialties when we onboard them. I think the first year will be a little dynamic because we'll be onboarding a lot of new specialties that don't exist today like oncology, transplant and stuff. So it will be a little dynamic, but we're hopeful that we'll be able to maintain at least a high 20% EBITDA margin.

Kunal Randeria: Right. As far as the revenue is concerned, this is potentially INR1,000 crore hospital in future?

Abhinay Bollineni: Correct.

Kunal Randeria: Great. And just one more, sir. Sorry, on CGHS again. In your payer mix chart, where do you capture CGHS?

Abhinay Bollineni: It fits in corporate.

Moderator: The next question is from the line of Vedant Nilekar from ICICI Securities.

Vedant Nilekar: I just had one question on Telangana and Andhra Pradesh clusters. So, could you please explain the bed additions? And in what particular facilities are they being added like quarter-on-quarter?

Abhinay Bollineni: So right now, we have completed all the capacity addition. There are only 2 hospitals, which is Kondapur, which will get commissioned over the next 6 months, which will be an incremental 550 beds. And then we're rehabilitating beds in the Secunderabad unit, and we may add another

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150, 200 beds in the Secunderabad unit. So that 1,000 beds in Secunderabad will probably can potentially become 1,200.

Vedant Nilekar: 1,000 beds will now become 1,200, is it?

Abhinay Bollineni: But that will take another 3.5 years.

Vedant Nilekar: Okay. And so the addition from -- so the difference from Q1 to Q2 in both Telangana and Andhra. So in Andhra, we have added around 100 beds, and what facility have we added those?

Abhinay Bollineni: In Srikakulam.

Vedant Nilekar: And in Telangana, the difference where has been these beds added?

Sachin Ashok Salvi: In Begumpet 65 beds were added.

Moderator: The next question is from the line of Avadhoot Joshi from Trivantage Capital.

Avadhoot Joshi: Abhinay, the question is on the payer mix on the new hospitals. So I just want to understand, see, I do understand it's not in our control, how we control it for the new hospitals. But if you consider new hospitals in the Tier 2 cities like Nashik and Bangalore and Thane, other 2 cities, how the payer mix grows over the period and how it has impact for both the things like ARPOB and the specialty we want to grow into. What is our understanding in the beginning of the phase and how we want to see it evolve over?

Abhinay Bollineni: Place like Nagpur and Nashik will have similar payer mix. So even if you look at our Nagpur hospital today, 25% will come from CGHS-related businesses. 20% to 40% will come from insurance and the remaining will be cash. So any Tier 2 market will have similar profile like we see in Andhra Pradesh as well.

So Nashik is very similar. When we got in, we know that this is how the business profile will look. But as far as Thane and Karnataka are concerned, we are more focused on cash and insurance kind of -- it's more a cash and insurance kind of a market, maybe around 5%, 10% at best will be CGHS and CGHS-related.

Avadhoot Joshi: So initially, when we have not empanelled at or we are in the process of empanelling at Bangalore and Thane, initially, the volumes would be lower from insurance and cash. The ARPOB will be lower as compared to what we expect going further over the period. Is that the correct understanding?

Abhinay Bollineni: Yes, ARPOBs will expand as over a period of time, the case mix changes. Deliberately, the pricing also has been kept on the lower side currently. As time progresses, the pricing will get corrected, it will improve there and also the case mix keeps changing.

Moderator: The next question is from the line of Alankar Garu

de from Kotak Institutional Equities.

Alankar Garude: Just one quick follow-up. In terms of positioning, we have a slightly more affordable positioning in AP, Telangana. You spoke about the difference in payer mix across the market. Similarly, KIMS Hospital
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how would you contrast your positioning across the newer markets of Nashik, Thane, Bangalore as well as Kerala with, say, Hyderabad?

Abhinay Bollineni: We are focused on a similar positioning, Alankar. We're trying to be more as affordable as we can in each of these markets. We were trying to be more similar to that we have in AP and Telangana.

Moderator: Thank you. Ladies and gentlemen, this was the last question for today. I now hand the conference over to the management for closing comments.

Bhaskara Rao Bollineni: So, thanks for all your insights. Your questions have given a lot more knowledge for future guidance. we were very, very confident wherever we started these new hospitals and we will deliver what was promised. what all we planned, we are moving in the same way, and there are no deviations. We are very, very confident.

And I'm sure the ramp-up is going well. The consultants are also very happy. what we have seen in Bangalore and as well as Thane is that more and more consultants are interested to join. So, we are very-very confident the things will move as we plan. Thank you very much for all your interactions.

Moderator: Thank you very much, sir. On behalf of IIFL Capital Services Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Feb 2026

13th February, 2026

To,
The General Manager,
Department of Corporate services
Bombay Stock Exchange Ltd (BSE)
Phiroze Jheejheebhoy Towers,
Dalal Street,
Mumbai - 400 001.
Scrip Code - 543308
ISIN: INE967H01025 To,
The Manager,
Listing Department
National Stock Exchange of India Limited,
Exchange Plaza, 5th Floor,
Plot No.C/1, 'G' Block
Bandra - Kurla Complex
Mumbai - 400 051.
Symbol - KIMS
ISIN: INE967H01025

Dear Sir/ Madam,

Sub: Transcript of earnings conference call with Analyst / Investors.

In Continuation to our letter dated 04th February 2026, the Company organized a conference call with the Investors/ Analysts on Monday, 09th February 2026 at 09:30 AM (IST). A copy of the transcript of the conference call held with the Investors/ Analysts is enclosed herewith and the same has also been uploaded to the Company's Website at <https://www.kimshospitals.com/investors> > Disclosures under Regulation 46 of SEBI (LODR) Regulations, 2015 > Analysts Calls schedule, PPT & Transcripts > Transcripts, Audio & Video Recordings > FY 25-26.

Kindly take the same on record.

Thanking you,
Yours truly

For Krishna Institute of Medical Sciences Limited

Nagajayanthi J. R
Company Secretary & Compliance Officer

Encl: As above

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"KIMS Hospital
Q3 FY26 Earnings Conference Call"
February 09, 2026

MANAGEMENT : DR. BHASKAR RAO BOLLINENI – FOUNDER AND
MANAGING DIRECTOR – KIMS HOSPITAL
D R. ABHINAY BOLLINENI – EXECUTIVE DIRECTOR
AND CHIEF EXECUTIVE OFFICER – KIMS HOSPITAL
M R. SACHIN SALVI – CHIEF FINANCIAL OFFICER –

KIMS HOSPITAL
D R. NITISH SHETTY – CHIEF EXECUTIVE OFFICER FOR
KIMS BANGALORE CLUSTER
M R. SREENATH REDDY – DIRECTOR – BUSINESS
STRATEGY AND M&A

MODERATOR : MR. RAHUL JEEWANI – IIFL CAPITAL SERVICES
LIMITED

KIMS Hospital
February 09, 2026

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Moderator: Ladies and gentlemen, good day and welcome to the KIMS Hospital Q3 FY '26 Earnings Conference Call hosted by IIFL Capital. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Rahul Jeewani from IIFL Capital. Thank you, and over to you, sir.

Rahul Jeewani: Yes. Hi, good morning, everyone. This is Rahul from IIFL Capital. I welcome you all to the third quarter earnings conference call of KIMS Hospitals. From KIMS, we have with us today Dr. Bhaskar Rao Bollineni, Founder and Managing Director; Dr. Abhinay Bollineni, Executive Director and CEO; Mr. Sachin Salvi, CFO; Dr. Nitish Shetty, CEO for KIMS Bangalore Cluster and Mr. Sreenath Reddy, Director, Business Strategy and M&A. Over to you, sir, for your opening comments.

Bhaskar Rao Bollineni: Good morning to all of you. A hearty and warm welcome to all of you. This happens to be the first meeting in the new year and I take this opportunity to extend the greetings for a healthy and happy year ahead for all of you. Last week, the union budget was presented, which is a quite budget devoid of any frills or fanfare, but conveys a clear resolve for fiscal consolidation. The health sector received a 7% hike over the previous year with a major push to mental health and making cancer and rare disease treatment more affordable. Primary focus was given to creating a skilled health care workforce to cater to the growing disease burden. That way the hospitals will get more skilled people in the new coming areas. Overall, it is a good budget, though it would have been better with increased allocations to health care sector.

Now let us move to financials and operational results for the quarter 3 – 2025-26.

I'm happy to share that this is a record-breaking quarter with the highest-ever revenue crossing the INR1,000 crores mark. Though Q3 is traditionally a weak quarter, I would say that we have raised fast with growing results despite the usual initial pressures associated with the new units because of our new inherent strength and enhanced network.

Q3 FY '26 highlights.

■ Total revenue of INR1,003 crores, a growth of 2.2% on year-on and 3.9% on quarter-on- basis.

■ The EBITDA of INR204 crores, a marginal decline of 0.4% on year-on-year and 2% on a quarter-on- basis.

■ The decline in EBITDA was mainly on account of EBITDA erosion caused by the newer units that commenced operations over the last 9 to 12 months.

■ EBITDA margin stood at 20.4% versus 25.9% in Q3 FY 25 and 21.6% in Q2 FY 26.

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■ PAT at INR52 crores in Q3 FY '26 against INR93 crores and INR72 crores in Q3 FY '25 and Q2 FY '26, respectively.

■ Consolidated EPS for Q3 FY '26 of INR1.3, a decline of 39.9% on a year-on-year basis and 20.1% quarter-on-quarter, respectively.

■ Cash and cash equivalents includes cash, bank balance deposits with maturity less than 12 months and investments in mutual funds at INR206 crores as on 31st December 2025.

Q3 FY 26 Financial Highlights-Consolidated:

■ Consolidated revenue from operations INR998 crores, a growth of 29.2% on year-on-year and 3.9% on a quarter-on-quarter basis.

■ Consolidated EBITDA pre-Ind AS, INR193 crores, a decline of 2.2% on year-on-year and up 3.7% on a quarter-on-quarter basis.

■ Consolidated EBITDA (Pre-INDAS and excluding other income) of INR188 crores, a growth of 4.6% and a decline of 4.3% on year-on-year and quarter-on-quarter basis respectively.

Operational highlights consolidated

Q3 FY 26 Highlights

■ Average revenue per operating debt (A

RPOB) grew by 20.5% and 10.3% on year-on-year and quarter-on-quarter basis, respectively.

■ Average revenue per patient (ARPP) grew by 13.9% and 9% on year-on-year and quarter-on-quarter basis, respectively.

■ IP volume (61,139) grew by 13.2% year-on-year and declined 4.9% quarter-on-quarter basis.

■ OP volumes (5,85,499) an increase of 24.5% year-on-year and a decline of 1.2% quarter-on-quarter.

Other Developments

KIMS entered into an agreement with Andrea Saba for its land to construct and operate a hospital at Chennai for 26 years. The construction is expected to be completed within 2 years.

At KIMS Mahadevapura, Bangalore, 25 transplants were performed just within 3 months, which started functioning.

At Thane, 100-plus life-saving periodic cardiac procedures were done within first 5 months. It's a great achievement for the new unit.

We organised an event on the occasion of World Cancer Day 4th February where good number of Cancer conquerors, their families and doctors participated. The Chief Guest was Padma Vibhushan awardee and famous Cine actor Shri Chiranjeevi.

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I don't intend to burden you with endless list of our clinical achievements. The point of note is that excellent medical care and latest technology is now available at tier 2 and 3 centres wherever KIMS is there. Even organ transplants are taking place at these small towns which was unheard of before.

There have been a galore of accomplishments during the year 2025. I will give brief highlights.

■ We now have 25 hospitals across 5 states.

■ In 2025, we launched 7 hospitals –, Bangalore – Mahadevapura and Electronic city, 2 in Guntur, Kollam in Kerala, Thane & Sangli in Maharashtra.

■ 100 Heart n lung transplants were done in 2025 under the leadership of Dr Sandeep Attawar and his team.

■ Over 430 plus renal transplants across the group led by Dr. Uma Maheshwara Rao and Satya Reddy and his team and his team.

■ Over 120 Liver transplants across the group done in this year under the leadership of Dr. Venugopal and Sachin Daga and Chandan and other group team members.

■ Over 100 organ donations are achieved by impressing the patient attenders to donate their kith and kin for their body so that at least 800 people got benefited out of this entire credit goes to our coordinator Mangadevi and Mr. Advik Bollineni.

■ Over 500 robotic surgeries done across and over 100 robotic Whipple surgeries done, which is a great achievement by KIMS Secunderabad.

■ KIMS Usha Lakshmi Breast unit got 2 Guinness book of world record for largest breast cancer awareness program.

■ Launch of India's first Tulsa Pro for Prostate Cancer care treatment

■ Launch of India's second MRgFUS for tremor treatment

■ Launch of South India's first Gamma Knife for brain tumours

Finally, I would like to say that the new units have started stabilizing and real growth on their path can be seen from coming financial year. We have more expansion in the offering and I would be able to let you know in the next quarter meeting.

Thank you for your continued trust and support.

I conclude now wishing you and your families best of everything in the new year.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Ameya from JM Financial. Please go ahead.

Ameya: So first question is on the newer assets. Is it possible for the management to provide the occupancy number for Thane as well as two Bangalore assets and also the trajectory for the EBITDA losses for these three units in the coming quarters, especially for quarter 4 and FY '26?

Abhinav Bollineni: Yes, may be difficult to gi

ve you specific details on occupancy of each of the assets because both -- all three assets are growing. But I think between Thane and Mahadevapuram in Bangalore and towards the end of Q1 next financial year, we should become EBITDA positive or EBITDA neutral. Electronic City might take a little longer by because we commissioned it only in the month of December. So I think by end of Q3, Electronic City also should turn EBITDA neutral or positive.

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Ameya: Sure. And for Bangalore, particularly or the Karnataka cluster, the ARPOBs are upwards of INR76,000 for this quarter. Should we consider some normalization in these ARPOBs because this is just the first few quarters of the hospitals getting started or you think these ARPOBs are maintainable going ahead?

Abhinay Bollineni: Sreenath, do you want to take that. So I think you should probably discount it a little because we have done a significant amount of transplant work in this quarter. And since the hospital is new, the base is still low, we should probably think of it similar to what it is in the Telangana cluster around INR70,000, INR75,000 crores ARPOBs.

Sreenath Reddy: Yes. To add to Dr. Abhinay, see, these are initial period, but we strongly believe that these trends should more or less continue. There could be some small rationalization, but the trend should continue.

Ameya: Sure. And the second question I have on the Andhra and Telangana cluster, the volumes growth has been for Telangana, it's in low single digit and for, I believe, Andhra, it has declined. Despite bed addition and only operating at 50% kind of occupancy, is there anything to read into this? You expect the performance to improve?

Abhinay Bollineni: No, I think if you look at Telangana, growth, it has been on a year-on-year basis, it's been 20% on a quarter-on-quarter last year same quarter. On the volume growth also from last year -- first 9 months of last year to the first 9 months of this year, there has been a 5% growth in volume and revenue has grown higher. But at a mature cluster to grow at 5% is healthy enough. As far as Andhra, in quarter 3, we had a strike with the state government. So we had not taken Aarogyasri scheme for almost a month, and that's why you see a significant dip in volume. But otherwise, January, things have bounced back and the revenues are as per how it was in Q2.

Ameya: Sure. So Telangana, what optimal occupancy should we assume going ahead at an annual level?

Abhinay Bollineni: Telangana, we should get to 75%, 80%. There are little constraints in Telangana because some beds, we have now shut down because of renovation. Once those beds come into operation, which will happen end of next year, that should contribute to growth. And the new Kondapur facility, like the Kondapur facility today is probably running at 90%, 95% occupancy. It should get commissioned in the next 3, 4 months. Once that gets commissioned, there will be more volume growth in the Telangana cluster.

Moderator: The next question is from the line of Damayanti from HSBC.

Damayanti: My question is on Maharashtra cluster unit. So first, for both Nashik and Thane unit, can you update whether -- like what is the status on NABH accreditation? The second point is the discussion which you are having with the insurance empanelment, what is the update? And since last quarter, have you signed any empanelment either in Nashik or Thane? And also, I just want to understand is NABH accreditation must before you really get empanelment with all the key insurance players?

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Abhinay Bollineni: No, NABH in Nashik, entry level is completed. NABH and Thane is in pipeline, it will take some more time. But both of them -- for both units or any other unit, NABH is not a prerequisite for empanelment with insurance company. There has been some progress i

n both units on

insurance empanelments. But the top 5 companies that we wanted to do, out of which 2 are done, 3 more are to be done. We are almost in the last leg of closure on all 3 for both Nashik and Thane. It should be done by -- before the end of this quarter, quarter 4.

Damayanti: Okay. And just want to understand, I believe your discussion or negotiation with insurance companies took a little longer than what we anticipated. So any point where you really had to negotiate a lot with the insurance companies? And what are the key pushback coming from them before they agree for signing the deal?

Abhinay Bollineni: I think it is -- we always want a better price. We want something higher than what is already being offered, and they obviously want to lower the price as quickly as they can. So that is something that's a bilateral discussion between the hospital and insurance company.

Damayanti: Okay. And you said of the top 5 insurance companies, 2 are already done, like you have closed

the contract. Is it that for Nashik unit or...

Abhinay Bollineni: Nashik and Thane for both.

Damayanti: Okay. And then second question I had on the overall ARPOB and ARPP trends. You did mention there was a component of transplant in new Bangalore unit, etc. But when we look at the trends, it seems exceptionally strong than what we had seen in last few quarters. So is transplant only the moving factor here or something else has changed? And how should we look at the trend in coming years.

Abhinay Bollineni: Are you referring to the group ARPOB?

Damayanti: Yes, the consolidated ARPOB and ARPP numbers.

Abhinay Bollineni: I think see Thane and Bangalore both are high ARPOB markets. And both of the -- all 3 hospitals got commissioned only in the last 6, 9 months. So definitely, the incremental revenue that's coming in from Thane and Bangalore, the ARPOB will be higher, which will pull the overall group ARPOB. And Telangana, also -- if you see Telangana was at 65,000 last quarter has now moved to 70,000 this quarter. So there is growth in 3 regions, Bombay, Hyderabad and Bangalore, all 3 of them being high ARPOB, which is pulling the overall company ARPOB.

Damayanti: Okay. So maybe not at, say, 20% level or so, but we can assume better trend than what we saw in 1Q, 2Q or any quantitative indication will be helpful?

Abhinay Bollineni: Difficult to quantify because these are all different states and each state is growing differently. So I think what we are delivering to you should get to, you should be sustainably growing that.

Damayanti: Okay. And my last question is, if I look at your Telangana cluster, I guess your growth number for revenue in last 2 quarters, they were somewhere in high single digit to low double digit. So

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will that be the range to look forward? Or as you mentioned, there is a headroom to grow better compared to what we saw in the last 2 quarters?

Abhinay Bollineni: I think we've always been indicating that Telangana is a very mature market with significant bed capacity by KIMS and other hospitals. As and when we add new hospitals, we might see this high single-digit move up to a double-digit number. But otherwise, just having mature hospitals to grow at a double-digit number from that cluster will be difficult.

But this year, because we are adding Kondapur, we're adding Kompali and subsequent years, we are adding a few more facilities. We might see double-digit growth for the next 3, 4 years. But again, moving forward, after that, it will be difficult to deliver double-digit growth only from Telangana.

Damayanti: Got it. My last question is actually a clarification. Did you mention for Telangana, you are looking to reach occupancy of 75%, 80%? And if yes, by when?

Abhinay Bollineni: It will eventually get there, maybe another three, four years on the current occupancy. But we'll keep as we keep adding more beds,

the occupancy percentage will fall down but by absolute numbers it will consistently get to that.

Moderator: The next question is from the line of Chirag Gupta from Allegro Capital.

Chirag Gupta: I just wanted to know the net debt position as of December 31?

Sachin Salvi: Chirag, it is about INR2,850-odd crores at a consolidated level as on 31st of December '25.

Moderator: The next question is from the line of Karan Vora from Goldman Sachs.

Karan Vora: So my first question is an extension of previous participant's question with respect to top 5 insurer empanelment. So can you highlight how many of the top 5 have been empanelled for Kerala cluster? And what's the progress on Bangalore? I know it's early days for Bangalore, but still just getting some sense. And what are the targets of reaching those top 5 insurer empanelment if they have not already been for Kerala?

Abhinay Bollineni: So on Kerala, I think Kannur, we are done with most of the impairments. Kollam might take another 2 quarters before the top 5 are fully done. Bangalore should take another 6 months to 9 months before the empanelment is done.

Karan Vora: Okay. Got it. And Kollam, how many are already on board?

Abhinay Bollineni: Top 5, none maybe one, I think I'm not sure.

Karan Vora: Okay. Okay. And Kannur, all top 5 are done?

Abhinay Bollineni: Yes.

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Karan Vora: Got it. And the second question is with respect to Bangalore. So in the previous presentations, previous quarter's presentations, the kind of capex we have given for Bangalore, it seems like these are asset-light expansion. So any rental number which you are calling out or as a percentage

of sales is also fine, which we might be paying to the owners of those land or properties?

Abhinay Bollineni: I think the total rent that we have, the commitment is around INR2.5 crores per month for both the assets put together. Half of the Mahadevampuram asset is owned by the company. The remaining half we pay lease. Electronic City is on a revenue share model. But I think the fixed commitment is towards INR2.5 crores a month.

Karan Vora: Okay. And sorry, you mentioned Electronic City is half owned, right?

Abhinay Bollineni: Correct. Sorry, Mahadevampuram is half owned. Electronic city is on revenue share.

Karan Vora: Okay. Got it. And the revenue share will be over and above the INR2.5 crores per month fixed rentals?

Abhinay Bollineni: No, no, it is part of it.

Karan Vora: Okay. Okay. Got it. And the last question is with respect to ARPOB. So whatever ARPOBs I think for Bangalore, you've already mentioned that there could be some moderation. But for other geographies, whatever ARPOB we see in Q3, should that be considered as the new base of ARPOB?

Abhinay Bollineni: Yes. For Karnataka and Maharashtra, we should still give it more time before it stabilizes. But other clusters, yes, you should more or less take that ARPOB.

Moderator: The next question is from the line of Kunal Randeria from Axis Capital.

Kunal Randeria: So, it's on your payer mix. Sir, your Aarogyasri contribution has gone down. Obviously, that is because incremental revenues are largely coming out of outside of Telangana market. So as we expand in other states, are you also getting empanelled for those state schemes?

Abhinay Bollineni: No, we only are empanelling state schemes in Andhra, not in other states.

Kunal Randeria: Okay. It's only in Andhra. Okay. So your institutional bed contribution as well as revenue contribution should keep coming down over the years?

Abhinay Bollineni: Correct, from state government schemes.

Kunal Randeria: Sure. That's very helpful. Secondly, again, on ARPOB, I'm not sure if it's covered earlier or not. But in the last couple of quarters, Andhra Cluster has seen very sharp ARPOB as well as ARPP growth. But there has been some moderation as far as the volumes are concerned, IP volume. So I just want to u

nderstand what is happening over here?

Abhinay Bollineni: No, even volume growth are growing very healthy. It's just that last quarter we had a strike with the state government on Aarogyasri payment because of which almost for 45 days, 45 to 60 days

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we had a, the entire one-month Aarogyasri was completely banned in all hospitals and 15 days before and 15 days later it took time for us to scale down and ramp up. So that disruption caused us decline in volume. But otherwise we don't see any issue in volume growth or revenue growth in Andhra.

Kunal Randeria: Got it, sir. And the reason for ARPP growth, I mean, is the case mix changing very heavily or just the payer mix?

Abhinay Bollineni: So even the case mix is some hospitals, we have started onco services like in ongole, in Vizag, we have started onco services. Transplant is picking up in a big way in Andhra. So it's a little bit of everything, case mix, pricing and the payer mix.

Kunal Randeria: Sure. And just one more, if I can. Sir, I noticed in this time down in the presentation, there are - there is no slide on expansion plans. So should I assume that what was there last time is the same thing?

Abhinay Bollineni: Yes, you should assume the same thing. We have no new hospitals in addition to what we have already announced.

Kunal Randeria: Right, sir. So Chennai is the only one that you would have announced. So other than that, there's nothing much coming.

Abhinay Bollineni: Yes.

Moderator: The next question is from the line of Anshul Agrawal from Emkay Global.

Anshul Agrawal: A few clarifications in the net debt number that you have mentioned, does that include all the payments made to our partners for bringing down the minority stake to 9%?

Sachin Salvi: So yes. So it is a net debt position as on 31st of December 2025. It includes all the debt which we have raised for the expansion. For minority, we do not get any -- we have not raised any debt. But of course, it is out of the internal sources. So to fund the internal sources, you have to raise working capital requirement.

Anshul Agrawal: So that's exactly what I wanted to understand. There are no payments due to get that minority stake to 9%, which have not already been paid?

Sachin Salvi: Yes.

Anshul Agrawal: Perfect. Second question, sir, that I had was, is there any update on the new units that were going to be commissioned in AP? I thought in the last slide, I think we mentioned Onngol was supposed to get 50 incremental beds added in Q3, which in the AP cluster doesn't seem to have

shown up. And Anantapur is expected to be commissioned in the current quarter. Could you help us with an update on these 2?

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Abhinay Bollineni: Onngol 50 beds got commissioned. That's what I just mentioned 3, 4 a week ago. Anantpur towards the end of March, we should commission the cancer center and add an incremental 75 beds of the 250 beds that we are supposed to add. Rajahmundry, were again stated towards the end of Q4. Kundapur 2 end of Q1, we are stated to start. And Kompali also from March, we will start commission the hospital.

Anshul Agrawal: Got it. Very clear. Also, just wanted to check, there's a dip in occupancy and profitability in the Kerala cluster. I understand seasonal trends, etc. But given that we are in the ramp-up phase in these 2 hospitals, any particular reason to call out for this dip in occupancy and profitability in the Kerala cluster?

Abhinay Bollineni: One of the hospital at Kerala -- both the hospitals are doing extremely well. In one of the hospitals, we had some onetime expenditure, which we had to write off in the last quarter. But otherwise, both hospitals are doing extremely well. Kollam is ramping up -- sorry, Kannur is ramping up quite well. In fact, December, Jan have been even more promising than the results of Q3

. Kollam is just 6, 7 months old. It should breakeven in the next 1, 2 quarter.

Anshul Agrawal: Got it. Just one last question from my end. In the previous con calls you'd mentioned that Thane, because of the ramp up we are seeing at least on a monthly run rate basis expectation is around break even somewhere in the month of January. Are the trends suggesting the same or should we wait for end of Q1 '27 for Thane to break even?

Abhinay Bollineni: No, trends are suggesting that towards the end of Q4 we should break even, but just to be on the safer side we have also indicated end of Q1. But right now things are going quite positive, there are some incremental costs that we're incurring on a month-on-month basis, but it's going better than planned.

Anshul Agrawal: Got it. Just one last question if I can squeeze in. For the Bangalore cluster, is Q3 an apt indicator for the overall cost for both hospitals or are there certain costs that you would think would sort of crop up in Q3 in the Electronic City hospital, or is it a full quarter of costs that have come up in the Bangalore cluster in the current quarter?

Sreenath Reddy: Yes, I can answer that. See, Electronic City the cost what you are seeing is only for 1 month, right? Because we just started in the month of December, but for the Mahadevapura unit it is the full cost. Yes, year-on-year there could be some smile minor increase in overall cost to the extent of 7% to 8%, but having said that most of the costs have been factored in. Electronic City per se the cost what you see is just 1 month.

Anshul Agrawal: Great. Thank you so much.

Moderator: Thank you. The next question is from the line of Rahul Jeewani from IIFL Capital. Please go ahead.

Rahul Jeewani: Yes, hi sir. Dr. Abhinay you indicated about Telangana being a mature market and a competitive market. But if we look at our occupancies, our occupancies are still let's say for first 9 month on an operating bed basis hovering somewhere around 52%, 53%?

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So with the scope which we have in terms of driving occupancy improvement, shouldn't our IP volume growth be slightly faster within the Telangana cluster because first 9 months the IP volume growth has been only let's say low to mid-single-digit. So just your thoughts in terms of how you see these dynamics playing out with respect to occupancies in Telangana and the potential for IP volume growth?

Abhinay Bollineni: See, Rahul, if you actually look at the Telangana occupancy, so occupied beds divided by the operational beds, correct? These are at around 52.5%. Now in this, see in Secunderabad we don't, we are renovating almost 200, 250 beds, which will only get commissioned in the next financial year. So actually on the beds that are available, we're running at 80%, 85% occupancy. It's just that those beds, it shows in the census beds, but they are under renovation. So that is one reason why we're not able to show more volume growth in the Secunderabad facility. As far as Kondapur is concerned, it is already running at 95%, 100% occupancy and we are losing patients because of space constraints.

Once we move to the new facility, there will be significant volume growth in that hospital.

Sunshine are the other two assets and they've been growing consistently on a year-on-year basis.

It is only since the bed capacity is spread over four facilities, Sunshine has been growing on a

healthy basis on a year-on-year basis even on volume.

These two facilities where there is more growth potential, we are limited with bed capacity, which should get sorted by one next quarter and one maybe in the next 18 months.

Rahul Jeewani: Okay, sure, sure Dr. Abhinay. So basically Secunderabad and Kondapur. So when would these 200 beds which you are renovating at Secunderabad, when would they come online?

Abhinay Bollineni: Towards the end of next year, second half of next financial

year.

Rahul Jeewani: You mean FY '27?

Abhinay Bollineni: Correct.

Rahul Jeewani: Sure Dr. Abhinay. And with respect to the new units, your presentation talks about the fact that the doctor onboarding at the Thane hospital is 90% over. So can you also talk about in terms of let's say the doctor onboarding for the two Bangalore hospitals in terms of where are we in terms of our targets for doctor additions in these two hospitals?

Abhinay Bollineni: Yes, I'll let Nitish take that.

Nitish Shetty: Yes, Rahul, most of the onboarding of the doctors in all specialties been done in both the hospitals. Probably a very few people might be added in the future as the volume growth, but at this moment of time considering all quaternary case specialties, tertiary case specialties, all doctors are on Board. We are close to 200 doctors between two hospitals already on Board.

Rahul Jeewani: Sure Dr. Nitish. So 200 doctors you have combined across both the hospitals as of now?

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Nitish Shetty: Yes, that's right. Close to 120 in Mahadevapura and other 90 doctors in the Electronic City unit.

Rahul Jeewani: Okay, and so for the first Bangalore hospital Mahadevapura now the costs or let's say the losses should keep coming down. But for Electronic City given that we only had 1 month of contribution, maybe for Electronic City we might still see an increase in loss going into fourth quarter?

Nitish Shetty: Not increase in the loss because we, December month, January month, January has come down and as the revenue ramps up which we are confident of, the losses only should come down.

Because all the doctors, all the fixed costs already been done.

So it's all a matter of the revenue ramping up and our revenue ramp up in Electronic City has been good and we are confident that the losses only will come down and eventually lead to the breakeven like indicated by Dr. Abhinay we are targeting somewhere in the second quarter of the financial year '26-'27.

Rahul Jeewani: Sure, sure, sure. And Dr. Abhinay you talked about Thane and Mahadevapura achieving break even in 1Q and Electronic City in third quarter of fiscal '27. So just to clarify, do you mean break even for the quarter or let's say break even on an exit run rate basis in these quarters?

Abhinay Bollineni: Towards the end of the quarter we will break even for that quarter.

Rahul Jeewani: Okay, so you will break even for the quarter. So Thane and Mahadevapura by 1Q FY '27 and Electronic by 3Q FY '27 break even for the quarter.

Abhinay Bollineni: Correct, correct. And Rahul, what we are indicating when we say doctor onboarding is completed, see doctor onboarding is a continuous process even in mature hospitals like in Telangana and Andhra. So what we've completed is for, Phase 1 of growth which is the first 24 months of growth.

We will keep adding more doctors for more growth in the future because all these hospitals are large format hospitals, they're more than 300 beds in each in each location. So it will be an ongoing process before we onboard doctors and that's why costs keep going up sometimes in between we find good doctors, we take them and the cost goes up. Break even might get delayed here and there a little, but as long as the revenue is growing and the losses are diminishing, it's in the right direction.

Rahul Jeewani: Sure. And given that all these 3 hospitals are growing, can you also talk about in terms of the operationalization plan for incremental beds at these 3 hospitals?

Abhinay Bollineni: I think we have now enough capacity first to breakeven. Our focus is to prioritize breakeven in all of these hospitals. We have enough capacity operationalized in all of these hospitals. All the capex for all the 3 hospitals are fully done, there is no incremental capex that we'll have to incur for more beds up to each of their capacities. It's about adding more manpower which is not

going

to take more than 2, 3 months. As and when we see the occupancy ramp up happen and we feel there is pressure on the system to add more beds, we'll operationalize the remaining.

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Rahul Jeewani: Sure. And last question before I join back. So now obviously if we look at these four hospitals, Nashik, Thane and the two Bangalore ones, maybe in Thane and the two Bangalore hospitals we have seen a very good traction in terms of revenue ramp up, but Nashik let's say still continues to be slightly behind in terms of ramp up.

So do you think that maybe a better strategy is to have or add hospitals largely in Tier 1 markets where let's say ramp up is relatively faster, easier as compared to let's say some of these Tier 2 Tier 3 markets like Nashik and if you can also talk about where let's say Nashik hasn't gone according to our plans? Thank you.

Abhinay Bollineni: Nashik yes, there is, it took us 13 months to achieve break even and in January we are EBITDA positive in Nashik, Rahul. We've achieved almost INR8.5 crores revenue and we turned EBITDA positive. We got empanelment of CGHS -- sorry ECHS in Nashik, we should start being able to take patients from next month.

The only difference in a Tier 2 and a metro market is the opportunity size is the same, in fact there is more opportunity in Tier 2 because of lesser competition, the ability to consolidate work there is much easier and fast much easier. But the only challenge is because the payer profile in Tier 2 markets you have a mix of cash, insurance and corporate. Insurance and corporate because they take time and it's a very price sensitive market, that is why the delayed ramp up. But otherwise fundamentally we don't see any issue in Thane or Bangalore because these two are very high cash paying markets.

Even though insurance empanelments are getting delayed, they are getting substituted by cash business, which may not be the case in smaller towns like Nashik. But you know we're now last month break even at the 13th month of operations has given positive more positive feedback in the entire system.

Now with the empanelment of corporates such as CGHS and ECHS in the next 2 months, we don't see any fundamental challenge in the growth story of Nashik. It was a little slow, likewise in Nagpur but overall, things in Nagpur have turned around significantly well in '24, 36 months. So Nashik we don't see any, we have similar confidence levels for Nashik too.

Rahul Jeewani: Sure Dr. Abhinay. So, with respect to the ramp up which we have seen on onco and transplants in these other 3 hospitals and the fact that for the Nashik market, obviously we cannot do onco, so has that also been a reason why Nashik has seen a slightly slower ramp up as compared to these other 3 hospitals?

Abhinay Bollineni: No, I wouldn't say that because even in Thane we have not done any transplant work yet. It's just that even on the broad specialties, we've just because of delayed empanelment from insurance which is almost 30% of the business and being a price sensitive market and the remaining 30% of the business there is from public sector companies and only 25%, 30% of the business is from cash.

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And with only one payer to scale up in Tier 2 markets is a challenge, like in the case with Andhra and most all our hospitals in Andhra. So if you actually look at just the cash-to-cash business of KIMS in Nashik today versus the other competition in Nashik, we're probably the largest in terms of cash as a payer in in that market already in less than 12, 13 months.

Rahul Jeewani: Okay, sure. Understood. I will join back the queue. Thank you.

Moderator: Thank you. The next question is from the line of Alankar Garude from Kotak. Please go ahead.

Alankar Garude: Hi, good morning everyone. So on a like-for-like basis, how would

you compare your tariffs in

Mahadevapura and Electronic City with other similar hospitals in those respective micro markets?

Sreenath Reddy: Yes, so between Electronic City and Mahadevapura internally there is no change in the pricing, both offer same pricing. Vis-a-vis in respect to competition, we are slightly priced at least at least I would say that around 15% lower in the initial days. We may rationalize it in the coming few months, but yes, we will be slightly at least for some more time, will be slightly lower compared to competition.

Alankar Garude: Got it, sir. The second question was can you take us through your thought process behind entering Chennai, which are the leading factors that drove this decision? And maybe a second question there would be are you open to looking at further expansion in Tamil Nadu?

Abhinay Bollineni: So, like we earlier mentioned, Alankar, we've always felt there is a lot of micro markets within these larger cities which are underserved, like in Bangalore, like in Bombay we felt there were many micro markets underserved. And very clearly with the way things are ramping up, that thought process of these being underserved is turned out to be true. Likewise in Chennai we feel there are at least two, three micro markets visibly underserved today.

We want to focus on being able to start new hospitals there and attract talent. And again,

strategically a lot of patients from Andhra go to Tamil Nadu for treatment, a lot of the people in Tamil Nadu at least 20% of the population there are Telugu speaking, the brand is well established, the doctor culture among the doctor community there is a strong goodwill and even now with Bangalore and Thane success that goodwill is even getting stronger because even outside the core markets we've been able to do quite well.

So, given the size and the opportunity for consolidating one more state, it will -- that's the reason why we looked at Chennai. Earlier we'd looked at Chennai, but we didn't get the right land parcel. Now that we got a very good land parcel, we said let's start the story in Chennai and that's the only missing link in consolidating South India.

So with Andhra, Telangana having such strong leadership position, now with Karnataka, Bangalore both the hospitals doing well and adding more facilities, Kerala doing well and adding more facilities, the only missing link in the South story would be Tamil Nadu and that's why we thought it's, it makes logical sense for us to enter that market.

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Alankar Garude: So does it mean Dr. Abhinay that apart from possibly looking at more expansion in Chennai, you'd also be open to Tamil Nadu, the other cities in Tamil Nadu or would it be just Chennai for the next few years?

Abhinay Bollineni: No, I think we're pretty open. We're not closing our mind on the other cities of Tamil Nadu. But at this point in time, we've not looked at anything, we're only focusing on Chennai as a market.

Alankar Garude: Got it. And similarly if you can comment about Karnataka state as well. We have two in Bangalore with more in the works. Any plans of looking at other markets in Karnataka?

Abhinay Bollineni: Nothing I think at this point in time, we have enough on the table for Bangalore. We want to do few more hospitals in Bangalore, we have a land which is already part of the company, we want to start work on that hospital too. We feel we can add more bed capacity in Bangalore the kind of doctor traction we've got with these two facilities. We are pretty confident that with more facilities and more micro markets being underserved we should be add -- adding more capacity here and expanding this. After that then we could probably potentially look at other Tier 2 markets in Karnataka.

Alankar Garude: Got it. And one final question from my side. In the past you've also spoken about adding more hospitals in Mumbai. C

an you share any update on your plans for Mumbai city?

Abhinay Bollineni: Mumbai city we're still exploring all the opportunities that are coming our way, nothing concrete at this point in time. But like we said, overall, we believe there's a large footprint that we can create in Bombay city. We are working towards achieving that over a period of time.

But for the next 12 months I think our focus is to first ensure all the hospitals that we've commissioned and we're going to commission in this financial year to ensure that they all breakeven and there is no drag on the system.

Alankar Garude: Got it. So basically any expansion plans, if at all, in Mumbai are unlikely to be finalized at least for the next 12 months?

Abhinay Bollineni: I wouldn't say that, Alankar, but Yes we're definitely exploring opportunities as and when something good comes up we might take it up, you may not. But our priority is on commissioning the current, I mean operationalizing and commissioning the current hospital.

Alankar Garude: Fair enough. That's helpful. Thank you and all the best.

Moderator: Thank you. The next question is from the line of Rahul Jeewani from IIFL Capital. Please go ahead.

Rahul Jeewani: Yes. Sir, now let's say large part of our current expansion is nearing completion. So the only large one which we have is Kondapur which should get commissioned. So how do you see your overall debt now trending from a next 1 to 2-year perspective? Has the debt number peaked out?

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Abhinay Bollineni: Yes, I think for the current expansion the debt numbers have peaked out. If we add more new facilities, then it could potentially go up. But for now we're almost done with most of the capex that we planned for 3 years ago.

Rahul Jeewani: Okay, sure Dr. Abhinay. And given that you said focus over from, let's say, a next 1-year perspective is to scale up these existing new hospitals, that would mean that debt should moderate unless and until we don't announce anything large in any of the new markets?

Abhinay Bollineni: Correct. Unless we don't announce anything large, I think debt should moderate.

Rahul Jeewani: Okay. And are we looking for something, let's say, a largish kind of an opportunity in any of the other markets apart from, let's say, a Mumbai market? And what let's say excites you still about

Mumbai?

Abhinay Bollineni: I think the way Thane has ramped up and the kind of confidence we're getting in Bombay, it's a pretty underserved market and I don't see why we shouldn't consolidate and add more capacity.

There is enough opportunity for two, three players to coexist in Bombay as a market. So, I think opportunity is quite large and we were waiting for us to do well in the first few hospitals.

Nagpur, Thane, Nashik we're pretty -- Nagpur anyways is doing extremely well, but Thane also is ramping up well. Nashik we're pretty confident with the way things are moving. So, I think overall Maharashtra being able to consolidate over the long run is a good opportunity beyond South.

Rahul Jeewani: Sure, Dr. Abhinay. And let's say these opportunities which you might be evaluating in Mumbai, would these be asset heavy or asset light expansions for you?

Abhinay Bollineni: I think our preference has largely been to own land and building, Rahul. So I think it will take into account owning the land and building. You know doing an asset light model may be challenging in a city like Bangalore than -- sorry in Bombay, the rental cost will be significantly higher, it will be difficult for us to absorb that cost. But if we're referring to the opportunity in Ghatkopar that's not true, we're not doing any hospital in Ghatkopar at this point in time.

Rahul Jeewani: Okay. And apart from, let's say, the organic expansion, any potential M&A opportunities in any of the markets which might -- where you might be interested in?

Abhinay Boll

ineni: Nothing sizable. There are a couple of small acquisitions, potential acquisitions that have come our way, but nothing significant and sizable at this point in time.

Rahul Jeewani: Okay, sure, Dr. Abhinay. Thanks for answering my questions.

Moderator: The next question is from the line of Alankar from Kotak.

Alankar: Just one question on Kondapur. You said you are operating at 90%, 95% occupancy. Now with this new hospital coming up, how should we assume a ramp-up of it? And I mean, what would be the plans for the existing hospital once the new building is up and running?

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Abhinay Bollineni: In 6 months, once the new building is ready, we will shut down the old hospital. And I think 20% -- 20%, 25% year-on-year growth at the new Kondapur facility is something that we are extremely confident about.

Alankar: So in terms of cost, would there be duplication of cost for 6 months? And I mean, how should we look at the EBITDA for both the hospitals combined, say, over the next year or so?

Abhinay Bollineni: There will be some duplication of costs like when we transition from the old Sunshine to the new facility. There has been some duplication that will continue to be there in Kondapur also for 3 to 4 months, before we fully shut down the old facility.

Alankar: Got it. And any sense on the monthly fixed cost at the new facility?

Abhinay Bollineni: It will only be incremental. If we're moving one of our old facility into a new facility. So we will -- obviously, a lot of new doctors will come in, a lot of new incremental staff to service those patient volumes will come in. So it will probably be higher than -- higher by around 25% from the current fixed cost, 25%, 30%. It also depends on when the new doctor teams are onboarding and stuff.

Alankar: Got it. But given that it's expected to be operationalized soon, I mean, we would already be in that process, right, of...

Abhinay Bollineni: Correct. But it's a very large facility. It's an 850-bedded hospital. So -- and the current facility is only 250 beds. So it will -- our doctor onboarding will happen in phases. It will not happen on day 1.

Alankar: Okay. So significantly higher beds, but just 25% higher cost, as you mentioned. Got it. Yes, that's it from my side. Thank you.

Moderator: The next question is from the line of Vedant Nilekar from ICICI Securities. Please go ahead.

Vedant Nilekar: Am I audible?

Abhinay Bollineni: Yes.

Vedant Nilekar: Thank you for this opportunity. I just had one question on the capex front. Now that most of our expansion is done, and whatever further bed capacity that we are going to add, what is our expected capex figure for FY '27 and FY'28?

Abhinay Bollineni: Capex for '27 and '28. '27, the full closure and capex will be another incremental INR500 crores to INR600 crores. And for '28, we have not yet put a plan together because there's no new hospitals that we're commissioning that. Initially the main capex will be adjusted.

Vedant Nilekar: Okay. Got it. Thank you.

Moderator: Ladies and gentlemen, that was the last question for today. I now hand the conference over to the management for closing comments. Over to you, sir.

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Bhaskara Bollineni: Yes, very good afternoon. And we have done a lot of knowledge by asking your questions. Let me clarify certain things beyond what all in addition to what Sachin, Abhinay and Sreenath and Nitish clarified. The debt portion is even though we have INR2,800 crores, we are supposed to get nearly INR600 crores from our creditors, from mostly the state and central government, which are sent for the payment. But because of lack of money there, that will definitely will come.

There is -- as I mentioned, that we have already at INR200 crores cash as of now with us. So with these things, I think moving forward, we may not see any inc

remental debt will be there on

the books, and it will start coming down quarter-on-quarter. If there is any new opportunities, expansions, which we are having that we will be able to look into the other means of funds, not through debt, that's one.

As far as the doctors onboard is concerned, the -- initially, we started a few specialties and it is an ongoing process of expansion of the new facilities with even the old facilities like Nellore and other places. We are adding new specialties when we add new specialties, then the doctors keep boarding. That is even incremental revenue instantaneously, because these are all already existing facilities. There will not be a requirement of any capex.

And these are all the things which we are doing for the growth, so that the growth we can expect in all the hospitals. There is a potential in every hospital that needs to add a little more of new specialties. And as one of the calls, I said, in about a year back that in 3 years, we will be able to double our top line, which we are on the -- better than what I promised.

So the -- whatever the things that we do, it will always be for the benefit of the investors. And also, we consider the -- many of you have been worried about the debt ratios. So we are very, very keen and confident that we will do good. And in the next few years, we expect a better growth than the last 2 years. Thank you. Thank you very much.

Moderator: Thank you. On behalf of IIFL Capital Services Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: May 2026

22nd May, 2026

To,
The General Manager,
Department of Corporate services
Bombay Stock Exchange Ltd (BSE)
Phiroze Jheejheebhoy Towers,
Dalal Street,
Mumbai - 400 001.
Scrip Code - 543308
ISIN: INE967H01025 To,
The Manager,
Listing Department
National Stock Exchange of India Limited,
Exchange Plaza, 5th Floor,
Plot No.C/1, 'G' Block
Bandra - Kurla Complex
Mumbai - 400 051.
Symbol - KIMS
ISIN: INE967H01025

Dear Sir/ Madam,

Sub: Transcript of earnings conference call with Analyst / Investors.

In Continuation to our letter dated 12th May 2026, the Company organized a conference call with the Investors/ Analysts on Monday, 18th May 2026 at 09:30 AM (IST). A copy of the transcript of the conference call held with the Investors/ Analysts is enclosed herewith and the same has also been uploaded to the Company's Website at <https://www.kimshospitals.com/investors> > Disclosures under Regulation 46 of SEBI (LODR) Regulations, 2015 > Analysts Calls schedule, PPT & Transcripts > Transcripts, Audio & Video Recordings > FY 25-26.

Kindly take the same on record.

Thanking you,
Yours truly

For Krishna Institute of Medical Sciences Limited

Nagajayanthi J. R
Company Secretary & Compliance Officer

Encl: As above

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"KIMS Hospitals Q4 FY '26 Earnings Conference Call"

May 18, 2026

MANAGEMENT : DR. BHASKAR RAO BOLLINENI – FOUNDER &
MANAGING DIRECTOR , KIMS HOSPITALS
DR. ABHINAY BOLLINENI – EXECUTIVE DIRECTOR &
CHIEF EXECUTIVE OFFICER, KIMS HOSPITALS
MR. SACHIN SALVI – CHIEF FINANCIAL OFFICER,
KIMS HOSPITALS
MODERATOR : RAHUL JEEWANI – IIFL CAPITAL SERVICES LIMITED

KIMS Hospitals
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Moderator: Ladies and gentlemen, good day and welcome to KIMS Hospitals Q4 FY '26 Earnings Conference Call hosted by IIFL Capital.

As a reminder, all participant lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Rahul Jeewani. Thank you and over to you, Mr. Jeewani.

Rahul Jeewani: Hi. Good morning, everyone. This is Rahul from IIFL Capital. I welcome you all to the 4th Quarter Earnings Conference Call of KIMS Hospitals being hosted by IIFL.

From KIMS, we have with us today Dr. Bhaskar Rao Bollineni – Founder and Managing Director; Dr. Abhinay Bollineni, Executive Director and CEO; and Mr. Sachin Salvi, CFO;

Over to you, sir, for your opening comments.

Bhaskar Rao Bollineni: Good morning. Hearty welcome to all of you. The closure of Financial Year 2026 coincided with the beginning of the ongoing Gulf War with ramifications across many countries in the world, including India. The recent appeal by Prime Minister for austerity measures underlines the grim scenario. Let us heed the call of the Prime Minister and ensure optimum utilization of resources till normalcy is restored. Let's hope and pray that better senses will prevail and war will soon come to an end. In the present world, war between any two countries is impacting many countries because of their interdependence.

I am aware that all of you are eagerly awaiting our financial results. I am happy to announce that KIMS registered a stable growth.

Q4 FY '26 highlights:

- Total revenue of INR 1,084 crore, a growth of 35.3% on year-on-year and 8.1% on quarter-on-quarter basis.
- EBITDA of INR 216 crore, a growth of 6.8% on year-on-year and 5.9% on quarter-on-quarter basis.
- EBITDA margin at 19.9% versus 25.3% in Quarter 4 Financial Year '25 and 20.4% in Quarter 3 Financial Year '26.
- PAT at INR 33 crores in Quarter 4 Financial Year '26 against INR 106 crore and INR 52 crore in Quarter 4 Financial Year '25 and Quarter 3 Financial Year '26 respectively.

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FY '26 highlights:

- Total revenue of INR 3,931 crore, a growth of 28.2% on year-on-year basis.
- EBITDA of INR 828 crore, a growth of 1.6% on year-on-year basis.
- EBITDA margin of 21.1% versus 26.6% in Financial Year '25.
- PAT at INR 242 crores in Financial Year '26 against INR 415 crores in Financial Year '25.
- Consolidated earning per share for Financial Year '26 of INR 6.03, a decline of 37.2% on year-on-year basis.

Q4 FY '26 Financial Highlights - Consolidated:

Consolidated revenue from operations is INR 1,075 crore, a growth of 34.8% on year-on-year and 7.7% on quarter-on-quarter basis.

Consolidated EBITDA pre-Ind AS is INR 131 crore, a decline of 33.5% and 32.3% on year-on-year and quarter-on-quarter basis respectively.

Operational Highlights - Q4 FY '26:

- Average revenue per operating bed grew by 13.7% and 1.7% on year-on-year and quarter-on-quarter basis respectively.

■ Average revenue per patient grew by 14% and 3.5% on year-on-year and quarter-on-quarter basis respectively.

■ IP volumes grew by 17.9% and 4% on year-on-year and quarter-on-quarter basis respectively.

■ OP volumes grew by 30.1% and 4.1% on year-on-year and quarter-on-quarter basis respectively.

FY '26 Operational Highlights:

■ Average revenue per operating bed grew by 14% on year-on-year basis.

■ Average revenue per patient grew by 11.4% on year-on-year basis.

■ IP and OP volumes grew by 15.4% and 25.4% respectively on year-on-year basis.

Thus, the operational parameters demonstrated a consistent upward trend. It augurs well that

new units at Bangalore, Thane, Kerala, Nashik and Seetamadara, Vizag contributed significantly to top-line growth.

The matured units maintained healthy growth pattern. Expansions necessarily involve heavy expenditure and it strains the profits in the initial phase. Likewise, the profit of the company for the year is impacted, though growth remains stable. However, the new units also will turn the tide sooner than expected and the prospects are already visible.

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'25-'26 has been a year of expansions. We opened one more hospital in Hyderabad at Kompally, two in Guntur, two in Maharashtra at Thane and Sangli, two in Bangalore, Mahadevapura and Electronic City, and one in Kerala at Kollam. We also opened fertility centers at Hyderabad and Guntur. Expansion has no meaning without clinical excellence and excellence is incomplete without empathy. All these new units reflect above concepts of excellence and empathy and already started making great imprints in their respective zones.

I am happy to state that we accomplished an important milestone of completing 10,000-plus successful brain tumor surgeries at KIMS.

An exclusive cancer block was recently inaugurated at KIMS Hospital, Ongole, to meet the medical needs locally without patients running far and wide.

We opened KIMS Cuddles, a mother and child unit at Nellore. On 4th February 2026, International Cancer Day program was organized to promote awareness. Megastar Padma Vibhushan Chiranjeevi took part as chief guest. Making the International Lupus Day, KIMS Hospital, Secunderabad, organized a unique ramp walk featuring lupus warriors on 11th May, attended by family members and healthcare professionals across the city. This happens to be the fifth such lupus ramp walk initiative in Asia.

Yesterday, that is 7th May, was World Hypertension Day. Hypertension or elevated blood pressure is a common condition with a prevalence of one in three adult Indians. This condition is dangerous, but silent and causes a huge disease burden in the forms of heart disease, brain stroke, renal failure, eye disease, and peripheral vascular disease. The danger is more since it often produces no symptoms by itself until target organ damage is already established and so it is appropriately called a silent killer. Large studies have shown that early detection and appropriate management can prevent the adverse effects of this disease. Early detection relies on screening programs across the populations and education to follow a healthy lifestyle.

Thus, high blood pressure is the gateway for mental health, many health issues, and timely action will enable mitigate many problems. Considering the paramount significance, we gave a call to all our unit heads across the globe to organize BP checkups in a massive scale of 1 lakh at random places like railway stations, bus stands, offices, etc. All the units spontaneously responded to the call in a resounding way despite the short notice and accomplished 2,04,000 numbers, more than double our expectations due to vigorous campaigning. Besides the numbers, it succeeded in creating awareness on a wide scale. We strongly believe that PREVENTION IS BETTER THAN CURE and this step demonstrate our philosophy.

Being a healthcare professional, I thought it fit to appraise you also, but the ill effects of hypertension on this occasion.

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Finally, it is said that THERE IS NO HAPPINESS IN MEDICINE AND THERE IS NO MEDICINE LIKE HAPPINESS. I thank you for continued support and trust. Thank you very much.

Rahul, we can open it for Q&A.

Moderator: Thank you. We will now be beginning the question-and-answer session. The first question comes from the line of Balamurali Krishna with Oman Investm

ent Advisor. Please go ahead.

Balamurali Krishna: So, my first question is regarding this bed addition or changes in the presentation. Earlier, we have a lot of bed addition in Andhra Pradesh which is not available now and also Telangana, Kondapur is elevated. So, is there any changes in plans? And also timelines are not available. So, how do we see this?

Vijay Hargunani: Hi Bala Krishna, this is Vijay here. So, the change is only because of we are coming up, we are planning for a QIP in a month or two. So, what has happened is to align with the consuls, the prospectors and everything, we have given this time only the information which is being informed to exchanges and all. But the earlier plan is on track and we will update the presentation next quarter.

Balamurali Krishna: Sir, secondly, in 2026, we have opened a lot of new assets. So, what is the contribution of those assets in the current revenue and what is the EBITDA range? Mostly, they are in EBITDA negative. So, what could be the approximate percentage of EBITDA?

Vijay Hargunani: So, for quarter-on-quarter basis, if you see the mature unit for us remains in the steady position of EBITDA of 30% approximately and new units which are recently started for Karnataka, specifically, the PES Hospital, there is a initial drag. That is only taking away the EBITDA. So, this is the change.

In terms of the numbers, if you are asking about the revenue of around INR 1,085 crore, the matured units in this quarter has contributed almost INR 862 crores of revenue and the balance revenue of about INR 224 crores is from the newer units. Newer units we mean the units which are in operations for less than one year, as on the date of financial statement.

Whereas the EBITDA, which we have reported almost INR 216 crores in this quarter, out of that INR 250-odd crores is from the mature unit. So, to say that almost 28.5% or 29% of EBITDA margin from matured unit we continue to maintain.

As far as the newer units for the current quarter, the EBITDA erosion is to the extent of about INR 32 crores. So, this is roughly or this is accurately in the terms of numbers for the current quarters.

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Balamurali Krishna: Thanks a lot for this explanation. So, when do you think, lastly, and when do you think that we can go to previous EBITDA numbers as these assets matures and we can take those numbers maybe down the line one year or two years? Or as we are expanding rapidly every year, so the EBITDA margin will settle at this range for the next three, four years.

Abhinay Bollineni: The better way to look at it is on a cluster-on-cluster basis. Every cluster we are now disclosing the EBITDA margins at each cluster. So, because we are adding so much bed capacity in different clusters, there will be some depression in EBITDA numbers. But as long as we are able to grow that on a quarter-on-quarter basis, that is the direction we should look at. That is the guidance we would like to give.

Balamurali Krishna: That is all. Thanks a lot.

Moderator: Next question comes from the line of Aman Goyal from IIFL Capital. Please go ahead.

Aman Goyal: So, my question is related to the new unit's loss trajectory. Can you throw some colors on the quarter-on-quarter losses on Nashik, Thane and newer Karnataka units?

Abhinay Bollineni: So, Nashik for Q4 is around INR 1 crore loss. Thane is around INR 5 crore loss for Q4. And the rest of the losses are from Karnataka. I think the losses in Maharashtra have narrowed down to just INR 6-7 crores now from both the new entities.

Karnataka, because PES just got commissioned in the month of Jan, there are some losses. Sorry, in December, PES got commissioned. Now that the ramp-up is happening in PES, maybe towards the end of the year, the losses from PES also should narrow down.

As far as Mahadevapuram is concerned, the losses are in the similar range of Thane, which again w

ill narrow down as soon as we get more and more insurance types.

Aman Goyal: My next question is related to the Karnataka geography. Since we are new to the region and there are lots of bigger hospital chain like Apollo, Manipal and there. So, how do you see the market as compared to the core cluster like Telangana and Andhra?

Abhinay Bollineni: Obviously, it is a new market, but we are very happy with the way the ramp-up has happened at Mahadevapuram. Now it is 8 months old, 8-9 months old. We are doing an average of INR 17 crores a month based on Q4 numbers. And we have very clear visibility of how it will go to INR 25-30 crores over the next 12 months.

So, we don't see any difference in how the ramp-up would have happened in a city like Hyderabad or in Bangalore. It is just that the timing for both the hospitals, there was a gap and because of which the losses expanded. And PES, we took a little longer to commission. And the only difficulty off late is the insurance empowerment. Once that gets solved, I think the growth trajectory, we are pretty confident in all the hospitals that we commission.

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Aman Goyal: I will join back the queue.

Moderator: Next question comes from the line of Nancy Yadav with Allegro. Please go ahead.

Nancy Yadav: Again, I also have a question on the new units. To be able to calculate margins, would it be possible for you to tell the revenues and the EBITDAs for Q4 for Mahadevapura, E-City and Thane? Mahadevapura you said is close to INR 17 crores per month. So, does it come to around INR 51 crores?

Abhinay Bollineni: No. Mahadevapura in Q4 is INR 49 crores. PES is INR 17 crores. Thane is INR 47 crores.

Nancy Yadav: And, sir, could you also tell the losses individually for Mahadevapura and Electronic City?

Abhinay Bollineni: Mahadevapura is INR 14, Electronic City is 25, Thane is 5 and Nasik is 1.

Nancy Yadav: And, sir, Electronic City is INR 17 crores in revenue. Sorry, I am not sure if I got that correctly.

Abhinay Bollineni: Yes, for the quarter.

Nancy Yadav: That is all I wanted.

Moderator: Next question comes from the line of Alankar Garude with Kotak Institutional Equities. Please go ahead.

Alankar Garude: Sir, in the 3rd Quarter call, you had mentioned that debt has peaked out and in the absence of any new expansion plans, debt should trend lower. Now, then within a month, you announced the QIP. Can you help us understand the reasons behind the planned INR 1,500 crore QIP? I mean, does it mean that you have zeroed in on any new expansion plans?

Abhinay Bollineni: Expansion plans, we are still exploring, Alankar. That is part of the whole strategy. But the idea is now that the debt has reached its peak, close to INR 3,000 plus crores, the idea is we retire some debt and use the cash flow to do more greenfield projects, which anyways will take three to four years before they get commissioned.

So, there are one, two small acquisition opportunities that can potentially come up over the next two quarters, but they are not significantly big. They are around 400, 500 beds, both put together.

But the idea is to retire a large amount of this, we go to retire debt and use the cash flow to fund greenfield expansion, which will anyways take three to five years before they get commissioned.

Alankar Garude: Which are these markets, Abhinay, you are talking about for both the organic expansion as well as the acquisitions?

Abhinay Bollineni: All of them are in our home markets, Karnataka, Kerala, Andhra, Telangana, mostly in these four markets.

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Alankar Garude: So, basically, you will be retiring debt and then eventually will be pursuing the greenfield, given that the cash flow requirement for the greenfield will be more back-ended. Retiring debt is something which is possible with the QIP. Is that the way to understand this?

Abhinay Bollineni: Right

t.

Alankar Garude: Fair enough. The second question is, qualitatively, particularly on the insurance empanelment, can you update us on the progress of the four new, relatively new hospitals, Nasik, Thane and the two Bangalore ones? And vis-a-vis your initial expectations, if you can just highlight how each of these units have performed.

Abhinay Bollineni: So, I think what we underestimated is the time it takes to now get insurance empanelments vis-a-vis how it was before. That has caused some delay in ramp-up, but otherwise, overall, we are very happy with all the four hospitals, the way we have been able to get clinical talent. Had we got empanelments like we could have in the past, the ramp-up would have significantly been better than what it is today. In spite of which, we are pretty happy with the trajectory of the revenue growth in most of these hospitals.

As far as empanelments are concerned, Nasik, we are still yet to get STAR, Medi Assist, and GIPSA. For Thane, we finally got GIPSA, and Medi Assist and STAR are almost through. That completes at least 70% of the volume of insurance. We are yet to get the empanelment, but at least the negotiations are more or less through. As far as Bangalore, it will take 3-4 more months before we are able to get the empanelment.

Alankar Garude: Just one quick follow-up there. You mentioned about delays in the insurance empanelment. Was it more specific to FY '26 and the standoff which happened with insurance providers for the industry, or this is something which we should expect to continue going forward?

Abhinay Bollineni: I think there is a new council, GIC, which also got introduced in the same financial year. And because of that, there led to a lot of confusion as to who empanels hospitals, will GIC do it, will companies do it directly. So, that confusion also led to a significant delay in empanelling hospitals. I think eventually, we will have to figure out a way to ease this and make sure that once a new hospital is commissioned, within 6 months, 9 months, we are able to get empanelment done.

Alankar Garude: That is very helpful.

Moderator: Next question comes on the line of Akshay Thakur with Helios Capital. Please go ahead.
Akshay Thakur: Sir, my question is related to capital raise. I wanted to know your strategic evaluation on, let's divide it in two parts. Say, for example, one would be taking it from the market. It could be debt or it could be QIP or any other source.

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Second would be doctor's participation model, which we used to have earlier. So, what is the strategic evaluation point between these two to raise capital from these two sources?

Abhinay Bollineni: So, right now, we have a debt of almost INR 3,000 plus crores. We want to use the capital raised from the QIP to retire debt. And most of the projects that we do at a project level, we put in an equity of 40% of the project cost and 65% is debt.

Now, doctor participation is usually a very, very small amount. It amounts only to 3% to 5% at best.

Akshay Thakur: So, in current expansion plans, do you evaluate this as well, this doctor participation model?

Abhinay Bollineni: Doctor participation model is always open. So, doctors are always open to invest as and when they feel like.

Akshay Thakur: That is my only question.

Moderator: Next question comes from the line of Karan Vohra with Goldman Sachs. Please go ahead.

Karan Vohra: So, the first question is with respect to the Kondapur expansion. So, I see that we have kind of increased the number of beds. So, just any thoughts on the kind of CAPEX we are planning and what are the timelines? When should these beds come online? And if they will come in phases, which is what we understand, then what will be the phase 1, 2, 3? What will be the timelines and the CAPEX for it?

Abhinay Bollineni: So, we will shift into the new Kondapur facility starting in the first

week of June. So, the new

hospital will have 800 beds in total. The old hospital, we will completely shut it down. We will transition over a period of one month. But we may continue to run the old facility for 2-3 months even after we transition. And then we will shut it down.

So, the new hospital will have 800 beds, of which in Phase 1, because we are already at an occupancy of 200 beds and we need headroom for growth, we will commission at least 400 to 500 beds in Phase 1. The remaining 300 beds, we will commission as and when the occupancy increases.

Karan Vohra: And CAPEX we have incurred for the whole project?

Abhinay Bollineni: Yes, except for, I would say around INR 50 crores to INR 75 crores, which we will incur not in the FY '27, but in FY '28 and beyond, which is to complete two additional floors, which we don't require at this point in time.

Karan Vohra: Sorry, INR 50 crores to INR 70 crores?

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Abhinay Bollineni: INR 50 crores to INR 75 crores over, we don't need it for the FY '27. We may need it in FY '28 or FY '29, based on how the ramp-up happens.

Karan Vohra: The second question is with respect to the Andhra Pradesh and Bangalore ARPOBs. So, Bangalore ARPOBs, I see, it looks like have risen significantly. Even the full year number shows like some 85,000 on the deck, if I am not wrong.

So, what has happened and is this the new base or that it is just a one-off where the mix was just too good and you go back to the 70s number, which was originally envisaged? And similarly, for Andhra Pradesh, what is the right run rate? Is 28,000 plus the new run rate or 24,000 to 26,000 is the one which we should think as a normal run rate?

Abhinay Bollineni: Yes, as far as Bangalore is concerned, 75,000 is a healthy number to assume. If you actually look at Q3, Q4 and full year, the ALOS keeps changing. As the ALOS keeps changing, the ARPOB keeps moving up and down.

But you should fairly assume, for all models, we should assume 75,000 as a base and work on it. It will take time before the hospital settles down because a lot of new programs are getting added. Transplants are happening on a very aggressive way. But for modeling purpose, we should just assume 75,000 annually.

Karan Vohra: And Andhra ARPOBs?

Abhinay Bollineni: Likewise, Andhra also is at 26,000, 27,000 in Q3. And Q4 also remains the same.

Karan Vohra: So, that is the base. That is the normal base.

Abhinay Bollineni: Yes.

Karan Vohra: And just a quick last question. So, Maharashtra, what was the Q4 occupancy? Because the implied number when I do it, it looks fairly high at 60%-65% plus to arrive at that 52.1% number for the full year, right? So, is that a fair number or is there some error?

Abhinay Bollineni: Maharashtra is 60.5% occupancy for Q4.

Karan Vohra: And is that sustainable? Because it looks like a massive ramp up from 35% in Q1 to like 60% plus in Q4. So, should we assume like a 60% or that kind of a number as the new base for Maharashtra occupancy?

Abhinay Bollineni: That is the new base in Q4.

Karan Vohra: That is helpful.

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Moderator: Next question comes from the line of Abhishek Gupta with Chrys Capital. Please go ahead. Mr. Gupta, please go ahead with your question.

Abhishek Gupta: In Telangana and Hyderabad particularly, which we do know is a more mature market, we saw IP volumes increased by 2% whereas the overall revenue mix was increasing by 12% plus, right?

So, most of it is value-driven. How do we sort of see this going forward in the next year or two?

Abhinay Bollineni: Sorry, Abhishek, could you come back? Could you come back again? You are saying on a year-on-year basis?

Abhishek Gupta: Yes. So, in Hyderabad, which is the mature market for the larger healthcare space, we saw the IP volumes increased by 2%, right? And the rest of it was driven by value. So, how do we sort

of see this number going forward in the next year or two?

Abhinay Bollineni: I think because as a cluster Telangana now, we have added Kompally, which was commissioned two months ago. The new Kondapur will get commissioned now. And in the next 18 months, expansion in Secunderabad will happen. So, once these expansions happen, the growth will be largely volume-driven for the next 4-5 years.

Abhishek Gupta: That's all from me.

Moderator: Next question comes from the line of Damayanti Kerai with HSBC. Please go ahead.

Damayanti Kerai: I have two broader industry-based questions. So, first, what is your observation on competition for doctors in some of your key markets, say in Bangalore, whether it is becoming more competitive or it is not an issue for you? So, if you can comment on that part.

Abhinay Bollineni: I think Hyderabad is the most competitive from all the geographies that we are present in. Even in the last one year, at least 4-5 hospitals got commissioned only in Hyderabad. But we have not seen any doctor, significant doctor acquisition. We lost one or two doctors, but nothing material that we are worried of. We are seeing competition intensify. In fact, we are also adding more capacity and we are pretty confident we will be able to get doctors from competition to be able to ramp up.

Damayanti Kerai: So, according to you, what is helping? Sorry, please complete.

Abhinay Bollineni: Likewise, in all the other markets, Karnataka, Kerala, we have been seeing good traction from all doctors. So, anything, any department that they want to go and be able to build, they are not seeing any significant resistance from doctors to join.

Damayanti Kerai: So, according to you, what are the key initiatives or key incentives from KIMS' side, which is retaining the key doctors? Because what we have seen across market, whenever new hospitals

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come up, I guess there are some tangible attrition from the incumbents, etc. So, what, according to you, is helping to retain the talent which you have in your hospitals?

Abhinay Bollineni: I guess they are very comfortable in our hospital and they are happy to continue to work here. They don't see any incremental value-add that they will get by moving from this hospital.

Damayanti Kerai: My second question is actually a continuation of earlier discussion on the insurance empanelment. So, if you can again, I think explain, I didn't get very well. What is the dynamic which has actually changed and that has caused to delay in the empanelment?

You mentioned the GIC involvement, etc. And then we have been hearing about the common empanelment initiative, etc. So, it seems like a lot of changes or discussions happening on that part. So, I just want to hear your thoughts or observations there.

Abhinay Bollineni: Yes, I think GIC, they have come up with an initiative, GIC, where common empanelments have to happen. And that is where we lost significant time in trying to understand which is the model to empanel hospitals. So, that is an initiative by all insurance companies. And it is going to continue. It will strengthen moving forward. But because it is a new initiative this year, there has been some confusion on how things should move forward.

And also given we have commissioned so many hospitals in different micro markets in different cities, and when we are going and approaching insurance companies for 10 empanelments at the same time, plus we are talking about renewals at the same time, that has kind of led to delayed response from them also. Not that they are not working towards it, but given that there are so many things that they have to work on from one single company, that is where we have to take

time and space for each hospital to finish first and then go to the other hospitals.

Damayanti Kerai: And what is your view on the common empanelment initiative? Will you be considering that? Or wh

at are your key resistance against that step?

Abhinay Bollineni: I think if IRDAI decides to do something, we have to only follow the rules that they put in place.

Damayanti Kerai: Because some of your larger peers, I think they indicated it is not legally applicable. And there are, I think, some concerns on pricing, et cetera. So, that is why I wanted to hear your view.

Abhinay Bollineni: I don't know. I don't think it is appropriate to comment on that now. Right now, we are pursuing our empanelments with each insurance companies.

Damayanti Kerai: I will get back in the queue.

Moderator: Next question comes from the line of Ankush with Sanctum. Please go ahead.

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Ankush: If we take some earlier calls, in the earlier call, it was given that December quarter is the quarter where we are finding that the new units have started, and it is one of the lowest margins in the December quarter. And there is some regular pickup in margins in the business.

So, just try to get your sense on that. How do you see the ramp up in the margins now in the upcoming quarters?

Abhinay Bollineni: Like I said, we should look at it on a cluster-on-cluster basis. It is difficult to look at it at a company level because there is a lot of new hospitals getting commissioned at different clusters.

On a cluster basis, every quarter there is margin expansion in almost all the clusters, including the mature ones. Even in Andhra, we are seeing margin expansion. In Telangana, we are seeing some margin expansion and good revenue growth.

Ankush: That's it from my side.

Moderator: Next question comes from the line of Satyam Kumar with AAA Holding Trust. Please go ahead.

Satyam Kumar: Actually, I have a couple of questions. First question is regarding the Bengaluru ARPOB. What I can see is like Bengaluru Hospital combined has reported around 85,000 of ARPOB. So, do you see this level sustaining going forward? And also occupancy, I understand. Hospitals have recently operationalized. So, occupancy is very low, like around 20-odd percent. So, how do you see this occupancy and ARPOB playing around? This is my first question.

Abhinay Bollineni: So, ARPOB, like I said, 85,000 for the full year, quarter-on-quarter, the ALOS has been changing. And given it is a new facility and other new departments are getting added, it will take time for it to stabilize the length of stay. For all practical purposes, you should assume a 75,000 ARPOB and model the ramp up based on that. As time passes, as the hospitals mature, the ARPOB will obviously expand.

And as far as occupancy is concerned, there has been a healthy growth from a quarter-on-quarter basis. We are seeing visibility that the ramp up will be quite healthy even in the coming financial year. Towards the end of the year, we should hit a 35-40% kind of an occupancy on the full bed capacity.

Satyam Kumar: And like, how are you planning to add beds in both the hospitals of Bengaluru? Like right now the operational bed is on the lower side. That is quite understandable. Like how you increase?

Like at what occupancy level you generally tend to increase the count of operational beds?

Abhinay Bollineni: 60% on the current occupancy, on the current operational beds, we will operationalize the other beds. See, operationalizing other beds means just adding more manpower. So, based on the need on ground, we will decide how to operationalize these beds. Because the CAPEX for all of these beds are completed.

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Satyam Kumar: And one last question, slightly for a longer term. So, like, a couple of years ago, you announced expansion plan. What I understand, most of the expansion has already been taken care of.

So, from here on, how do you see expansion going forward for, let's say, next three, four years, since you are also doing QIP? So, what kind of expansion you are aiming for? An

d also, what

kind of top line and EBITDA growth from growth perspective, slightly for medium term, what is your target, if you would like to share?

Abhinay Bollineni: I think the expansion will continue to happen. We will keep adding more beds on a year-on-year basis. For the next 12 to 24 months, a lot of the consolidation of the current hospitals will happen, which will also expand margin for us to strengthen the current hospitals. And by then, the debt level also will come down to, in spite of the QIP level, will come down to a significant low number.

We will also use the next 24, 36 months to get the greenfield projects going. So, we obviously

have plans to expand in Karnataka, in Kerala, Tamil Nadu, we are going to add one hospital. And obviously, continue to expand in Andhra and Telangana. So, similar phase of expansion will happen, but because these are all greenfield projects, they will again get culminated up once after three or four years.

Satyam Kumar: So, any growth perspective in your mind, like what kind of top line or bottom line you are aiming for? Anything you would like to share at this moment?

Abhinay Bollineni: Forward-looking numbers, we are not able to share, but I think it should be similar to how we have been able to grow in the past.

Moderator: Next question comes from the line of Karan Vohra with Goldman Sachs. Please go ahead.

Karan Vohra: So, just trying to get a sense on what is the key, one key challenge for like each of our geographies, if you can highlight, it will be helpful. Just one biggest top of mind management concern for each of the five states you are present in.

Abhinay Bollineni: Right now, the only challenge I see is the insurance empanelment. Whatever we have learned in Thane, Bangalore, Nasik, all new clusters, we have not seen it any different from how things are in AP and Telangana. We are attracting talent, building hospitals. Ramp-ups are very similar. The experience has been very similar. The only challenge we faced in the ramp-up was the insurance empanelment.

Karan Vohra: So, basically applicable only to the new hospitals, which we are kind of operationalizing, not to the existing ones. No challenges in any of the existing hospitals.

Abhinay Bollineni: No, we are not--

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Karan Vohra: Second question is with respect to the doctor pay and strategy in existing versus new geographies. Are we kind of having some different strategies in newer markets like Bangalore or Kerala or Thane versus what we do in Andhra Pradesh, Telangana? Are we paying up? Are we like, where do we stand in terms of paying to doctors? Are we top quartile? Are we like just above average, average, any color on the strategy and pay ranges will be helpful.

Abhinay Bollineni: It is the same thing that we are following in AP and Telangana. The same model has been extended to Maharashtra and Karnataka and Kerala. We have not changed anything.

Karan Vohra: And are we like top quartile in terms of pay versus other larger peers, above average, average, any color there?

Abhinay Bollineni: We pay based on what we believe that the teams can do. Now, I don't know how to classify them under the kind of category that you have been asking. But I think, yes, we have been paying pretty well, whatever we think is healthy enough for ramp up of the hospital. That is how we look at it.

Karan Vohra: And just a quick bookkeeping one. So, Kondapur margin hit any qualitative number, any quantitative number or range will be helpful in FY '27?

Abhinay Bollineni: I think last time when we did the Sunshine Movement, there was a INR 15 to 20 crore or INR 25 crore kind of a number when Sunshine moved from its old hospital to the new facility. But because this is a much, much larger facility, we should think of it on similar lines a little more.

Moderator: Next question comes from the line of Harith Ahamed with Avendus. Please go ahead.

Harith Ahamed: Good morning, Dr. Abhinay. Can you talk a bit about the acquisition in Palakkad, the kind of revenue, EBITDA profile there and the potential that we are seeing, and then whether we will be consolidating this in our P&L?

Abhinay Bollineni: We will consolidate this in our P&L, Harith. It is like how we have in Kollam. It is a 250 bedded hospital, sorry, 300 bed hospital. The revenue potential would be around INR 12 to 15 crores per month over a period of time, where currently this month of May, we should be doing around INR 6.5, 7 crores with maybe a 5% negative EBITDA. So, by next month, we should break even in that facility, June, July.

Harith Ahamed: And following up on your comment on the net debt reduction post the QIP, so is there a broad level that you can guide for from the current 3,000-odd crore net debt level that we have post the QIP and assuming factoring in some of the CAPEX plans that we have for the year and also the ramp up that we are expecting at some of the currently loss making hospitals, some broad level for FY '27 end?

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Abhinay Bollineni: I think may be to the tune of INR 1,000 crores is what we should retire debt. And the rest we will use for losses and additional CAPEX in the current facilities, CAPEX that we needed to complete in some of the existing facilities that we have commissioned.

Harith Ahamed: And lastly, on the Telangana cluster, given such a large contributor to our overall profits, and then the Kondapura expansion being a fairly large one, in FY '27, should we expect a growth for

the cluster overall? Or will it be in line with our recent trends? Or will it be a soft year overall for the cluster?

Abhinay Bollineni: With the cluster, I think there should be a healthy growth rate. It won't be soft, maybe around 10%-12% EBITDA growth we should look at for Telangana cluster. Historically, it's been higher, but given Kondapur and Kompally that just got commissioned, it should be in that range.

Moderator: Next question comes from the line of Rahul Jeewani with IIFL Capital. Please go ahead.

Rahul Jeewani: Sachin, can you please call out the EBITDA loss from the new units for the entire year, as well as the base business EBITDA, the way you did for 4th Quarter?

Sachin Salvi: Sure, I will do that. So, from the mature units for the entire financial year, we did a revenue of INR 3,335 crores, to be exact, out of that INR 3,931 crores of revenue which we have reported. The EBITDA which we have done from the mature unit is about INR 956 crores out of INR 827 crores of EBITDA which we have reported. So, that says from the newer units, so the units which are in operation for less than one year, we did in total INR 597 crores of revenue and EBITDA erosion for the entire year is about INR 128 crores.

So, that also says that from the mature unit, we continue to do about 29.5% of EBITDA margin on the overall base revenue which we continue to do for these mature units in the past.

Rahul Jeewani: And can you also reiterate the commentary in terms of the break-evens for these new units on a monthly basis which you had called out in your previous call? So, has Thane already achieved EBITDA break-even on a monthly basis and what are your expectations in terms of when do you think both the Bangalore hospitals should be able to achieve a break-even?

Sachin Salvi: So, at a March exit, I mean, yes, please go ahead, Dr. Abhinay, if you want to.

Abhinay Bollineni: No, so Rahul, as far as Thane is concerned, March exit number, we were EBITDA neutral. April also, we were EBITDA neutral. But also, we added some doctors now again in the month of May. So, there might be some drag there.

But it is going in the right direction. We are seeing growth on a month-on-month basis. The only roadblock currently for that growth is the empanelments. Once that is done, I think there will be more tra

ction for growth.

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As far as Bangalore is concerned, like you said, Mahadevapuram, within 12 months of commissioning, we commissioned it in October. Twelve months from then, we should break, there was one month we were EBITDA positive in Mahadevapuram. EBITDA neutral, not positive. But I think before October, we should become EBITDA positive, single-digit positive in Mahadevapuram.

As far as PES is concerned, again, we are pretty confident with the trajectory. End of the financial year, we should get to a EBITDA positive kind of a number.

Rahul Jeewani: End of the financial year, which means March 27 for Electronic City. And let's say, obviously, this year, you will have some impact from Kondapur, but all these other new units should scale up. So, this entire EBITDA loss, which you had for the year to the tune of INR 120 crore kind of a drag from these new units, what kind of an expectation you have in terms of, let's say, the combined losses from these new hospitals going into FY '27 and '28?

Abhinay Bollineni: It is difficult to quantify that now, Rahul. We are just waiting for these empanelments to be done. And then the ramp-ups can happen faster. But obviously, it will be less than half the number.

Rahul Jeewani: I will join back the queue.

Moderator: Next question comes from the line of Aditya Chheda with InCred Asset Management. Please go ahead.

Aditya Chheda: I would like to know the management's perspective on balancing the current cost of debt and the cost of equity that we are looking to raise. So, how do you view the capital structure and your view on the optimal mix for the capital structure and fund raise?

Abhinay Bollineni: No, I think, given the feedback from most of our investors, they all believe that a healthy net debt to EBITDA ratio should be 1 is to 2. So, now that it is slightly higher than 1 is, it is at 1 to 3, we wanted to bring it down to that healthy number. And that is the purpose of doing this QIP. And also, it will ease out some cash flow for us when we want to do the new greenfield expansion.

Moderator: Ladies and gentlemen, as there are no further questions, we have reached the end of the question-and-answer session. I would now like to hand the conference over to the management for closing comments.

Bhaskar Rao Bollineni: So, thank you very much for all the relevant questions. And as we all already ramped up and given the knowledge that our entire aim is to see that whatever the last five years we have been achieved, I keep on telling that in the next few years, in the next five years also, we will try to achieve the same growth. That is why we are trying to do these expansions, Greenfield, Brownfield, as well as the QIPO.

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The reason is, and also we were mentioning all the time that our debt EBITDA should not cross more than 1 is to 2, and also the asset ratios. And keeping all those things, the PAT this quarter and the EPS and all those things, ROE, ROC has come down. So, we realize, we know that this financial year, it will cross our expectations of all these things.

That is why we are raising this fund, so that to keep the company healthy and also, at the same time, we will do a steady expansion plans. With that, I think whatever is the losses that we have been, nearly 120 EBITDA eroded, and that also comes down in the next year. And moving forward, I think this will not be much.

So, we can see the company should be able to, and we are anticipating the company should go on a healthy note in the next 3 to 5 years. Thank you very much for all the questions again.

Moderator: Thank you. on behalf of KIMS Hospitals and IIFL Capital that concludes this conference. Thank you for joining us. You may now disconnect your lines.